

Rainer Sachse

Personality Disorders

A Clarification-Oriented Psychotherapy Treatment Model

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Personality Disorders

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About the Author

Rainer Sachse is Head of the Institute for Psychological Psychotherapy (IPP) in Bochum, Germany. He studied psychology from 1969 to 1978 at the Ruhr University of Bochum, Germany, and went on to gain his doctorate in psychology and a postdoctoral qualification for a full professorship, and later becoming a professor of clinical psychology and psychotherapy. At the end of the 1990s, Prof. Sachse developed a dual action theory of personality disorder which led to the creation of clarification-oriented psychotherapy, a therapy approach which he continues to use and develop today. His main areas of interest are personality disorders, psychosomatics, clarification-oriented psychotherapy, and behavioral therapy, and he has written extensively about these themes.

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Foreword

Personality disorder has been increasingly identified as a mental condition that influences the treatment and recovery of patients both in medical psychiatric and in psychodynamic and cognitive behavioral treatments. Despite significant research, personality disorders still remain relatively underrecognized and misperceived in both psychiatric and psychological evaluations, either due to the predominance of primary comorbid disorders, such as mood, trauma, substance misuse, or eating disorders, or because of insufficient strategies for distinguishing and integrating personality-based function into assessment and treatment. In addition, the shortage of general, effective guidelines for identifying and approaching personality disorders has left many clinicians and therapists to their own devices when facing these problems with their patients. Some personality disorders are significantly influenced by deficits in neuropsychological processing, by attachment patterns, and by trauma, in addition to cognitive and interpersonal patterns, while others can be identified primarily in terms of psychological processing and relational motivation and regulation.

This volume provides a most instructive and comprehensive outline for identifying and treating clients with these personality disorders, which first and foremost present with problems in psychological and relational functioning. It fills a significant gap through its aim to create an integrative strategy for clarification and for motivating patients to seek to change. The author presents a well-organized approach, including detailed descriptions of each disorder and their array of functional patterns, with thorough therapeutic strategies for a broad range of problems in different phases of the treatment.

The clarification-oriented psychotherapy approach focuses on self-regulation and motivation in interpersonal interactions and relationships. By identifying motives and schemas, the author provides constructive and informative strategies for therapists to understand and manage the complexity of patients' internal and interpersonal functioning, which can otherwise easily distract therapists and invite unconnected or misguiding perceptions of patients' mental functioning. The informative and detailed therapeutic guidelines can help direct therapists' efforts to identify and clarify patients' complex motives and the different behavioral and intentional aspects of their functioning, and these guidelines include specific examples that are anchored in a solid theoretical frame of reference. In addition, the author identifies the co-occurring normal aspects of personality functioning within the framework of psychopathology, as well as subcategories of disorders that present with a range of behaviors and levels of functioning. This is very important for alliance building and for engaging patients' sense of agency and motivation toward change and improvement.

In sum, this is a very well-structured, informative, and readily accessible book that provides unique and valuable guidelines for therapists treating those with personality disorders, with a clarification- and schema-focused approach. Given the integration of empirical studies

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with detailed clinical descriptions of each disorder, this is a useful and inspiring resource, and it is to be hoped that this book can be made available to therapists and clinicians in many countries.

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Chapter 1

Essential Basic Concepts of Personality Disorders

1.1 Introduction

The concept of personality disorders (PDs) has a long history, and in consequence, widely differing ideas have developed around it. These ideas vary greatly from one another and are barely compatible (e.g., see Benjamin, 1996, 2003; Clarkin & Lenzenweger, 1996; Derksen, 1995; Fiedler, 2007; Fowler et al., 2007; Magnavita, 2004; Oldham et al., 2005).

Recent developments of this concept suggest that PDs should be conceived of as based on two factors: One should first conceptualize generally what PDs in fact are in a psychological sense, and then, on the basis of this general concept, one should clearly define the characteristics of the individual disorders (see Livesley, 1998, 2001; Livesley & Jackson, 1992, 2009; Livesley & Jang, 2005; Livesley et al., 1994, 1998; Hentschel, 2013). Some considerations of this are also dealt with in the American Psychiatric Association (APA) *Diagnostic and Statistical Manual of Mental Disorders,* fifth edition (DSM-5; APA, 2013).

The concept of PDs presented here adopts an equivalent approach: A general model of the psychological functioning of PDs is introduced, with the individual disorders then being elucidated on the basis of this model. Moreover, therapeutic implications are derived from the general as well as specific models (see Döring & Sachse, 2008a, 2008b).

The purpose of this book is not to trace and discuss conceptual developments, however. Rather it is to illustrate a **treatment concept of PD** – that is, the concept of **clarification-oriented psychotherapy** (COP; in German: *klärungsorientierte Psychotherapie*, or KOP). For this purpose, essential basic concepts of the approach to PDs will be emphasized to reveal the ideas that are suggested for the concept described here.

1.2 The Term *Personality Disorder*

It was initially suggested that there were some disorders that were very comprehensive, profound, and treatment-resistant. As a result, these disorders were seen as **disorders of the overall personality** (see Kernberg, 1978; Kretschmer, 1921; Schneider, 1923).

According to current psychological concepts (Fiedler & Herpertz, 2016; Millon, 2011), one must still assume that these disorders are complex, and that owing to their specific psychological

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constellations, they remain relatively difficult to treat (see O'Donohue, Fowler, & Lilienfeld, 2007). However, the disorder at issue is not necessarily considered a disorder of the personality. Instead, it has become clear that features which characterize a PD are often already present in a lighter form in almost every person and are largely considered as normal and ordinary. As a result, more severe forms appear to be only extreme forms of ordinary psychological occurrences (Fiedler, 2007) and therefore are variations of a norm that are not necessarily considered as pathological.

In this context, a tendency in psychology could be observed to depathologize and normalize PDs. However, it is still obvious that these disorders generate great costs for the person concerned and that it makes sense to treat them therapeutically. Nonetheless, it is important to refrain from stigmatizing those affected. Unlike Emmelkamp and Kamphuis (2007), we do not view PDs as a "chronic psychiatric disorder . . . characterized by pathological personality traits" (Sachse, Sachse & Fasbender, 2010, 2011; Sachse, Fasbender, Breil, & Sachse, 2012).

It is essential to see PDs as an extreme form of ordinary, normal psychological processes, which generate such great costs for the person concerned that psychotherapy is useful.

Therefore, in this book clients with PD will not be classified or designated as infantile, immature, pathological, seriously disturbed, or temperamentally deficient. It is important to get away from such negative evaluations (this is important to create a good therapeutic relationship with the client!). Furthermore, such a diagnosis may affect the therapist's stance and interventions. Basically, it would make sense to dispense with the term *personality disorder* and replace it with *interaction disorder*. However, since the term has been adopted into the language, it is easier to stay with the term personality disorder as long as one knows what is intended by it.

1.3 Style and Disorder

Individuals with a minor personality style exhibit characteristics of a psychological entity in a mild form, whereas individuals with a major clinical disorder exhibit these characteristics in a severe form.

An important implication of this approach is the assumption that there are no distinct criteria according to which a style becomes a disorder. Basically, there are no empirically valid criteria which specify precisely when a style becomes a disorder (see Caspar et al., 2008; Foster & Campbell, 2005, 2007; Krueger et al., 2007; Livesley et al., 1994; Ronningstam, 2005; Samuel & Widiger, 2011; Watson, 2005; Widiger & Samuel, 2005; Widiger & Simonsen, 2005; Widiger et al., 2005). Thus, during the process of psychotherapy, it is sensible to negotiate with the client as to whether they consider their disorder to be so disruptive that therapy is indispensable.

1.4 Making Diagnoses

An important aspect of depathologization is that one does not make diagnoses of PD to label people: If one makes an official diagnosis (i.e., one that is passed on to the authorities), one

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should always be aware that it can certainly be used against the client, and one should be careful about this. For internal communication between professionals, that is, in supervision, diagnoses serve exclusively to help understand exactly what the client's disorder is in order to be able to deal constructively with the client.

The sole purpose of diagnosis is to derive meaningful therapeutic measures to help the client (Sachse, 2017).

Therefore, it makes sense in principle that a therapist
- gives a diagnosis,
- is aware of the fact that this is always a more or less well-proven hypothesis – that is, a working hypothesis for the purposes of psychotherapy,
- establishes a diagnosis as early as possible in the process (and as a first hypothesis),
- never overlooks a client's PD.

And in this case, it may well make sense to speak, for example, of narcissism as a disorder, although the client only exhibits a style: Because it can be helpful even then to be sufficiently prepared for games, motivational problems, etc.

In general, it appears to be expedient to consider a personality style or a disorder in the therapy process – that is, to diagnose it and to consider it in the therapeutic procedure if
- aspects of the style or disorder cause the costs the client does not want to incur, and/or
- aspects of the style or disorder become relevant in therapeutic interaction – for example, by leading to manipulative behavior that significantly influences interactions with the therapist.

As a rule, however, even mild styles are relevant, so therapists should generally
- be mindful of PDs,
- be capable of quickly detecting and validly diagnosing any PD,
- be able to handle the PD in a constructive manner.

1.5 Resources

When treating PDs, the focus is no longer solely on deficits, but moves toward the client's **resources.** It becomes more and more obvious that PD always implicates resources: The clients are, due to their schemas, very sensitive regarding certain information, they are competent in certain actions, etc. Most of the time, the problem is not that they do not have any resources but that they are unwilling or unable to use them constructively.

These resources are good therapeutic tools, if one wants to change schemas, develop alternative action patterns, etc.: In the modification phase of the therapy, the orientation of these resources can be very helpful. Plus, the focus on resources helps to get away from the tendency to pathologize and stigmatize the client.

This approach also has consequences for the definition of **therapeutic objectives**. Healing as an objective is not useful (aside from not being even realistic), because clients do not have to abandon all aspects of their personality. Clients just have to learn to use their resources more

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sensibly; they have to learn to use their structures (such as schematas, motives, manipulations etc.) more constructively and how to benefit from them.

1.6 Personality Disorders as Relationship Disorders

Scientific studies have shown that at the center of PD are problems on a relationship and interaction level (see research by Sachse, Sachse, & Fasbender, 2010, 2011; Sachse, Fasbender, Breil, & Sachse, 2011, 2012). Thus, in certain theoretical perspectives, PDs are understood as **relationship or interaction disorders**. This conception is also represented in the approach taken by COP. Within this approach, it is accepted that PDs are indeed psychologically complex disorders affecting many psychological functional areas; however, the central point of PD is a relationship disorder. This assumption indicates why clients with PD are difficult clients in the therapy process: They bring their interactional problems with them into the therapeutic relationship.

1.7 Therapist Expertise

Since clients with PD put significantly greater demands on therapists (Sachse, 2006a, 2006b, 2009), therapists need a much higher level of expertise for the treatment of PD clients than for the treatment of Axis I clients. The therapists need to master a highly flexible relationship structure, they have to deal with so-called tests to be able to apply confrontational interventions, to deal with manipulative interaction behavior, to be able to clarify affective schemas, etc. And above all, they must be able to quickly recognize a PD and generate a hypothesis about its type. This requires the ability to process information and model it quickly and confidently. It also requires highly strategic knowledge, rapid action planning, and intervention formulating, etc.

1.8 Relevance of DSM and ICD

In the author's view, diagnostic systems such as those of the DSM-5 (APA, 2013) and the World Health Organization (WHO) *International Classification of Diseases,* 10th edition (ICD-10; WHO, 1992) do allow a diagnosis of PDs. However, clients with PDs have characteristics that make diagnosis very difficult. These include

- a high degree of distrust,
- a tendency to reveal very little of themselves,
- a strong tendency to produce images, and
- a poor understanding of their own problems.

Therefore, traditional diagnostic systems can only ever provide diagnostic hypotheses, especially at the beginning of therapy (see Sachse, 2006a, 2006d; Sachse & Breil, 2011; Sachse, Sachse, & Fasbender, 2011; Sachse, Fasbender, Breil, & Sachse, 2012).

In addition, there are other problems with the DSM and ICD:

- The DSM and ICD do not even begin to take note of the state of development of psychological concepts in the field of PDs.

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- The criteria for the disorders are minimal, with many of the criteria that have now been empirically verified not being taken into account.
- No central criteria – the "core" of the disorder – are defined, although it is clear both empirically and theoretically that not all characteristics are equally relevant.
- Essential in-depth characteristics, such as motives, schemas, etc., are not taken into account at all.
- The criteria do not take into account the predominant position of conceiving of PDs as relationship disorders.
- The criteria for determining when a disorder is a disorder are in no way empirically validated and are completely arbitrary. From a therapeutic point of view, they are in part incomprehensible.
- The clusters specified by the DSM-5 are nonempirical and result in summarizing disorders that are completely heterogeneous and that form neither theoretically nor therapeutically meaningful patterns.

In this respect, the DSM-5 is hardly a step forward; and to refrain from even defining PDs can certainly not be regarded as an advance.

In the author's opinion, a therapist can make use of the DSM and ICD criteria as heuristics. However, it is our obligation as scientific practitioners to describe, analyze, understand, and treat disorders appropriately based on our understanding and expertise. It is not our task to standardize them, to make discussions more difficult, and to inhibit developments.

In addition to the DSM, rating systems can also be used in the therapy process (Sachse, 2015a, 2015b; Sachse, Kiszkenow-Bäker, & Schirm, 2016; Sachse & Kramer, 2015a, 2015b, 2015c; Sachse & Schirm, 2015; Sachse, Schirm, & Kramer, 2015a, 2015b). The main advantage of these procedures is that they are based on the concrete interaction behavior of the client – that is, they take into account how the client acts and not only what they express in terms of content.

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Chapter 2

Characteristics of Personality Disorders

2.1 Introduction

As a kind of advanced organizer, essential general characteristics of PDs are summarized in this section (also in distinction to Axis I disorders): Thus, the question is, what characterizes clients with PD and what are the particular features they bring into the therapy?

2.2 Ego-Syntony and Ego-Dystony

Fiedler (2007) spotlighted the aspect of ego-syntony: A disorder is **ego-syntonic** if the person concerned does not perceive essential aspects of the disorder as disruptive, problematic, and necessitating change; moreover, these aspects are perceived as part of the self – as part of one's identity. By contrast, a disorder is **ego-dystonic** if the person perceives essential aspects of it as disruptive: The person does not want to suffer these aspects of their disorder and experiences them as foreign and necessitating change.

Contrary to Axis I disorders, which are usually ego-dystonic, PDs are usually ego-syntonic: the degree of ego-syntony depends on the type and severity of the disorder (e.g., avoidant PD is a little ego-syntonic, whereas obsessive-compulsive PD is highly ego-syntonic).

2.3 Motivation for Change

The degree of motivation to change strongly depends on the ego-syntony (motivation to change is the tendency to actively want to change aspects of oneself).

Motivation to change implies that

- The person realizes that their belief system produces costs;
- That these costs are relevant, and they do not want these costs;
- That changing can reduce costs and make objectives attainable;
- That these costs are self-produced, and that the client must actively pursue their objectives. (Sachse, Langens, & Sachse, 2012)

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If a disorder is ego-syntonic, the person will be able to recognize the costs and realize that they do not want to suffer them, but the person will not realize that they produce these costs themselves. Thus, the person is motivated to cut costs, meaning they want a cost reduction, but are not inclined to do anything for it. Often these individuals are motivated to remain the same (stabilized), which means that they do not want to change their belief system, they just want to avoid the costs.

This means that at the beginning of the therapy, most of PD clients are not motivated to change concerning aspects of their PD.

In other words, motivation to change is not a basic therapy precondition for clients with PD but has to be established during therapy.

However, the clients may be therapy-motivated: They may be motivated to reduce costs or to have the costs reduced by the therapist. The problem is that therapists sometimes consider this motivation to be the same as motivation to change, which is not true.

2.4 Clients Are Motivated to Seek a Particular Relationship

Clients with PD show strong relationship motives and often come to therapy to get these desires satisfied by the therapist. Due to their high relationship motivation and high ego-syntony, PD clients rarely undergo therapy to do actual therapeutic work, therefore they do not accept therapeutic strategies, something that therapists perceive as sabotage.

The strong relationship motivation implies that clients with PD often access therapy primarily for a certain relationship offered by the therapist – in other words, they look for a certain kind of relationship.

Clients might send double messages: "I am miserable, I need help, do something!" and "I can't do anything, I can't collaborate, it is all too difficult!" If therapists then proceed in line with the motto "Something must be done," they will greatly impede the therapy, because the clients are not yet motivated to actively collaborate therapeutically.

Due to the extreme relationship motivation of PD clients, a therapist must concentrate on strong complementary relationship structuring at first; only when a therapist has sufficient *relationship credit* may they move on to specific therapeutic strategies.

2.5 Interaction Maneuvers

Individuals with PD will very likely use so-called interaction games: They use these nontransparent, manipulative actions much more than clients without PD. With these manipulative interaction games (see Tedeschi et al., 1973, 1985; Tedeschi & Norman, 1985; Tedeschi & Riess, 1981), they often cause difficulties for their interaction partners.

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It is important to understand that we (Sachse, Sachse, & Fasbender, 2011) do not regard manipulative actions as such as to be problematic: According to impression management theory (see Mummendey, 1995; Tedeschi et al., 1985), manipulation is an ordinary interaction behavior and an aspect of social competence. The problem is not the manipulation per se but rather the degree of manipulation or the amount of it: With a high degree of manipulative action, one will upset long-term interaction partners.

Clients with PD are characterized by such highly manipulative actions, and they will bring these actions into the therapeutic interaction. As a result, they involve therapists in interaction games. If therapists do not recognize this and/or are unable to constructively deal with it, they will get into great therapeutic difficulty.

The degree of manipulation depends on the type of disorder – clients with histrionic PD are highly manipulative, whereas clients with obsessive-compulsive PD are less manipulative), as well as on the degree of the disorder.

2.6 Tests

Clients with PD sometimes use so-called tests (Silberschatz et al., 1989, 1990; Weiss et al., 1986): Tests are behavior patterns by which the client tries to ascertain whether the therapist is genuine, whether their relationship offer is serious, whether they are reliable, etc. To test these, the therapist will, for example, be criticized, although, for the client, it is not about criticism, it is about ascertaining how the therapist will respond. If the therapist passes the test, they gain a lot of relationship credit; however, if the therapist fails the test, the client may abandon the therapy. Thus, tests are, in a sense, acid tests for the therapy, and a therapist should know what kind of tests may lie ahead, and how they can be dealt with.

2.7 Problems Encountered by Therapists

The consequences of some client characteristics are typical problems for therapists: To start with, clients with PD show no desire to change: They do not regard their behavior as problematic and do not intend to change their structures (such as schematas, manipulations etc.). Clients are, however, often motivated to try to stabilize themselves: They access therapy to stabilize themselves with the help of the therapist, meaning they are undergoing therapy so that they need not change. Therapist often have trouble realizing this: because, on the one hand, the clients do not explain it and on the other, the therapist may not know how to analyze the client's motivations during the process. Further, clients with PD are usually relationship motivated: They access therapy to get a certain kind of relationship. Due to their stabilization motivation and relationship motivation, they are indeed motivated for therapy, but they lack any motivation to refine their assumptions, motives, objectives, etc. If the therapist targets this refining effort, the client will very likely block the therapist, and clients have a number of strategies to systematically avoid refining and to systematically block the therapist. If the therapist does not know and understand those strategies, they will feel helpless and react angrily toward the client, and the therapist's interventions will then exacerbate the problem. Generally, clients will not thematize their relationship issues; thus, it is hard for the therapist to conceptualize these

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issues. The therapist will fail to understand that the client's problem is not just panic or the like, but a massive interaction problem. Because therapists often fail to recognize the real problem and/or have no suitable interventions through which to make the problem clear, they feel helpless, blocked, and frustrated in the long term. As a result, the therapist will only concentrate on the problems and objectives the clients explicitly reports – for example, panic, addiction, somatization disorders, etc. The therapist then applies specific methods to treat these disorders and realizes that these do not work as they usually do (or do not work at all, they exacerbate the problem, new problems arise constantly, or the client does not use the approach at all). The therapist, who has only a limited conceptualization of the client's problem, therefore does not understand these internal therapeutic problems. Even if the therapist realizes the client suffers from a PD, they often do not know which therapeutic approaches are necessary, due to the lack of specific know-how required for these clients. As a result, the clients very often cannot be helped effectively.

Clients with PD are not difficult patients just because they lack certain prerequisites which would be helpful for therapy. The main problem for the therapist is the client's relationship issues which are brought into the therapy: The clients do not thematize or refine these issues, they act them out in therapy. As a consequence, the therapist, suddenly, without being able to prevent it and often without understanding it, goes from being part of the therapeutic team to being part of the problem.

For the development of an effective concept of therapy for clients with PD, it is therefore of crucial importance to provide the therapist with the means to deal with these interaction problems. Thus, an effective therapy for these clients must be one that is process-oriented as well as interaction-oriented.

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Chapter 3

What Is Clarification-Oriented Psychotherapy?

3.1 Introduction

Since both the theory of PDs and the therapeutic strategies described here are based on a concept of therapy from COP, we would like to give a brief overview here of what is meant by COP. This will make the concepts presented easier to understand. COP is a psychologically well founded, highly empirically validated form of psychotherapy that pursues two major lines of approach.

One of these lines of approach relates to clarification: On the basis of a trustful therapist–client relationship actively established by the therapist, the real motives of the client that they are presently unaware of are clarified with a view to eliminating the client's state of alienation. Further, clarification also aims at representing and clarifying dysfunctional client schemas that are co-determining the problems encountered.

The second major task of COP deals with processing and modifying these clarified schemas therapeutically, which enables the client to behave more constructively and flexibly during their daily routines, exhibit less or no disturbing symptoms, better face up to everyday situations both cognitively and affectively, thus leading to a more satisfied self-regulative life.

The key objective of COP focuses on (re)establishing functional self-regulation (Baumann & Kuhl, 2005). The client should be put in a position that enables them to access their motives, appropriately deal with situations, and take decisions that both comply with and face reality, and which are compatible with their motives. Moreover, the client should be able to process information and make decisions without disturbances caused by any dysfunctional schemas, symptoms, or unreasonable behavioral costs.

3.2 Aspects of Clarification-Oriented Psychotherapy

To accomplish the objectives listed in the previous section, COP comprises a number of subdomains that involve tasks therapists have to cope with and that require their expertise to be brought to bear in various different fields.

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3.2.1 Relationship Formation

One domain of expertise in COP concerns the therapist's active and well-directed approach to establishing a relationship. For this purpose, a therapist can adopt strategies of **general relationship formation** or strategies of **complementary relationship formation**. By adopting these strategies, the therapists can build up a trustful therapist–client relationship that forms the basis for all clarification and processing work that follows (Sachse, 1995, 1999, 2006b).

A particularly important aspect of COP is the so-called complementary relationship formation. The concept of **complementary relationship design** was developed by Franz Caspar and Klaus Grawe (Caspar, 1996; Caspar & Grawe, 1982a, 1982b; Grawe & Caspar, 1984). In this context, it states that with respect to the client's *interactional plans*, a coach should act in a complementary way – that is, satisfying the client's needs. The concept is based on the assumption that people in their biographies learn interactional plans (Caspar, 1996) or interactional schemas (Soygüt, Nelson, & Safran, 2001), which they as clients bring into interaction and which strongly influence the relationship behavior of the clients toward the coach.

In COP, complementary relationship formation means that a therapist recognizes and reconstructs the client's central motives for relationships and aligns the relationship formation in such a way that these motives are satisfied as much as possible within the framework of the therapeutic rules (for the therapeutic rules, see, e.g., the subsection No Complementarity at the Game Level, in Section 9.3.1).

Like the classic motives of seeking performance, power, and affiliation, relationship motives are "internal incitements" of behavior (Atkinson, 1964; Heckhausen, 1963). They are defined by subjective goals and subjective expectations: In this case, the goals are focused on certain types of relationships an interaction partner should realize. Here, we distinguish among appreciation, importance, solidarity, reliability, autonomy, and boundaries.

It can be assumed that people have fundamental motives for relationships which they seek to satisfy through their actions (Sachse, 1997a, 1999, 2001a, 2004a, 2013a; Sachse, Sachse, & Fasbender, 2010). These motives are elementary and positive. Satisfying them also has a positive effect on the client's system: The client is contented and feels well treated and understood.

Significant relationship motives (described in more detail below) are

- Appreciation
- Importance
- Dependability
- Solidarity
- Autonomy
- Boundaries

These motives are not all equally important for all people. While appreciation is central to some, it is less important to others, and there are people to whom autonomy is extremely important, while solidarity is significant to others, and so on.

Therapists who want to act in a complementary manner must first find out which relationship motive is of central importance for the respective client and then try to act complementary to that relationship motive. The motive of seeking appreciation implies the need to receive positive feedback from other people about one's own person. The motive of seeking importance implies the need to play a significant role in another person's life and to receive signals

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that saying one is a source of enrichment for another person. The motive of seeking a reliable relationship is the need to receive signals that a relationship is stable, lasting and predictable. The motive for seeking a collaborative relationship is a need to get help and support when you need it. The motive of seeking autonomy is the need (even *in* relationships) to be able to exist and be allowed to exist as an independent person, to be able to make and be allowed to make one's own decisions, and to be able and allowed to have one's own spheres of life. The motive of looking for safe boundaries is the need to define your own territory, one that has safe borders, and for which you can decide yourself who is allowed to enter it and who is not!

3.2.2 Confrontations

Clients, especially clients with PDs, often generate costs for their actions and behavior without realizing that they are responsible for those costs themselves: Accordingly, they have no motivation for change, because they see no reason to change themselves or their belief system. As described in Section 2.2, PDs must be understood as ego-syntonic disorders (and the model of dual action regulation also makes clear why this is the case; see Section 4.2). Thus, these are disorders that do not disturb the person who has them, which are not considered by the person to be problematic or needing to be changed. This means that clients who come into therapy with PDs are not motivated to change, because of their PD, and as a result, they have no concrete **therapeutic goal** with regard to this disorder: They do not want to work on changing their PD, and they do not demonstrate any compliance with the therapist's corresponding interventions. Without a therapeutic goal, however, a therapist cannot work on changing a disorder at all: Without a therapeutic goal, there is no meaningful therapy.

Therefore, if you want to work on changing a PD, you must first create a therapeutic goal (a motivation to change).

If clients want to take the trouble (and it is one!) to clarify and modify their system, then they urgently need to develop a therapeutic goal – that is, they need to build up a motivation for change with regard to aspects of the PD.

But they can only do this when they realize

- that there are high costs, especially high interactional costs;
- that they really do not want to incur these costs, which massively contradict important motives;
- that they generate these costs themselves through their actions and behavior and ultimately through their own goals, assumptions, and strategies;
- that they can substantially reduce their costs by changing these objectives, assumptions, and strategies;
- that they can do this with the help of the therapist.

However, this is precisely what clients with PDs do not recognize on their own initiative (as the model of dual action regulation makes clear; see Section 4.2), rather this knowledge must be actively conveyed to them by the therapist. This means that a therapeutic goal is not a prerequisite for a therapy, but rather the second objective in therapy (the first being the development of relationship credit). And the second phase of the therapy serves to achieve this second therapy goal. In that second phase, the client has to build up a motivation for change and develop a therapeutic goal through the therapist's interventions.

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To do this, the therapist must make it clear to the clients through appropriate interventions:
- that they are actually incurring costs;
- that they do not want these costs;
- that they generate these costs themselves through schemas, etc.;
- that they can reduce these costs by changing the schemas, etc.;
- that they can do this in therapy with the therapist.

However, the therapist must make all this clear to the client, because the client will hardly be able to see this for themselves: In the case of clients with PDs, therapeutic goals do not arise automatically or on their own, but only through a targeted approach realized by the therapist! Clients with PDs recognize that they are incurring costs, but the fact that they generate those costs themselves is hardly recognizable at all due to the ego-syntony of their disorder.

All of this makes it very clear that the therapist's task is to communicate these insights to the client, so it depends crucially on the therapist's interventions whether a client develops a motivation for change and a therapeutic goal, or not.

If one looks at these tasks the therapist has to accomplish, it becomes clear that the therapist via their interventions must draw the client's attention to aspects of their disorder which the client does not recognize and does not want to recognize (according to the dual action regulation model; see Section 4.2) but rather strongly wants to avoid. Therefore, this makes it clear that all interventions that a therapist can carry out here are, by definition, **confrontational interventions** and therefore all these interventions result in a loss of relationship credit. A therapist who carries out such confrontational interventions must have sufficient relationship credit to be able to afford such strategies. If they make such interventions without adequate relationship credit, they can badly endanger the therapist–client relationship.

Even attracting the client's attention to costs they are incurring can be confrontational, but making it clear to the client that they generate those costs through their own actions and behavior, in particular through their own manipulative actions, is very likely to be highly confrontational: Because clients usually do not want to be exposed at this point in particular, and usually react highly aversively when someone tries to makes this a topic of discussion.

It follows, therefore, that all interventions and strategies that serve to make the disorder ego-dystonic and create a motivation for change are very likely to be confrontational.

Confrontational interventions are necessary, for without such interventions, the disorder will not become ego-dystonic, and clients will be unable to develop any motivation to change.

So, therapy cannot be limited to the creation of relationship credits. It must always use the relationship credits to carry out confrontational interventions.

Confrontational interventions are the second essential therapeutic element in therapy with PD clients.

3.2.3 Clarification of Dysfunctional Schemas

Clarification – that is, the conscious and valid representation of dysfunctional schemas, is the central part of COP. From the perspective of COP, many problems are caused by the fact that clients develop dysfunctional schemas in their biographies – that is, certain **assumptions** (or

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basic beliefs) that are unfavorable and result in costs that are incurred by the client. People make a number of assumptions: assumptions about reality, assumptions about themselves, assumptions about relationships, etc. Some of these assumptions are realistic, derived from experience, and withstand everyday testing (empirical tests). But many assumptions are not realistic; they do not reflect reality well, or they are wrong. They do not stand up to examination, but unfortunately, they are no longer examined by the client; they are believed. And some assumptions are unfavorable and lead to problems, including misinterpretation of situations, giving rise to making unfavorable decisions, causing disturbing emotions, etc.

It is precisely these problem-causing or problem-determining assumptions that are tackled in COP. That involves identifying, clarifying, and changing them (Sachse, 1996, 2003, 2005, 2008a; Sachse, Breil, & Fasbender, 2009; Sachse, Breil, Fasbender, Püschel, & Sachse, 2009; Sachse & Fasbender, 2010, 2014; Sachse, Fasbender, & Breil, 2009).

Unfortunately, however, people do not retain assumptions in the same way as normal memories. Rather, assumptions form **schemas**, and in addition to their contents (i.e., the assumptions), there are also other important psychological characteristics of the schemas (Beck, 1979; Herrmann, 1965; Norman, 1982; Piaget, 1954; Power & Dalgleish, 1997; Rumelhart, 1980; Sachse, 2014c; Teasdale & Barnard, 1993). Schemas are automatically activated (triggered) by situations (i.e., in a bottom-up manner) – and once activated, they have a strong influence on the processing of information (top-down). Therefore, the assumptions underlying schemas will to a large extent determine current interpretations of situations and thus emotions, actions, and behavior.

If the schemas contain unfavorable (i.e., dysfunctional) assumptions, then the schemas lead to wrong and problematic interpretations of situations and thus to problematic actions and emotions. In that case, it is important

- to identify and make clear that schemas are involved in a problem;
- to elaborate these schemas and their contents (the assumptions) – that is, to clarify them exactly;
- to work on and change these schemas.

However, everyday experience, therapy experiences, and process research studies now show that people cannot clarify (i.e., name or express in speech) large parts of their schemas without therapeutic help. Often they can name only some assumptions or indicate them in response to questionnaires, while, underlying, deeper assumptions are no longer accessible to them.

Process research studies have shown

- that it is very difficult for clients to clarify schemas;
- that clients need special support from therapists;
- that therapists need special therapeutic techniques to encourage clarification;
- that clarification processes still take time

(Sachse, 1988, 1990a, 1990b, 1990c, 1991a, 1991b, 1992a, 1992b, 1992c).

In schemas, it is possible to differentiate between **contents** and **function**: Each schema has a specific content – for example, a structure of certain assumptions – and these contents make the schema specific. These include assumptions such as: "I'm a failure," "I'm unattractive," "in relationships you are not taken seriously," "I have to be the best," and so on. And, each schema has psychological functions – for example, it is automatically activated by stimuli and it then controls information processing, etc. (Sachse, 1992a, 2003).

Schemas are activated (triggered) by activating stimuli (bottom up), and then they control a person's information processing (top down). Schemas can influence all types of information processing: interpretations of situations, interpretations of personal relevance, coping skills, etc.

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Furthermore, schemas must be acknowledged to have a filtering function: They either let all the information pass through, or they reinforce those pieces of information that are consistent or compatible with the contents of the schema.

We distinguish among four types of schemas (Sachse, Breil, Fasbender, Püschel ,& Sachse, 2009; Sachse, Fasbender, Breil, & Sachse, 2011; Sachse, Püschel et al., 2008):

- Two types of **dysfunctional schemas**:
  - Self-schemas
  - Relationship schemas
- Two types of **compensatory schemas**:
  - Normative schemas
  - Rule schemas

We assume that a client is often unaware of the assumptions of a schema, or they are not completely clear to them, or they are not able to express them well, or that they cannot grasp them precisely. Although the schema contents are in a cognitive code, the client cannot express the contents in speech, at least not exactly, or precisely, or validly. Rather, it is necessary to translate schema contents into language, and express them in accurate and valid formulations, to communicate the contents during the therapy process so that the contents can be fully understood by the client, so the contents can be checked for coherence and problem relevance, and so the contents can be questioned, checked, and refuted if necessary, using cognitive techniques.

The conversion (or translation) of (rather implicit) schema contents into explicit verbal statements is what we call clarification or explication, and the process is called the **clarification** or **explication process**.

In this context, we assume that

- The clarification or explication process is implemented by the client and must be implemented by them: Only the client has access to their schemas, and only the client can translate implicit meanings into explicit meanings in a coherent way.
- The clarification or explication process must, however, be guided or controlled by a therapist through appropriate interventions.
- That client and therapist work together on the clarification process: The client as an expert regarding the contents and the therapist as an expert in the process.

(Sachse, 1984, 1986a, 1986b, 1987, 1990a, 1990b, 1990c, 1991a, 1991b, 2003, 2008a; Sachse, Fasbender, & Breil, 2009).

At the **content level**, one can distinguish between **five subprocesses** of the clarification process (Sachse, Fasbender, & Breil, 2009):

- no problems in focus;
- intellectualization;
- aloof reporting;
- concrete reporting;
- explication.

No problems in focus: The client concentrates on contents that do not personally affect them or that do not touch their problems. The topics they raise have nothing to do with them or their problems. The client thus implicitly follows the *guiding question* (i.e., the question steers the client within the therapeutic framework): Which topics can I use to avoid addressing my problems?

Intellectualization: Although the client has their own problems in focus, they think mainly about how they can be rationalized – that is, they search for (psychological or other) theories

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that can explain away their problems (though the theories do not do that). The client thus implicitly follows the *leading question*: How can I rationalize my problems?

Abstract reporting: The client describes aspects of a problem, but does so in an unspecified, general, or abstract way, without reference to concrete problem situations. The client thus implicitly follows the *leading question*: What are my problems in general?

Concrete reporting: The client describes their problems and identifies them with concrete, relevant situations that illustrate their problem in a characteristic manner. The client thus follows the implicit *leading question*: In which situations do my problems manifest themselves and how?

Explication: The client works on the clarification of current processing processes triggered by a situation and on the clarification of their schemas. The client follows the *leading questions*: What situations trigger a particular schema for me? Why do those situations trigger exactly that schema in me?

All of these processes, which can be defined in terms of contents, are based on two essential psychological functions:

- the perspective the client adopts in each case;
- the processing mode the client uses to process information.

The individual processes of the explication process can also be understood as steps in the process, and these steps follow each other logically, are subject-related, and thus provide an ordered sequence of explication processes (see Figure 1).

In this way, a differentiation can be made between a **preexplication phase** and an **explication phase**, which follow each other and together form the entire explication process.

Based on the results of the process research studies mentioned above, we have to assume the following:

- Different clients enter the explication process at different points: Some in intellectualization step, others in the concrete reporting step, etc.
- All clients enter the process at one of the steps in the preexplication phase.
- All clients have to go through the steps of concrete reporting and focus on the situation.

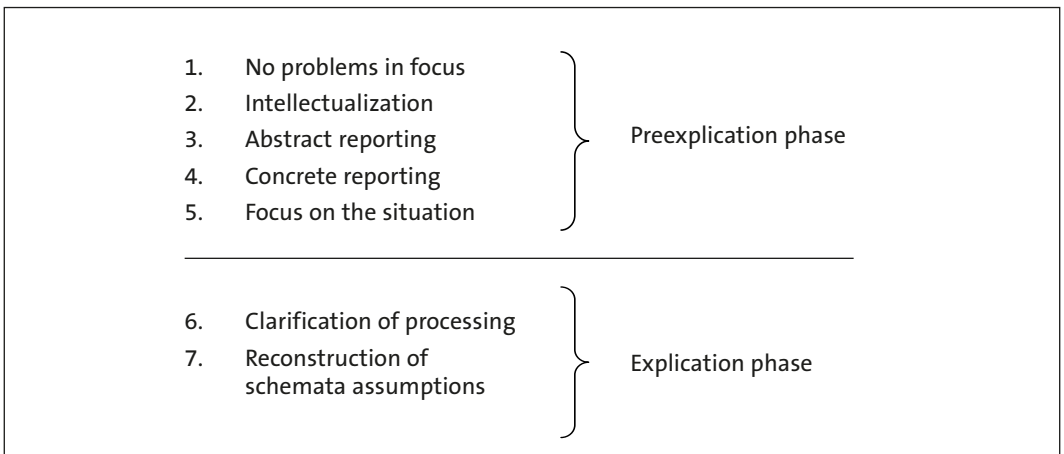


Figure 1
The steps in the explication process.

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A client indicates which of these process steps they are at, by the specific statements they make in the therapy process: We call that their current **processing mode**.

In the explication process, a client passes through the steps from top to bottom, and therefore we call each further step toward reconstruction, a deepening of the processing mode. If a client remains at the same level of the process with two successive client statements, we call this remaining at a constant level of processing mode. If the client moves from one client statement to the next away from the step of reconstruction (upwards), we call this a flattening of the processing mode.

As the process research results clearly show, therapists must control or guide the client's explication process. They thus encourage the client to deepen their processing mode with the help of certain types of interventions.

The inducement which a therapist offers for making an intervention, we call a **processing proposal** (PP): The therapist makes a suggestion to the client of what the client should do, and which direction-setting question the client should follow.

Analogous to the definition on the client side, therapists can now choose

- to make deepening PPs;
- to make constant-level PPs;
- to pursue (but unfortunately, also) flattening PPs.

The process research study results consistently show that therapists with their PPs have a strong influence on the explication process for clients.

Both the empirical results and our therapeutic experience show very clearly that therapists have to support clients in their clarification process very actively. Therapists have to be process-directive; therapists have to stimulate and incite processes, keep them running, raise questions, and guide clients back to the topic or subject and the process, etc. And therapists have to control the clarification process step by step. They have to know at which clarification step (in which subprocess) the client is at the moment, and then they have to try to bring the client into the next subprocess – that is, encourage the client to go to the next clarification step. In this way, the therapist guides the client from step to step until a reconstruction of relevant schema elements is achieved.

In practice, empirical study results and practical experience show that the progress accomplished is not linear, but if the client is at level X, several attempts by the therapist may be needed to take the client to the next level, and more often than not, clients do not remain on one level, but fall back to a lower level by themselves. Thus, it is a laborious undertaking to lead clients to a constructive clarification process. Therapists have to carry out interventions (as we say, make PPs) again and again, to help clients through the process and keep them involved in the process.

Therapists should in any case make PPs with a view to guiding the client's process effectively. But they should also make the appropriate offers. This means that they must make different interventions – make different types of deepening PPs – depending on the phase (subprocess) in which the client is in each case.

3.2.4 Therapeutic Processing of Dysfunctional Schemas

If relevant schemas have been sufficiently clarified, they must be systematically worked on therapeutically. They must be disputed, refuted, and thus inhibited, and alternative functional schemas must be developed. COP uses the so-called one-person role play (OPRP; German: *ein-personen-rollenspiel*, EPR) for this purpose.

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OPRP is a therapy technique in which a client is instructed to act as their own therapist and in that role to question, dispute, and debate dysfunctional schemas, and develop alternative assumptions (see Sachse, Püschel, Fasbender, & Breil, 2008). In the sense of this used by Grawe, the procedure serves the purpose of schema processing, schema clarification, resource activation, and the motivation of clients (Grawe, Donati, & Bernauer, 1994). OPRP is a therapeutic framework within which a therapist can implement different techniques, in particular:

- different cognitive intervention techniques for the processing of cognitive schemas;
- different affective techniques for processing affective schemas;
- different motivational techniques to increase the change motivation of the clients.

The method can thus always be used in a meaningful way

- if cognitive and/or affective schemas contribute significantly to a client's problem (e.g., in the case of depression, anxiety, or PDs);
- when it comes to significantly strengthening the motivation to change of clients.

In OPRP, the therapist defines two positions for the client:

- **Client position** (CP): Client as client.
- **Therapist position** (TP): Client as their own therapist.

To make both positions apparent to the client and to make it easier for the client to move from one position to the other, the therapist arranges two chairs opposite each other (so that two persons sitting opposite to each other can look at each other) and defines on each chair one of the positions for the client. The therapist places their own chair at right angles to the two chairs.

The spatial separation of the two client positions with the help of the two chairs is advantageous because clients often have difficulty, especially at the beginning, to adopt a different perspective and to step out of their accustomed frame of reference. The spatial separation of these two positions makes it easier for the client to distance themselves from their own belief system and bring about a mental and spiritual change of position Thus, changing the chair serves to help the client,

- to differentiate between aspects of their disorder;
- to distance themselves from their own assumptions;
- to adopt new perspectives consistently and thoroughly;
- to rethink these new perspectives, and in this context activate memories as a consequence;
- to try to believe these positions, to play with them, and let them have an effect on themselves.

OPRP proceeds in a series of three steps (with the therapist taking the role of "supervisor"):

First step: The client sits in the client's position (CP) and expresses the dysfunctional assumption.

Second step: The client sits in the therapist's position (TP), elaborates with the supervisor the counter strategies against the dysfunctional assumption, and then directly tells this to the "client," who they imagine to be sitting in front of them on the opposite chair.

Third step: The client sits in the CP again and together with the supervisor checks the coherence of the counter strategy.

The cycles of these three-step sequences in OPRP are discussed in more detail below. The first step begins when the therapist and client have succeeded in identifying and extracting an assumption that can be worked on – that is, an assumption that is now to be examined and changed in the role play and which can be changed.

The second step of the cycle begins when the therapist requests that the client to assume the therapist's position (TP). With the assistance of the therapist now as supervisor, the client then

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works on the formulated assumption. The client and supervisor have quite a number of therapy strategies at their disposal. The client is guided to be their own therapist and as such to deliberately distance themself from the "client's" assumption. When the client sits on the therapist's chair, the supervisor says,

> You are now your own therapist. That means you strongly disagree with your client. It is now your job to find something that will help your client. I support you in this. First of all, we can consult on points that might be helpful. Your client says … [the supervisor repeats here the assumption uncovered in the CP]. What could help your client?

Then the supervisor and client–therapist formulate a (more or less long) **counter strategy**. This process can be quick or lengthy, depending on how well the TP is able to distance themselves and how quickly they can develop objections and counterarguments.

The third step of OPRP starts when the client switches back to the client position and then examines the counterarguments that the client–therapist has previously voiced. When the client sits in the client position again, the supervisor gives them the instruction:

> You are now a client again. Your therapist says, … [supervisor repeats the counter strategy]. Now let that have an effect on you and think it over. What aspect of that is conclusive for you? … And what doesn't convince you?

This plausibility check is carried out in two steps. In the first step, the client should determine which of the aspects of what was uncovered convince them and why. This is important so that the aspects of their disorder which are already convincing are first thoroughly appraised and analyzed; in this way, the supervisor brings in the harvest, so to speak. In doing so, the supervisor should by all means take the time to review the new findings with the client several times, to anchor them in the client's memory, and already at that point deduce some initial consequences.

In the second step of the plausibility test, the client should then determine which aspects of what was uncovered do not convince them (and why not), whether there is any "but"– that is,an emotional resistance or a (diffuse) discomfort. The therapist as supervisor here makes it clear here that OPRP is not about persuading the client or imparting contents to the client that they can not in fact accept. Rather, all this is about finding contents that the client will be able to integrate. Therefore, the therapist as supervisor instructs the client here to very carefully and critically examine the counterarguments put forward by the "client"-therapist. However, should the client find a "but," the therapist as supervisor and client clarify in detail what this but is; in doing so, new schema aspects are often worked out which are then the starting point for a new cycle.

3.3 Empirical Findings

3.3.1 Process Research Metrics

Within the scope of COP, various rating systems for recording client and therapist actions in the therapy process have been developed at the micro and macro levels and tested for reliability and validity.

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- Scales to measure the client's depth of processing and the therapist's constructive process directions (see Hammink, 1997; Kayser, 1993; Lookman, 1993; Oosterling, 1999; Sachse & Maus, 1987, 1991; Sachse, 1992b; Sachse & Takens, 2003; Silberg, 1994; Takens, 1995, 1996).
- Scales for the recording of different client actions or behavior in the therapy process (processing of contents, explication, avoidance, interaction games, etc.), as well as for the recording of constructive therapist actions (directions, understanding, relationship formation, processing of avoidance, etc.): the so-called Processing, Contents, Relationship (PCRS) scales (see Kramer & Sachse, 2010, 2013; Sachse, 2010, 2013b; Sachse & Schirm, 2015; Sachse, Schirm, & Kramer, 2015a, 2015b; Sachse, Schülken, Leisch & Sachse, 2011; Sachse, Schülken, & Leisch, 2006).

3.3.2 Process Research

In a series of process research studies, the strategies postulated by COP such as direction, complementarity, confrontation, etc., could be empirically confirmed, so that the approaches adopted in COP can be regarded as very well empirically validated (Bullmann, 2006; Frohburg & Sachse, 1992; Kramer & Sachse, 2010, 2013; Kramer et al., 2015; Lewis, Weiland-Heil, & Sachse, 2018; Reicherts & Montini Lirgg, 2006; Sachse, 1988, 1990a, 1990b, 1991c, 1992a, 1992b, 1992c, 1993, 1994, 1995, 1997a, 1997b, 1997c; Sachse & Atrops, 1989, 1991; Sachse & Kramer, 2015a, 2015b, 2015c; Sachse & Maus, 1987, 1991; Sachse & Neumann, 1986, 1987a, 1987b; Sachse & Rudolph, 1992a, 1992b; Sachse & Sachse, 2009, 2016a; Takens, 2001).

3.3.3 Research on Success

In several studies, the effectiveness of COP in general (Sachse, Schülken, Leisch, & Sachse, 2011), for depression (Sachse, Sachse, & Diermann, 2019), for psychosomatic disorders (Sachse, 2018; Sachse & Sachse, 2016e), and for PDs (Freudenberg, Weiland-Heil, & Sachse, 2018; Sachse, 2018; Sachse, Fasbender, Breil, & Püschel, 2009; Sachse & Sachse, 2016b, 2016c, 2016d, 2016f, 2016g) could be proven.

Overall, there were significant to high effect levels – that is, precisely for those variables in which the disorder-specific orientation of the COP was meant to achieve effects. Therefore, COP can be regarded as a very effective form of therapy.

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Chapter 4

General Psychological Function Model for Personality Disorders

4.1 Introduction

In this section, a general psychological function model for PDs is presented: the **model of dual action regulation**. This model specifies which psychological variables matter for PD, how these variables interact, and what kind of consequences result from these interactions (see Sachse, 1997a, 2000a, 2001a, 2001b, 2002, 2004a, 2004b, 2004c, 2005, 2006a, 2006b, 2013a, 2014a, 2016a; Sachse, Sachse, & Fasbender, 2010). The dual action regulation model represents a theoretical framework for PDs.

4.2 Dual Action Regulation Model

4.2.1 Introduction

The dual action regulation model is based on the assumption that PDs may be understood as relationship disorders (or, in other words, interaction disorders). To be sure, PDs are indeed complex disorders involving disturbances in behavior, reasoning, feelings, certain patterns of information processing, etc. However, it can be assumed that dysfunctional beliefs about relationships, dysfunctional intentions regarding interactions, and dysfunctional types of relationship structuring constitute the core of the disorder (Figure 2).

The model is called the dual action regulation model, because it is proposed that there are two action levels:

- the **motive level**, or level of authentic action regulation;
- the **level of disfunctional schemas**
- the **maneuver level** (or "game"-level), or level of manipulative or nontransparent action regulation

At the motive level, the person pursues their actual motives and objectives, their approximation objectives – that is, those objectives whose achievement will bring positive affects and satisfaction. These effects satisfy the motive and result in a long-term satiation of the motive, whereby their relevance decreases. At this level, the person acts authentically – that is to say,

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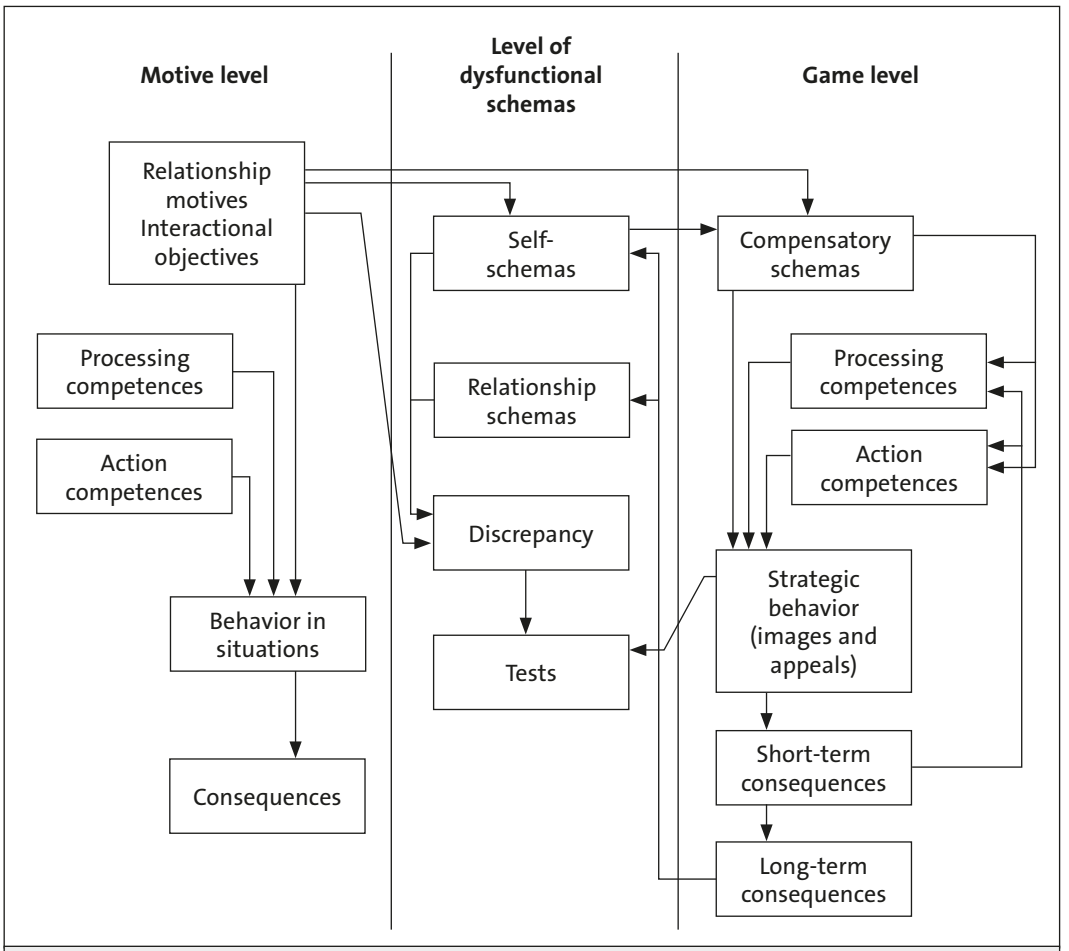


Figure 2
The dual action regulation model.

the person acts in such a way that the interaction partner is able to recognize their objectives. The person means what they say, and their behavior is consistent with their motives.

At the game level (maneuver level), the person is already pursuing interactional objectives which do not represent their real motives but are explicit, often extrinsically motivated objectives, in the sense of being for the purpose of avoidance (i.e., avoiding particular themes or questions). The norms and rules followed usually do not represent what the person actually wants and what would actually satisfy them. In this sense, the person does not even act authentically. They also delude their interaction partner. They will communicate images of themselves that are either exaggerated or simply untrue (see the subsection Game Behavior: Images and Appeals, in Section 4.2.4). By transmitting these images, they more or less systematically deceive the interaction partner. On the basis of those images, the interaction partner is compelled to act in response to their appeals. As the interaction partner trusts these images of the person and accepts them, they will therefore act on the basis of false assumptions. They will be compelled to perform actions they would not have done without the images (i.e., without

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the deception). The appeal itself, however, is transparent, which means the person clearly communicates to the interaction partner what to do and what not to do.

This is a crucial aspect of manipulative behavior: Through manipulative actions, an interaction partner is led to perform actions which the manipulator thinks the interaction partner would not do on their own initiative.

4.2.2 The Level of Authentic Action Regulation, or Motive Level

The dual action regulation model proposes that a person has a number of **central relationship motives** already in childhood (or adolescence), such as the motive of seeking appreciation, importance, dependability, solidarity, autonomy, and boundaries or territoriality (see Langens, 2009; Sachse, 1999, 2006a; Sachse, Breil, & Fasbender, 2009; Sachse, Fasbender, & Breil, 2009).

The motive of seeking appreciation is based on the desire to obtain positive feedback from other (relevant) persons, the feedback that one is ok, and loveable, has positive qualities, and is appreciated as a person. The motive of seeking importance is based on the desire to play a crucial role in the life of another person and the acknowledgement of that: One wants to be meaningful for someone. The motive of seeking dependability is based on the desire to be confident in the stability, durability, and resilience of relationships. The motive of seeking solidarity is based on the desire that one will get help, support, encouragement, protection, etc. from another (relevant) person, when needed. The motive of seeking autonomy is based on the desire that other (relevant) persons respect that one will make one's own decisions, will behave in a self-determined way in essential matters, and will take responsibility for oneself. The motive of seeking boundaries and territoriality is based on the desire that other (relevant) persons accept that one can define boundaries for personal domains which may be protected and which other persons must not invade without permission (e.g., the domain of one's own body, own room, own desk, etc.).

These motives constitute a hierarchy, meaning that for a particular person, one motive will be central (high on the hierarchy), some motives are quite important, and others are relatively unimportant (the central motive also characterizes the type of PD). Furthermore, at the level of interactional objectives, these motives are made concrete, or are rather operationalized, in concrete, situational objectives. Thus, the motive of seeking importance may be operationalized in objectives such as:

- I want to get attention.
- I want to be recognized by others.
- I want to be taken seriously.
- I want to be respected by others.
- I want to be seen by others.
- I want to get signals of affiliation.

The person will then behave in a way, especially in interactional situations, that will satisfy their motives or rather satisfy the deduced interactional objectives, or more precisely, try to have them satisfied by others (because they are, after all, relationship motives that can only be satisfied by other persons!). For that, the person uses processing and action competences.

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Depending on how effectively the person behaves, they will experience positive consequences (i.e., the behavior of others is **motive complementary** = motive satisfying) or negative consequences (i.e., the behavior of others is not motive complementary or is even motive offending).

This level is called the **level of authentic action regulation** because the person usually behaves in a way that renders their interactional objectives transparent for the interaction partner, so that the interaction partner knows (or at least can deduce) what objectives the person has, is able to anticipate the effects, and therefore knows what they are getting themselves into. Usually, the satisfaction of motives is only obtainable if the interaction partner is aware of the person's intentions (e.g., a woman cannot react, even positively, to a man approaching her, if she cannot identify his behavior as an approach, and if she thus does not understand his intentions!).

To act in a way that the behavior results in positive consequences, the person needs **competences**. One can distinguish between action competences and processing competences (Sachse, 1999). **Action competences** are composed of knowledge about action strategies ("What can I do in certain situations to achieve certain objectives?") and the ability to modify and adjust strategies flexibly. The action competence of a person is larger, the larger their action strategy repertoire is and the more flexible (and faster) they can adapt these strategies.

Processing competences include the ability to analyze and understand situations, especially social situations, well and quickly. Furthermore, they especially include the ability to engage in social empathy: The ability to put oneself in somebody else's shoes, quickly understand what the other wants, which behavior they will react to positively, etc. Thus, processing competences have a lot to do with a theory of mind (Fodor, 1978; Förstl, 2007; Premack & Woodruff, 1978). Individuals who are able to quickly form reconstructions of an interaction partner are able to adjust their behavior (if they have appropriate strategies!) toward them and can thus obtain positive interaction effects. Individuals who do not have this ability will constantly blunder.

If relationship motives, or rather interactional objectives, are satisfied through complementary behavior, a feeling of satisfaction will occur (Brunstein, 1993; Brunstein et al., 1995). As motives are consistently satisfied over a long period, their relevance will decrease: They will descend in the motive hierarchy, other motives will become more important and will take over the "executive position" (Kuhl, 1983, 2001).

However, if central motives are frustrated over a long period, they will remain high in the hierarchy or will even ascend (Kuhl, 1983, 2001). The more massively they are frustrated the more important they become! If they stay crucially important, they will dominate the executive position for a long time, which may mean other motives cannot be satisfied sufficiently. The result will be a state of more or less great dissatisfaction.

This means that frustrated motives stay high on the motive hierarchy!

As will be shown, the behavior of PD clients leads to a lack of satisfaction of their central relationship motives. And because they do not follow other motives, these other motives will not be satisfied either: Thus, individuals with PD usually show a state of high, general dissatisfaction.

4.2.3 The Level of Dysfunctional Schemas

Schemas are stable beliefs that people learn in the course of their life. Schemas are activated (bottom up) automatically by situations and then influence (top down) the processing of current

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information and regulation of action (Beck, 1979, 1999; Beck et al., 1996; Crocker et al., 1984; Mandler, 1979; Rumelhart, 1980; Sachse, 2006c, 2014b; Sachse, Fasbender, Breil, & Püschel, 2009; Sachse, Schülken, Leisch, & Sachse, 2011; Sachse, Püschel, Fasbender, & Breil, 2008). Dysfunctional schemas are those whose activation generates unfavorable, cost-producing processing and regulatory processes.

Self-Schemas and Relationship Schemas

At the second level of the model, we work from the assumption that there are dysfunctional schemas which strongly determine a person's information processing and action regulation (see Beck et al., 1990). For the term and function of schemas, see Sachse (1996, 1999, 2003, 2006b) and Sachse, Püschel, et al. (2008).

We work from the understanding that there are two different types of dysfunctional schemas in persons with PD: self-schemas and relationship schemas (see Sachse, Breil, & Fasbender, 2009). **Self-schemas** are those that contain assumptions about oneself such as "I am a failure," "I am not important," etc., as well as contingency assumptions and valuations about oneself. **Relationship schemas** are those that contain assumptions about how relationships work and what to expect from relationships, as well as contingency assumptions and valuations about relationships (e.g., "One will be denigrated in relationships," "Relationships are not reliable," etc.).

These schemas in this form are **dysfunctional** because they lead to negative expectations, but also, in particular, because they lead to negative interpretations of situations, to negative affects, etc. They determine a fast, highly automated information processing and lead to something that we call a **hyperaversive reaction**: Minimal situational triggers quickly provoke an intense (affective) reaction. Thus, someone who has a schema such as "I am not important" will most likely react heavily to any hurt and be offended by even minimal inattention on the part of the interaction partner.

Development of Dysfunctional Schemas

A key assumption in the model is that dysfunctional schemas in a person's life evolve through interactional experiences which children and adolescents have with relevant caregivers with regard to their central relationship motives (see Nowacki, 2009). We work from the understanding that individuals, in childhood or adolescence, have had highly relevant experiences with important intimate partners (usually parents) concerning the satisfaction of central relationship motives. These experiences strongly affect, for example, the development of self-schemas and relationship schemas, probably for biological reasons.

For example, the child has positive experiences relevant to the importance motive when they

- are taken seriously by the parents;
- get attention from the parents;
- get signals such as "we like spending time with you" and "we like being with you";
- get signals such as "you enrich our lives."

If the child gets this kind of feedback consistently for a long time, the child will develop positive schemas. In other words, a positive self-schema will develop, as conveyed by statements such as

- "I am important."
- "I have a lot to offer, etc."

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And a positive relationship schema will develop, such conveyed by

- "I will be taken seriously in relationships."
- "I will be noticed and will get attention in relationships, etc."

However, if the child gets negative feedback concerning their importance motive, they will receive messages such as:

- "We don't have time for you."
- "We don't care about you."
- "We don't listen to you."
- "We don't pay attention to you."
- "We don't take you seriously, etc."

Or even messages like

- "You're bothering us."
- "It would be better for us, if you weren't here."
- "Without you I could have had a career, etc."

If the child gets this kind of negative feedback consistently for a long time, then a negative self-schema will evolve, such as that conveyed in the statements

- "I am not important.."
- "I have nothing to offer."

Or even:

- "I am toxic for others."
- "I am a bother to others."

Furthermore, a negative relationship schema will develop as conveyed in the statements:

- "I will not be taken seriously in relationships."
- "I will not get any attention in relationships."
- "Nobody will listen to me in relationships, etc."

The systematic frustration of relationship motives does not just have an influence on schema development: Frustrated motives also stay high in the motive hierarchy, and subjectively, due to frustration, the motives become even more important. Through systematic frustration in the first place, a relationship motive becomes the dominating motive!

Such schemas, one can assume, evolve slowly: Schemas are formed through conclusions drawn from experiences. Thus, a negative schema may develop from just a little negative but long-term feedback.

As schemas are developed, they will, if activated, strongly determine information processing and behavior. Schemas often lead to hyperaversive reactions: When someone has the schema "I will not be respected in relationships," they will then react hyperaversively to all behavior of others that can be interpreted as lack of respect. That means, the person will react quickly and intensely, becoming sad or angry, depending on their schema.

Thus, schemas can lead to intense reactions triggered by minimal stimuli!

This level of dysfunctional schemas is is characteristic for individuals with PD (Beck et al., 1993), and clients with certain specific PDs show types of schemas characteristic for those. Therefore, a schema such as "I am not important" is characteristic of clients with histrionic PD; a schema such as "I am a failure" is characteristic for clients with narcissistic PD (see Sachse, 2002, 2004a, 2004c, 2006c, 2006d).

As dysfunctional schemas obviously evolve from frustrated motives, it is to be expected that the central, frustrated motives of a person, which still remain high in their motive hierarchy, and

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the central negative schemas of that person are formulated oppositely. If the frustrated motive is that of seeking importance, then the related schema will be "I am not important." Central relationship motives and central dysfunctional schemas are thus strongly associated in terms of content: The one is derived from the other.

Thus, dysfunctional schemas are, in a sense, the negation of the relevant relationship motive.

4.2.4 The Level of Nontransparent Behavior, or Game Level

Solutions

We assume that the development of a PD is by no means complete with the formation of negative schemas, however. We rather believe that a person exposed to difficult and frustrating interaction experiences in the sense described in the section Development of Dysfunctional Schemas, above, will not usually passively give up hope, but will actively try to find a solution. And these solutions are the most exciting part of PDs.

An essential part of PDs may be seen as a solution to early interactional problems.

These solutions constitute the second action level in the dual action regulation model: the level of nontransparent behavior, or the game level.

Development of an Nontransparent Action Level

The development of this action level begins with a difficult, persistent interaction situation for children and adolescents: Important interaction partners frustrate permanent important relationship needs. They then have to react to this situation. Children whose central relationship motives are consistently frustrated, for example, have two options:

1. They can give up and stop acting, start adjusting themselves, or go through a depressive development.
2. They can try to develop a behavior that helps them to get the interaction partner to satisfy at least some aspects of their motives.

We assume that only the second strategy leads to the development of a PD or at least to a personality style. We also assume that people, children as well, are usually active, problem-solving organisms. Thus, the second strategy is much more probable than the first one.

Individuals, then, slowly, step by step, develop strategies to get important interaction partners to satisfy certain interactional objectives: These strategies develop if (and only if) they are successful, and thus the individuals view these strategies as good solutions for difficult situations. That is why the strategies become ego-syntonic: they are perceived as part of the self and not as disturbing, dysfunctional, or problematic.

Ego-syntony results from the fact that certain central aspects of the PD are perceived as helpful and positive, therefore they cannot be seen as problematic.

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This strategy, of encouraging interaction partners to satisfy certain aspects of important motives, has far-reaching implications:

- The interaction partners (usually the parents) who do not satisfy the child's motives obviously have no intention of doing so or even have the intention of not doing so.
- Therefore, the child has to make them do this, although they do not have any intention of doing so or even have the intention of not doing so.
- But by making them do it, the child cannot behave authentically, because that obviously had not worked so far.
- To make them do it may consequently only mean to behave strategically and manipulate them: Make them do something they actually do not want to do.

The term *manipulation* is not meant to be judgmental but is used here in its psychological sense. Somebody is manipulated when through strategic behavior they are made to do something they do not actually want to do. And these strategies are usually nontransparent: The one who is manipulated is not aware of the manipulator's objectives; they do not see what it is about. Based on terms used in transactional analysis and self-handicapping research, we call this kind of manipulative behavior **game action** or just **games** (see Berne, 1961, 1967; Mummendey, 1995).

Manipulative behavior serves to make interaction partners behave in a way they normally would not (or rather in a way that the client thinks they would not of their own volition)ing strategies the purpose of which the other person cannot see through.

Children can use positive or negative strategies:

- **Positive strategies** are those where the different intentions and motives of interaction partners are used to obtain positive effects. A child may not receive attention as a person (e.g., because the father has no interest in the child), but as the father wants family members to make good impressions on others, the child can use this intention for its strategic purposes. It can be funny and entertaining at a family celebration. Because the father likes this behavior (as it matches his objectives) he will give the child attention for it.
- **Negative strategies** are those where norms and avoidance aims of others are used to obtain effects. A child who does not get attention as a person may get attention when simulating a headache. The father may have the norm "One has to care for sick children otherwise one is a bad parent" and will thus care for the child. He actually does not want to do it, and it is no pleasure for him to do it, but he does it anyway to avoid having a bad conscience or to avoid negative feedback from others (e.g., his wife).

Positive strategies for satisfying motives are said to be positive because interaction partners usually react positively to them: What the person does reflects their intentions, and thus they think it is proper. Thus, the person is easily manipulated and will not react angrily. This is one of the great advantages of positive strategies: One can use them for a long time without making someone angry. (In the long term, however, these strategies may incur costs if they are used excessively. The interaction partner may feel they are being used and no longer react positively.)

A disadvantage of positive strategies is they do not have a great impact: If a person does not want to react to the strategy, it is easy for them to ignore it. The strategies are not compulsory.

Negative strategies on the other hand do have a great impact; they are compulsory. The interaction partner will not be willing to infringe the norm involved and may therefore feel

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forced to react. This can quickly make them angry, and this is therefore the disadvantage of negative strategies: They provoke anger in interaction partners very easily. Usually, one can only use them briefly and in small doses, otherwise negative consequences may occur.

These strategies are manipulative (note again: manipulative is not meant to be judgmental, but is just meant in a psychological sense) because the interaction partner is being deluded about the action's objective. The child who is funny using this strategy, is not being funny for the sake of it (even though the father believes this to be the case) but to get his attention (which the father does not realize). In response, the child simulates something that is untrue and leaves the interaction partner in the dark about their real intentions. This is manipulative: The interaction partner is made to do something for reasons they are unaware of. The strategic behavior in this respect is also nontransparent.

Usually, negative strategies are much more manipulative than positive ones. The father who cares for the child having a headache, for example, is highly deluded about the actual problem. In additionally, positive strategies can still work even if the interaction partner realizes what the strategy is (e.g., the father can still react positively even though he realizes that the funny child just wants his attention). However, negative strategies will usually fail if the interaction partner realizes the real intentions (e.g., if the father realizes the child is not really suffering from a headache, he will most likely turn away angrily).

Children soon find out which strategic behaviors work well and learn to use them:

- They learn by trial and error: A certain behavior brings attention and will, therefore, be maintained.
- They learn by modeling: Aunt Emma gets attention for being funny, and this can be imitated.
- They learn because they understand the system and realize which strategy causes a certain behavior, and who expects what from them – since satisfying expectations is the best way to guarantee positive reactions.

Over time, children, in terms of strategic behavior, develop greater processing and action competences. They learn **action competences** as they acquire more and more strategies enabling them to achieve their goals. Furthermore, they learn to adjust, modify, and use them. They develop **processing competences** because they learn which person can be manipulated (and which not) based on certain characteristics, and they learn to use certain strategic maneuvers in a given situation. They learn to assess people and understand situational demands.

If a person has good strategies and is very skilled, they will be able to behave in such a way that interaction partners will most likely behave complementarily to their strategies, meaning positive action consequences will occur. The longer and the more intensive the person practices, the better their skills at strategic behavior. Some clients even become experts!

As has been said, we refer to this level of nontransparent behavior, based on Berne (1961, 1967; Tedeschi et al., 1985) as the game level, which we think is an apt formulation, because the manipulative strategies appear to resemble games in which someone makes a move, and the interaction partner answers with a countermove – that is, a real game between adults on a certain level (see Sachse, Sachse, & Fasbender, 2010, 2011; Sachse, Fasbender, Breil, & Sachse, 2012).

It is an essential feature of PDs that the individuals affected develop manipulative strategies bringing with them high interactional costs in adulthood.

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Compensatory Schemas

On the game level there are **compensatory schemas,** which serve to compensate for dysfunctional schemas (on the schema level) or to ensure sure they do not prove to be true. On the one hand, there are compensatory self-schemas with positive (or compensatory) exaggerated self-assumptions; on the other hand, there are compensatory normative schemas which hold (mostly exaggerated) objectives (in the form of "have to" sentences). Additionally, there are compensatory rule schemas which hold (mostly exaggerated, as well) instructions for others. Sachse, Breil, and Fasbender (2009) refer to these schemas as "compensatory" because their sole objective is to iron out the contents of self-schemas and relationship schemas on the schema level or make sure those specified contents and consequences do not occur.

Compensatory Self-Schemas

Compensatory self-schemas are those that hold positive or even exaggerated positive statements about one's own person (to balance out negative self-schemas). A person with negative self-schemas such as

- "I am a failure."
- "I am not able to satisfy expectations."
- "I am incompetent, etc."

sometimes develops a compensatory schema (either of an illusionary or realistic type) using assumptions such as

- "I am successful."
- "I am highly intelligent."
- "I am highly efficient."

The two self-schemas are separate and will usually inhibit each other (see Sachse, 1999, 2001b, 2002, 2004b). If one of the schemas is activated, the other one will be inhibited. The person switches between the schemas and thus switches from one state of mind to the other. Compensatory self-schemas contain no goals but condition statements – that is, statements about the self.

Normative Schemas and Avoidance Goals

Another kind of compensatory schemas are **normative schemas**. These include instructions about how the person should be. They thus contain the person's goals (in the sense of explicit objectives; see Püschel & Sachse, 2009). Normative schemas are therefore interactional objectives on the game level.

Normative schemas are those that contain instructions to the self such as "Be successful," "Be the best," "Be the most important," "Avoid incurring blame," and "Avoid any kind of situation where you could be criticized."

Thus, normative schemas contain objectives in the form of rules: "Be XYZ," "You have to do XYZ," "You may not do XYZ," etc. These instructions can be more or less binding for the person, depending on how the contingency level is defined (see Sachse, Püschel, et al., 2008).

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First of all, the contingency level of normative schemas contains assumptions about which negative consequences, as threatened by the dysfunctional schemas, will not occur when following the instructions. For example:

- "If you are the best, nobody will repudiate you."
- "If you are the best, nobody will turn away from you."

These schemas are called compensatory schemas because they specify the nonappearance of the consequences (which are threatened by the schemas). The contingency level may also threaten consequences in the case of the nonfulfillment of the norm: These are either the same consequences as specified by the dysfunctional schema or even worse consequences. These consequences may range from light to medium: "If you're not successful, nobody will like you." Or they may threaten massive existential disasters such as "If you make any mistakes, you are absolutely morally reprehensible and no longer belong to the human race." Thus, the normative schemas may include threats with consequences that are even worse than those of the dysfunctional schemas.

It is important to notice that all of these norms define objectives that compensate for the assumptions of the dysfunctional schemas. If the schema, for example. is "I'm a failure," then the normative schema will be something like "Be successful," "Present yourself as intelligent," "Be the best." These are all objectives aimed at falsifying the negative assumptions of the dysfunctional schemas or ensuring that they are not true. The normative schemas are closely linked to the dysfunctional schemas, and, even more important, the objectives are all by definition avoidance objectives!

One does not want to be successful because it is so much fun (which would be an approach objective), and one does not want to be the best because one would enjoy it. Instead one wants to be successful and the best because one wants to show that one is not a failure! Normative objectives do not strive for positive states or positive affects, they strive for the abolition or avoidance of negative states or affects.

This also holds true for objectives such as "Get attention" or "Be the most important." One wants these because one thinks (due to the dysfunctional schemas) that one is not important or does not get any attention. Therefore, one is not intrinsically motivated, does not strive for positive affects, but one always strives for the abolition or avoidance of negative affects.

All objectives of normative schemas are avoidance objectives.

Avoidance objectives operate psychologically differently from approach objectives (see Ebner & Freund, 2009; Elliot & Covington, 2001; Kuhl, 1983, 2001): The pursuance and achievement of approach objectives satisfies central motives and leads to a state of satisfaction (Brunstein, 1993, 1995, 2001; Brunstein et al., 1995, 1998), to a decline in the motive hierarchy (Kuhl, 1983, 2001), and thus to a decrease in effort. However, the pursuit and achievement of avoidance objectives leads to a reduction of anxiety and tension (C-), but not to a satisfaction of motives and thus not to a state of satisfaction (Brunstein, 1993, 2001; Brunstein & Schultheiß, 1996; Brunstein et al., 1996, 1998; Kuhl, 2001; Kuhl & Koole, 2005): Despite the pursuit of the avoidance objectives, the central motive of appreciation ranks high in the motive hierarchy.

While the attainment of approach objectives leads to positive affects and to a satisfaction of motives in the long term (and thus to a fall in the motive hierarchy) as well as to a feeling of satisfaction, the attainment of avoidance objectives only leads to the disappearance of negative affects and emotions (of tension, discomfort, or anxiety) and not to a satisfaction of motives,

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and thus not to a feeling of satisfaction, with no decline of the motive in the motive hierarchy in the long term. Although dysfunctional schemas are derived from relationship motives, and normative schemas are derived from dysfunctional schemas, the person is mainly busy following the normative objectives. Although the person will achieve a reduction in negative affects and therefore an affirmation of the behavior, they will not achieve any satisfaction for the underlying motive. These motives remain high in the motive hierarchy and stay dominant, no matter what the person does.

The pursuit of compensatory objectives on the game level will not satisfy underlying relationship motives, so that they remain high in the motive hierarchy.

The strategic behavior meant to satisfy the objectives on this level always creates effects but never leads to a real satisfaction of motive. This behavior is thus insatiable (in the sense used by Gollwitzer, 1987, 1999). Because the activation of dysfunctional schemas always leads to negative affects and effects, one always has to act again to achieve the avoidance objectives. The achieved effects always generate relief but never change the system as such.

All compensatory objectives are insatiable.

Another important aspect of this which ensures that compensatory behavior will not lead to a lowering of motives in the hierarchy is that the user of the strategic behavior will only get feedback for the behavior but never for themselves as a person. They will not get attention as a person merely because they are funny and entertaining.

However, the relationship motives on the motive level all propel the desire to obtain feedback as a person or more precisely, to be important as a person. Through strategic behavior, they will never achieve any importance as a person but will only receive feedback for their behavior.

Again, the effects one can achieve through strategic behavior cannot satisfy the central relationship motives: No matter what is done on the game level, no matter how hard one tries, no matter who one wants to manipulate, it will never be enough to satisfy the central relationship motive and achieve a state of satisfaction.

There is another aspect to be considered here as well: Even if the person has a partner who gives them feedback as person ("You are really important to me"), the schema ("I am not important") will still operate as a filter. The person will not be able to believe the feedback. Thus an "authentic feedback" if given does not stand a chance to satisfy their motives.

Nevertheless, the strategic behavior is satisfying, and will thus be intensified constantly: Because when you do something to get attention, you will be given attention. In this way, the negative affect generated by the schema "I do not get any attention" is reduced. However, this means as well that the objectives pursued at the game level have been attained (at least from time to time). This always leads to an affirmation: Although the motive is not satisfied, one at least satisfies the avoidance objective – and that is better than nothing. A client once put it this way: "Getting attention is like being hungry and consistently getting something tasty to drink" – a very apt formulation.

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Rule Schemas: Instructions for Interaction Partners

Aside from self-schemas and normative schemas, there is another kind of compensatory schema at the game level: **rule schemas** (see Sachse, Breil, & Fasbender, 2009).

Rule schemas do not contain rules which the person themselves must obey but instead rules which others, the interaction partners, shall obey! Rule schemas contain interactional expectations such as that "Others have to treat me respectfully" or "A partner has to pay attention to me on a 24/7 basis."

At the contingency level of these schemas, there are no disasters which could occur for the person themselves but only consequences for the interaction partner – for example, "If someone doesn't treat me respectfully, I can react angrily" or "If my partner doesn't pay attention to me, I will make a scene."

Rule schemas especially compensate for the negative relationship expectations of dysfunctional relationship schemas. If the person has a schema such as "I will not be respected in relationships," the person will develop an expectation of interaction partners at the game level that contradicts this assumption: "A partner must respect me and they'll be sorry if they don't!"

If interaction partners go against these rules, the person will not react with a sense of hurt or resentment (as happens with activation of dysfunctional schemas) but with anger and annoyance. Because this schema says

- The person deserves respect and may expect to get respect, and
- The person has the right to be angry and punish the rule violator.

Therefore, the hyperaversive reaction by activation of rule schemas is not about being hurt but rather about being angry. This means a person will be absolutely furious even if an interaction partner only slightly disobeys a rule.

It is mainly these rule schemas and the resulting behavior that bring massive interactional conflicts for individuals with PD: Interaction partners will not accept having to comply with the rules their partner enforces (especially not when the partner does not behave at all reciprocally), and they will not tolerate massive reproaches for trifles.

Thus, the more distinct the rule-setting behavior of a person, the greater their interactional conflicts.

Game Behavior: Images and Appeals

To get people to do something they actually do not want to do, they have to be prepared for it. The interaction partner needs to be put in a state in which they are ready to accept the interactional objectives of the person. This preparation is done by conveying so-called **images** (see Tedeschi et al., 1985).

An **image** is something that the person wants to establish in the interaction partner. The interaction partner has to get a *certain idea* of the person. This image should only contain certain components – leading the partner to develop certain belief and conviction systems.

The person may thus convey the image "I am weak and helpless." This image is a good preparation if the person wants the partner to assume responsibility, because for someone who

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is weak and helpless, a responsible and helpful person has to take responsibility. The person can attempt to make the partner establish the conviction that the person

- cannot cope on their own,
- is not resourceful,
- suffers in their present state and condition.

However, merely transferring images is not enough to attain interactional objectives, because the interaction partner is not only supposed to believe something, they also have to do something. The transfer of images is thus a preparation for the transfer of **appeals**: The interaction partner has to develop a certain conviction about the person and then behave in a certain way based on that conviction.

The person sends appeals to the interaction partner. Appeals are demands to do certain things or not do certain things. Appeals can be explicit and expressed openly; however, most appeals are indirect and implicit. They are expressed covertly between the lines.

In the case of **positive appeals**, interaction partners are supposed to do something that serves the interactional objectives of the person. There are several different possibilities. For example, the partner could:

- confirm the person's point of view;
- confirm the person is "normal";
- confirm the person is great, helpless, etc.;
- side with the person against another;
- be available for the person 24/7.

In case of **negative appeals**, interaction partners should refrain from doing something – that is, something that would be prejudicial for the person's interactional objectives. This also plays a crucial role in therapy. Clients try to motivate the therapist

- not to question their point of view;
- not to touch on unpleasant subjects;
- not to ask searching questions;
- not to get too close, etc.

Images and appeals are the building blocks of manipulative strategies: These strategies can be made up of many images and appeals and can get very complex.

Images and appeals appear in typical combinations which are then called **complex interaction games** (see the subsection Images, Appeals, and Interaction Games, in Section 9.2.4).

A Consequence of the Motive Level: Permanent Dissatisfaction

Based on motivation theories, two things are evident: First, the central relationship motive is not satisfied through game behavior; only avoidance objectives are satisfied. A high degree of dissatisfaction has to be the result because no matter what the person does and how hard they try, satisfaction of the motive cannot be attained. Furthermore, the result will be diffuse dissatisfaction, since because the person cannot understand why dissatisfaction occurs, they will not be able to integrate the dissatisfaction or effectively resolve it (see Brunstein, 1993; Brunstein et al., 1995, 1998).

Second, as a result of the frustration, the central relationship motive remains high in the hierarchy or even becomes more relevant. Consequently, the executive functioning of the

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person will not be available to satisfy other relevant motives: Thus, other motives are frustrated as well, so that the state of diffuse dissatisfaction will even be intensified.

It must therefore be assumed that

- A strong, diffuse dissatisfaction is a characteristic of PD.
- The degree of dissatisfaction correlates with the severity of the PD.

Ratio Between Authentic Behavior and Game Behavior

In our conception of PD, we assume that a PD is not an all-or-nothing concept but may range in severity from a minor personality style to a severe clinical disorder (see Kuhl & Kazén, 1997). In the case of a personality style, individuals only show a few characteristics, whereas in the case of a disorder, individuals show many characteristics in a distinct form. Where or when a style becomes a disorder can only be determined with careful consideration.

Individuals going through such a development toward game behavior develop (unfavorable) interactional objectives at the game level, manipulative strategic behaviors, and corresponding competences.

It is of great importance, now, how much this game level dominates all of the behavior of the person – in other words, what is the ratio between authentic behavior and game behavior? If a person shows authentic behavior to a greater extent and their game behavior is less significant, it is a matter of personality style. They show slightly negative schemas and manipulate only occasionally, and they are still able to behave in a very genuine way. In our experience, this applies to the vast majority of people. A fully functioning person is probably a giant illusion, so that one should be careful in using terms such as pathological or speaking derogatorily of manipulation. We all live in houses made of very thin glass, and we should be wary of even picking up a stone.

The more pronounced the game level and the weaker the authentic level, the stronger the severity of the PD: Individuals with a major disorder can hardly ever behave authentically; they almost always behave in a strategic and manipulative mode.

Long-Term Negative Consequences

The negative long-term consequences of strategic behavior are linked to the question of reciprocity. If a person behaves in a highly authentic way despite manipulative behavior, they may still behave reciprocally in their relationships. The person ensures the partner is satisfied as well, and both partners equally promote the relationship. If a partner sticks to this reciprocity rule, a relationship can be stable for a long time, and their occasionally manipulative behavior will not have a negative impact on it. Negative long-term consequences will be held at bay or will occur only after a long time.

Nevertheless, the more a person resorts to game behavior, and the less they act authentically, the more strikingly will the reciprocity rule be violated. Someone who massively manipulates only cares about their own objectives and is uninterested in the objectives of their partner (they

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sometimes even have the idea the partner must serve them, although they may not even be aware of this idea). If the person behaves like this, the partner sooner or later will no longer behave complementarily to the strategic behavior. Instead they will get angry and act accordingly. The complementary behavior then changes into an angry behavior. The positive short-term consequences turn into negative long-term consequences. How fast this change occurs depends strongly on how much the partner puts up with said behavior, and thus depends on their schemas.

The long-term consequences of strategic behavior are usually negative, with the PD system generating (more or less) high costs for the client. The more distinct a disorder, the higher the costs.

Client's Belief System Is Not Adaptive

Individuals presenting a very severe degree of PD do not take any responsibility for their behavior when a sudden change in relationships occurs. Despite multiple negative feedbacks and multiple failed relationships, the person will not realize that the problem could be due to their behavior, and they will not even think about reassessing their behavior, objectives, and views. Apparently, their belief system is not adaptive. The question then is, why is their belief system nonadaptive? Why do people even after massive negative feedback fail to consider reassessing their behavior or their system? Why do they fail to realize they are part of the problem? Why do they fail to revise their behavior?

A central point is that people cannot easily perceive their strategic, manipulative behavior as problematic, because this behavior is always effective (in the short term) in bringing relief and a certain satisfaction. At first, the behavior is always successful; the negative consequences only occur after a while; in general, such a constellation makes their behavior resistant to change.

Another important aspect of this is that these individuals must perceive the whole development of their strategic behavior and the involved objectives and competences as a solution for a serious biographical problem. The whole development of the game level has helped them to get at least a little bit of what they needed. It has also helped them to compensate for massive negative schemas, and it has often helped them to be successful. The whole game level is thus perceived as a win, as something that has had positive effects. The person developed these strategies to receive attention, appreciation, etc., and in their view, they have thus accomplished something.

We, therefore, have to assume that all aspects of the game level are highly ego-syntonic. They are perceived as part of the self, as positive, and not as problem. If the person considers all aspects of the game level to be correct, then in the case of conflict or negative feedback, they have little reason to attribute the problems to themselves, or their behavior, objectives, or attitudes: The person will not even consider that they might be a part of the problem. Quite the contrary, for them, two other much more obvious reasons come into focus:

- The interaction partner initially behaved complementarily at the game level but suddenly got angry: There is probably something wrong with them; they are obviously the reason for the conflict!
- The assumptions of their schemas, which suggest themselves as a source of attribution, because the schemas say "I am not important" and "I will not be respected in relationships," etc.

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If a partner suddenly behaves aggressively and unsympathetically, the schema will most likely interpret this as confirmation: "This shows again how unimportant I am and that I am treated badly in relationships." Thus, the schema accounts for the new experience the person has actually created by their dysfunctional manipulative behavior, as confirmation of the schema assumption: This is a type of self-fulfilling prophecy typical of PD. In the end, the person creates the conditions the schema fears.

The person thus attributes the cause of the negative effects internally, while regarding it as a stable, apparently unmodifiable cause and not as the actual cause which would be modifiable: This processing stabilizes the problem again.

It is another characteristic of PDs that the person does not learn from negative experiences and does not use these to reflect on or modify their belief system.

Tests

In this section, the problem of **interaction tests** or short tests will be described in more detail. Tests are actions used by a person to find out certain things about an interaction partner. The person wants this because they hope the interaction partner has certain positive features, and at the same time, they fear the partner does not have these features.

Tests for this reason serve to create certainty at points where the person is distrustful.

A testing behavior causes the interaction partner to be systematically checked and examined; they are subjected to a test situation in which they can (in the person's view) act appropriately (passing the test) or wrongly (failing the test). The person does not use tests to annoy the interaction partner but rather to get clarity about the relationship!

Such test situations are usually difficult for the interaction partners, since only difficult and demanding tests are useful in finding out whether the partner is really good.

Tests are always **relationship tests** to examine the attitudes and features of the interaction partner in relation to the relationship structuring. There are different things that can be tested – that is, whether

- The partner stays approachable in critical situations.
- The partner is reliable.
- The partner shows solidarity.
- The partner is competent.
- The partner is strong enough.
- The partner can gain acceptance.

Tests can be of varying difficulty for an interaction partner. **Soft tests** barely cause the partner any trouble: One might ask a personal question and watch the reaction. Does the partner answer honestly (then they can be trusted) or are they evasive (and so cannot be trusted)? By soft tests only a lesser degree of certainty can be achieved.

Hard tests, on the other hand, may create difficulties for interaction partners: For example, a client may criticize the therapist very strongly ("You acted irresponsibly!") and watch the therapist's reaction. If the therapist stays approachable and tries to discuss matters, then the

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client is important to them; if the therapist gets angry and is disapproving, then they are not important. Hard tests thus create a high degree of certainty.

Moreover, there are **very hard tests** as well: These are usually personal depreciations or personal insults. The client may say: "You are have no personality!" and watch the therapist's reaction. If they remain friendly and try to understand why the client does this, they are reliable ("They will stay with me even in a crisis"); on the other hand, should they get angry and be disapproving, the person feels that they cannot rely on them.

It is important to see that these tests are not made to provoke or hurt the therapists or interaction partners. That is not their intention. It is their intention to get information about the therapist. These tests are meant to find out how the therapist behaves under extreme conditions.

The realization of tests is typical of people with PD since they face a fundamental dilemma. On the one hand, they show very strong relationship motives, and their schemas tell them that these needs cannot be satisfied. Their schemas thus create a high degree of distrust concerning the satisfaction of needs. And therefore, people with PD are caught in a dilemma: On the one hand, they hope to get their needs satisfied by the therapist – for example, to be important to the therapist; on the other hand, they are highly distrustful about getting their needs satisfied by the therapist because they may, for example, think "I will not be taken seriously in relationships."

This dilemma even becomes highly acute if the client meets a therapist who creates a good relationship structuring, which is accepting, empathic, etc., because both the motive and the schemas are triggered now so that the client must make a decision between Opening up to the relationship or avoiding the relationship.

It is obvious that tests are the ideal escape from the dilemma. As a client makes a test, they will get clarity: Either the therapist passes the test so that the client can allow themselves to engage in the relationship (at least a little bit more) or the therapist fails the test, which means the client keeps their distance or ends the relationship. Tests are an escape from a dilemma. Therefore, clients with PD tend to be highly effective at testing; and tests are especially made use of in the event therapists behave inappropriately.

Furthermore, tests should be even harder, the higher the internal contradictions in the client. And this is true of histrionic PD clients perceiving moderate discrepancies between motives and schemas. With medium-negative schemas, testing usually takes the form of professional criticism leveled at the therapist but usually never gets personally offensive. Borderline clients with extremely negative schemas face a major discrepancy; they usually test extremely hard making use of very insulting, personal degradation.

Many clients with PD also use tests in therapy situations. These tests impose very high interactional demands upon the therapist: If therapists do not pass these tests, the therapist–client relationship deteriorates, and the client may abandon the therapy.

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Chapter 5

Diagnostic Features of Personality Disorders

5.1 Introduction

An essential question is what kind of psychological aspects of their disorder are relevant indicators and thus defining features of PDs, and which features qualify for a diagnosis of PD. In this section, these essential aspects of PDs as deduced from the dual action regulation model will be discussed. These features can be assessed by therapists in therapy sessions. As these features appear in the interactional behavior with therapists, the client can hardly falsify them intentionally; The therapist's assessment in this respect is thus fairly reliable (see Sachse, Sachse, & Fasbender, 2011).

5.2 Relationship Motives

People with PD are always characterized by the fact that they have certain relationship motives which rank and stay high in their motive hierarchy. Narcissists, for example, are characterized by a central appreciation motive, whereas histrionic PD clients have a central importance motive. Any PD can be characterized by a set of relevant relationship motives. These aspects of PDs are described in more detail later in Chapter 7.

5.3 Dysfunctional Schemas

People with PD feature characteristic, dysfunctional schemas: both self-schemas and relationship schemas. **Dysfunctional self-schemas** are schemas which include negative assumptions about the self. Narcissistic PD clients make typical assumptions such as

- "I am not ok."
- "I am a failure."
- "I am not competent."

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Histrionic PD clients make typical assumptions such as
- "I am not important."
- "I have nothing to offer."

Dysfunctional relationship schemas are negative assumptions about how relationships work or what to expect from relationships. Narcissistic PD clients make typical assumptions such as
- One will be evaluated in relationships.
- One will be criticized in relationships.

Histrionic PD clients make assumptions such as
- One will not be taken seriously in relationships.
- Nobody cares.

5.4 Compensatory Schemas

Compensatory schemas serve to compensate dysfunctional schemas. A distinction is made between **norms** and **rules**. Norms and rules are also **interactional objectives**; however, from the viewpoint of motivation theories they are avoidance objectives. Their achievement does not generate positive affects or satisfaction but merely a reduction of negative affects (see Ebner & Freund, 2009; Elliot & Covington, 2001; Kuhl, 1983, 2001).

Norms are schemas which contain instructions for the self; they compensate negative self-schemas. Narcissistic PD clients usually show norms such as
- Be the best.
- Avoid mistakes.

Histrionic PD clients show norms such as
- Be the most important.
- Get attention.

Rules are schemas by means of which the person formulates expectations of others in terms of instructions. Using rules, a person defines how they want to be treated by others. Rules usually compensate negative relationship schemas.

Narcissistic PD clients often have rules such as
- Nobody may impede me.
- Everybody has to treat me respectfully.

Histrionic PD clients have rules such as:
- Others must listen to me.
- Others have to care for me.

5.5 Interaction Games

People with PD resort to characteristic, manipulative interaction behavior. Interactional strategies can be divided into **images** and **appeals** (see also the subsection Game Behavior: Images and Appeals, in Section 4.2.4). Clients create so-called images to induce others to fulfil their own **interactional objectives**. They present themselves in a certain way to create a certain image for the interaction partner – that is, the partner shall believe certain things about the

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person. On the basis of the images the client makes appeals: These are usually imparted indirectly and implicitly and serve to make the partner do certain things.

Narcissistic PD clients often send images such as

- "I am great."
- "I am super."
- "I am highly competent."

And appeals such as

- "Praise me."
- "Validate me."
- "Don't criticize me."

Histrionic PD clients send images such as

- "I am very entertaining," or
- "I am weak and helpless."

With appeals such as

- "Care for me."
- "Pay attention to me."
- "Notice me."

5.6 General Model and Specific Disorders

Basically, the components represented show up in all PDs. However, each individual disorder also shows specific severities of these components. What these look like depends on the relevant disorder: which relationship motives are dominant, which schemas the person shows, which interactional strategies the person uses at the game level, and which types of tests the person uses, which all depend on the respective PD. A client with schizoid PD has central relationship motives that differ from those of a client with narcissistic PD; they show different schemas, realize other types of nontransparent strategies, and use other tests.

Therefore, the general dual action regulation model has to be individually elaborated for every single PD. It thus not only includes general characteristics but may also be tailored to address any individual disorder so that it is to be viewed as a general disorder–specific model. In this way, any disorder has its own specific model but at the same time maintains its general characteristics. Moreover, every specific model leads to a specific approach on a concrete therapeutic action level, although general therapeutic principles are still maintained: At the beginning of a therapy, the therapist should behave complementarily with the motive level of the client suffering from a disorder, although the type of complementarity toward a narcissist differs completely from that for a schizoid client. Thus, even on the therapeutic level, principles are consistently valid; however, what these therapeutic principles in fact comprise in each individual case depends to a great extent on the respective PD.

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Chapter 6

Therapeutic Strategies for Clients With PD: Consequences of the Model

6.1 Introduction

Basic therapeutic strategies can be derived from the dual action regulation model and are based on the therapeutic concept of COP (Sachse, 2003, 2004d, 2006e, 2007a, 2008a, 2008b, 2013b, 2014b, 2015a, 2016c; Sachse, Breil & Fasbender, 2009; Sachse & Fasbender, 2010, 2013; Sachse, Fasbender, & Breil, 2009; Sachse & Langens, 2014a, 2015; Sachse, Langens, & Sachse, 2012; Sachse & Rudolf, 2008; Sachse & Sachse, 2009, 2011; Sachse, Sachse, & Fasbender, 2011). These are represented below.

6.2 Therapy Phases

We work from the assumption that the therapy of clients with PD should take place in phases. Any therapy phase is characterized by certain therapeutic key aspects: In each phase, the therapist follows certain therapeutic process objectives involving particular strategies. As these objectives are achieved, the therapist proceeds to the next phase, in which they focus on new objectives including other therapeutic strategies. Certainly, these phases cannot be exactly defined, and to some extent their transitions are fluid: When therapists have already achieved subgoals regarding certain issues, they may start addressing the objectives of the next phase while still working on the remaining objectives. Furthermore, the therapy phases do not take place linearly: It may be necessary for a client to go through certain phases several times.

In addition, a therapist may work on many client schemas at the same time, but the phases being processed for a given schema may differ. Schema X, for example, is not embarrassing for the client, and thus the therapist and client are already in Phase 4 with this schema, whereas schema Y is difficult for the client, so that in that case, therapist and client are only concentrating on Phase 1.

Following phase sequences has proven successful in evaluations of therapy success. Experience has shown that it is important to stick to the order of process objectives to achieve

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therapeutic effects; therapists who fail to do this usually are not successful, since they cannot create the conditions necessary to apply certain strategies, and consequently clients react with insufficient compliance.

In each phase, the therapist realizes certain therapeutic strategies at the relationship level, the content level, and the processing level; however, the priorities change, with the strategies at the relationship level dominating in Phase 1, while Phase 2 is characterized by content and processing level strategies. In Phases 3 and 4, content strategies take priority.

Overview of the phases

Phase 1: Complementary relationship

Phase 2: Development of a therapeutic goal

Phase 3: Clarification

Phase 4: Processing of schemas

Phase 5: Transfer

6.3 Therapeutic Strategies for Phase 1: Model and Relationship

In this section, the therapeutic strategies are presented that should be used in Phase 1. Note: these strategies are used in other phases as well, but are mostly used in Phase 1.

6.3.1 Model Development

In the first phase, it is essential for the therapist to develop and elaborate a model of the client. Model development takes place at the very start of the therapy, with the therapist making use of all information levels accessible to them based on their knowledge and available heuristics (see Becker & Sachse, 1998; Sachse, 2017).The therapist should develop first hypotheses as quickly as possible and then prove and substantiate these hypotheses. Hypotheses must essentially focus on

- The client's concerns, whether the client has a **therapeutic goal**, and if so which one
- Whether the client substantially sends **images** and **appeals** and which ones
- Whether the client tries to **manipulate** and entrap the therapist and the strategies they use and the objectives they have in mind
- The client's **central relationship motives** (including their concrete operationalization)
- The client's **central self-schemas** and **relationship schemas**
- The client's **interactional objectives** at the game level

Based on this information and further disorder criteria, the therapist should also diagnose any PD (if a PD exists). This will then provide further search heuristics for the therapist's orientation.

Certainly, the model development cannot be finalized in Phase 1 but will extend to all phases. However, the better the early model is, the less it has to be modified later on. Usually, client models become more and more elaborated as the therapist acquires more information

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about the client. Moreover, the therapist will not obtain a lot of information in the beginning as the client will not disclose crucial information before they trust the therapist. Thus, a model has to be continuously revised and upgraded, and a therapist must be able to manage that development process.

6.3.2 Complementary Relationship

Complementary (i.e., motive satisfying) relationship structuring is always the first thing a therapist has to accomplish in the therapy process (as soon as the therapist has reconstructed the relationship motives). This is due to the high relationship motivation and high ego-syntony in PD clients. The therapist also needs a large amount of relationship credit for confrontations, thus they need to obtain credit first.

The concept of complementary relationship formation goes back to the work of Caspar and Grawe (Caspar, 1986, 1996, 2007a, 2007b, 2008; Caspar & Grawe, 1982a, 1982b; Grawe, 1982, 1986, 1992; Grawe & Caspar, 1984). In COP, however, unlike in Caspar, complementarity does not mean fulfilling plans, but rather satisfying motives for relationships within the framework of therapeutic rules (Sachse, 1997c, 1999, 2001a, 2003, 2004a, 2006d, 2013a).

Purpose of Complementarity

Primarily, but not exclusively, the first phase of therapy is about the complementary relationship structuring. In this phase, the therapeutic strategies at the relationship level dominate. The dual action regulation model suggests that clients come highly relationship motivated to therapy. They thus have a strong desire to realize a certain type of relationship.

This is the primary desire in clients with PD. They want the therapist to behave in a certain way. All other therapeutic goals (e.g., processing comorbid Axis I disorders) are of a secondary nature. PDs are relationship disorders, and this becomes therapeutically clear because clients have certain relationship desires.

If therapists behave complementarily to these relationship desires, they obtain a large amount of relationship credit (especially in the sense of personal trust, but also in the sense of competency trust; see Sachse, 2006a). They assume the therapist is not threatening and really wants to help. This is a vital requirement for any therapeutic cooperation and for compliance: Clients only allow themselves to get involved in clarification, therapeutic strategies, processes of change, and especially confrontation when they have developed this kind of personal trust in the therapist.

If the therapist shows no or insufficient complementarity, the client will not develop personal trust. Consequently, they will not permit themselves to be involved in therapeutic strategies, will not cooperate, will resort to avoidance and sabotage in an indirect, nontransparent way. Our experience in this respect widely confirms these assumptions.

Complementary relationship structuring is a key element for the therapy of PD clients: A sensible therapy is not possible without complementary relationship structuring. Complementary relationship structuring results in a large amount of relationship credit.

Clients trust the therapist and allow themselves to participate in the clarification processes. The therapist also builds up enough credit to use for confrontational interventions.

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As supervisors, however, we have often seen, mostly in classical behavioral therapies, what happens when the therapist does not behave complementarily but directly offers therapeutic strategies: The clients do not cooperate, they misunderstand the therapist, feeling too bad about lessons or showing every conceivable type of avoidance and sabotage (see Sachse, 1997a, 1999).

What Is Complementary Relationship Structuring?

As said before (see Section 6.3.2: Complementary Relationship), complementary (i.e., motive satisfying) relationship structuring has priority in therapy Phase 1. The primary objective of this phase is for the client to establish confidence in the therapist – primarily personal trust (but trust in their competences as well).

The client should develop certain beliefs about the therapist:

- the belief that the therapist is loyal, and the client can rely on them;
- the belief that the therapist will not harm, depreciate, define as deficient, criticize, or evaluate the client;
- the belief that the therapist will take the client seriously and accept, respect, and understand the client;
- the belief that the therapist will not infantilize the client, and will preserve the client's autonomy and respects the client's boundaries, and will not be intrusive.

These beliefs can be realized to some extent through a general relationship structuring – that is, through empathy, acceptance, respect, etc. However, to a great extent they can also be realized through complementary relationship structuring (see Sachse, 2000b, 2004b, 2006e, 2013b, 2016c). Complementary relationship structuring means a therapist examines the six relationship motives:

- Appreciation
- Importance
- Dependability
- Solidarity
- Autonomy
- Boundaries and territoriality

and clarifies which of them ranks high in, and is vitally important to, the client's motive hierarchy. As soon as the therapist has obtained this information, they should behave in a way (within the framework of therapeutic rules; for the therapeutic rules, see, e.g., the subsection No Complementarity at the Game Level, in Section 9.3.1) to adequately satisfy that motive (Sachse, 2006a, 2006c). Because the client recognizes their motives are satisfied, they very likely will develop one or more of the above-described beliefs about the therapist.

Altogether, complementary relationship structuring always includes two aspects:

1. The therapist should feed the client's central relationship motive well.
2. The therapist should trigger (activate) the dysfunctional self-schemas and relationship schemas (which are connected to the relationship motive) in the least possible way. Every triggering impairs the therapist–client relationship and takes away relationship credit.

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Not Triggering Dysfunctional Schemas

The dual action regulation model assumes that clients with PD always have related relationship motives and dysfunctional schemas. If for a person "importance" is a central motive, they will also have a self-schema like "I am not important" and a relationship schema like "One will not be taken seriously in relationships." Thus, central schemas are negations of the central motive.

Every activation of these negative, dysfunctional schemas triggers interpretations contradicting the motive: When an interaction partner triggers the schema "I am not important" this will lead to immediate frustration of the person, since this will be interpreted as contradicting to their motive. Certainly, such an activation of negative schemas will put stress on the relationship as the person feels they are not being seen as important. Therefore, if a therapist acts in a way that triggers a dysfunctional schema, they cause relationship credit to be taken way.

This happens in therapy as well: If the therapist triggers the schemas "I am not important" or "One will not be taken seriously in relationships," the client feels they are not important to the therapist. This means, every activation of dysfunctional self-schemas and relationship schemas in the therapy impairs the relationship and consequently reduces personal trust.

In the therapy process, a therapist should try to behave in a way such that the client's dysfunctional schemas are triggered as seldom as possible.

If a therapist, therefore, wants to establish personal trust with a client in the therapy process, they have to behave complementary (i.e., need satisfying) to their motives and avoid triggering dysfunctional schemas, because personal trust cannot be established at all, if the client often interprets the therapist as not taking them seriously or as not seeing them as being important. What is more, the more often and acutely the therapist triggers those schemas, the harder the establishment of personal trust.

Because it is sometimes quite easy to trigger a schema, it is virtually impossible for a therapist to work with a client and not trigger one occasionally. However, if the therapist normally behaves in a quite complementary fashion and circumvents schema activation – for example, by making appropriate comments – we can say from experience that an occasional blunder committed by the therapist will not have a detrimental effect – it just should not happen too often. It is thus important that a therapist develops a model regarding the relevant schemas for a patient, to be able to judge any aversive reactions of the client (Becker & Sachse, 1998; Sachse, 1996).

Complementarity at the Motive Level

This section gives a short overview of complementary behavior at the motive level; as this is therapeutically important, more detail will be presented in a subsequent chapter. According to the dual action regulation model, relationship motives are defined very broadly and generally; nevertheless, these are operationalized to form interactional objectives: Interactional objectives are the concretely, situationally defined objectives a person tries to achieve.

The model proposes that clients do not try only to obtain global and general information but also to acquire specific, situational, and concrete information as well. A person with a high importance motive not only wants to hear that they are important but also wants to receive attention in concrete situations, wants to be taken seriously in various ways, with different contents, etc.

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A person with a high appreciation motive not only wants to know they are generally okay as a person but also wants to hear they have great abilities the interaction partner takes notice of and appreciates. For that reason, the therapist must realize more than just general attitudes; they must realize concrete, validating behavior satisfying the concrete interactional objectives of clients. Therefore, complementary behavior means more than just realizing basic variables, rather it means giving clients concrete positive feedback regarding concrete behavior and concrete contents. Concrete interactional objectives of the client (forming part of the motive) must be fed by the therapist.

Complementary (i.e., motive satisfying) behavior at the motive level means behaving complementarily to the general, comprehensive concepts as well as to the concrete, operationalized objectives. Consequently, a therapist must know not only the comprehensive motive of clients but also the concrete interactional objectives.

Thus, complementary behavior respecting the importance motive means letting the client feel they are important to the therapist and that the therapist appreciates being with them (within the framework of therapeutic rules; for the therapeutic rules, see, e.g., the subsection No Complementarity at the Game Level, in Section 9.3.1). Therefore, a very helpful attitude toward the client is to show appreciation of them, and be interested in the contents of the client's communications, and interested in listening to them. This attitude helps the therapist to behave authentically and positively toward the client.

However, complementary behavior means even more than just giving the client a general feeling in a relationship. Complementary behavior that respects the importance motive also means

- Giving the client as much attention as possible, orientating oneself toward the client: Look at them and send signals that one is listening to and fully focusing on them.
- Showing the clients that one is interested in what they are saying, that one is attentive and understands.
- Showing the client that one takes them seriously, takes their problems and sorrows seriously and is prepared to deal with them.

Behaving complementarily with respect to the appreciation motive means, to start with, valuing the client as a person and signaling that they do not have to exert themselves to be valued and accepted for what they are. But this is not enough. When going into detail, the client will want more; They will have concrete interactional objectives the therapist should help to accomplish. It is not enough for clients to get the information that they are okay as a person and that they need not do anything for this. That information is psychologically completely insufficient. It is thus important to

- signal to the client that one values and appreciates aspects of their personality (e.g., their efforts, capacities, and successes);
- signal to the client that one values their efforts, goodwill, openness, compliance, etc.

Therefore, one could tell the client, for example, that

- one is impressed by how fast they have finished their studies;
- one is delighted that the client often thinks for themselves in the therapy;
- one thinks it is notable the degree to which the client is very active in their company, etc.

As therapist, one behaves complementarily at the motive level as long as

- one confirms and positively expresses contents capable of being perceived as okay, functional, and unproblematic;
- one perceives the client's wishes as okay, functional, and unproblematic;
- one is able to realize complementary behavior authentically and without having disturbing feelings and having to resort to dissimulation.

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In our opinion, the desire of clients to be considered important, to be taken seriously and praised for their efforts and successes cannot be regarded as problematic or dysfunctional. All of these wishes are appropriate, functional, and nonstrategic (and, therefore, not found at the game level!).

Contents not resulting in problematic behavior can be reaffirmed positively by the therapist. Therapists may thus easily behave complementarily with respect to many contents, constructions, objectives, and wishes of the client.

Complementarity at the Game Level

As clients pursue compensatory objectives and are successful with those, their compensatory behavior is reinforced. Otherwise, however, this does not have any positive effects but rather stabilizes a dysfunctional system. From this insight, it follows conclusively that a therapist should not do anything that reinforces or stabilizes the client's dysfunctional system. Consequently, the thesis has been formulated that a therapist shall not behave complementarily at the game level (Sachse, 2004b). However, closer analyses have shown that any general rule to not act complementarily at the game level cannot be easily realized, and for that reason, this rule shall be reformulated here.

Considering the dual action regulation model more closely, we see that a therapist may behave complementarily to (be affirmative of) different aspects of the game level:

1. A therapist may confirm a client's compensatory, positive self-schema and may affirm clients describing themselves as great, efficient, intelligent, reflected, successful, etc.; the therapist may thus positively assert the client's compensatory view.
2. A therapist may affirm a normative schema and agree that a client should strive to be the best, avoid mistakes, etc. In this way, the therapist can confirm the client's compensatory objectives.
3. A therapist may reaffirm a rule schema and confirm that the client formulates rules, the client may approve those rules, and, in this way, confirm the client's assumption that they are entitled to draw up these rules.
4. A therapist may react to appeals from clients and may thus be manipulated and consequently made use of for the purposes of the client's belief system.

In general, it can still be said that a therapist should possibly not behave complementarily at the game level: However, should it be important in a given case to behave complementarily in a game aspect, the therapist should only do so under the following conditions:

1. There is no good alternative at that time.
2. The therapist strategically and deliberately decides to do it.
3. The therapist only uses complementary behavior to the extent it is absolutely necessary.
4. The therapist only behaves complementarily for the shortest time possible.
5. And above all, the complementarity does not contradict any therapeutic rules and does not sabotage other therapeutic objectives (for therapeutic rules, see, e.g., the subsection No Complementarity at the Game Level, in Section 9.3.1).

As one looks at the list of aspects of the game level toward which a therapist may in any way behave complementarily, it becomes quite clear that the assumptions are almost exclusively

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valid for Case 1 in the box above: A therapist may confirm a client's compensatory positive self-schema. If the therapist does this, it usually has the following consequences in the therapy process:

- The client feels deeply understood and esteemed.
- The client offers a large amount of relationship credit.
- The client realizes they need not fight for the therapist's appreciation, consequently the client's compensatory self-projection slowly diminishes more and more.
- The client will be more inclined to deal with aspects of the burden of their PD, – for example, deal with their dysfunctional negative self-schema.

Therefore, this therapeutic behavior can have short-term positive consequences (relationship credit) without sabotaging important therapeutic objectives in the long term. The findings of the plan analysis (i.e., the exact and detailed analysis of interactional plans on different concretization levels) will fully apply (see Caspar, 1996), and therapists can orientate themselves by it. This is especially important when working therapeutically with narcissistic clients.

However, it is to be noted that even on the level of compensatory self-schemas, there are images (and appeals) from clients to which a therapist should not react complementarily. Therefore, a therapist should always analyze the images and appeals (see the subsection Game Behavior: Images and Appeals, in Section 4.2.4) and – if this is possible (see below) – decide reasonably in each individual case. Therapists should also bear in mind that there can be "toxic" images, although it is generally to be assumed that images based on compensatory self-schemas are rather harmless.

However, it gets much more problematic when affirming contents of normative schemas: In this case, therapists must be careful not to strengthen such dysfunctional statements that may aggravate problems or lessen the motivation to undergo therapy. If clients have the idea they must do something, this should always be questioned by the therapist; any affirmation of this idea will result in increasing the pressure.

Even more problematic are rule schemas: Usually, rules should never be affirmed by the therapist, because clients could easily conclude that this was a legitimization of an unsocial behavior. As clients set rules for others, interaction problems are normally the result; a client should realize this and question their schemas. In the event that a therapist affirms these schemas, the client's motivation to deal with them will decrease.

Nevertheless, major problems will arise when affirming the manipulative behavior of clients: Therapists should never ever let themselves be functionalized by clients. They should never confer special rights on clients (unless they are very sure these will not be detrimental to the therapy or used against the therapist), never let the client determine the rules of therapy, and never take a position of solidarity with the client against others, etc. On this level, complementarity is only conceivable in justified, exceptional cases!

In this case as well, an exact analysis of appeals (see the subsection Game Behavior: Images and Appeals, in Section 4.2.4) can bring clarity. There are appeals to which a therapist can easily behave complementarily but should not do so without having made a detailed analysis.

Assuming that complementarity with respect to the game level is conceivable on the level of affirming compensatory self-schemas, this solves a content problem that therapists often have in the therapy process. It becomes evident that on this level, the boundaries between the motive level and the game level are often vague. If the therapist accepts the client as a person, this will not cause ambiguity, but if the therapist affirms that the client is efficient and productive, uncertainty may arise as to whether this is a mere desire at the motive level which the therapist affirms, or it is an objective of a compensatory schema at the game level (in which case, the

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affirmation would be a complementarity to the game level). In individual cases (as clarified in supervision), it is difficult for therapists to decide exactly whether an affirmative behavior is complementarity at the motive level or already complementarity at the game level.

Another aspect of their disorder that has been clarified is that at the start of the therapy, clients with PD, due to their strong compensatory schemas, expect affirmation of compensatory assumptions and are not satisfied with the information that they are okay as a person. Rather they expect that the therapist values their special efforts, competences, etc. This means that many clients with PD expect an explicit affirmation of compensatory assumptions from the therapist and can apparently only be fed in this way. As a result, therapists only get sufficient relationship credit if they act accordingly.

6.3.3 Explicating the Relationship Motives

According to the dual action regulation model, it seems useful to enable clients to represent their central relationship motives. If clients know what is centrally important to them, they can reflect whether they really want and need the interactional objectives at the game level ("Do I need to make an effort and be a success when my central motive is appreciation?") and whether they have available new or other actions to satisfy their central motives. The representation of essential motives creates internal standards that clients can orientate themselves by, based on which they can detect what they need and what bothers them. These standards are essential for an effective self-regulation.

Explication means that the therapist understands the client's relevant relationship motives and that they signal their understanding or rather induce the client to clarify their relationship motives with the therapist. The therapist tries to understand the client's behavior at the relationship level – that is, they try to understand which interactional desires the client has, and explain these motives to the client.

6.3.4 Directing and Internalizing

Although relationship structuring takes priority in the first phase, a therapist should do more than just realizing interventions at the relationship level. Already at the beginning of the therapy, the therapist should try to carefully direct the client's processing efforts. To this end, they should realize interventions at the processing level. In this way, the therapist should initiate a clarification or explication process by the client – that is, the client should, step by step,

- internalize their perspective and thus deal with their own coping processes – in other words, their cognitions, emotions, and affects;
- clarify these cognitions, emotions, and affects step by step, thus reconstructing which schemas they belong to;
- and in this way, work out their problem-relevant schemas step by step (which, according to empirical analyses, is a hard and long process that must be supported by the therapist).

The therapist has to direct this process to a high degree through specific processing offers. Directing means a therapist shows the client what they should to do next in the therapy process: In this way, the therapist realizes the so-called processing offers (Sachse & Maus, 1991).

However, at the beginning of therapy, the therapist expects that the client will not accept the therapist's processing offers, so the therapist must realize these interventions to set **markers**:

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These are specific interventions drawing the client's attention to important content or questions. Due to these repeatedly realized interventions in the form of directions, the client is advised regarding

- what they could do concretely in the therapy process and how they could concretely continue with their work;
- what they should meaningfully do, in the therapist's opinion.
- In this phase, the therapist does not always assume that the client readily pursues the offers, but they must make these offers very often in order that
- the client actually notices those offers;
- the client understands that the therapist means business;
- the client understands that these offers are important;
- the client understands that they could accept these offers;
- the client decides to take up the offers.

Directing interventions have a cumulative effect: Only if the offers are presented often will they be effective; it is to be noted they will not take effect immediately but rather are more likely to be delayed and only occur at all through constant repetition.

Thus, the therapist should make their constructive processing offers right from the start (see Sachse, 2003) – for example, by

- asking the client to describe problematic situations;
- asking the client to concretely describe situations;
- interrupting the client during lengthy narrations and discussing certain aspects of their disorder in more detail;
- getting the client to show which aspects are relevant for them.

Especially when clients show strong, dysfunctional processing, the therapist should take directing measures. They should, for example, take action if a client tends to highly intellectualize their problems, and then try to induce the client to link problems to concrete examples to enable internal processes to be initiated and clarified:

- If a client is inclined to be unconcrete, the therapist consistently draws attention to concrete representations.
- If a client loves to tell stories from their week, the therapist chooses a current, concrete event that the client shall deal with exclusively.

During this phase, therapists take careful directing actions: They make the client offers

- to deal with concrete, problematic situations;
- to stick to these situations and describe them concretely; and
- based on these situations, to initiate and analyze internal processes.

If the client refrains from getting involved in these offers, the therapist should not force them; instead the therapist lets the client "roam," but after a short time starts over again by making a similar offer and does this repeatedly until the client realizes the therapist's offer is important and they are ready to accept the offer.

The name of the game is: Make directing offers; if the client doesn't accept them, let them roam; but again and again repeat the relevant offer after a short time.

A central aspect of directing is **internalization**. In the process, the therapist induces the client to direct their perspective inwardly to what they think, feel, and want. The therapist can do this by asking:

- What does this situation trigger in you?

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- What is crossing your mind in situation X?
- What do you feel in situation X? etc.

Again, the therapist induces internalization over and over again until the client accepts the offer, and the therapist can initiate a clarification process. The therapist uses internalization as a **marker**. Even if the client does not answer the questions, the therapist sets markers that indicate that

- The client should deal with internal processes.
- The therapist thinks theses processes are important.
- The client realizes they cannot answer the question and that this may be an indication of unclear problems.

6.3.5 Understanding and Clarifying

Certainly, therapists must work at the content level in this phase as well. They must actively try to understand what the client says and means, and they must communicate what they understand by making appropriate interventions (see Sachse, 1993).

In the process, the therapist has to realize that clients with an ego-syntonic disorder have their schemas, motives, and objectives only present in a rudimentary form and for that reason will probably only refer to peripheral topics (at the beginning). It is thus important for therapists to constitute and reconstruct good models (see Section 6.3.1: Model Development) about central contents. The therapist must try to find relevant "tracks" to be followed to get to the real interesting areas – that is,

- to interactionally relevant situations;
- to situations in which relevant schemas of the client become explicit;
- to situations in which the manipulative strategies of the client become explicit.

Depending on the client's initial situation (state of problem discernment, degree of avoidance, quality of reflection), it is possible in this phase to go through initial clarification processes with the client. Clients can identify relevant situations, realize that aspects of their behavior are problematic, and (in real, constructive cases) can already identify incipient stages of schemas.

6.3.6 Dealing With Avoidance

Further interventions at the processing level concern the careful handling of avoidance behavior in this phase: Sometimes a therapist may take a countermeasure to alert the client to the importance of a certain topic (see Sachse, 1995, 1997b, 2007d; Sachse, Fasbender, & Sachse, 2011). If the client, for example, says, "I don't know," the therapist immediately replies by saying, "I know it's difficult to answer this, but I think the answer is important. Please look again at the question and see if something comes to mind." If the client is not responsive, the therapist lets them roam and may repeat the intervention after a while.

6.3.7 Passing the Tests

Clients usually make interactional tests at the beginning of therapy, and most of the tests occur during therapy Phase 1. At this time, the therapists must always be prepared to undergo and pass such testing and must always be aware that

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- Tests, even if they comprise provocative and aggressive behavior on the part of the client, are never meant to provoke the therapist.
- Tests only serve to assess whether the therapist remains friendly, approachable, and reliable even in case of such provocations.

During testing, the therapist should therefore

- remain friendly and realize that testing is not meant personally but is rather an offer to establish a good relationship;
- try to understand the client's point of view, taking the client seriously, offering the client the opportunity to clarify what is actually happening at the moment;
- try to work out with the client why the client adopts this kind of behavior.

6.3.8 Resource Activation

An important therapeutic strategy that the therapist may already use in this phase as well, especially for clients with distance disorders (see Section 7.3: Proximity and Distance Disorders, below), is the activation of resources. In this way, the therapist can

- frequently point out positive aspects of their personality, competences, abilities, possibilities, strengths, etc. of the client; make these salient for the client; and emphasize the client can use these resources (working with the client on when and how to use them);
- signal confidence in the client, revealing that one thinks the client is strong – strong enough to face their problems – that one thinks the client is competent – competent enough to solve their problems, etc.;
- ascribe motivation to the client (Sachse, Langens, & Sachse, 2012), thus making clear that the therapist assumes the client wants to face and work on their problems and does not want to put up with the condition anymore, etc.;
- express astonishment where necessary and point out they are rather surprised the client resigns at this point, because they think the client would be capable of tackling the problem, etc.

6.4 Therapeutic Strategies in Phase 2: Game Level and Tests

6.4.1 Developing a Therapeutic Goal

Therapeutic Objectives

PDs must be understood as ego-syntonic disorders (the dual action regulation model emphasizes why; see Section 4.2); therefore, they are disorders which the person themselves do not perceive as disturbing and problematic. For that reason, clients with a PD are not motivated to change with regard to their relevant disorders and consequently have no therapeutic goal for their PD. They do not want to work on altering the disorder and consequently show no compliance when therapists make relevant interventions.

Without a therapeutic goal, the therapist cannot work on addressing the disorder. It thus follows there is no useful therapy without a therapeutic goal.

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It thus applies that a therapeutic goal (motivation to change) has to be developed before a PD can be worked on and altered (see Sachse, 2015c; Sachse & Langens, 2015; Sachse, Langens, & Sachse, 2012)!

If the client wants to clarify and modify their system, they must urgently develop a therapeutic goal – that is, they must find a motivation to change regarding the aspects of their PD.

This can only be done if the clients realize they

- are incurring high costs, particularly high interactional costs;
- really do not want to suffer these costs and that the costs massively contradict important motives;
- generate these costs themselves, through their behavior, objectives, assumptions, and strategies;
- can massively reduce the costs when changing these objectives, assumptions, and strategies;
- can do this with the help of the therapist.

This is what PD clients cannot realize on their own (as is clearly evident from the dual action regulation model) and what must actively be communicated by the therapist. Therefore, a therapeutic goal is not a prerequisite for a therapy but rather the second therapy objective (the first is developing relationship credit). The second therapy phase serves to achieve the second therapy objective.

Confrontation

In this phase, the client has to establish a motivation to change and a therapeutic goal through the interventions of the therapist. For this purpose, the therapist must point out to the client that they

- are incurring costs;
- do not want to incur these costs;
- produce these costs themselves through their schemas, etc.;
- can reduce these costs, if they alter their schemas, etc.;
- can do this in therapy together with the therapist.

All this has to be emphasized by the therapist, because the client will not be able to realize it on their own. PD clients never undertake developing a therapeutic goal themselves, they must be developed through well-directed actions by the therapist. If at all, clients with PD may realize they are incurring costs, but due to the ego-syntony of the disorder, they are unable to realize that they produce these costs themselves.

Obviously, it is thus the duty of the therapist to convey these insights to the client. It depends on the therapist's interventions whether a client develops a motivation to change and a therapeutic goal. The therapist must draw the client's attention to the fact that they are incurring costs, do not want these costs, produce these costs themselves, and can reduce these costs by altering their system.

When examining this duty of the therapist more closely, it becomes clear that the therapist must direct the clients attention to facts the client does not realize and (according to the dual action regulation model) does not want to realize, and even wants to avoid by all means. For

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that reason, all interventions the therapist can make in this phase are by definition confrontational interventions, and therefore all of these interventions will reduce the relationship credit. A therapist making these confrontational interventions must have sufficient relationship credit available. If the therapist makes these interventions without enough relationship credit, they put the therapist–client relationship at high risk.

It may even be confrontational to direct the client's attention to their costs; however, drawing the client's attention to the fact that their behavior, especially their manipulative behavior, has costs, is in all probability highly confrontational, because at this point, clients usually do not want to be seen through, and they react hyperaversively if someone raises this issue.

All interventions and strategies which are used to make the disorder ego-dystonic and create a motivation to change are very likely to be highly confrontational. But confrontational interventions are necessary: Without them the disorder will not become ego-dystonic, and clients will not develop any motivation to change.

A therapy should, therefore, not be limited to creating relationship credit. The therapist must use the relationship credit to make confrontational interventions (Sachse, 2016d).

These strategies must first be realized in Phase 2 – that is, not before the therapist has obtained enough relationship credit through their complementary behavior. And for that reason, this complementary behavior must continue in Phase 2 so that the therapist, who has lost credit through confrontational interventions, may again obtain new relationship credit.

It is certainly the client who decides whether an intervention is confrontational or not, because whether an intervention is and to what degree depends on how much it impedes the client's intentions. An intervention is less confrontational, if the client is confronted with aspects of their disorder they already see or do not strongly avoid. However, when drawing the client's attention to aspects of their disorder they refuse to believe, this will be highly confrontational.

A distinction can thus be made between confrontations having different degrees of hardness:

- **Soft confrontations** are those that call the client's attention to aspects of their condition that are less unpleasant to them or aspects the client has already recognized. These interventions affect the client's intentions only slightly and do not cost a large amount of relationship credit.
- **Hard confrontations** are those that call the client's attention to aspects of their disorder of a highly unpleasant nature, which they do not want to touch upon or which they refuse to believe. These interventions strongly impair the client's intentions and cost a lot of relationship credit.

Dealing With Defensiveness

Therapists must be capable of coping with defensive and aggressive behavior of clients. Thus, the therapist should

- formulate the confrontation in as friendly way as possible (never reproachfully!!) and by suggesting that "there is an aspect of your situation I noticed and it could be worthwhile to talk about it";
- if the client reacts aggressively or defensively, the therapist makes clear that
  - they noticed this, but they might also be wrong (what in fact is the case?);

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- they will not pursue this any further at the time; and
- they do not expect the client to deal further with this at the moment.

If expedient, the therapist emphasizes what they meant and what they did not mean. Then they allow the client to continue, and after a while, they confront them again, etc. The therapist here again sets markers and again and repeatedly makes clear they have noticed something, and they invite the client to talk about it.

Between the confrontations, the therapist should endeavor to achieve complementarity, win new relationship credit, and make clear that their confrontations are not aimed at making the client angry but merely to clarify and understand certain things about the client.

Effects of Confrontations

In this therapy phase, confrontational interventions are of significance due to their effects, and these occur although therapists usually do not realize many of them. Mostly, therapists use one to two hard confrontations and three to four soft confrontations per session. Well placed, they are highly effective. Usually, clients can hardly ignore appropriate confrontations: Basically, there are only three reactions they can have: ignoring the confrontation, a defensive-aggressive reaction, or a concerned-reflective reaction.

Mostly, clients begin with the first or second reaction: They do not react at all, they emphasize that the therapist is wrong, that the therapist misunderstands the client, and often the therapist always misunderstands the client, that the therapist does not care about the client or wants to wrongly ascribe something to the client, etc. If the therapist handles this in the above-described manner and politely explains that it is only their point of view and that they might be wrong, but at the same time, they leave it standing (i.e., do not revoke it) and do not force the client to deal with it right then and then let the client roam, this sort of relationship crisis may only last a few minutes (provided, of course, that the therapist has enough relationship credit). As the therapist will then continue behaving complementarily, they will confront the client again after a while using the same strategy.

Every time the therapist does this, they set a marker: They call the client's attention to something, and the more often the therapist does this in different situations, the less the client can ignore the fact that the therapist is right. The more often the therapist politely confronts, explains, and emphasizes that they do not want to provoke the client, the less able the client will be to ignore the fact that the therapist is only confronting them to draw their attention to something important, to something which might really be problematic (i.e., cost intensive), and which should probably be dealt with. In this way, these consistently made confrontations work cumulatively: The client has fewer and fewer reasons to react defensively and will realize that the therapist might be right. Consequently, the client will change over to the third reaction 3, and they will start to deal with the issue at hand.

Certainly, there are clients and confrontations where the first reaction does not even occur, and they are forthwith cooperative. However, we can say from experience that this is quite the exception. Confrontations usually lead to relationship crises that the therapist has to master and withstand before a constructive processing of the contents can take place.

The second reaction means the client now thinks about whether the therapist could be right and, step by step, the client begins to deal with the contents of the confrontations. At first, it is still very unpleasant for them and they avoid rushing ahead. The therapist should give the client time, showing they accept the contents (e.g., that the client is manipulative), can understand it, and still accept the client.

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This is a sensitive phase: The client feels their way into contents that are highly embarrassing for them and needs to be gently guided by the therapist. The therapist holds them to "hot" contents but does not exert any pressure. The therapist verbalizes a lot to emphasize understanding and acceptance, and once in a while asks questions to keep the client in the process.

What can happen here is self-incrimination by the client that at once needs to be therapeutically processed. The client feels bad because of their manipulations; or a client realizes that their dysfunctional behavior scares others away and may even think something like "I have ruined my life." It is now very important for the client to understand that they have acquired their behavior during their biography (see Section 6.4.4: Biographical Work), learned it as solutions for difficult situations, and did not choose it deliberately. Consequently, they are only responsible to some degree. Certainly, they have made mistakes and might have scared away other people. But it is now vital to correct and adjust those mistakes in future. Sometimes, it may be necessary to cope with and get over mistakes and their negative consequences through grief work (see Sachse, 2003).

If the client realizes exactly how they behave and to what extent they have caused the costs themselves that they have incurred, this usually results in a high motivation to change (a first motivational boost). Clients realize they have to do something and must actively change something to reduce their costs.

A therapist should then show them positive ways of doing this and emphasize that clients are able to constructively change their behavior, schemas, etc. (and can in fact do this by resorting to their resources and abilities) and in this manner increase their quality of life significantly. However, this will only be successful, if, together with the therapist, they meticulously clarify and understand their schemas in the first step and then thoroughly process relevant schemas.

In this way, the therapist emphasizes two things:

- clients can reduce their costs efficiently;
- provided they actively strive for an alteration (together with the therapist).

Our experience with confrontations is that during the process, clients increasingly learn that the therapist means well, and as soon as they fully understand that, a new quality in the therapist–client relationship arises: The therapist can confront the client with anything without triggering a reaction in the client. Confrontations will no longer deduct any relationship credits but rather become regular interventions: A phase is reached in which highly constructive work with the client becomes possible!

6.4.2 Replenishing the Relationship Account

A therapist who has made confrontations costing a large amount of relationship credit and resulting in relationship crises must again make sure to earn new relationship credits. For that reason, therapists during this phase must continue to establish a complementary relationship structuring. Especially after confrontations, the therapist needs to be highly attentive and must signal that they

- accept and appreciate the client as a person;
- will not depreciate the client in consequence of the confrontation's contents;
- still appreciates the client's resources, abilities, efforts, etc.;
- take the client seriously, regard them as important, etc.

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6.4.3 Clarification

Clarification processes play an increasingly crucial role in Phase 2. The therapist directs the client toward concrete situations and tries to internalize the client's perspective, and this is usually more and more successful. Some schema elements may have been clarified with the client already at this point – that is, clarifying that clients react aversively when not taken seriously; clarifying that they are touchy in response to criticism, etc.; and clarifying to some extent that they have negative assumptions about themselves and what these are called. Clients often understand quite early that they assume things such as "I'm not important" or "I'm not okay," etc.

6.4.4 Biographical Work

More often than not, initial insights into schemas enable biographical work to commence even in this early phase. Biographical work does not mean that clients must disclose details of their biography or work on experiences from early childhood or the like. What is meant here is that therapists, by well-aimed, careful guidance, lead their clients to understand how certain schemas have been formed and developed in their biography.

Biographical work does not begin until therapist and client have extracted a first schema assumption from the client's current situations and processing work – for example, an assumption such as "I'm a failure." Then the therapist sends the client into their biography with a concrete and differentiated search request: to find out how the client has formed this schema, due to what feedback, and due to what interpretation.

The therapist does not do this because they expect changes in the schema but rather to initiate certain client processes:

- To help the client to realize the schema was built through concrete feedback and interpretations in concrete, historical situations.
- To help the client to realize they are not responsible for the schema and did not deliberately choose the schema.
- To help the client to realize the contents of the schema trace back to certain people and certain contexts to which they were more or less exposed: Therefore, the contents (in this form or extent) are neither true nor accurate but rather depend on certain people and contexts.
- To help the client to realize they were at the mercy of these people and contexts, that they believed and adopted the contents imparted, but that these contents are not at all imperatives.
- To help the client to realize that schemas can thus be questioned, put in context, and changed. What they have believed in they can cast doubt on, what they were told they can change.

This kind of biographical work thus creates an essential prerequisite for the client to be able to decide to actively go against their schemas: It creates an essential basis for a motivation to change.

Other aspects of their disorder can be handled in a similar way – for example, the manipulative behavior of the client. The client can realize here as well that they have developed this kind of behavior as a solution to dilemmas that had to be coped with in certain situations. However, they have not been responsible for these situations and thus should be able to see they had no alternative than resort to this kind of solution. It thus follows that they must not feel guilty in

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this respect. On the other hand, there is no denying that this manipulative behavior has been causing them a lot of trouble currently and for that reason needs to be changed.

6.4.5 Dealing With Avoidance

In the second phase, the active handling of avoidance plays an important role as well: The therapist takes countermeasures. Moreover, the therapist may also confront the client with avoidance by calling the client's attention to an avoidance strategy and offering to talk about it, why it is important for the client to avoid this topic, why it is difficult for the client to stick to the topic.

6.5 Therapeutic Strategies in Phase 3: Clarification of Schemas

6.5.1 Clarification of Schemas

In this phase, the clarification of schemas is crucial: Therapist and client work out a valid representation of the problem-relevant, dysfunctional schemas. The clarification of schemas and motives is the third crucial aspect of therapy with PD clients. This clarification of dysfunctional schemas takes place according to the rules and strategies of COP (for a closer look, see Sachse, Breil, & Fasbender, 2009).

During this clarification, therapist and client systematically work out which current internal processes (cognitions, interpretations, affects, emotions, desires, impulses to act) the client shows in problem-relevant situations (due to their schemas), and based on this material, they reconstruct step by step the underlying schemas.

During this, therapists make interventions

- that activate relevant schemas and so trigger the current coping processes;
- that lead the client to internalize their perspective so that they can represent their processes;
- that introduce and follow the issue leading to relevant schemas;
- that keep the client engaged in the clarification processes;
- that help the client with cognitive representation – that is, with the translation of the schemas into cognition, or rather into language.

As the aspects of schema clarification have been explicitly stated and elaborated in detail in Chapter 3 of this book and in other publications, this shall not be dealt with here again (see Sachse, Breil, & Fasbender, 2009).

6.5.2 Dealing With Alienation

Another important aspect of clarification refers to alienation (alienation of one's own motives). Depending on the client, dealing with alienation may also happen in other phases (see, e.g., Sections 9.4, 10.4.4, or 12.3.6) We have to assume that a lot of clients with PD have relatively bad access to their motive system – in the terms used by Püschel and Sachse (2009): to their

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"implicit" motive system – and that they consequently do not know what they want and what is good for them in essential aspects of their life.

As clients often present with alienation (from their own motive system), it is necessary to therapeutically clarify their relevant motives.

Alienation is a term characterized by Kuhl (Kuhl & Beckmann, 1994) by the fact that people differ in how good their access to their motive and need system is: There are people who have good access to their motive system and consequently have a good (conscious) representation of their motives and preferences. However, there are also people who have bad access and who have barely any representations. These people show a high degree of alienation, an alienation from their motive system. A high degree of alienation entails

- that the person does not know what they really want, what is good for them, what they do not want;
- that the person is very likely to make decisions, follows norms, etc., which miss their actual motives, resulting in a nonsatisfaction of central motives (Kuhl & Beckmann, 1994) for a long time;
- consequently, the person will be massively dissatisfied without knowing the reasons for the dissatisfaction (Brunstein, 1993, 2001; Brunstein & Maier, 2002; Brunstein & Schultheiß, 1996; Brunstein et al., 1995, 1998; Schultheiß & Brunstein, 2001);
- which is why they cannot do anything about the dissatisfaction, often do more of the same, which actually increases the alienation and dissatisfaction (Brunstein et al., 1995);
- they follow motives that massively contradict their real motives, and they consequently develop a strong internal conflict system which is massively depressive so that they develop psychological disorders (Kuhl & Koole, 2005).

Further consequences of alienation are (Kuhl & Beckmann, 1994):

- The person has no internal standards to orientate themselves with.
- In decision situations, this leads to high state orientation and difficulties in decision making.
- Moreover, the person consequently orientates themselves using external standards (by rules, norms, or expectations of others).
- They make so-called intrusion errors where the external standards they adopt can no longer be distinguished from their own decisions based on their own motives, thus they assume external norms as their own preferences (which increases their alienation).

People with PDs always show a higher degree of alienation. Alienation of clients with narcissistic disorder is of medium degree on average, clients with dependent disorder show a high degree of alienation, and clients with obsessive-compulsive disorder show an extreme magnitude of alienation.

Thus a reduction of alienation needs to be worked on with all PD clients. For disorders with low alienation, it is usually enough to use the strategy of explicating relationship motives. This strategy makes central motives salient and consequently reduces alienation. For clients with massive alienation, one should apply special training. Such a training must be actively introduced by the therapist because clients usually do not realize their alienation so that the therapist must explain this problem, make the client conscious of it, and suggest appropriate training.

During such training, the therapist will systematically work through a number of situations with the client, concerning

- relationships;

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- leisure time;
- job;
- life goals.

For every situation, the therapist works by asking the client certain questions, for example:

- "What do I want?"
- "What is good for me?"
- "What is important to me?"
- "What do I want in this situation?"
- "What do I want from the other person?"
- "What behavior in the other would be good for me?"
- "Which aspects of the situation can I enjoy?"
- "What would I miss if it were not there anymore?"
- "What do I not want in this situation?"
- "Which aspects of this are not good for me?"
- "What behavior in others makes me angry?"
- "Which change in the situation would make me happy?"

As a homework assignment, the client can be instructed to observe certain situations and ascertain which aspects of the situation are good and which are bad for them. As a result, a representation of motives and needs can be worked out step by step.

This

- increases their orientation to their own, internal standards,
- increases self-regulation,
- decreases dissatisfaction,
- increases the ability to make decisions,
- decreases the state orientation.

6.6 Therapeutic Strategies in Phase 4: Working Through Schemas

The dysfunctional schemas of clients have to be treated in all cases. However, this is only possible if a stable therapist–client relationship has been established, and the client has defined a therapeutic goal. Therefore, schema treatment is to be dealt with in this therapy phase. OPRP is suitable for the treatment of schemas (see Section 3.2.4: Therapeutic Processing of Dysfunctional Schemas; also see Breil & Sachse, 2009; Sachse, 1983, 2006e, 2014c, 2016e; Sachse & Fasbender, 2013; Sachse, Püschel, et al., 2008; Sachse, Fasbender, Breil, & Sachse, 2011). The therapist defines two positions for the client (by placing two chairs facing one another, the therapist sitting at the side). A client position in which the client represents their dysfunctional assumptions, and a therapist position in which the client

- questions their assumptions,
- argues against their assumptions,
- disproves their assumptions,
- formulates alternative assumptions.

The OPRP proceeds in a series of three steps:

First step: The client sits in the **client position** (CP) representing the dysfunctional assumption.

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Second step: The client sits in **therapist position** (TP), and together with the supervisor, they work out counter strategies against the dysfunctional assumptions and present them to the client imagined to be sitting on the opposite chair.

Third step: The client sits in client position (CP) and, together with the therapist as supervisor, verifies whether the counter strategy is plausible.

The cycles of triple sequences in the OPRP are described in more detail in Section 3.2.4.

6.6.1 Client in Client Position: Working Out a Treatable Assumption

The OPRP starts when therapist and client have successfully extracted a **treatable assumption** – that is, an assumption that shall be examined and changed in the OPRP. A treatable assumption should meet the following criteria:

- It should be an aspect of a problem-relevant, dysfunctional (or compensatory) schema. If a client has a negative self-schema, for example, then a treatable assumption could be "I'm a failure." However, the assumption may originate from Schema Level 2 (e.g., "If I'm a failure, I will be expelled and alone") or from Schema Level 3 (e.g., "being alone is awful"). The assumption can be cognitive and can have strong affective parts.
- It should preferably be central for the schema.
- It should be formulated as concretely and precisely as possible.
- It should preferably be short; the longer and more complicated an assumption, the harder it is to treat.
- It should not be a reality statement such as "I'm afraid" or "when XYZ appears, I'm paralyzed." Statements such as situational descriptions, statements about one's own actions and own reactions cannot be examined via OPRP.

Assumptions that can be expediently worked on are, for example:

- "I'm a failure."
- "I did everything wrong in life."
- "No one is interested in me."
- "Relationships are not reliable."

6.6.2 Client in Therapist Position: Treating the Assumption

The second phase of the cycle begins when the therapist lets the client switch to the therapist position (TP). Now, the client examines the assumption with the help of the supervisor. For this purpose, client and therapist have available many therapy strategies.

This phase begins with giving instructions to the client. These include some very important elements like the instruction to be one's own therapist and consciously distance oneself from the client's assumption. As the client sits on the therapist chair, the supervisor says:

> You are now your own therapist. That means, you disagree with what your client thinks. It is your duty to find something that helps your client. I will help you with that task. As a start, we can discuss what might be helpful. Your client says . . . [the supervisor repeats here the assumption uncovered in the TP]. What could help your client?

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The supervisor and client–therapist will then formulate a **counter strategy**. This process may be quick or take a while, depending on how well the client in the TP can keep their distance and how fast they can develop counterarguments.

Finally, the client–therapist explains the counter strategy to the client–client imagined to be sitting on the other chair. It has proven beneficial if the TP client really talks as if the CP were sitting opposite. That means the TP should look at the CP (not at the supervisor!) and should communicate the counter arguments in their own words. The supervisor may give assistance concerning verbalization and the convincingness of the arguments. It seems to be beneficial when the TP also explains why the counter strategy disproves the assumption. For example, against the assumption "I'm incompetent," one could argue "It can't be true you're completely incompetent, as you graduated with honors."

6.6.3 Client in Client Position: Examining the Counterarguments

The third phase of the OPRP begins when the client is again in the CP and examines the counterarguments the client–therapist has made. When the client again sits in the CP, the therapist's instruction is "You are now client again. Your therapist says . . . [therapist repeats the counter strategy]. Please open yourself to this. What convinces you of that? . . . And what doesn't convince you?"

The coherence analysis takes place in two steps. In the first step, the client shall ascertain the aspects of what has been uncovered that convince them and why. It is important for the already convincing aspects to be considered and analyzed; the therapist should always take time to repeatedly go through the new insights with the client to anchor them in the client's mind and draw first conclusions.

In the second step, the client shall ascertain which aspects of what has been uncovered do not convince them (and why) and if there is an emotional resistance or a diffuse discomfort. The therapist emphasizes that the OPRP is not about talking the client into something or communicating contents they cannot accept. It is rather about finding contents the client is capable of integrating. Thus, the therapist instructs the client to precisely examine the counterarguments. If the client finds a "but," therapist and client clarify what this but is about; during the process, new schemas aspects are worked out which are the basis for a new cycle.

It is this coherence examination that makes the OPRP a learning, self-regulative system. When the client develops new ideas and interpretations, they can prove by themselves whether these are effective. If they are, they can integrate them; if not, they have to continue searching, analyzing, explicating, etc. Insufficient coherence implies there are further, not yet clarified and worked out assumptions that must be worked out to be treatable in therapy. Detailed descriptions of the OPRP can be found in Sachse (1983, 2006c) as well as in Sachse, Püschel, et al. (2008).

6.7 Therapeutic Strategies in Phase 5: Transfer

Clients have to assume that the dysfunctional schemas as well as the manipulative strategies are highly automatized. Even if a client is conscious of what they are doing, and even if they decide

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to no longer behave like this, and actively fight against the schemas, and even if they develop alternative behavior and alternative schemas, the client will not be able to simply implement these in everyday life, as the old structures will keep breaking through. Thus, clients have to actively practice the new behavior and schemas for a while in their everyday life. They have to actively inhibit old behavior and processing resulting from old schemas, they must fight against them, and substitute for them deliberately and intentionally – that is, they have to train for new behavior and schemas in everyday life.

Theoretically, one has to assume that schemas once acquired cannot be "deleted" but can only be inhibited in therapy. They can be inhibited to the extent that they are hardly activated and much less easily so in situations in which they previously were activated very quickly and reliably. Instead, alternative schemas are activated, leading to different processing by the client and thus different cognitions, emotions, and behavior. Therefore, the relevance of the old, dysfunctional schemas is distinctly reduced.

An essential consequence of the assumption that inhibition is behind this process is that an inhibition of dysfunctional schemas builds up slowly. Additionally, inhibition and development do not occur by chance but rather require extensive training efforts.

At the beginning of the therapy, the dysfunctional schema can be easily activated. It is very well established and has a very high activation potential; by contrast, any positive schema, if it exists at all, is very weak; the negative schema is stronger than the positive one (see Figure 3).

The systematic inhibition of dysfunctional schemas during therapy is essential, for the activation potential decreases gradually. A new schema is established or a weak, already existing schema systematically trained and thus gradually established with a greater activation potential. It is to be assumed that there will not be sufficient therapy time to inhibit a dysfunctional schema to be entirely silent, and probably not sufficient time to strengthen a functional schema to become dominant. It is most likely that the client will have to work on this actively in their everyday life after therapy as well. For a long time (probably 6–12 months), the client must constantly and deliberately fight against the old schema, must control and dispute it, and actively and consciously foster and make the new schema salient.

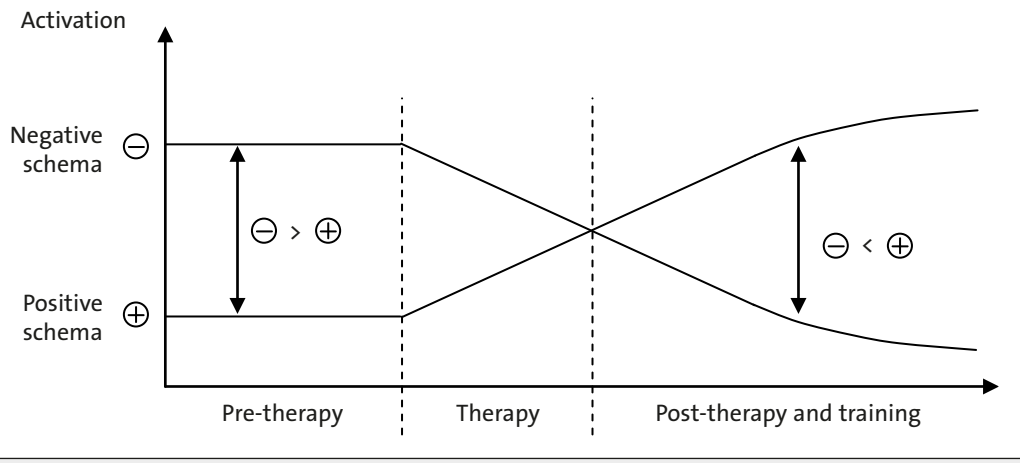


Figure 3
Progress of activation levels of positive (+) and negative (-) schemas.

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That means the client has to work on new schemas in everyday life. They have to fight against old schemas in real situations and consciously foster new ones for a long time.

This shows again how important a high level of motivation of the client is. Some day, the situation will have been stabilized, and the new schema can be activated more easily and quicker than the old one. The client will then be in a new self-regulation mode. They will no longer be impaired by old schemas, and their cognitions, emotions, and behavior will operate automatically on a new level. Therefore, it is also evident that a phase of active analysis of schemas will be necessary.

The state of new, functional self-regulation can only be accomplished by going through a phase of self-control, with an active analysis of schemas.

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Chapter 7

Types of Personality Disorders

7.1 Introduction

An essential question is how to summarize and classify different PDs. The DSM (APA, 1994, 2013) distinguishes between three clusters: However, there is the major disadvantage that they are neither empirically founded nor comprehensibly and theoretically deducted. The clusters combine disorders which resemble each other superficially at the most, but otherwise are highly heterogeneous. In the opinion of the author of this present work, the DSM clusters cannot be understood to be a useful summary of PDs.

In recent years, a great deal of neuropsychological research has been done into PDs. The results have shown that there are PDs showing strong to very strong functional neuropsychological deficits, and many PDs that do not show any such disturbances. These aspects are useful when making a first distinction among different PDs.

7.2 Pure and Hybrid Personality Disorders

Disorders without (noteworthy) functional neuropsychological deficits can be seen as purely psychologically interpretable disorders (or **pure disorders**): Pure PDs can be understood as relationship disorders functioning as indicated by the dual action regulation model. These disorders do not comprise any other disorder aspects.

Disorders that can only be ascribed to some extent to psychological factors (such as schemas, interaction games, etc.) and in which considerable neuropsychological aspects are involved (e.g., neuropsychological regulation deficits, control deficits, information processing deficits) are called **hybrid disorders**. The term *hybrid* emphasizes that psychological as well as neuropsychological factors induce the disorder, so that hybrid disorders can only be partly understood as PDs, because only parts of the disorder are considered to be interaction disorders. Disorders of this type have additional disorder aspects that cannot be interpreted as interaction disorders – for example, aspects of an emotion regulation disorder or an information processing disorder. The considerations usually associated with PDs apply to these disorders only in part or with restrictions; other therapeutic procedures that are not at all linked with PDs are additionally indicated.

The pure PDs are:

- Narcissistic PD
- Histrionic PD

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- Dependent PD
- Avoidant PD
- Schizoid PD
- Passive-aggressive PD
- Obsessive-compulsive PD
- Paranoid PD

The hybrid PDs include aspects of interaction disorders and additional disorder aspects are:

- Borderline PD
- Schizotypal PD
- Antisocial PD

Borderline disorder is also a PD: The client's behavior can definitely be characterized by relationship motives, and it may be highly manipulative, and the client may massively test the therapist and show typical dysfunctional assumptions (see Buchheim, 2011; Beck et al., 2001; Breil & Sachse, 2011, 2016; Butler et al., 2002; Jacob et al., 2011; Kramer & Sachse, 2013; Kramer et al., 2015; Osofsky, 2011; Paris, 2011; Sachse & Kramer, 2016). However, this is only one aspect of borderline disorder. To a great extent, this disorder can be understood as an emotion regulation disorder – a disturbance characterized by triggering and regulating emotional processes (see Fiedler, 2001; Hartocollis, 1977; Herpertz, 2011; Herpertz et al., 1997, 1998; Koenigsberg & Siever, 2011; Levine et al., 1997). Borderline disorder may also be understood as an identity disorder (see Baumann et al., 2011; Herpertz, 1999) or as a dissociative disorder (see Bohus & Unckel, 2000; Dulz & Lanzoni, 1996; Dulz & Schneider, 1999; Fiedler, 2002; Reddemann, 2000) partly caused by massive traumatizations (see Famularo et al., 1991). For that reason, therapy aimed purely at aspects of these PDs makes little sense. Essential in this case are measures used in the dialectical behavior therapy (see Bohus, 2002; Bohus & Haaf, 2001; Bohus et al., 1996, 2000; Linehan, 1993, 1996; Linehan & Kehrer, 1993; Low et al., 2001; Petersen-Ostroga, 1999; Scheel, 2000; Shearin & Linehan, 1992; Westen, 2000).

Schizotypal disorder can be understood as a disorder with deficits in information processing playing a central role and may be interpreted as a schizophrenia spectrum disorder rather than as a PD (see Beech & Claridge, 1987; Beech et al., 1989, 1991; Claridge et al., 1992; Ferraro & Okerlund, 1995, 1996; Hall & Habbits, 1996; Kotsaftis & Neale, 1993; Moritz & Mass, 1997; Peters et al., 1994; Venables & Bailes, 1994).

Antisocial PD can be a PD but may also be a neurophysiological disorder based on a reduction of the frontalis activity (see Bauer et al., 1994a, 1994b; Blair, 2007; Buikhuisen, 1987; Cloninger, 1987; Damasio et al., 1990; Deckel et al., 1996; Franqué & Sachse, 2016; Gödecke-Koch & Emrich, 2001; Gorenstein, 1982; Hare, 1984; Hoffman et al., 1987; Kandel & Freed, 1989; Lilienfeld, 1994; Lueger & Gill, 1990; Minzenberg & Siever, 2007; Morgan & Lilienfeld, 2000; O'Connor et al., 1994; Raine, 1997; Raine & Yang, 2007; Rogers, 2007; Yeudall & Fromm-Auch, 1979).

7.3 Proximity and Distance Disorders

On the basis of the proximity–distance dimension, pure PDs may further be divided into two classes.

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7.3.1 Proximity Disorders

Proximity disorders are characterized by the fact that clients with this kind of disorder establish proximity to interaction partners: They enter relationships, relationships are important to them; they bond, they actively establish proximity to a partner and have a need for proximity. To some extent, they use manipulative strategies to establish proximity and bond to their partner. On the other hand, they use proximity to rope interaction partners in for other objectives – that is, they manipulate through proximity. The following PDs belong to the proximity disorders:

- Narcissistic PD
- Histrionic PD
- Avoidant PD
- Dependent PD

Usually, the therapist can easily establish a therapeutic relationship with clients with a proximity disorder. Provided they treat the client correctly, they may easily gain relationship credit, and, consequently, are able to confront the client relatively soon. (These statements apply on average, and there are certainly clients where this might be more difficult to accomplish; in that case however, establishing a complementary relationship is still normally easier than with those clients who have a distance disorder.)

On the other hand, therapist will be integrated into the client's system relatively quickly. The client will use the therapist–client relationship to manipulate the therapist, and the therapist needs to be aware of this.

7.3.2 Distance Disorders

Distance disorders are characterized by the fact that clients with such a disorder keep their distance from interaction partners. They barely enter into relationships, relationships are not very important to them, they avoid relationships, they avoid proximity, and they actively create distance. They defend their boundaries and only let selected people into their territory. To some extent, they use manipulative strategies to keep interaction partner at bay and to prevent bonds.

The following PDs belong to the distance disorders:

- Passive-aggressive PD
- Schizoid PD
- Paranoid PD
- Obsessive-compulsive PD

It is quite difficult to create a therapist–client relationship with these clients. Establishing a relationship is usually troublesome and tedious. Therapists are often under the impression that nothing changes in the relationship. Since the therapist only gains relationship credit slowly, confrontational interventions can only be made very late. With these clients, the establishment of a trusting therapist–client relationship is already a central therapeutic task. It is a therapy objective, rather than a prerequisite of the therapy.

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7.4 Differences Between Proximity and Distance Disorders

7.4.1 Trust and Building Up the Relationship

As has been described in the previous sections, clients with proximity and distance disorders differ in how they establish relationships. Clients who have a proximity disorder develop trust in the therapist relatively quickly provided the therapist treats them complementarily. Thus, relationship structuring is relatively easy with these clients, and consequently, the therapist may confront and direct them relatively quickly.

Clients with distance disorders have trouble establishing trust. As a result, the phase of structuring the relationship can be very long. For a long time. the therapist may be unable to direct or confront the client. and clarification processes are quickly on the "brink of what may possibly be done."

7.4.2 Affects and Processing Processes

However, clients with proximity and distance disorders do not only differ with respect to their behavior in relationships. Clients with proximity disorders rarely use schemas regulating their affective behavior. They usually allow their emotions and affects to show and often gain positive experience by means of which they exhibit positive affects to a relatively high degree. Positive affects are a prerequisite for thinking in an intuitive-holistic mode or as Kuhl (2001) expresses this, "to have access to the extension memory." Positive affects are thus a prerequisite to being able to think in a complex and creative way (Sachse, 2014b, 2014c; Sachse & Langens, 2014a, 2014b, 2014c).

In contrast, clients with a distance disorder often have "control schemas": They have a high degree of self-control; they control their emotions and affects, often interpret situations negatively, and consequently experience negative affects frequently. However, in doing so, they not only restrict their spontaneity but also impede any intuitive-holistic thinking and limit their creativity and capability to perceive complex issues. They actually limit their competences and thus impair themselves even further.

7.4.3 Therapeutic Consequences

When dealing with clients with a distance disorder, the therapist should keep in mind that

- They should work long and patiently on the relationship structure.
- They should never pressure the client.
- Transparency is a central variable, and therapists should always be transparent in what they are doing, what they want, and what they do not want.
- Therapists should give the client a high degree of control.
- However, therapist should never let themselves be controlled by clients and should never behave complementarily at the game level.

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7.4.4 State of Research

Proximity and distance disorders also differ significantly with respect to their

- theoretical concept development;
- empirical research;
- therapeutic conceptions.

Distance disorders are found less frequently in therapy than proximity disorders, and for that reason, little research has been done on them. All in all, conclusions about distance disorders are considerably less elaborated and empirically confirmed than assumptions about proximity disorders.

Another research problem is that authors often examine an entire DSM cluster instead of individual disorders. Many studies examine, for example, Cluster A of the DSM, which in the opinion of the author of this present work, creates a major problem because the disorders dealt with in this cluster (passive-aggressive, schizotypal) are disorders of a very different nature and cannot be clustered homogeneously (see Esterberg et al., 2010; Gooding et al., 2007; Kendler et al., 2006; Miller et al., 2001; Williams et al., 2005).

7.5 Further Specific Therapy Approaches

It is the objective of this book to view PDs in the framework of the dual action regulation model, and this also means dealing with those aspects of each disorder which are highly practical and therapeutically relevant. In an endeavor to provide a consistent and coherent concept, other therapeutic approaches have been deliberately ignored. It is certainly possible in this way to interpret and treat disorders differently.

For other therapeutic approaches the reader may also refer to:

- For narcissistic PD: Levy et al. (2007), Menges (1999), and Millon (1981, 1996)
- For histrionic PD: Blagov et al. (2007), Horowitz (1991, 1995), and Mentzos (1980, 1982)
- For dependent PD: Bornstein (1992, 1993, 2007) and Greenberg and Bornstein (1988a, 1988b)
- For avoidant PD: Alden (1989), Herbert (2007), Millon (1981, 1986), Pilkonis (1977, 1984), and Renneberg et al. (1990)
- For passive-aggressive PD: Morey et al. (2007), Perry (1989), and Perry and Flannery (1982)
- For schizoid PD: Mittal et al. (2007), Turkat (1990), and Turkat and Banks (1987)
- For paranoid PD: Bernstein and Useda (2007), Vaillant and Perry (1988), and Weintraub (1981)
- For obsessive-compulsive PD: Bartz et al. (2007), Hoffmann (1984), Salzman (1980, 1989), and Salzman and Thaler (1980)

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7.6 Overview of Disorders

Before a detailed description of the individual pure PDs is given, an overview of disorders shall be presented. This tabular compilation serves for fast orientation as well as to compare the disorders (Table 1). As one can see, all proximity disorders either show appreciation and importance or dependability and solidarity as central relationship motives, whereas all distance disorders show autonomy and inviolability of boundaries as central motives (Table 2). As can be seen, proximity disorders accept bonds and proximity, while the distance disorders cannot do this. Proximity disorders are much more manipulative than distance disorders. Proximity disorders also comprise more self-schemas than distance disorders which to a great extent show relationship schemas. The proximity disorders involve more pronounced game structures than distance disorders, consequently therapeutically processing the game level is more relevant for proximity disorders. In contrast, complementary relationship structuring plays an even greater role for distance disorders than for proximity disorders. Establishing a trustful working relationship is more difficult and time consuming with distance disorders than with proximity disorders but cannot be dispensed with. Confrontational interventions play a greater role for proximity disorders; with distance disorders, they can only be realized relatively late in the therapy process.

Generally speaking, therapeutic work with distance disorders requires much more patience, tact, perseverance, and a large amount of empathy. The therapeutic work with proximity disorders is rather dynamic and requires the ability to deal with conflicts, and to confront and strongly direct clients.

Therapists should work out the disorders they can deal with best, and they should never work in areas they do not like or cannot handle (for their own and the client's sake). When working with PD clients, therapists have to be highly authentic as a person; this can only be done if the therapist can respect and appreciate the client. If they are unable to do so, they may easily lose their empathy but not their authenticity, and this will impair the client.

In the opinion of the author of this present volume, working with PDs has something very existential to it: One processes very central schemas and works on life scripts or plans. One gets very close to clients and works on extremely relevant contents. One accompanies clients through existential crises, addresses central questions of meaning and central aspects of human existence. This is extremely fascinating but at the same time extremely challenging as well.

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Table 1. Characteristics of Proximity Disorders

	Narcissistic personality disorder	Histrionic personality disorder	Avoidant personality disorder	Dependent personality disorder
Relationship motives	Appreciation, importance, solidarity, autonomy	Importance, dependability, solidarity	Appreciation, importance	Dependability, solidarity
Schemas	Positive/negative self-schema, special treatment, VIP status, rule setting, territoriality	"I'm not important, relationships are not reliable, relationships are not solidary."	"I'm not attractive, I'm socially incompetent, I will disgrace myself in social activities, I have nothing to offer."	"Relationships are not reliable, relationships are not collaborative" "I can't live on my own; it is a disaster to be left by someone."
Tests	+++	++++	+	++
Proximity + vs. distance ⊖	++++	++++	++++	+++++
Bonding + vs. avoidance of bonding ⊖	+	++	++++	++++++
Alienation	+++	++++	+	+++++
Characteristics	Sensitive to criticism, empathy can be switched off	Dramatic, escalation, alienation	Lack of social competences, self-fulfilling prophecies, persistent schemas	Total subordination, difficulty to make decisions or take a position, alienation, conflict avoidance
Ego-syntony	Changing from + to ++++	+++	+	+++++
External perspective	+	++++	++++	+++++
Manipulation	+++++	+++++	+	+++++

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Table 1 (continued)

	Narcissistic personality disorder	Histrionic personality disorder	Avoidant personality disorder	Dependent personality disorder
Interactional objectives at the game level	Be better than everyone else, show little of yourself, don't be dominated, only expect the best, make demands	Get attention at any costs, be the most important, demand this from your partner, give responsibility to others	Keep yourself in the background, don't attract attention, don't disgrace yourself	Present yourself as indispensable, show yourself as weak and needy, avoid conflicts, avoid fights, always agree, only criticize if nothing can happen to you
Games	Dumb game, Super Hero	Poor sucker, victim of other people and circumstances, direct control, Sleeping Beauty	Almost none	Dumb game, poor sucker
Complementarity on part of therapist	Do not treat adversely; respect, accept, feed, normalize, and create compliance	Take as important and take seriously, accept; avoid everything that could be interpreted as lack of interest; follow therapeutic rules	Take seriously, signal understanding, treat respectfully	Be loyal, don't get involved in games
Treating the game level	Very important	Very important	Not very essential	Very important
Treating the schemas	Very important	Important	Very important!	Very important

Note. +/⊖ = weakly pronounced; ++/⊖⊖ = slightly pronounced; +++/⊖⊖⊖ = distinctly pronounced; ++++/⊖⊖⊖⊖ = strongly pronounced; +++++/⊖⊖⊖⊖⊖ = very strongly pronounced; ++++++/⊖⊖⊖⊖⊖⊖ = extremely pronounced.

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Table 2. Characteristics of Distance Disorders

	Passive-aggressive personality disorder	Schizoid personality disorder	Obsessive-compulsive personality disorder	Paranoid personality disorder
Relationship motives	Inviolableness of boundaries, autonomy, appreciation	Appreciation, importance, reliable relationships, collaborative relationships	Appreciation, importance, solidarity, autonomy	Inviolableness of boundaries, autonomy, appreciation, solidarity
Schemas	"Others do not respect my boundaries, others restrain my autonomy, I don't get appreciation, overt resistance aggravates the problem."	Relationships don't bring anything, flight into autonomy, "I can only rely on myself, I can't handle social situations."	"I'm not okay, I'm toxic, one will be devalued in relationships; if I follow norms I'm safe."	"My boundaries won't be respected, others want to harm me, if I don't react strongly at once I will be pushed aside."
Tests	+++	+	+++	++++
Proximity (+) vs. distance (⊖)	⊖⊖⊖	⊖⊖⊖⊖⊖	⊖⊖⊖⊖	⊖⊖⊖⊖⊖
Bonding + vs. avoidance of bonding ⊖	+	⊖⊖⊖	⊖⊖⊖⊖	⊖⊖⊖⊖
Alienation	+++	+++++	++++++	+++
Characteristics	Passive refusals, with overt compliance	Distance strategy through lack of nonverbal communication, social competence and rule deficits	Following norms serves as protection, and allows for appreciation; *Rule setter!* massive alienation	Hypersensitive reactions to everything that can be interpreted as a devaluation, boundary violation, or impairment
Ego-syntony	changing	++++ but changing	+++++	++++
External perspective	++++	++++	+++++	+++++
Manipulation	++++	+	++	+++

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Table 2 (continued)

	Passive-aggressive personality disorder	Schizoid personality disorder	Obsessive-compulsive personality disorder	Paranoid personality disorder
Interactional objectives at the game level	Don't let yourself be patronized, fight back but only react openly against something if it is safe, don't even put up with trivialities	Stay alone, keep away from others, don't be dependent, you're all right on your own	Always follow your norms, if anyone attacks your norms put up resistance, don't mind your emotions and needs	Be attentive, clearly show how strongly you can fight back, don't show weakness
Games	Poor sucker, heroic poor sucker, always me, martyr	Few	Moses game	⊖
Complementarity on part of therapist	Respect, no pressure, don't violate boundaries, high transparency, give the client control	Understanding, show empathy for the client's frame of reference, no pressure, respect and accept, careful offers	Accepting and respecting, no pressure, no boundary violations, give the client control, do not discuss norms!	Treat respectfully, appreciate resources, total transparency, respect boundaries, no pressure, give the client control
Treating the game level	Important but difficult	Not important	Important but very difficult	Important but very difficult
Treating the schemas	Important	Important	Important but difficult	Important but difficult

Note. +/⊖ = weakly pronounced; ++/⊖⊖ = slightly pronounced; +++/⊖⊖⊖ = distinctly pronounced; ++++/⊖⊖⊖⊖ = strongly pronounced; +++++/⊖⊖⊖⊖⊖ = very strongly pronounced; ++++++/⊖⊖⊖⊖⊖⊖ = extremely pronounced.

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Chapter 8

Narcissistic Personality Disorder

Nothing is so common-place as the wish to be remarkable.
Oliver Wendell Holmes

8.1 Description and Types of NPD

One characteristic of **narcissistic personality disorder** (NPD) is that it can be subdivided into three subtypes (Sachse, Sachse, & Fasbender, 2011): **successful narcissism** (SNPD), **failed narcissism** (FNPD), and **unsuccessful narcissism** (UNPDs). First of all, a general description of NPD will be given, followed by a characterization of these subtypes (see Döring & Sachse, 2008a, 2008b; Sachse, 2000a, 2001b, 2004b, 2006f, 2007c, 2008b, 2014d, 2015b, 2016e, 2016f; Sachse & Fasbender, 2013; Sachse & Müller, 2016; Sachse, Sachse, & Fasbender, 2011; Sachse & Schirm, 2015).

Generally speaking, SNPD persons are successful, strong-willed, and capable of roping in other people for their purposes (Akhtar, 1996; Horowitz, 1996; Levy et al., 2007; Millon, 1996; Ronningstam & Gunderson, 1996). On the other hand, NPD clients exhibit a high level of self-doubt and emotional vulnerability and are sensitive to criticism.

Conspicuous characteristics of NPD persons are described by Millon (1996) and Ogrodniczuk (2013) as well as Ronningstam (1996, 2000, 2005, 2010, 2011; Ronningstam & Gunderson, 1996). SNPD clients are viewed as people who are self-assured in an exaggerated way and in general are quite successful, able to get their own way, recruit others for their purposes, etc. (see Akhtar, 1996; Döring & Sachse, 2008a, 2008b; Horowitz, 1996; Levy et al., 2007; Millon, 1996; Ronningstam & Gunderson, 1996).

However, there are two problems linked with how NPD has been viewed hitherto:

1. NPD clients are often regarded as persons who exclusively exhibit a feeling of greatness; more often than not aspects of self-doubt, vulnerability, depressiveness, and negative self-esteem are overlooked (see Gabbard, 2009; Heisel et al., 2007; Levy et. al., 2007; Pimentel, 2007; Ronningstam, 2009; Russ et al., 2008); but these must be seen as an integral attribute of narcissism.
2. The disorder is usually understood to refer to the so-called successful narcissists while other subtypes of NPD are ignored (see Russ et al., 2008).

With our conception, we aim at establishing as comprehensive as possible a picture of NPD, as well as discussing the therapeutic consequences resulting therefrom.

When making a closer analysis of what characterizes NPD persons, it turns out that former definitions mainly refer to **successful narcissists** while the fact that not all NPD persons

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are successful has been disregarded. For that reason, we put forward the proposition that the definition of NPD must be changed with a view to being able to also encompass failed and unsuccessful NPD. It is of vital importance, in particular from a therapeutic viewpoint, to make this distinction because there are fundamental differences between meaningful strategies to be adopted for these three disorder categories (see Sachse, 2002, 2004a, 2004c, 2006d, 2007c).

8.1.1 The Former Definition: SNPD

SNPD clients are people who in general appear to be very self-assured. They seem to exactly know what they want, are capable of getting their own way, set rules others should obey, are convinced of their capabilities and achievements, and seem to be highly performance oriented. They often manage without difficulty to rope in other people for their purposes and objectives, they tend to show off, but as a rule disclose very little of their own personality; they want to be admired but react (sometimes even extremely) sensitively to criticism. They behave highly in a very target-oriented way, but at the same time are quite hedonistic. Dealings with these persons are not always easy: As long as they are praised and admired, they are relatively easy to handle but when corrected or even criticized, they become difficult, and if their rules are not followed, they will show their true face. Some tend to be aggressive and speak derogatorily. Although SNPD clients allow themselves to get involved in partnerships, they have major problems in forming real bonds; they usually prefer to "have one foot in their autonomy," figuratively speaking.

As per classical definition, an NPD client is a highly autonomous, self-assured, and performance-oriented person who sets a multitude of rules and strongly reacts to criticism.

8.1.2 Diagnostics: DSM Criteria for SNPD

Previous closer evaluations of the DSM-IV criteria for NPD have shown that these criteria deal exclusively with the grandiosity aspects of an SNPD client, whereas the vulnerable aspects are neglected (see Gabbard, 2009; Levy et al., 2007; Ronningstam, 2009). The current DSM-5 criteria (APA, 2013) vary little from those of the DSM-IV.

8.1.3 Further Characteristics of NPD

From the empirical research literature on NPD, it can be seen that the disorder should better be regarded as dimensional rather than categorical, and under dimensional consideration there is no longer any cutoff point between *disturbed* and *undisturbed* (see Cooper, 2005; Foster & Campbell, 2007; Livesley et al., 1994; Markon et al., 2005; Paulhus, 1998; Ronningstam, 2005; Ritter & Lammers, 2007; Watson, 2005; Widiger et al., 2005). The transition from personality style to disorder is fluent and a priori cannot be clearly defined.

When looking at the empirical research literature on the subject of NPD, it is found that the focus has mainly been on successful narcissists – von Hippel et al., 2005 that is, primarily collecting or compiling the characteristics of SNPD clients.

According to this, NPD clients in general are characterized by:

1. High to excessive self-esteem:
   - A highly positive self-appraisal (Emmons, 1984; Kernis & Sun, 1994; Morf & Rhodewalt, 1993; Raskin & Terry, 1988; Raskin et al., 1991; Watson et al., 1987)

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- An excessive self-esteem experience (Colvin et al., 1995)
- High self-confidence (Brown & Zeigler-Hill, 2004; Campbell & Foster, 2002; Rose, 2002)
- A feeling of grandiosity (Raskin et al., 1991)
- A tendency to overestimate their academic capabilities (Farwell & Wohlwend-Lloyd, 1998)
- Making exorbitant predictions about their own performance level (Farwell & Wohlwend-Lloyd, 1998)
- Judging themselves as more intelligent than they are according to objective tests (Gabriel et al., 1994)
- A greater tendency to judge themselves as better than the "average" person (Campbell et al., 2002)
- Overestimating their contribution to tasks performed by a team (John & Robins, 1994)
- Having an increased tendency to recall from memory information that improves their self-esteem (Rhodewalt & Eddings, 2002)

2. High self-doubt and fluctuations in self-esteem:
   - A greatly diminished self-esteem after negative events or experiences such as the lack of success or any feeling of social exclusion (Rhodewalt et al., 1998; Rhodewalt, 2005; Zeigler-Hill et al., 2010); fluctuations between the positive and negative schema
   - Exhibiting a highly explicit self-esteem and, at the same time, showing a low implicit self-esteem (Jordan et al., 2003; Zeigler-Hill, 2006)
   - Self-esteem fluctuations (Rhodewalt et al., 1998; Sedikides et al., 2004; Stucke & Sporer, 2002)
   - Being filled with great shame in the event of failures (Martens, 2005)
   - Distinctive fantasies about own successes and own power (Raskin & Novacek, 1991)

In contrast, in this present work, we do not make a distinction between grandiose and vulnerable NPD as has been done in the literature (Besser & Priel, 2010; Fossati et al., 2010; Rohmann et al., 2012): In our opinion, both forms are manifestations on a dimension of positive versus negative self-schemas.

3. Tendency toward self-protection:
   - Overestimation of positive feedback (John & Robins, 1994)
   - Whenever possible, keeping up positive illusions about themselves and minimizing negative information (Farwell & Wohlwend-Lloyd, 1998; John & Robins, 1994; Morf & Rhodewalt, 2001; Campbell et al., 2002)
   - Showing a stronger tendency than nonnarcissists to attribute success internally while they ascribe failures to external reasons (Campbell et al., 2000; Kernis & Sun, 1994; Rhodewalt & Morf, 1998)
   - Having difficulties in learning from their own mistakes (Campbell et al., 2004)
   - Showing defensive tendencies in response to negative feedback (Kernis & Sun, 1994), in particular adopting defensive strategies such as self-handicapping and playing down the success of other people as well as strategies aimed at demanding positive feedback (Hepper et al., 2010; Hirt et al., 1991)
   - Showing low tolerance toward real self versus ideal self discrepancies (Morf & Rhodewalt, 1993)

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- Showing a higher aggressiveness level in case they are "put to shame" (e.g., when performing a task very poorly; see Baumeister et al., 2000; Bushman & Baumeister, 1998; Sander et al., 2008; Stucke & Sporer, 2002; Twenge & Campbell, 2003), as well as in case they are devalued (Campbell & Baumeister, 2006)
- Increased hostility when they feel inferior to others (Bogart et al., 2004; Bushman & Baumeister, 1998)

4. Performance-oriented actions:
   - Higher levels of creativity (Raskin, 1980)
   - Making better achievements if they think there might be a chance that the resulting effect can be used to improve their self-esteem (Wallace & Baumeister, 2002)
   - Seeking self-esteem improvements primarily in competitive domains such as in surpassing other people (Crocker et al., 2003; Zeigler-Hill et al., 2008)
   - Being able to work well under pressure (Foster & Campbell, 2005; Wallace & Baumeister, 2002)
   - Performing duties more persistently that are classified as intelligence tasks, when there are no alternative opportunities to improve their self-esteem. But if there are other opportunities, their perseverance diminishes (Wallace et al., 2009)
   - Showing strong tendencies to be motivated extrinsically rather than intrinsically (Kasser & Ryan, 1996)
   - Distinctive fantasies about own successes and own power (Raskin & Novacek, 1991)
5. Search for appreciation and recognition:
   - A feeling of being appreciated (Rhodewalt & Morf, 1995)
   - A strong desire to be admired (Campbell, 1999)
   - Grandiose tendencies to try to impress others (Buss & Chiodo, 1991)
   - An inclination to boast about their achievements and behave arrogantly (Paulhus, 1998)
   - Highly competitive behavior (Rhodewalt & Morf, 1995)
   - Inclination to wear expensive outfits, high-priced watches, and conspicuous clothes (Vazire et al., 2008)
   - A strong relation between NPD and workaholism (Clark et al., 2010)
   - Showing a tendency to cheat in performance tests (Brunell et al., 2011; Brown et al., 2009; von Hippel et al., 2005)
6. Egocentricity:
   - A higher degree of self-centeredness (Campbell et al., 2000; Farwell & Wohlwend-Lloyd, 1998)
   - Self-referentiality (Emmons, 1987; Raskin & Shaw, 1988)
7. Interactional aspects:
   - High dominance (Brown & Zeigler-Hill, 2004; Dickinson & Pincus, 2003; Ogrodniczuk et al., 2009; Emmons, 1984; Raskin et al., 1991)
   - A tendency to overestimate their attractiveness (Gabriel et al., 1994; Holtzman & Strube, 2010; Rhodewalt & Eddings, 2002)
   - Higher impulsivity (Vazire & Funder, 2006)
   - Increased hostility (Bushman & Baumeister, 1998; Raskin et al., 1991; Rhodewalt & Morf, 1995), increased anger potential (Twenge & Campbell, 2003; Stucke & Sporer, 2002), and increased aggressiveness (Bushman & Baumeister, 1998; Reidy et al., 2008)

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- Being regarded as less likeable (Kasser & Ryan, 1996)
- React more strongly and aggressively in road traffic situations when they are annoyed by the behavior of other drivers (Lustman et al., 2010)
- An increased need for power (Carroll, 1987)
- Extraversion (Bradlee & Emmons, 1992; Emmons, 1984; Paulhus & Williams, 2002; Vazire et al., 2008)
- Increased outgoingness and open-mindedness (Bradlee & Emmons, 1992)
- High levels of independence (Emmons, 1987; Stolorow, 1975)
- Showing increased tendency to deal rather playfully with romantic relationships (Campbell et al., 2002; Foster, 2008; Foster et al., 2006)
- Feeling safe in close, affiliated relationships (Emmons, 1984; Foster & Campbell, 2005)
- An increased tendency to be less strongly bonded in relationships and rather seeking alternatives to existing relations (Campbell & Foster, 2002; Neumann & Bierhoff, 2004),
- Displaying a tendency to establish romantic relationships with partners who admire them (Campbell, 1999; Farwell & Wohlwend-Lloyd, 1998)
- Showing an increased level of aggression in case they are embarrassed (e.g., when performing a task poorly; see Baumeister et al., 2000; Bushman & Baumeister, 1998; Sander et al., 2008; Stucke & Sporer, 2002; Twenge & Campbell, 2003) as well as in cases where they are devalued (Campbell & Baumeister, 2006)
- Increased hostility when they feel inferior to others (Bogart et al., 2004; Bushman & Baumeister, 1998)

8. Empathy:
   - Being barely able to emotionally empathize, while they have no difficulty with the dimension of intellectual understanding of empathy (Watson et al., 1984); although NPD clients are not incapable of showing empathy, they are able switch it off (Baskin-Sommers et al., 2014) as has been verified in examinations with psychopaths (Meffert et al., 2013)
9. Emotionality:
   - Increased affect lability (Bogart et al., 2004; Emmons, 1987)
   - Sensation seeking (Lasch, 1979)
   - Daydreaming (Raskin & Novacek, 1991)
   - An increased suicide risk (Bronisch, 2000; Heisel et al., 2007; Links et al., 2003; Ronningstam & Maltsberger, 1998)
10. Alcohol:
    - An increased tendency toward alcohol dependency and addiction to gambling (probably more likely to be found with UNPD clients; Lakey et al., 2006; Luhtanen & Crocker, 2005)
11. Therapy:
    - Strongly feeling excluded and injured by others in therapy discussions, and being distrustful (Dimaggio et al., 2008); building up a stable therapist–client relation is deemed to be difficult (Ronningstam, 2012)
12. Stability:
    - The trait of narcissism appears to be relatively time stable (Vater et al., 2014).

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Medium forms of NPD thus correlate with conditions of well-being, and minor degrees of depressiveness, loneliness, anxiety, and neuroticism. In moderate forms, NPD seems rather to be a protective factor against depression (Baumeister et al., 2003; Campbell, 2001; Kohut, 1977; Ronningstam, 2005; Russ et al., 2008; Sedikides et al., 2004; Stone, 1998; Wink et al., 2005). To some extent, NPD clients show traits that resemble those of bipolar clients (positive alternating with negative moods) which may lead to wrong diagnoses (Akiskal, 1992; Fulford et al., 2008).

Furthermore, with regard to gender bias, NPD clients are more often men than women (Bushman & Baumeister, 1998; Farwell & Wohlwend-Lloyd, 1998; Foster et al., 2003; Joubert, 1998; Ladd et al., 1997; Ritter et al., 2008; Stinson et al., 2008).

As with all PDs, NPD may range from a minor personality style to a major disorder. And the same as other disorders, a narcissistic style is rather to be viewed as a resource than a burden: These people are more efficient, straight, and well-organized; show good perseverance; and are ambitious, contented, and successful (see Campbell, 1999, 2001; Horten & Sedikides, 2009; Johnson , Cohen, Kasen, & Brook, 2005, Kohut, 1977; Lelord & André, 2009; Oldham & Morris, 2010; Ronningstam, 2005; Russ et al., 2008; Stone, 1998; Wink et al. 2005).

However, if NPD is seen as, and approaches, a major disorder, then it involves higher costs – for example, Axis I disorders, substance abuse, interpersonal problems, and increased suicide risk (Bachar et al., 2005; Campbell et al., 2002; Caroll et al., 1998; Ehrenberg et al., 1996; Links et al., 2003; Luhtanen & Crocker, 2005; Morf &Rhodewalt, 2001; Rathvon & Holmstrom, 1996; Ritter et al., 2008; Ronningstam, 1996; Ronningstam & Maltsberger, 1998).

We can say from experience that moderate disorders occur most frequently in outpatient psychotherapy. This is why we would like to start with describing the moderate forms to give a first impression (see Döring & Sachse, 2008a, 2008b; Sachse, 2001a, 2002, 2004a, 2004b, 2006a, 2006b, 2007c, 2008a).

Clients with NPD always have a negative self-schema, with assumptions like "I'm incompetent," "I know nothing," "I'm a failure," etc. Thus, clients with NPD always have negative self-schemas (even if they are very successful), and consequently they show a more or less high degree of self-doubts, sharing thoughts like "Can I really make it?" "Am I really good enough?" "I could fail on this task," etc. Other than people who are just effort motivated – in terms of "hope for success," as expressed by Heckhausen (1963) – NPD clients always move on a level of self-doubts, a fear of failure.

In our experience, these self-doubts always stem from their relevant biography and are due to negative feedback from highly important caregivers (mostly the father), a combination of high expectance ("You must be successful!") and doubts ("I don't think you can do it!"), or even the fear of children and youths to be (strongly) depreciated as a consequence of insufficient success. We have never seen clients in therapy who had received only positive (or exaggeratedly positive) feedback – as Millon (1996) assumed they would. As seen at the start of Section 8.1: Description and Types of NPD, three types of NPD can be distinguished (Sachse, Sachse, & Fasbender, 2011):

- SNPD clients, who strongly compensate for their self-doubts through effort and thus actually become successful;
- UNPD clients, who do not really compensate but rather establish illusionary self-schemas;
- FNPD clients, who resemble SNPD clients but have, for example, chosen the wrong program of study (due their high alienation and high expectation orientation) and got stuck in it someday.

SNPD clients realize that some day that they can compensate for their self-doubts through effort and success, and they then decide to try hard and actively prove that their self-doubts are

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untrue (see Section 8.2.2: Successful Narcissists). Due to the success they achieve (if they are good enough), they develop a positive schema parallel with the negative schema, which (for compensatory reasons) is sometimes very positive and includes assumptions like "I am great," "I am highly talented," "I have extraordinary abilities," etc.

Successful narcissists tend to

- Try very hard to be successful
- Boast about these successes
- Compete fiercely with others
- Be highly sensitive to criticism
- Have a highly demanding attitude, think of themselves as being better, and expect special treatment
- Be arrogant, hardly listening to others
- Be highly egocentric and believe themselves to be the absolute center of the universe

Although being successful and often even very successful, they keep having the feeling that they are missing something, are discontented but do not understand why. Their behavior often creates high costs. They wear themselves out in their job, develop psychosomatic complaints (especially cardiovascular problems), neglect important relationships, have too little time for themselves, always worry about career and success, etc. (see Bornstein & Gold, 2008; Daig et al., 2009; Miller & Campbell, 2008; Miller et al., 2009). It is usually these costs that cause NPD clients to undergo therapy, not the PD as such (see Ritter & Lammers, 2007). However, they do not know what they could change, or they may even think they cannot change anything and hope the therapist will show them a way out.

When they come into therapy, they often tell the therapist that they do not have any problems or have everything under control. They talk at length about their success, can hardly be interrupted, and do not respond to any questions the therapist asks. They tend to test therapists to see whether the therapist measures up to them or whether they are good enough for them. They feel quickly and easily criticized, defined as deficient, and tend to become reactant if the therapist gives or offers advice. Sometimes they even react sensitively to the word *problem* (because they surely have no problems) and, basically, already know what to do and already understand their problems. Often, they are reluctant to assume the client role and avoid tackling their negative self-schemas.

For all these reasons, they must doubtlessly be regarded as difficult clients that a therapist must by all means treat correctly to be able to work with them effectively (see Gabbard, 2009; Kernberg, 2007). Inexperienced therapists make many mistakes here, and some of these mistakes may even cause the client to leave therapy; other mistakes lead to power struggles or turn the therapy into a forum for discussions.

UNPD clients in practice do not make real compensation efforts and consequently do not achieve success (see Section 8.2.4: Unsuccessful Narcissists). They either avoid the effort associated with compensation or think they do not have the competence needed to be successful. Consequently, they give up before they have even started. However, as their self-schema is highly negative, often even more negative than that of SNPD clients, they also require compensation for this. They strongly compensate through illusionary positive self-schemas (through assumptions like "my competence goes unnoticed" → "I'm actually highly talented." → "Basically, I could become a lawyer." → "Actually, I am a lawyer").

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A typical unsuccessful narcissist is a person (usually male) who

- Is between 25 and 40 years old when starting therapy
- Dropped out of school and/or several training courses
- Still lives with their mother, has no partner and only a few friends
- Has occasional jobs or even no job
- Spends a lot of time with computer games
- Still believes themselves to have great competence that has not been recognized yet
- Has illusionary ideas about the future: One client (no graduation, 34 years old) once told a therapist: "Mr. B., I know that I can't be federal chancellor anymore, but I will certainly become a federal minister"

These clients are hard to treat in therapy as they are very afraid of facing their desolate situation and because, similar to SNPD clients, they have high demands ("I won't take any job that pays less than $4,000 per month"), so that it is difficult to process negative schemas and be coached to learn to become competent (a suggestion that is often regarded as "defamatory").

FNPD clients are people who are usually successful in school and afterwards go to college because their parents (usually the father) want them to (due to a lack of autonomy and orientation toward high expectations, they are responsive to their parents' wishes) (see Section 8.2.3: Failed Narcissists). As these clients are only extrinsically motivated, their motivation is not be sufficient to cope with the high demands of their studies, and their academic achievements are poor. Due to their negative self-schema and the high expectations of their parents, the client will be filled with a deeply felt fear of failure, ultimately resulting in anxiety over grades and examinations. These clients avoid exams, seminars, etc. However, as they still endeavor to satisfy their parents' expectations, they are unable to terminate their studies. Consequently, they are in a fix: They can neither finish their studies nor abandon them.

FNPD clients are thus positioned between SNPD and UNPD clients: They are indeed motivated to make an effort, but this not sufficient, and moreover, they are not autonomous enough to make and stick to their own decisions.

In our experience, failure in successfully completing study or training is most frequently found among FNPD clients, and we will go into more detail here in this respect. However, failures may also be encountered later in life – for example, at the beginning of professional practice. This may be the case when demands during study were rather low and could be easily achieved by the client, but the professional requirements called for in practice were much more demanding and could not be fulfilled.

All three types are variants of narcissism so that all three types meet certain criteria. Furthermore, they differ in aspects that are highly relevant therapeutically, as well. As is known, SNPD clients can be treated very well. FNPD clients are much harder to deal with, and other therapeutic approaches are needed. UNPD clients seem to be **highly therapy resistant** (at least with respect to the strategies that are available). The three types of NPD mostly differ in their effortful behavior and consequently with respect to their (occupational and social) success; however, they also differ as far as psychological attributes are concerned, and these psychological attributes significantly influence how and where therapy must start.

It is important to keep in mind that all three types of NPD described here are very specific for Western cultures, as the distinction strongly implies differences in effortful behavior – and this also implies that NPD clients choose effortful behavior as an appreciation-acquiring strategy which is doubtlessly, and in all probability, noticeable in our society. However, such a behavior is not at all obligatory, as many other strategies may also be applied, such as establishing a

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macho image, etc. Although our analyses may not be fully applicable to other cultural environments, they are certainly true for our society.

8.2 Definition of NPD, Based on Dual Action Regulation Model

The definition of each PD in this work is based throughout on the relevant aspects of the dual action regulation model. To define the three PD subtypes, one can proceed as follows: first of all, defining the general criteria that apply to NPD; and then following this, defining the specific criteria for (a) an SNPD client, (b) a UNPD client, and (c) an FNPD client.

8.2.1 Definition Criteria for NPD

First of all, within the framework of the dual action regulation model, the definition criteria are determined for NPD before the three subtypes are dealt with, because if one wants to define different subtypes of NPD, it is not only necessary to define differentiating features – that is, type-distinguishing features (which is trivial) – but it is also necessary to define the common features – that is, characteristics that are true for all types of NPD (which is significantly more difficult).

The latter is difficult because all features that are especially true for SNPD clients can no longer be used as general definition criteria. However, even from the DSM-5 definition (APA, 2013), it is evident that most of these criteria only apply to SNPD clients and not to UNPD clients. It is, therefore, not to be recommended that therapists only orientate themselves according to the DSM. In contrast, the dual action regulation model is a good alternative (see Sachse, 2001a, 2002, 2004b, 2006a; Sachse, Sachse, & Fasbender, 2010). The following criteria are most helpful in defining a comprehensive concept of NPD, as all criteria apply to successful, unsuccessful, and failed NPD clients.

Strong Appreciation Motives

The first criterion refers to the central relationship motives: All NPD clients show a strong appreciation motive (in their implicit motive system). It is all about getting appreciation from other people. The relationship motive of appreciation ranks and remains high in the motive hierarchy of NPD clients.

> For narcissists, their relationship behavior is thus about appreciation (although many narcissists are no longer aware of it).

Aside from appreciation, there are often further motives which are of importance, but these are not as important as appreciation. For unsuccessful and failed NPD clients, these motives are significantly less pronounced than for SNPD clients. These include

- importance,
- solidarity, and
- autonomy.

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Negative Self-Schemas

The second criterion refers to self-schemas. Self-schemas are schemas which refer to aspects of the self – for example, to one's own competences, abilities, characteristics, etc. It is highly characteristic of people with NPD that they have two parallel self-schemas, which do not combine to form a single one, which are activated through different stimuli, and for which the activation of one schema inhibits the other. A negative schema stemming from older biography (negative self-schema) and containing negative assumptions about the self; a schema stemming from more recent biography and containing positive assumptions about the person (positive self-schema).

Narcissists have developed a **negative self-schema** due to negative feedback regarding the appreciation motive in their biography.

The negative self-schema may range from strongly negative to slightly negative, but it usually includes assumptions such as:

- "I'm not ok (as person)."
- "I can't be loved or liked."
- "I'm not competent."
- "I can't meet the expectations of important people," etc.

However, part of the self-schema is always the assumption that "because of my deficits, I cannot be (sufficiently) appreciated as a person." This deficit assumption is the basic message of the negative self-schema.

In our experience, all NPD clients, even SNPD clients (!),show a negative self-schema (next to and parallel to a positive self-schema!) (see Fossati et al., 2005; Jordan et al., 2003; Zeigler-Hill, 2006). In our opinion, this is, therefore, also a definition criterion for NPD (not only for UNPD client).

Since criticism triggers the negative self-schema and leads to a negative state of mind (unless the NPD clients do not control this state intentionally), all NPD clients react sensitively to criticism. Criticism affects them personally, and does not leave them cold. Even minor criticism may upset the client psychologically (through activation of the negative schema).

This also implies that NPD clients usually cannot make a distinction between personal and objective criticism. Because any kind of criticism triggers the schema and is aversive, regardless of whether the criticism was meant personally or not.

Positive Self-Schemas

All NPD clients develop a positive self-schema (compensating for the negative self-schema). This contains positive assumptions about the person. These assumptions can be realistic (thus more or less coinciding with real evaluations) or illusionary (thus more or less differing from reality – e.g., be exaggerated).

Alternating Between Positive and Negative Schemas

NPD clients alternate between positive and negative schemas: If the positive schema is activated through certain stimuli (e.g., positive feedback, success), the person will be in a positive

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state of mind. They think about the success they have had; are in a positive, affective state; are rapprochement motivated; etc.

However, if the negative schema is activated through certain stimuli, the person will be in a negative state of mind: They thinks about their failures, are in a negative state of mind, are avoidance motivated, etc.

People with SNPD often alternate between these states. Consequently, they show mood changes (depending on schema activation) (see Bogart et al., 2004; Emmons, 1987). Unfortunately, this alternation between depressive and hypomanic moods often leads to a misdiagnosis of NPD as bipolar disorder (see Akiskal, 1992; Fulford et al., 2008).

Permanent Self-Doubts

It may be assumed that the negative self-schema experienced by NPD clients is only fully inhibited if the positive schema has been strongly activated. However, in everyday life, the positive schema normally should be activated only moderately. Consequently, the negative schema will not be fully inhibited. As a result, the negative schema continuously sends self-doubts to the system. Therefore, the person is, often subliminally, constantly bothered by self-doubts – for example, "Am I really good enough?" "Do I really meet expectations?" "Am I really competent enough?" "Can I make it?" Will I fail?"

Compensation Efforts

The constant discrepancy between the strong appreciation motive and the negative self-schema creates a strong motivational tendency toward compensation. For SNPD clients, compensation means to find something that upgrades the person, something that falsifies the assumptions of the negative schema. The person has to find something that enables them to maintain the positive schema. People can use different strategies here: SNPD clients use a lot of effort as a compensation strategy to be able to exhibit real success; on the other hand, UNPD clients establish an unrealistic positive self-image that shows how they could be.

Negative Relationship Schemas

NPD clients also develop a negative relationship schema during their biography. This contains assumptions that in relationships one will be

- evaluated,
- devalued,
- criticized, and
- will not be appreciated.

The assumptions of the relationship schema thus imply that relationships are potentially associated with devaluation and are thus potentially dangerous. As a result, NPD clients have a strong tendency toward personal distance: They try to maintain a high degree of autonomy (for safety reasons), "holding one's cards close to one's chest." NPD clients keep their distance (see Campbell et al., 2002; Campbell & Foster, 2002; Foster, 2008; Foster et al., 2006; Neumann & Bierhoff, 2004). After a short while, an interaction partner knows how successful, great, etc. the NPD client is, but even after a year has passed, they have not learned anything about the NPD client's problems and weaknesses.

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All narcissists show negative relationship schemas: The assumption about being evaluated and potentially devalued is essential.

Rule-Setting Behavior

NPD clients assume (rightly or wrongly) that they are special: They consequently deduce that they deserve something special. They also claim for themselves the right to determine how others have to deal with them. That means NPD clients set rules for how to interact with them, for example:

- a rule that others have to get out of the way when they want to drive fast,
- a rule that others shall not interfere, and
- a rule that others have to follow their instructions, etc.

NPD clients show a (more or less pronounced) rule-setting behavior: The rules are highly egocentric and inflexible.

Using Others for One's Own Objectives

NPD clients show (more or less elaborated and competent) manipulative strategies aimed at using others for their own purposes and objectives.

NPD is a highly manipulative disorder: Manipulations usually concern taking advantage of interaction partners with a view to achieving one's own objectives.

Egocentrism

NPD is the most egocentric of all PDs. These individuals perceive the world extremely from out of their own ego – that is, they are under the impression everything has to be about them and everybody has to focus on them. Subjectively, NPD clients view themselves as the center of the universe. Although basically, all PDs are highly egocentric, NPD clients are at the top of the scale (see Campbell et al., 2000; Farwell & Wohlwend-Lloyd, 1998).

8.2.2 Definition Criteria for Successful Narcissists

Central Relationship Motives

Following the general definition of NPD, SNPD shall now be defined in this section based on the dual action regulation model. Aside from the motive of seeking appreciation, three further relationship motives are also important to the SNPD client (see also Section 3.2.1: Relationship Formation):

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- The motive of seeking **appreciation**: This motive leads the SNPD client to strive for positive feedback about their self, to seek appreciation as a person, to want feedback indicating they are loveable and have positive characteristics, etc.
- The motive of seeking **autonomy**: This motive focuses on being able to make one's own decisions about one's life, being independent as person.
- The motive of seeking **importance:** This motive focuses on playing an important or essential part in the life of others, being needed, etc.
- The motive of seeking **solidarity**: This motive requires others to be on one's side, to be there if one needs them, etc.

SNPD clients have a strong desire for autonomy (either in their autonomy motive or as what has been described as an "escape into autonomy"). They wish to be praised, be successful, and depend on others concerning these aspects; however, it is exclusively by their own decisions that how that success is achieved, in what areas, or through which strategies. And this is also important to them: Nothing shall be imposed on them; they make their own decisions and assume responsibility on their own.

This is certainly an aspect that greatly contributes to the success of SNPD clients. They do not rely on others to help or save them; whatever they want to do, they actively deal with. SNPD clients are highly action motivated. They are highly decisive and only consider pros and cons if, and as long as, it is absolutely necessary.

Successful narcissists are highly autonomous and highly action orientated.

Dysfunctional Schemas

Negative Self-Schema

Even SNPD clients *always* have a negative self-schema: However, this schema varies among different SNPD clients from slightly to strongly negative. In any case, this negative schema forms the basis for self-doubts and a high degree of sensitivity to criticism. The negative self-schema gives rise to assumptions such as:

- "I am not ok."
- "I'm a failure."
- "I'm incompetent."
- "I'm unintelligent."
- "I fail at difficult tasks."
- "I can't meet expectations."

These assumptions can be negative to a higher or lesser degree (depending on experience or conclusions based on experience).

Relationship Schemas

Relationship schemas include assumptions about how relationships function, what to expect in relationships and how one is treated in relationships. Even SNPD clients have negative relationship schemas such as:

- One is devalued, rejected, and criticized in relationships.
- One is controlled in relationships.

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These schemas induce clients to have reservations about relationships and often refrain from being linked by bonds: They want to stay autonomous in relationships to be able to break off the relationship, if necessary.

Problematic as well are assumptions from the solidarity motive, such as:

- Relationships are not collaborative.
- One can not rely on others.

These assumptions form the basis of respective compensatory normative schemas, like "Only rely on yourself!"

Compensatory Schemas

Positive Self-Schemas

As a result of using compensation strategies, NPD clients develop a positive self-schema containing more or less positive assumptions about the self; usually, the assumptions of this schema are negations of the assumptions of the negative schema. (It is possible but not obligatory that these positive assumptions are formulated positively in an exaggerated way.)

The positive self-schema includes assumptions such as:

- "I'm competent."
- "I'm great."
- "I'm highly efficient."
- "I can take on any challenge."

Here as well, the schema can be more or less positive, may contain relatively realistic self-assumptions (realistic in that assumptions, e.g., about intelligence or competence largely coincide with capabilities) or (for some SNPD clients) may contain exorbitant self-assumptions. However, for SNPD clients, their positive self-schema always shows a great many realistic, valid assumptions.

Parallelism of the Self-Schemas

Both self-schemas, positive and negative, always exist in parallel: Positive experiences trigger the positive self-schema and create a positive state of mind. The person activates their positive assumptions, feels great, is rapprochement motivated, accepts challenges, etc. Depending on the extent to which the positive schema is activated, the person's overall condition is more or less positive. This may range from a slightly positive mood (e.g., in the case of a little success) to an almost hypomanic state (in case of extremely positive feedback). Thus, schema activation has a great impact on the overall condition of the person.

Therefore, a positive schema (coexisting with the negative self-schema) is a definition criterion for SNPD clients. But since the schema also contains the easily triggered negative schema, SNPD clients must be characterized as being (highly) sensitive to criticism and largely unable to make a distinction between personal and objective criticism.

Normative Schemas

To compensate for the negative self-schemas, the person develops normative schemas – that is, schemas that include their own objectives and behavior rules for themselves. These are objectives containing instructions for the person to do something with a view to avoiding negative consequences. SNPD clients assume things such as rules to always:

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- Be the best!
- Always win!
- Show off your success!
- Show everyone how great you are!
- Accept any challenge!

For narcissists, normative effort objectives are avoidance objectives; as a result, narcissists to a great extent are not intrinsically but rather extrinsically effort motivated (see Deci, 1975; Deci & Ryan, 1991, 2000). Nonetheless, they can also show intrinsic motivation.

Further normative schemas are:

- Avoid criticism and devaluation!
- Avoid failures!
- Avoid being vulnerable!
- Avoid being controlled!

On this level, there can be autonomy objectives such as:

- Stay independent!
- Always avoid deep bonds!
- Keep your control!
- Make sure you can rely on yourself!

These autonomy objectives are **avoidance objectives** and not an expression of the autonomy motive: The person does not want to be autonomous, they are rather under the impression they must be autonomous. We thus see this as an escape into autonomy.

Based on our previous analyses we assume that it is in fact this escape into autonomy that constitutes a major obstacle to bonding: Clients who show this kind of tendency cannot allow themselves to be unreservedly involved in a relationship but always need the feeling to be free to live alone; they treasure their autonomy and react (often highly) aversively to anything that restricts their autonomy.

Rule-Setter Schema

Aside from the normative schema, SNPD clients often develop other compensatory schemas: rule schemas. Rule schemas include rules for interaction partners about how the SNPD client wants to be treated by others, the behavior they deserve, and what they should expect.

These rules of SNPD clients are

- egocentric – the rules are always about the SNPD client's self;
- flexible – adjusted to the client's needs;
- perhaps not even mandatory for the client themselves – the client stands above the rules and may abrogate them for themselves (note: the rules set by a person with an obsessive-compulsive PD are to be observed by everybody, they are rigid and unchangeable, and the client themselves must obey these rules).

On the contingency level of these schemas, penalties are found that the SNPD client imposes if an interaction partner neglects the rules.

It may be of interest to examine how SNPD clients decide on these these rules. SNPD clients assume they have accomplished a lot in their biography and that they have even been successful in spite of great resistance. In fact, this may usually be correct. However, SNPD clients deduce from that that they are entitled to deserve special rights – for example, VIP treatment

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and the right to define social rules and how others must deal with them. They also see themselves as having

- the right to demand service right away if they want to pay;
- the right to be dealt with first at store checkouts, since their own time is much more precious than that of others;
- the right to expect that slower drivers get out of their way, etc.

Moreover, they assume themselves not only to be entitled to demand all of this but to also have the right to punish "rule breakers." To berate service people if they do not respond at once, insult people who do not get out of their way, and oppress or reproach slower drivers. It is this interaction behavior that brings many costs for NPD clients in everyday life, as interaction partners, of course, do not at all tolerate such a behavior.

Manipulative Behavior

SNPD clients develop manipulative behavior to a great extent. This basically involves using interaction partners for their own objectives. They use others – for example, to listen to stories about how great they are, what they have achieved, etc., with a view to being admired and appreciated ("Super Hero game"). Or they use other people to carry out work for them, relieve them of annoying duties, cover for them, etc.

SNPD clients use games – for game structures, see Volume 1 of the series by Sachse, Sachse, & Fasbender (2010) – such as:

- Super Hero: to demonstrate success, competence, and generally show off;
- Rule Setter: to manipulate others and achieve one's own objectives;
- Dumb Game: to free oneself from annoying duties.

UNPD clients also play the

- Victim Game: to exculpate and enforce intentions and delegate responsibility;
- Always Me: to enforce intentions and delegate tasks.

Within the framework of manipulative strategies, SNPD clients send characteristic images:

- I am great.
- I am competent.
- I can do (almost) everything.
- I accomplish every task.
- I am very intelligent.
- I am very successful.
- I am the best, etc.

They also send corresponding appeals:

- Praise me!
- Appreciate me!
- Find me great!
- Affirm me!
- Don't question me!
- Don't criticize me!

The interaction of images and appeals in complex games is called a *game structure* (see the subsection Game Behavior: Images and Appeals, in Section 4.2.4; and see Sachse, 1999, 2000a, 2001b, 2002, 2004a, 2004b, 2005, 2007c).

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Characteristics of SNPD Clients

Real Compensation

Due to the negative self-schema, SNPD clients show high levels of motivation for real compensation: They want to falsify the assumptions of the negative self-schema. This compensation motive is, as one can assume, a very strong force, probably even stronger than the appreciation motive itself would be. It might be this force that enables SNPD clients to accomplish maximum performance and pursue objectives for many years and against massive difficulties. Therefore, the interesting question arises (socially interesting as well) of whether one has to be narcissistic to be really successful: Is one really capable of putting up with enduring the hard times of a career such as that of a professor, principal consultant, manager, etc., without having the power of a NPD client?

SNPD clients are confident enough to create such compensations through their behavior and they are willing to put in the necessary effort. The result is a highly effortful behavior that, when accompanied by competence, leads to real successes. And one may assume that confidence in their compensation abilities even increases with the success they have achieved.

Objectives

SNPD clients usually pursue great, ambitious objectives; they pursue long-term objectives; and they pursue these objectives very persistently. If these individuals also have a high intelligence and competence level, they will be occupationally successful – even very successful.

Intrinsic Motivation

Due to their high levels of autonomy and high degree of self-determination, SNPD clients realize achievements in areas they are interested in and which are compatible with their motive system. Therefore, their effortful behavior is motivated extrinsically as well as intrinsically. As a result, they are able to maintain this effortful behavior for a long time. Notably, these efforts are not only focused on (extrinsic) success but (at least to some extent) are also fun and satisfying in themselves. Accordingly, their behavior is not at risk of becoming really aversive, boring, or too demanding.

Alienation

When it comes to delivering performance, SNPD clients show some degree of alienation: They often can no longer decide which aspects of their involvement are pleasant and which they only work on to achieve success. However, with respect to other aspects of life, they show little alienation and know exactly what they want, are often hedonistic, and can definitely enjoy their life.

Costs

However, unrealistic SNPD clients may tend to push themselves to their limits: They ignore the limits to their efforts and their exhaustion, and may suffer desolate physical conditions. When having a high rule-setting preference, they also tend to get permanently upset, and as a consequence may even develop coronary heart disease (see research on Type A behavior, especially

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the relevance of overt aggression; Friedman & Rosenman, 1959, 1974; Friedman et al., 1984; Johnston, 1992; Thompson, 1988; Williams, 1985).

Another relevant cost factor are interactional problems, especially due to the pronounced rule-setter behavior: problems with partners, friends, colleagues, and bosses.

Ego-Syntony

On average, SNPD clients show high ego-syntonySNPD, but this may vary. If the SNPD client is in a state of mind determined by the positive self-schema, their ego-syntony will usually be very high. Clients are then under the impression that everything is all right, and there is little or no need to take action.

However, if the client is in a state of mind determined by the negative self-schema, their ego-syntony will be low; their system may even be highly ego-dystonic, with the client being under the impression that nothing is all right and that there is a need for action. In this condition, there may be a distinct motivation to change.

Perspective

SNPD clients are usually able to adopt an internal perspective and thus follow the internalizing interventions the therapist makes. If they have trouble with that, therapists should coach them to gain this perspective by frequently realizing internalizing interventions, or the therapist may need take into consideration the possibility that avoidance may be involved.

Avoidance

SNPD clients often show a strong avoidance behavior: They do not want to admit that they have problems, do not want to realize they cannot cope with certain challenges, and above all they do not want to be confronted with their negative self-schema. Normalizations can be helpful here: The client must understand that

- From the therapist's point of view, their problems are not defamatory.
- It is wise (and not a weakness) to seek professional help.
- The therapist trusts the client to be capable of solving their problems.
- Their problems must be faced and understood before they can be solved ("I don't have to tell this to an intelligent client like you").

One can often motivate clients to face the negative self-schema by introducing it as follows:

I [therapist] think you have achieved a lot in your life. But I'm also under the impression that you have developed many self-doubts in your biography. On the other hand, you have proven that these self-doubts are wrong and absolutely unfounded. From a psychological viewpoint, we know that self-doubts when acquired cannot simply be deleted; even if they have been proven to be wrong long since, they keep impairing us in life. And my impression is that this is true for you, too. So, please let me suggest that we take a closer look at your self-doubts and eliminate them once and for all

If one realizes such an intervention in Phase 2 – that is, when enough relationship credit is available and a therapeutic goal has been established, we can say that most clients will at that point embark on a clarification of aspects of their negative schema.

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8.2.3 Definition Criteria for Failed Narcissists

When analyzing FNPD clients in detail, the following points are clear.

Relationship Motives

Even FNPD clients show the central motive of seeking appreciation and furthermore the motives of seeking importance and solidarity. Autonomy plays only a minor part (see point no. 5 below).

High-Expectation Orientation

Clients are strongly orientated toward meeting the expectations of people important to them (usually the father): "Dad wants me to become a lawyer, so I will become a lawyer." Like SNPD clients, these individuals do not merely endeavor to meet general expectations that they be successful, but they even try to fulfil specific expectations ("Become a lawyer!" "Become a doctor!"). They have thus adopted explicit objectives of reference persons to a great extent and will adapt their behavior accordingly. Furthermore, they usually consider these adopted objectives to be their own.

High Alienation

Trying anxiously to fulfill the expectations of others and confusing external objectives with their own objectives result in a high degree of alienation (Beckmann, 2006; Kuhl & Beckmann, 1994). FNPD clients have poor access to their own motive system, are thus largely unaware of their own preferences, and tend to pursue objectives which are incompatible with their motive system. FNPD clients do not know what they really want and are unable to figure it out (due to their high external orientation). They live their life external to their own motive system and pursue (with great effort) objectives that are not their own, and which may perhaps be incompatible with their motive system.

Obligation Toward Expectations

FNPD clients feel highly obligated toward specific expectations and have a great deal of trouble freeing themselves from these expectations; if they do not meet the expectations, they are convinced they have failed, and failure must by all means be avoided. Consequently, FNPD clients are unable (without professional help) to challenge these expectations: They are unable to find out whether they even want to meet the expectations or if there may be other options for them. In many cases, they are even unaware of their high-expectation orientation, as they mistake the adopted objectives for their own.

Low Degree of Autonomy

Consequently, FNPD clients are far lesser autonomous than SNPD clients: They let themselves to a great extent be determined in specific ways, and they strongly pursue rules of authorities

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and do not dare to oppose, break, or revolt against those rules. This high degree of dependency on authorities is characteristic of FNPD clients.

Choice of Wrong Occupation

A tendency to meet specific expectations, combined with low autonomy and a low tendency toward rebellion, often leads FNPD clients to make occupational or study choices they are not really convinced are correct. They study law or medicine because their father wants them to (for example, father wants the son to be very successful, attain a high status), although they have never really scrutinized whether they want it too, whether the chosen study coincides with their interests and abilities, whether they really want to do the job. Consequently, many FNPD clients make wrong study choices. They find the contents of their studies to be uninteresting and to demand too much of them, and they are not intrinsically motivated to cope with that.

Usually, the FNPD client does not have problems during their years in school. They can meet the demands and challenges relatively well and get appreciation for it. However, as soon as they take up their advanced studies, something they actually do not want and which may not be in line with their competences, problems will occur. The demands on them greatly increase, and the client must accomplish a lot to come up to expectations. They must put in a great deal of effort.

As the activities they have to perform are incompatible with their true motives, the clients are not intrinsically motivated. They do not enjoy the activities. They must make great efforts for something that is not worth their while. As a consequence, the activity will become increasingly aversive, unpleasant, boring, and tedious. An avoidance tendency emerges and becomes stronger with time.

A second avoidance tendency may result from the fact that due to more and more time and energy needing to be spent, the FNPD client develops major resistance against extrinsic control. They refuse to be determined by the expectations of others anymore. However, they cannot free themselves from the compulsion and consequently experience it as increasingly burdensome. Their resistance increases and with it the tendency to refuse to perform. Many FNPD clients are not even aware of this mechanism. Since they are very afraid of failure and of having to admit they are unable to meet expectations, they strongly avoid facing the conflict. Therefore, many FNPD clients keep thinking they are still highly motivated to finish their studies, even if they are no longer in a position to do so.

A third avoidance tendency is experienced when the final and most important exam is to be taken. Now is the time to show whether one can succeed or not; now failure may be imminent. As a consequence, exam and assessment anxiety and in the end concentration disorders are gradually encountered. These anxieties induce the clients to avoid the challenges. The closer the exam gets, the stronger this tendency becomes.

Stuck With Conflict

As a consequence of these avoidance tendencies, the FNPD client will be less concerned with the contents of their studies, fear of failing their objectives will increase, and the tendency toward avoidance will grow. A vicious circle is established with the consequence that the person will no longer be able to work constructively. They cannot resolve this conflict. On the one

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hand and due to their strong expectation orientation and deep fear of failure, they are highly motivated to read, study, and attend seminars, etc. On the other hand, however, due to aversion to the subject, to a tendency toward rebellion, and their anxiety about disgracing themselves through failure, they show a strong tendency to avoid their studies, exams, and tests.

Within this conflict, the FNPD client sways to and fro. If the rapprochement tendency is substantial, they will sit down and start working. However, through immediate confrontation with aversive contents, their avoidance tendency grows and increasingly causes concentration difficulties resulting in their finally abandoning their work, taking a walk in the park, and having a very guilty conscience.

Impossibility of Escaping From Conflicted Motives

However, the FNPD client is still authority fixated and full of fear of failure when being unable to meet expectations. Consequently, they refrain from dropping out of college, are unable to escape this situation, and try to muddle through. They

- postpone tests,
- get sick notes,
- prolong the duration of study,
- do not tell their parents the truth about the situation and hope everything will be all right.

Some FNPD clients are rather persevering in sticking to this over a long time even presenting to their parents wild figments of their imagination. We once had a client whose preliminary medical examination was due, and he made his parents believe that he had safely passed his final examination and was already worked as an assistant physician. Certainly, getting out of such a story would be more than problematic.

An FNPD client tends to behave like the person who falls off a skyscraper and at every level, they pass on the way down, thinks "went well so far!" However, like that person, an FNPD client with this strategy will inevitably hit the ground.

Exculpating Strategies

If the FNPD client really fails, they tend to use exculpating strategies such as

- I failed because of my exam anxiety.
- The university structure leaves you alone, etc.

because they are unable admit this failure to themselves and especially to the authority figure(s) in their life.

8.2.4 Definition Criteria for Unsuccessful Narcissists

UNPD clients are people who show the earlier-defined characteristics of NPD clients in general (see Section 8.2.1: Definition Criteria for NPD). They have a negative self-schema, a compensatory positive self-schema, and a strong appreciation motive, as well as a (more or less distinct) rule-setter behavior.

However, what distinguishes UNPD clients from SNPD clients is their effortful behavior. UNs either do not show effortful behavior at all and do not make compensatory efforts (we call this a Type 1 UNPD client), or they fall significantly short of their (intellectual) possibilities (we call this a Type 2 UNPD client; Sachse, Sachse, & Fasbender, 2011).

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Entirely unsuccessful narcissists (Type 1) are usually those who drop out of school or barely finish it, abandon one to three apprenticeships, are dismissed from several jobs, are unemployed, stay at home and play computer games, and have only a few social contacts and no partner. At the age of 30 or 40, many of them still live with their parents. It appears they have more or less accepted their misery. To put it more drastically, they are "up shit creek," and it smells somewhat tangy, but it is warm where they are, and it would be very stressful to get out.

Relatively unsuccessful narcissists (Type 2) have earned their degree, but although being sufficiently intelligent, they earned it only with great difficulty. They are not very success orientated, do as little as possible, do not face challenges or they systematically avoid them, avoid efforts and remain far behind in realizing their potential. They live on their own and at first glance do not seem to be unsuccessful: They have a job, a partner, a "normal" life. But when taking a closer look, it is evident that they have jobs for which they are overqualified, do not take an opportunity to climb up the career ladder, do not do more than is absolutely necessary, and systematically avoid making any greater effort. They just barely live up to expectations.

It is thus clearly obvious that Type 1 UNPD clients are the greater problem, and, therefore, we will concentrate primarily on this group in the following sections.

Motives

UNPD clients show a strong appreciation motive and more often than not an importance and solidarity motive. Autonomy may also play a role sometimes.

Negative Self-Schema

The negative self-schema of UNPD clients is very pronounced, and these clients' self-efficiency expectation is very low. On average, the negative self-schema of UNPD clients is much more negative than that of SNPD clients (as far as we know, this is due to significant devaluation influences in their biography). What must be considered, however, is their low degree of self-efficiency expectation. These clients are under the impression they can only achieve little or even nothing, can barely succeed, or due to a lack of competence, must try very hard to achieve anything.

Illusionary, Positive Self-Schema

The positive self-schema of UNPD clients is illusionarily positive and contains assumptions about what the client could have achieved or what they are (probably) capable of. The UNPD client has assumptions such as:

- "Basically, I'm intelligent."
- "If my parents had encouraged me more and my teachers had not bullied me, then I would be a lawyer today."
- "So, I'm actually a lawyer."

The client's assumption can differ extremely from the real situation.

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Unrealistic Objectives

UNPD clients usually claim they pursue objectives, but compared with their capabilities and due to their insufficient training and knowledge, these objectives are massively unrealistic. For example:

- A client without any qualifications wants to become an investment advisor.
- A client without any qualifications dabbled in selling real estate (and failed).
- A client without any qualifications said, "I want a job in which I make at least $4,000."
- A client without any qualifications told his therapist: "In 3 years, I'll go to Hollywood and be a star!"
- A client without any qualifications told his therapist: "I know I can't become federal chancellor anymore, but I will become a federal minister for sure!"

Avoidance of Effort

UNPD clients show a high to extremely high avoidance of effort: They avoid all activities which require effort, greater involvement, or any kind of endurance.

Exculpating Behavior

UNPD clients use exculpating assumptions to a high degree, and use them to explain why they have no real success. They make assumptions such as:

- "Nobody ever encouraged me."
- "Everyone was against me."
- "The school system is unfair."
- "I never got a chance."
- "Teachers sabotaged me."
- "Life is unfair."

Rule-Setter Behavior

UNPD clients often display a rule-setting behavior that creates massive interactional problems. Interestingly, UNPD clients sometimes expect the same VIP status as SNPD clients ("Since I'm actually a lawyer, I want to be treated like one"). While interaction partners let the SNPD client get away with this behavior, they think it very inappropriate when dealing with UNPD clients – resulting in correspondingly negative social reactions.

Unsuccessfulness

UNPD clients are either occupationally totally unsuccessful and have no school-leaving qualification or do not graduation, have no training, etc., or they remain way behind what they could actually achieve in an occupation.

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Characteristics of UNPD

Unrealistic Positive Self-Concept

Since UNPD clients are not inclined to achieve effort compensation and thus do not acquire any competence, they cannot create a realistic, positive self-schema that is based on real success or real competence. However, since the UNPD client has a negative self-schema and usually continues to get negative feedback from others, they urgently need compensation – but real compensation is unavailable. For that reason, UNPD clients develop two illusionary compensatory strategies, which are closely interlinked and interwoven – that is,

- assuming themselves to have hidden or latent competence and capabilities, and
- pursuing exculpation.

These clients claim to have a number of latent abilities and competences – that, actually, they are highly intelligent, have high general and specific competence, are actually very diligent, and the like. Or they claim that with their competence, they will come a long way. With UNPD clients, all these latent competences and "bills to be honored in the future" may diverge from reality to various extents. Usually, these are illusionary and sometimes even utopian.

A second important strategy is exculpation: The clients make clear they could not have used their intelligence and competence to achieve success and make greater efforts, because they haven't been encouraged sufficiently by others; have been hindered, bullied, etc. by parents, teachers, others; and are thus not responsible for their failures (the "exculpation" strategy).

With the help of these strategies, UNPD clients establish an illusionary self-schema which says:

- I'm basically highly intelligent.
- I'm actually an unrecognized genius.
- I'm actually very successful etc.
- If I had been more encouraged and less hindered by others, I would have been a Nobel Laureate today.

Their statements in therapy are just like this: Clients in their 30s, without any qualifications, and who have abandoned apprenticeships, tell their therapists things like the above. Their self-schema is clearly a positive, compensatory one, and it is obviously illusionary as it is not based on any real competence, success, or efforts. And it is – more or less – far from reality. They thus do not only fool themselves but others as well and sometimes in an absurd way that is highly inconsistent with reality. But it is especially this degree of absurdity that shows how vital the compensation is.

With the help of these strategies, UNPD clients may also try to keep criticism at bay (i.e., persistent negative feedback from parents, partners, etc.), because they obviously cannot be held responsible for the failures they have experienced. Others are to be blamed for having prevented them from achieving success. And, therefore, nobody is actually allowed to criticize them. However, by using exculpating strategies these clients do not assume responsibility for their failures and therefore the following applies (see Sachse, 2009): Whoever does not assume responsibility for their actions is unable to bring about changes in their condition.

Abrogation of Responsibility and State Orientation

Due to these exculpating strategies, clients with UNPD strongly tend to externally attribute any kind of responsibility for their failure and their desolate situation. They cannot be held responsible for anything, because they never had a choice, everything was inevitable, and they

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are the victim of circumstances and of other people. And they continue to be the victim of circumstances because today the situation is so confused and stagnant that nobody could escape it. Thus, nobody must blame them for being in a desolate situation, and likewise, they are not responsible for not being able to change the situation.

Avoiding any responsibility in this way is something that distinguishes UNPD clients from SNPD clients: SNPD clients endeavor to assume responsibility. When they experience failure, they blame themselves for it, have another go at it, and try even harder. It is thus evident that SNPD clients are strongly (sometimes extremely) action orientated. They quickly make a new attempt, do not ruminate over situations, but take action. In contrast, UNPD clients are strongly state orientated: They think about what they could do, analyze and consider carefully – but never start acting.

Rule Setting and VIP Status

Interestingly, UNPD clients, the same as SNPD clients, also develop a need for VIP status and rule-setter structure. They stake out a claim to achieve VIP status from the assumption that in actual fact they are as good and competent as successful people, meaning they deserve the same treatment. If in actual fact one is a manager or Nobel Laureate, one has the right to be treated accordingly. And if someone is entitled to receive such treatment, relevant claims can be put forward. Unfortunately, if made by a person who has nothing to show to substantiate them, interaction partners react even more negatively to such demands than they do with really successful persons. By adopting these strategies, UNPD clients make themselves highly unpopular.

In the opinion of UNPD clients, rules may be set by them because

- they are (basically) something special, and especially
- they have been treated unfairly by society – that is, society owes them.

And that is why they are allowed to define egocentric rules that clearly determine how others should treat them. They may, for example, expect to be served immediately in a restaurant, otherwise they will react clamorously; they may expect special rights in relationships and take offense if they cannot enforce them, etc.

Such rule-setting behaviors result in UNPD clients themselves contributing greatly to their failures. By behaving in such a way, even the highly successful clients may offend others, but the "losers" normally cannot be expected to get away with it and must know that "the Empire strikes back." A client of one of our therapists had a job as a janitor in a school. The principal came and said, "Please sweep the yard." The client answered: "I won't sweep the yard; that's beneath my dignity." As a consequence, he lost his job. Apparently, this occurs quite frequently, and UNPD clients lose their jobs because they think they do not have to follow social and work rules or comply with instructions from their boss, they do not arrive in time, they neglect routine work, or they think they are entitled to pick and choose what they do. This is a well-established strategy of self-handicapping adopted by UNPD clients.

8.3 Therapeutic Strategies

First of all, general therapeutic strategies are described here that are applicable to all types of NPD clients, followed by a brief description of specific procedures for the three types of NPD.

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8.3.1 Therapeutic Strategies for NPD in General

Therapeutic Strategies for Phase 1

The therapy for NPD clients can also be divided into phases, and every phase has various therapeutic priorities.

Complementarity at the Motive Level

Complementary therapist behavior at the motive level is of vital importance when dealing with clients suffering from NPD: The therapist does not stand a chance in establishing a therapist–client relationship and winning relationship credit without showing complementary behavior (see Sachse, 2000b, 2001a, 2004b, 2006c).

For that reason, therapy always begins with complementarity (Phase 1): The therapist starts at the point where the clients stands at the moment and helps them step by step to develop a therapeutic goal. Complementary behavior has several aspects which are addressed below. This involves some general rules focusing on the therapist's complementary behavior.

1. Never Define the Client as Deficient

Therapists must always bear in mind that clients with NPD are extremely appreciation motivated, on the one hand, but, on the other, they fear being devalued in relationships. It is essential that in relationship structuring, the therapist bears these two aspects in mind.

> The central principle of complementary behavior in treating NPD is: The therapist must never and by any means define the client as deficient!

NPD clients highly fear devaluation, to be identified as a sham. Thus a NPD client will only be ready to work with the therapist if they are absolutely sure the therapist will not define them as deficient. This is the most important aspect when establishing a complementary relationship. The client must trust the therapist, and be convinced that the therapist

- will always be convinced the client has many positive aspects aside from their negative ones;
- always respects and appreciates the client as a person;
- always thinks the client is okay;
- never uses client-specific information against the client; and
- never denigrates the client or considers them to be abnormal, weak, worthless, or deficient because of negative aspects of their self.
- It is thus essential for the therapist to
- always recognize, appreciate, and praise the positive self-definitions of the client;
- always normalize problem aspects; and
- always emphasize that problem aspects are negligible in relation to available resources.

However, the therapist also makes clear they understand the client wants to work on their problems and that the therapist approves that very much. The client wants to develop, refine themselves, and is not satisfied with their current conditions. This speaks greatly in favor of the client. Further procedures for complementary relationship structuring are based on this general principle.

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2. Therapist Follows the Client's Mission Definition but Will Then Direct Them Toward Therapeutically Relevant Aspects

The therapist must always meet the client at the point where they are currently situated. If they contradict the client, confront them with problems, or explain that the client must define a problem or that the client must agree with the therapist's rules to receive therapeutic help, this will certainly cause immediate resistance on the part of the client. An unpleasant power struggle between therapist and client will be the result. It is therefore mandatory for the therapist to absolutely avoid such a struggle for power. Note: Do not cause reactance! To begin with, the therapist should instead accept the client's therapeutic goal and definitions and then lead the client stepwise to work on therapeutically sensible issues.

If the client, for example, says that they do not have any problems and are going into therapy for their personal development, the therapist will reply saying they think this is an excellent and reasonable objective, but to know how and where to develop oneself first requires knowledge about what makes the client a bit dissatisfied. In this manner, the therapist initializes the analysis of the problem. Nevertheless, the therapist shall not call the problem a "problem." It does not matter what else it is called – for instance, if the client wants to call it "tra la la," then "tra la la" it is. But the therapist will, in fact, carefully continue working as they would do with a normal problem: It will be clarified, explicated, and processed. But always in a way that the client will not lose face (since, officially, this is not a problem).

If the client wishes to be coached, then coached they will be. But the therapist always makes clear that relevant causes must first be identified and understood. Thus, the therapist always resorts to a sort of of judo technique: They use the client's momentum and carefully guide them to follow a constructive path. Never argue against the client. Never provoke reactance. Always create compliance. Direct the client, but do not block them.

3. Therapist "Feeds" the Client as Well as They Can

The therapist to the greatest extent possible substantiates the contents presented by the client: for example, the therapist

- praises the client's efforts and shows themselves to be impressed;
- respects special efforts and shows amazement;
- if the client theorizes, the therapist praises their great reflexivity;
- if the client criticizes the therapist, the client's autonomy and critical faculty is to be praised.

At this point, the difference between complementarity at the motive level and at the game level becomes diffuse: NPD clients often demand validation with respect to their compensatory positive self-schema. As explained in the subsection Game Behavior: Images and Appeals (Section 4.2.4), if in individual cases, there is no objection to validating an image (although there may be), the therapist may validate this image to signal that

- the therapist recognizes and validates the client's efforts,
- the client can be sure the therapist perceives and never forgets these resources,
- the client does not have to fight for the therapist's appreciation – that is, for the therapist, this is a matter of course.

Thus, the therapist may behave complementarily to client images (if such validation will not sabotage other objectives in therapy).

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The therapist follows the rule: Feed, feed, feed! The client has to feel validated, appreciated, respected, and valued. Do not think you could overdo praising the client. This is barely possible, but if it occurs, the client will correct you.

Never miss opportunities to signal the client that you
- think they are competent,
- think they are successful,
- think they are smart and sophisticated.

And with NPD clients, it is usually easy to find resources that can really be appreciated and respected so there is no need to deceive the client.

4. Therapist Emphasizes All Resources of the Client

The therapist emphasizes they think that
- The client has achieved a lot in life.
- The client has done very well in life.
- These efforts are special.
- They deserve a lot of appreciation for that.
- They will gain a lot of appreciation for that, etc.

In this way, the therapist not only makes positive client aspects salient but also responds to the client's need for validation. This validation produces relationship credit, as the client recognizes they are valued rather than devalued by the therapist.

It is also of significance to make existing resources salient to signal "You have already achieved a lot, you are great already, you are respected even now etc."

Accordingly, there is no real need for them to achieve more, prove themselves more, gain even more appreciation. Clients with NPD always look ahead to the future (as Yoda said of Luke Skywalker in *Star Wars: The Empire Strikes Back*: "All his life he has looked away . . . to the future, to the horizon. Never his mind on where he was"), and they are never really aware of the present, of what they have already accomplished. They barely value what they have achieved. The therapist should counteract this.

5. Leaving Room for Self-Presentation

A NPD client who touches on a problem in a therapeutic conversation must point out to the therapist very quickly that there is more about them than just this problem, that they are not deficient but in fact really great. The problem will thus be dealt with only briefly and what follows is a lengthy description of the great things the client has done and can achieve. In this case, the therapist should by all means let the client carry on. The client needs to describe themselves as competent, etc. So, do not interrupt the client. Leave them room! They are in great need of imparting this information. Afterwards, you will induce them to again deal with relevant issues.

6. Creating Compliance

If one wants to work therapeutically with NPD clients, one must create compliance for every aspect of the therapeutic work, for every objective, etc. The client has to agree, must perceive the therapeutic step as sensible, and must be willing to take that step. Working against the client and their reactance will be in vain. Therefore, challenging the client's reactance must

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be avoided to the greatest extent possible. Absolutely avoid power struggles, status struggles, arguments, etc. These will considerably impact the relationship negatively and obstruct the therapeutic work.

Compliance is best achieved by entering the client's system and pointing out to them that

- They actually do not want the costs.
- They do not wish to encounter these effects.
- They certainly wish to be given a thorough therapy.
- It is surely not necessary to explain to them that good solutions need a thorough analysis of the problem, etc.

All objectives and interventions should be attached to the client's system: The client must understand them, accept them, and want them. Thus, an essential aspect of structuring the complementary relationship with narcissists is to establish compliance again and again. If a client is unwilling to accept an objective or procedure, the therapist must put it on hold. The client must never be forced to do something they do not want to do! (At the most, the client can be confronted with the costs that could arise when rejecting certain therapeutic proposals – for example, that the problem may probably remain unsolved; however, the decision about what shall be done must always be left to the client.)

Explicating the Relationship Motives

As NPD clients are usually unable to access their central relationship motives or any other motives, it is an important therapeutic strategy to explicate central relationship motives and other motives and make them transparent. Only if the client accepts that they are in need of appreciation as a person can they perceive that

- Their current strategies are basically hardly successful.
- They are not as effort motivated as they thought.
- There are other central motives that need to be followed.
- There are other authentic modes of behavior for gaining the appreciation of others.
- They have already received a lot more appreciation as a person than they realize.
- They have not duly appreciated this appreciation.

A therapist should, therefore, take every opportunity to raise the client's awareness about aspects such as:

- "Basically, it's very important for you to be appreciated by other people."
- "You wish others would show solidarity and stand by your side."
- "You wish to be able to rely on others."

Explication strategies are not described further here (for more details, see Sachse, 2003).

Clarification

The first phase is about making relevant dysfunctional and compensatory schemas explicit to the extent that the client allows it, and such a clarification should be focused on what can possibly be done and achieved. As soon as NPD clients refrain from impressing the therapist, they often have a good internal perspective and are capable of doing good clarification work – provided the therapist can motivate them to do so.

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Dealing With Tests

The tests adopted by NPD clients are based on the same logic as every other test (see Section 2.6: Tests). The client does not use tests to provoke the therapist or sabotage the therapy, and the therapist must be aware of this. Using a test is part of the client's problem; it is an aspect of the disorder. Basically, the client must establish certain relationship constellations to be able to build up a relationship with the therapist. Thus, the therapist should

- not lecture the client about who determines the rules, and
- not tell the client something like "if you have reached my intellectual level, I will be glad to consider you a colleague," or the like.

It does not make any sense to start a power struggle with the client; this only destroys the therapeutic alliance.

Thus, the therapist should behave complementarily to these tests. If the client says, "You look young, have you ever had a client like me before?" the therapist should keep calm and answer,

> Yes, I have worked with many clients of your age and position. But if you have doubts whether I'm the right therapist for you, I suggest that you try it out; let us have some trial sessions so that you can see whether you're satisfied with me or not.

If clients bring in their own ideas and describe themselves as competent, the therapist will praise them: "I'm glad to work with a client who has so much professional competence. It will make our work much easier." If therapists are criticized, they should deal with it by, for example, saying: "You're unhappy that I can't tell you more about X at the moment. I understand your wish for more precise information, but for [such and such] reasons I can't be more specific now." The therapist is responsive to the client and emphasizes that

- They respect and accept criticism.
- They will not react angrily to criticism and will give it consideration.
- They have good reasons to act exactly like this and not otherwise.
- They would gladly explain these reasons to the client.

In all tests, it is important for therapists

- to stay approachable,
- not to react angrily,
- to respect and accept the criticism,
- to remain sure of themselves and continue with what they think is important, and
- otherwise to comply with the client to the greatest extent possible and integrate the client's ideas into the approach, should this be feasible.

Therapeutic Strategies for Phase 2

Making Game Structure Transparent

General

It is highly important therapeutically for all PDs to make interactional games transparent. Clients can only decide to change their behavior when they know

- what they're doing,
- the consequences of their behavior,

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- that they have caused their behavior and consequences themselves,
- that they are able to change their behavior, and
- that relevant decisions are needed for this.

If the therapist wants their client to change their behavior and gain control of their own system, the therapist must explain what the behavior means and the consequences it may entail. If therapists make these aspects transparent they usually act confrontationally: They work against the client's intentions. The client usually does not want these aspects to be revealed and refrains from dealing with them. If the therapist still tries to induce the client to work on these aspects, relationship credit will certainly be lost. For that reason, therapists should only realize these interventions if enough relationship credit is available.

Confrontation With Costs

A confrontation with system costs is usually the first confrontation, the therapist makes: Basically, the therapist provides elucidation about the costs the client has already identified themselves. Making salient means the therapist

- refers to the client's information about costs,
- elaborates the existing costs,
- explicates hidden or further costs,
- keeps addressing these costs,
- makes the client aware of the fact that their behavior and actions have caused these costs, and
- consistently points out that the client does not want these costs, regards them as too high, and no longer accepts them.

In this case as well, the therapist should anchor their interventions to the client's system. It is the client who specifies the costs, not the therapist. However, the therapist

- increases the client's awareness of the costs,
- establishes a link between objectives and behavior, on the one hand, and costs, on the other.

Therefore, the therapist makes very clear that the client not only has to bear the costs but that they have themselves caused these costs to be incurred – that is, they are responsible for these costs.

Confrontations With Intentions

The therapist confronts the client with regard to their interactional intentions on the game level. For example, they explain that the client

- wants to exert control,
- wants to determine the rules,
- is sensitive to criticism, and
- wants to be appreciated and praised, etc.

The therapist combines these aspects with other aspects and may, for example, bring to the client's attention the fact that they

- want to be successful at any price, and consequently endanger their health;
- want to determine the rules and consequently create social conflicts;
- react to criticism by being offended and that this is a sign of not being self-confident but having self-doubts;
- demand much more from others than they give back, consequently their relationships are

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not reciprocal, and nonreciprocal relationships lead to conflicts;
- demand special rights, but these special rights collide with the beliefs of others;
- are always anxious to invest effort, but in this way, they neglect other interests.

Confrontations should make the client see that their system functions properly; the therapist wants to make sure the client realizes that their
- assumptions and behavior are disputable,
- assumptions and behavior are not as problematic as thought, and
- assumptions and behavior can be treated therapeutically.

In this way, the therapist wants to create an ego-dystony of the disorder, and together with the client, they endeavor to define a therapeutic goal.

Confrontation With Games and Manipulation

Eventually, the therapist confronts the client with the fact that they are manipulative. These confrontations require a large amount of relationship credit. If the therapist has enough relationship credit available and can confront the client in an empathic, normalizing, and not devaluating but sympathetic way, the client will usually be able to accept the confrontation. It is important for the therapist to make costs salient and request from the client a decision as to whether they wish either to keep everything the way it is, or with the help of the therapist, to attempt to change their behavior.

Therapeutic Strategies for Phase 3

Clarification

In this phase, the clarification of schemas is dealt with. The clarification of schemas is a very complex and time-consuming process. During this process, the therapist must give advice and support to the client and direct the process constructively. Based on the so-called reporting level, the client must internalize the perspective, concentrate on schema-determined current processes, and from these processes elaborate the relevant schema contents.

At the end of the clarification process stands a valid reconstruction of schema aspects. Such a reconstruction enables the client to understand their schema and, as has now been clarified, therapeutically work on it, with the help of the therapist.

The client's clarification process as well as the interventions made by therapists that direct the process have been described in sufficient detail elsewhere (see Chapter 3). In this context, reference is only made to the respective publications (see especially Sachse, 2003; Sachse, Fasbender, & Breil, 2009).

Explicating Schemas

A therapist may also explicate schemas and rules. Explication means the therapist addresses implicit contents, expresses contents explicitly, and formulates contents in appropriate terms, without any euphemisms or exaggerations;
- does this in a very accepting manner, without any valuations, just reflecting what the client means (but has not said explicitly); and
- does this very sympathetically, emphasizing that they find it understandable and reasonable from the client's viewpoint; here, the therapist neither confirms the contents nor comments on them in any way.

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Provided such an explication has been successful, it usually is not confrontational for the client. The client does not feel questioned or confronted with unpleasant contents, and for that reason, they will not show reactance. (However, neither will they observe discrepancies. Although explication avoids the negative interaction effects of confrontations, it will not be able to bring about their desired effects.) Thus, explication is not about exerting the effects of confrontations but is rather aimed at disclosing relevant schemas and rules but not yet processing them.

Such a disclosure may be highly successful, especially with NPD clients, if the therapist behaves with acceptance when explicating. In the process, it may be ascertained that the client in fact

- thinks they have special rights,
- feels legitimized to define rules,
- feels offended when others do not make way when they are in a hurry, etc.

When the therapist behaves in this way, it may happen that the client discloses their schemas and rules quite early in the therapy process. The therapist is thus able to quickly establish a model of the client. But the therapist must be cautious and handle disclosed contents carefully and not use such contents against the client.

Biographical Work

Biographical work serves to answer the question of how relevant schemas have arisen in the client's biography. The aim of this procedure is to provide a historical relativization of schemas; the client shall become aware that the schemas were established through specific feedback received from certain people in specific contexts, and that they do not represent "the truth" but instead are highly questionable. This serves to strongly motivate the client to work on these schemas.

Biographical work means for therapist and client to address the following questions:

- In what kind of situations have I been in as a child or adolescent?
- What were the strategies that I had to develop?
- What strategies did I actually develop?

On the one hand, biographical work may deepen the client's understanding of a schema, while, on the other, the client may recognize that the strategies they adopted were important and necessary for them, and were not deliberately chosen but rather had to be developed. For the therapist, it is now important to make clear that the client needed these strategies as a child, but that they are not a child anymore and are now free to decide whether they still intend to employ these strategies or wish to develop new strategies that will cause them to incur fewer costs.

Therapeutic Strategies for Phase 4

The dysfunctional schemas of clients have to be treated therapeutically. However, this is only possible if

- a stable therapist–client relationship has been established,
- the client has defined a therapeutic goal

Schema processing is thus reserved for a later therapy phase (phase 4). OPRP is suitable for the treatment of schemas. The therapist defines two positions for the client, by placing two chairs facing one another, with the therapist sitting at the side: a client position in which the client represents their dysfunctional assumptions, and a "therapist" position in which the client

- questions their assumptions,

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- argues against their assumptions,
- disproves their assumptions, and
- formulates alternative assumptions.

For more details of OPRP, see Section 3.2.4: Therapeutic Processing of Dysfunctional Schemas, in this book, as well as Sachse (1983, 2006c) and Sachse, Püschel, et al. (2008).

Additional Interventions: Trojan Horses

Especially important interventions for NPD clients are so-called **Trojan horses**. These are interventions which induce the client to follow a sensible therapeutic strategy and do not create reactance but rather compliance. The intervention connects to the client's system but then directs the client to perform sensible processing work.

These interventions are called Trojan horses because they do not attack the client's "fortress" directly but rather induce the client to allow the intervention to enter. The client should perceive these interventions as sensible and be motivated (not forced) to follow them. In this context, the interventions must be suitably structured so that they take advantage of the client's assumptions by attaching to them instead of provoking them.

For example, a NPD client complains about two employees. The therapist believes the client is involved in the conflict due to lack of empathy. Therefore, they want to complete an empathy exercise with the client. For this purpose, the client must, however, agree to the exercise and perceive it as sensible. The therapist may, for example, say,

It might well be that your employees are particularly difficult. I can't tell whether this is so, since I don't know them. But if you say so, it must be like that. As you know, what is needed in this case are special strategies for how to handle these employees. To develop such strategies, you must first understand how these employees think. So, we should find out together what these employees want, what objectives they pursue, etc. What do you think they want?

> In this manner, the therapist has attached an empathy exercise; the client is not under the impression they shall perform an exercise because they are deficient,"but rather may experience this exercise as sensible.

Therapeutic Strategies for Phase 5

Even with clients suffering from NPD, the therapist usually must make explicitly sure that a client transfers any therapeutic changes to their everyday life. Therapists may thus agree with clients to do various kinds of homework – for example:

- If the client from now on wants to treat his female partner differently, less manipulatively, and more authentically, what exactly does he want to do then? In what situation does he want to realize a certain behavior? Exactly when does he want to realize it?
- If the client now wants to actively go against and overcome their self-doubts, what alternative cognitions might they develop and use for that? When exactly (when certain thoughts occur) do they want to use such counter cognitions? When do they want to behave in this way?
- If a client decides to terminate a certain rule-setting behavior, they must decide when and where and what kind of alternative is to be adopted – for example, stop becoming angry when a car in front of them goes slowly (e.g., "It only costs me 15 min; there is no need to get angry" or "That a careful driver is overcautious, which is regrettable but can't be

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changed, so I don't care," etc.). In this case as well, what shall be done in a given situation must be agreed with the client.
- If a client does not want to perform in a given situation, they should take active control of that decision – for example, through cognitions such as "I could present a paper at such-and-such a conference but that would be rather time consuming, and I wish to reduce my work load. I do not need this success and I have proven my expertise long before. Therefore, I do not accept the invitation."

It is important for the therapist to point out that
- Clients must fight their schemas actively in everyday life for some time.
- They must be ready for that and endeavor to do it.
- They must prepare for this well, carry it out, and subsequently discuss it with the therapist.
- This constitutes a great challenge the client can and must face.

8.3.2 Therapeutic Strategies for SNPD

Basically, the therapeutic approaches presented in Section 8.3.1 can be adopted on a one-to-one basis for SNPD clients.

From a motivational point of view, the main problem with SNPD clients in particular is their fixation on effortful behavior and the success stimuli associated with it: Through this they generate high personal costs, in the sense of exhaustion, overstressing, unawareness of strain, etc., which lead to serious health costs. Furthermore, they neglect the satisfaction of motives other than the appreciation motive, and they also fail to satisfy even the appreciation motive, because they follow avoidance objectives and because their negative self-concept again and again "shreds" and thus ruins their success. This leads (despite a great deal of success) to a chronic feeling of dissatisfaction.

If one systematically goes through the components of the rapprochement tendency, one can see which components should be weakened and which should be strengthened to create alternative behaviors: behavior and actions that do not primarily focus on effort and success but rather on satisfying other motives, which brings about recovery and relaxation.

8.3.3 Therapeutic Strategies for FNPD

A psychological problem arises for FNPD clients, especially when they are stuck – when they are unable to perform but are also unable to dismiss their previous objectives. In this case, the clients are in conflict, and the solution of this conflict is therapeutically highly significant. Aside from processing their negative self-schema, the client must primarily be freed from the weight of expectation by emphasizing that they will not fail when they make other choices, and by helping them to develop alternative perspectives.

Working on Expectations

To begin with, it is a central therapeutic task to emphasize (through clarification processes and confrontations) that the client feels extremely bound by the current and internalized expectations of relevant others. They try very hard to live up to these expectations and think it would be a disaster to fail, or rid themselves of, these expectations.

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As soon as clients have realized this, they must be induced to work on *questioning these expectations and their being bound by them*. This can be done with the motivation techniques of OPRP (Section 3.2.4; and see also Breil & Sachse, 2009; Sachse, Püschel, et al., 2008). The therapist emphasizes that

- The client, as an adult, need not orientate themselves to these expectations anymore.
- Pursuing these expectations produce enormous costs, especially an enormous heteronomy.
- The client *can* distance themselves from these expectations.
- The client need not orientate themselves to these standards but rather may set their own standards; this also implies that they need not define themselves as a "failure" when refusing to meet these expectations.

This therapeutic work is difficult and troublesome as the clients tend to stick strongly to these expectations and define themselves through them. If they do not meet the expectations, they think of themselves as being failures, a nobody, etc. Consequently, it is not at all easy to get the clients (e.g., when assuming the therapist role in OPRP) to question these assumptions and develop alternatives. However, we do not think there are suitable alternatives. If the client does not get rid of this heteronomy, they not only will be unable to reduce this extreme fixation on performance targets, but will also give up their primary objectives (e.g., completing subject of study X), and consequently, they will remain stuck and unable resolve their conflict. The therapist does not have a choice: The FNPD client's extremely strong commitment to expectations and external standards must be treated therapeutically.

Treating Negative Self-Schemas

The strong desire to compensate for the negative self-schema can only be reduced in SNPD clients by making the negative self-schema itself a therapeutic starting point and processing it.

Treating Alienation

Due to the great desire to fulfil expectations, combined with strong alienation with respect to their own objectives, FNPD clients strongly tend to choose objectives that are not their own, and pursuing them does not bring any intrinsic satisfaction. Typically, FNPD clients study subjects they did not chose themselves They study law because their father wanted them to become a lawyer; they study to become a teacher because their parents wanted them to.

It is therapeutically essential here to start with that high alienation and elaborate what the clients really want:

- Do you really want to study this subject?
- Do you want to study at all?
- Or do you want to do something totally different?

When it has been clarified that they actually want something totally different, therapists should encourage them in this respect and work with them to show the positive consequences of such an alternative choice. (However, it is essential that the clients, through processing of their expectations (1. Working on Expectations, above), are capable of allowing themselves to actually do something else.)

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8.3.4 Therapeutic Strategies for UNPD

As a therapist and especially as a supervisor of many other therapists, the author of this present work (R.S.) has been dealing for many years with the development of therapeutic strategies for UNPD clients, but unfortunately, it must be said that in contrast to outcomes for SNPD clients (where we have achieved very good therapy successes) and FNPD clients (where therapy success is relatively good), these efforts have not been very productive. Despite very intensive therapeutic endeavors, one rarely sees therapeutic progress (only in approximately 25–30% of cases). However, therapists should always try everything possible even in cases where their approaches do not have positive effects.

The aim of therapeutic approaches for UNPD clients is to strengthen their rapprochement tendency toward effortful behavior and reduce their avoidance levels to such an extent that the client at least realizes enough effortful behavior to have both feet on the ground and be independent.

An essential problem already arises with complementary relationship structuring: Because in NPD, complementarity means praising the client for everything that is good and acceptable. However, UNPD clients barely have any success, resources, competences, etc. It is easy for therapists to feed SNPD clients, because they are successful; however, for UNs the "food with which to feed them is in extremely short supply": The client is often jobless, sits at home and plays computer games, and barely has any social contacts. It is very hard to say anything appreciative to such a client, which does not sound totally trivial. The difficulty of behaving complementarily also means the therapist can barely win any relationship credit. In all probability, credit may be obtained when the therapist emphasizes that the client will not be evaluated at all and need not meet any expectations in therapy.

Basically, the negative self-schema of all NPD clients must be processed. To do that, the schema must first be clarified, and for that purpose, the client must face up to the schema. However, for UNPD clients, this is very difficult; because they have no real compensation strategies, they perceive confrontations with schema assumptions as an extreme threat. They strongly avoid this in therapy. Consequently, the schema assumptions cannot be treated. Additionally, the therapist would need a very large amount of relationship credit to confront the client with negative schema contents.

Confrontations: As a rule, clients have to realize and work on their unfavorable strategies. To do so (as has been described in Chapters 3 and 4), confrontational therapeutic strategies are necessary. In our experience, these strategies are unavailable to UNPD clients. Because they have already been confronted with them by their parents or others, they often react hyperaversively to such interventions, the therapist cannot obtain from UNPD clients the amount of relationship credit that would be necessary to make such interventions. It is thus extremely difficult to guide clients who have rule-setter strategies and – without offending and devaluing them – make clear that their strategies are highly harmful to themselves.

Realistic objectives: Basically, UNPD clients can only make progress if they develop for themselves realistic life perspectives which they will need to pursue for a long time. They would then be able to develop competence and find a job they can cope with. They would also then be able to live an autonomous life. However, it is extremely difficult to elaborate realistic objectives with UNPD clients: If the therapist calls the client's attention to this or gives appropriate advice, the client with usually react with irritation.

In our experience, it is best for the therapist to proceed as follows:

1. The therapist accepts the client to a high degree and emphasizes that they will still appreciate the client even if they stay as they are. In this manner, the therapist also

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points out that they want to help the client in case they want to change something but that they do not expect anything from the client! The client does not have to do anything for the therapist.

2. The therapist tries to behave highly complementarily at the motive level and tries to praise the client for everything that is okay.
3. The therapist is very empathic and tries to understand the client's point of view.
4. The therapist carefully tries to explicate the schemas of the client; they clearly signal they think the schemas stem from their biography, are obsolete, and should be questioned.
5. The therapist tries carefully, step by step, to confront the client; the therapist always emphasizes that they only want to highlight given aspects because it might be favorable for the client to question certain aspects and possibly change them.

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Chapter 9

Histrionic Personality Disorder

9.1 Description and Types of HPD

Similar to NPD, we subdivide **histrionic personality disorder** (HPD) as well into two types (Sachse, Fasbender, Breil, & Sachse, 2012): **successful histrionic PD** (SHPD) and **unsuccessful histrionic PD** (UHPD).

As it does for NPD, the DSM essentially addresses the successful type of HPD (see APA, 2013). The following characteristics have been described in the empirical literature:

- High attractiveness, primarily of women with HPD (Bornstein, 1999).
- Having diversified and supportive social surroundings, especially in the case of attractive women suffering from HPD (Bornstein, 1999).
- Self-dramatization and emotional instability (Slavney, 1978) as well as excessive, labile emotional reactions (Millon, 1996).
- Drastic changes in mood (Slavney, Breitner, & Rabins, 1977; Slavney & Rich, 1980).
- High extraversion (Süllwold, 1990).
- A very low tolerance to frustration (Millon, 1996).
- Seeking excitement and being quickly bored (Millon, 1996).
- From 17 to 27 years of age increasing conflicts with partners (Chen et al., 2004).
- Impaired capabilities to assume roles and perspectives, as well as a lack of empathy (Standage, Bilsbury, Jain, & Smith, 1984).
- Lower self-confidence, greater dissatisfaction in marriage, and higher inclination to engage in extramarital affairs, than found in a control group (Apt & Hurlbert, 1994).
- Women with HPD resort more frequently than men with HPD, to overt sexual seduction practices to express histrionic needs in relationships (Lilienfeld, Van Valkenburg, Larntz, & Akiskal, 1986; Stone, 1993).
- Concealed dependency needs (Bornstein, 1998); in contrast, Baker, Capron, and Azorlosa (1996) are of the opinion that HPD clients show higher scores than dependent persons, and comparably higher scores than healthy people with respect to independence and performance orientation.
- In comparison with a control group, indications were found that clients with HPD showed deficits with respect to attentiveness, memory performance, language, abstraction capability, and behavioral planning (Burgess, 1992); a fundamental conviction of patients with HPD is that they are unable to care for themselves (Beck et al., 2004).
- Ascertained with the aid of a special test, the most relevant dimensions of HPD are affect lability, narcissism, and insecure attachment (Bagge & Trull, 2003).

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- There are indications of associations between HPD and higher anxiety, as well as lower avoidance, with youths (Nakash-Eisikovits et al., 2002).

9.1.1 Prevalence and Gender Differences

Prevalence of the HPD in the population has been found to be approximately 2% (2.1% found by Nestadt, Romanowski, Chalel, & Merchant, 1990; and 2% by Grant et al., 2004). With regard to prevalence surveys, the age of the test persons should probably be taken into account. For example, Cohen et al. (1994) found that the prevalence rate of HPD was lower in an older cohort (2.2% in those ≥ 55 years of age) versus a younger age group (4.3% in those < 55 years).

The prevalence rate of 24% in random clinical samples (Blashfield & Davis, 1993) is significantly higher than that found in the general population. Accordingly, Johnson, Bornstein, and Sherman (1996) also found that there were more people with a HPD among ambulant psychiatric patients and inpatients than among college students or persons in adult education ("community" grown-ups). Occasionally, prevalence rates have been found to be significantly even higher: 44% according to Millon and Tringone in 1989 (unpublished raw data) and 63% in Morey (1988).

Whereas Nestadt et al. (1990) found that the prevalence of HPD was equally high in men and women, especially if structured interviews were used as the instrument, and while other studies have also indicate that there are no gender differences with respect to the frequency of HPDs in clinical and nonclinical random samples (Hamburger, Lilienfeld, & Hogben, 1996; Grant et al., 2004), there are other studies that have indicated that women are more often diagnosed with HPD than men (Schotte, Doncker, Maes, Cluydts, & Cosyns, 1993; Zimmermann & Coryell, 1989). This is probably due to the assumption that female behavioral examples of clients with HPD are viewed as better examples of the histrionic criteria and judged to be more representative of the HPD construct than male behavioral examples. Significant differences in the diagnoses made may therefore stem from putting the focus more on the female gender when determining HPD criteria (Sprock, 2000).

9.1.2 Progression

Different results are available regarding the stability of the symptoms from early youth on. Some studies find that the symptoms of a HPD as a rule are most pronounced in the early youth and will recede as time goes on (Bernstein et al., 1993; Johnson et al., 2000). However, results published by Crawford, Cohen, and Brook (2001), on the other hand, show that the typical symptoms exhibited by histrionic male and female clients are very stable over a time period of 18 years (from early youth until early adulthood). Approximately 40% of the sample patients with a HPD even experienced more pronounced symptoms in early adulthood than during adolescence. Seiverwright, Tyrer, and Johnson (2002) observed a significant reduction of histrionic characteristics occurring during the early to middle years of adulthood. The reason for this could be that the sexual seduction strategy diminishes, and patients change over to adopting other manipulative ways of behavior that were not investigated in the study.

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9.1.3 Comorbidity

Regarding HPD, high levels of comorbidity have been found with other disorders:

- Major depression (Corruble, Ginestet, & Guelfi, 1996; Dyck et al., 2001): According to Johnson et al. (2005), an increase of histrionic symptoms in childhood constitutes a long-term risk of developing depression.
- Dysthymia (Pepper et al., 1995): According to Cohen and Cohen (1996), comorbidity with mood disorders amounted to 27–75% of cases of HPD.
- Anxiety disorders (Blashfield & Davis, 1993): As found by Cohen and Cohen (1996), the comorbidity of HPD with anxiety disorders even amounted to 65–92% in three age groups (13, 16, and 22 years). It has also been shown that patients with panic disorder and those with comorbid panic disorder and major depression exhibited significantly higher rates of a comorbid HPD than the control group or patients only affected by major depression (Rost, Akins, Brown, & Smith, 1992).
- Somatization disorders (Reich, 1987; Stern, Murphy, & Bass, 1993): The prevalence rates for HPD are also high with clients affected by somatization disorders. They vary between 7.1% and 81.8% in various studies, presumably depending on the diagnostic instrument applied (Rost, Akins, Brown, & Smith, 1992).
- Dissociative disorder (Boon & Draijer, 1993; Millon, 1994) and substance dependence disorders (22–25%, in Cohen & Cohen, 1996) are also common comorbidities.

As shown by Johnson, Cohen, Kasen, and Brook (2006), long-term studies indicate there is a correlation between histrionic symptoms at a young age and in early adulthood, and the risk of developing an eating disorder, recurring binge-eating disorder, or recurring purging behavior during the middle years of adulthood.

Furthermore, a comorbid HPD may also increase suicide risk in the case of bipolar disorders. According to Garno, Goldberg, Ramirez, and Ritzler (2005), clients with bipolar disorder and comorbid HPD attempt suicide significantly more often than patients suffering from a bipolar disorder in the absence of HPD.

Patients with HPD have also been shown to have significant comorbidities with antisocial PD, NPD, borderline PD, and DPD (Flick, Roy-Byrne, Cowley, Shores, & Dunner, 1993; Johnson & Bornstein, 1992).

9.1.4 Further Characteristics of HPD

SHPD clients are people who like to be the center of attention, who are good at narrating stories, who dramatically amplify stories, and put on a great show. Usually they are well to flashily dressed, heavily made up, often have a highly erotic charisma, and are always dramatic. They greatly stage themselves (*histrion* is the term for a stage actor in classical Rome; see Bornstein, 1999; Millon, 1996; Slavney, 1978; Süllwold, 1990).

People with a HPD are often exaggeratedly emotional. When they are happy, they are excessively happy, and when they are sad, they are deeply grieved. Interaction partners sometimes perceive their emotionality as spurious (see Slavney & Rich, 1980; Slavney et al., 1977).

People with a HPD are very outgoing and extroverted. They easily approach other people, make contact, are sociable, have a huge circle of acquaintances, and do many things with them. They have difficulty being alone, are quickly bored, and have an urge to always be doing something. Only action brings them satisfaction (see Bornstein, 1999; Chen et al., 2004; Lilienfeld et al., 1986; Millon, 1996; Stone, 1993).

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Histrionics primarily want attention from interaction partners. Others shall notice, hear, and see them, and they want to play a central role in the life of others. They often expect from interaction partners to be always noticed, highly respected, cared for etc., and react by being offended if interaction partners behave otherwise.

People with a HPD are also highly manipulative. They use diverse strategies to get attention from interaction partners, induce others to be with them, care for them, etc. They use so-called positive strategies such as flattery, being attractive, being entertaining, being sexy, etc. However, they use negative strategies as well such as complaining, producing symptoms like headaches, dizziness, angina, etc. (see Sachse, 2002, 2004a, 2007a).

If individuals have a histrionic personality style, it can be a substantial resource and source of social competence. Their strategies result in having many contacts, getting a lot of attention, and asserting many wishes in social interactions. Life is exciting, entertaining, and thrilling. However, if a major disorder is concerned, it may get on the nerves of interaction partners after a while. Others turn away, and histrionics achieve the opposite of what they actually want: They are isolated, avoided, get little attention, etc.

The following aspects impede therapy with HPDs:

- *The disorder is highly ego-syntonic*. The clients do not attribute their relationship problems to themselves but to others. Consequently, clients show little to no motivation to change in therapy concerning their histrionic disorder. They want to reduce their (interaction) costs but do not realize that they must change themselves to achieve this. They often state intentions such as "help me so that I can control my partner better!" or simply "wash me – but don't get me wet." Therapist must constructively deal with this low motivation to change and high stabilization motivation.
- *If clients start therapy, they usually do it because of relationship problems.* They have conflicts with their partners, are left by their partners, are bullied, etc. This motivates them to attribute their (current) problems externally and thus hold their interaction partners responsible for the problems.
- *The clients are especially relationship motivated at the beginning of the therapy.* They want therapists to offer a relationship that satisfies their importance-seeking motive; they are only interested in this and in stabilization and are not interested in anything else. Therefore, it makes no sense at all to offer these clients therapeutic measures or even manuals at the beginning of the therapy. If the therapist does this anyway, the client will sabotage any cooperation, often causing annoyance for the inexperienced therapist and resulting in an impairment of the therapeutic relationship.
- *The client involves the therapist in interaction games.* They demand more sessions, longer sessions, extra appointments, private phone numbers, etc., and test the therapist relatively more often. They barely respond to internalizing questions, and consequently relevant schemas are difficult to clarify; they barely deal with therapeutic suggestions at the beginning of the therapy and they do not behave compliantly (they refuse cooperation). All of these behavior patterns make people with HPD difficult clients to cope with appropriately, for inexperienced therapists and therapists without special training.

Due to these different factors, therapists must have special strategies available and procedures to reach and have access to the clients, and be able to work with them constructively.

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9.1.5 Successful and Unsuccessful HPD

There is less research literature and conceptual input for HPDs than for NPDs (see Crawford & Cohen, 2007; Fiedler, 1998; Millon, 1996; Sachse, 2002, 2004a, 2007a, 2007b).

Similar to NPD, HPD can be subdivided based on accurate and detailed therapeutic analyses: We make a distinction between the SHPD and UHPD types (see Sachse, Fasbender, Breil, & Sachse, 2012).

Successful here means something different for histrionics than for narcissists, because histrionics are importance motivated rather than appreciation motivated. With narcissists, *success* primarily refers to effortful success, whereas with histrionics, it means behaving in a manner that one is respected, valued, and noticed and gets attention, etc. (and thus refers to *social success*). Respectively, *unsuccessful* means that this kind of feedback is missing.

SHPD clients use positive as well as negative strategies. Usually they employ a number of positive strategies which they may apply flexibly; if these do not work any more, then negative strategies can be used (with high interactional impact). In this way, they achieve objectives without provoking interaction partners in the long term. As a result, SHPD clients have many friends and acquaintances and always have partners. UHPD clients, on the other hand, mainly use negative strategies. As a result, they often alienate partners and friends and have little success socially in the long run.

The following negative strategies make UHPD clients difficult in interactions:

- They whine and complain.
- They show depressive symptoms in a highly manipulative way.
- They grumble and criticize.
- They show many symptoms.
- They show strong comorbidities with other disorders – for example, somatization.
- They very frequently play interaction games, especially the **poor sucker game** (see the subsection Images, Appeals, and Interaction Games, in Section 9.2.4).

UHPD clients are also very difficult for therapists to deal with, especially those who appear to be "whining depressive." They do not respond to therapeutic proposals ("nothing of that is possible"), they complain ("my whole body hurts, I can't take it anymore"), and send double-bind messages – that is, "Help me, and do it fast and effectively!" and "Leave me alone with your questions and suggestions!"

Inexperienced or untrained therapists often react aversively to UHPD clients. They feel sabotaged, blocked, even that they are being "taken for a ride," and more often than not, get angry or give up. Therapists may quickly face diagnostic problems with UHPD clients, if they let themselves be guided solely by the ICD or DSM: Many criteria of these systems do not even apply to UHPD clients.

In our experience, this often leads to serious misdiagnoses: UHPD clients are sometimes diagnosed as "whining depressed" or "agitated depressed" persons, which may easily give rise to grave therapeutic mistakes. In our opinion, therapists should therefore not rely solely on the ICD or DSM (see Sachse, Sachse, & Fasbender, 2010) but rather make their analyses based on the dual action regulation model.

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9.2 Definition of HPD, Based on Dual Action Regulation Model

The concept adopted in COP with respect to HPD shall be elucidated here (see Sachse, 2002, 2004b, 2007c, 2008b, 2016b; Sachse & Fasbender, 2013; Sachse, Fasbender, Breil, & Sachse, 2012; Sachse & Sachse, 2016b, 2016c, 2016d; Sachse & Schirm, 2015).

9.2.1 Central Relationship Motives

HPD as well is characterized by typical relationship motives that rank high in the motive hierarchy. People with HPD feature have especially three central relationship motives (see also Section 3.2.1: Relationship Formation):

- The **importance motive**: This motive is about playing a central role in the life of other people, having a positive meaning for others, being valuable and an enrichment for others, being taken seriously, being noticed, receiving attention, being heard – and all this without having to do anything for it.
- The **solidarity motive**: The motive of seeking to receive help and support from others and receive protection; sometimes the need that others care for oneself.
- The **dependability motive**: The motive of seeking to have a reliable relationship that lasts, withstands stress, and will not be called off.

Of special significance in this context is the importance motive, which takes priority. Importance is defined by interactional objectives such as

- Getting attention
- Being taken seriously
- Being seen and heard
- Being respected
- Having a certain meaning for others
- Having a sense of belonging

9.2.2 Dysfunctional Schemas

People with HPD feature dysfunctional self-schemas and relationship schemas caused through frustration of central motives in their biography. Dysfunctional schemas contain fundamental, problematic statements about the self or about relationships, and result in persons incurring costs in the present (see Sachse & Fasbender, 2010; Sachse, Fasbender, & Breil, 2009; Sachse, Breil, & Fasbender, 2009).

Self-Schemas

The **most central self-schema of HPD** is "I'm not important." This schema plays an essential role in the HPD processing system. Further crucial self-schemas are

- I'm no source of enrichment for others.
- I don't occupy a role in the life of others.
- I have nothing to offer.

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- I'm boring.
- I'm not interesting.
- I'm not being part of something.

Based on their biographical experience, some clients have developed additional negative schemas, which we describe as **toxicity schemas**. Toxicity schemas give rise to assumptions about being somehow harmful to others as a person – that is, being annoying, a bother, or being someone who impairs them or is really harmful. The clients are under the impression they are not only "unimportant" for others, but they think of themselves as being "negatively unimportant."

Relationship Schemas

Central relationship schemas cause clients to assume they

- don't get attention in relationships;
- are ignored in relationships;
- are not respected in relationships;
- are not taken seriously in relationships;
- are not seen or heard or noticed in relationships.

These self-schemas and relationship schemas make clients feel an aversion to others and to be hypersensitive. They interpret everything and all events as "I'm ignored," "I don't get attention," "I'm not heard," etc., irrespective of whether an interaction partner has really meant it that way. And they overreact or react aversively to everything they perceive in that context.

Another schema is "relationships are not characterized by solidarity." This gives rise to assumptions like

- One cannot rely on partners.
- If one needs help, one won't get it.
- Partners are not there when needed.
- Nobody cares for me.
- I'll be let down.
- Nobody is on my side.

9.2.3 Compensatory Schemas

Compensatory schemas serve to compensate for dysfunctional self-schemas and relationship schemas. Normative schemas basically compensate for dysfunctional self-schemas, whereas rule schemas compensate for dysfunctional relationship schemas (see Sachse, Breil, & Fasbender, 2009).

Compensatory Normative Schemas

Based on their dysfunctional schemas, people with HPD assume their essential relationship motives will not be satisfied "just because" – that is, without any demands or strings attached to them as a person, but that they can only achieve their objectives through actions. Consequently, they establish normative schemas specifying every interactional objective that must

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be achieved in performing actions. Accordingly, they follow the interactional objectives that are specified by the normative schemas but which in actual fact are avoidance objectives. With some objectives such as "Be the most important," it is not immediately obvious that they are in fact avoidance objectives. However, on closer examination, it is evident that the superlative used results from the fact that the schema "I'm not important" can best be falsified by "Be the most important" (not merely important). Therefore, objectives containing superlatives are always avoidance objectives. (However, it depends on the action level, which concrete behavior strategies the client uses, because an objective such as "Be the most important" may be realized by different strategies, see below.)

People with HPD show normative schemas with assumptions such as

- Be the most important!
- Be the center of attention!
- Absolutely try to get attention!
- Never stress or burden others!

There also normative schemas where the avoidance disposition is obvious – that is,

- Avoid being ignored!
- Avoid not being taken seriously, not being noticed, etc.!

Compensatory Rule Schemas

People with HPD develop compensatory rule schemas by means of which they instruct interaction partners how to treat them. Such schemas include, for example:

- Partners (and therapists!) have to give me absolute attention!
- Partners have to make clear that I'm the most important (the only one)!
- I expect absolute attention on a 24/7 basis!
- Others must take me seriously!
- Others must listen to me!
- Others must always be there for me, must support me, must care for me!
- Others must always be fully available for me!
- I expect that the other will stay with me!
- I deserve respect in relationships!

The contingency level of the schemas contains assumptions about the consequences the interaction partner (legitimately) must be aware of, should they not follow the rules.

Conversely, it can be deduced that the client when annoyed about the interaction partner's behavior must at this point necessarily have defined a rule. Should the client be annoyed about the behavior of an interaction partner (or therapist), they usually have established a rule concerning the interaction partner's behavior.

Rule schemas also trigger hyperaversive reactions: If a therapist consults their watch, and the client interprets this as "The therapist doesn't give me enough attention," their dysfunctional schema may be activated, causing the client to quickly become sad or disappointed. Nevertheless, a rule schema may also be activated (even simultaneously with the self-schema). The client may easily become very angry, thinking, "Even you ignore me! I don't even get the attention (that I deserve) from you!" etc.

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9.2.4 Manipulation

Positive and Negative Strategies

People with HPD develop positive and negative manipulative strategies. Positive strategies are those that appear to be positive (at first) for interaction partners but which have no great impact (interaction partners may ignore them). On the other hand, negative strategies are obligatory for interaction partners but may easily provoke them.

People with HPD develop **positive strategies** such as

- being entertaining,
- being interesting,
- being attractive,
- being sexy,
- having an erotic charisma.

And they develop **negative strategies** such as

- producing symptoms,
- exercising control,
- whining and complaining,
- being needy and worse off than others.

Images, Appeals, and Interaction Games

As can be assumed from the term (see the subsection Game Behavior: Images and Appeals, in Section 4.2.4), an image is an illustration of themselves that persons want to create for their interaction partners. An appeal is a request that matches the image addressed to the interaction partner and calls for certain things to be done for the person. Images and appeals appear in typical combinations which are then called **complex interaction games** (for images, appeals, and game structures, see Volume 1 of Sachse, Sachse, & Fasbender, 2010).

The following complex interaction games are realized by clients with HPD:

- *Poor sucker:* I'm badly off, I feel totally unwell, and this is unbearable. I can't help myself. Look, how bad I feel! Don't make it more oppressive for me! Be easy on me! Please, help me!
- *Victim of circumstances or victim of others:* The circumstances are very unfavorable and bad for me! Others treat me badly. That's unfair. I'm not to blame for the trouble. Say that it is unfair! Say that it is not my fault. Exculpate me! Show solidarity with me!
- *Always me:* Negative events always hit me, but it is not my fault! Notice and confirm that I'm badly off and at a disadvantage. Be gentle with me.
- *Dumb game:* I don't know how to do XYZ, I can't do XYZ. But you can do it very well! You do XYZ for me!
- *Sleeping beauty game:* Within the framework of this game, the clients use rather subtle images. They behave passively and do not change or initiate anything. They signal that they can not do anything. The interaction partner is then expected to try even harder, according to the motto: "Cut through the brier hedge and kiss me awake!" Save me! Ask me again!

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Demanding Availability

Popular manipulative strategies the therapist is confronted with early in the therapy concern the concept of "availability" and go back to a rule schema of the kind "Others always have to be there for me, must support and care for me." HPD clients

- call for extra appointments: They need extra appointments because they could be in a crisis, they could feel bad, etc. (extra appointments, evening appointments, appointments at weekends, and so on);
- demand longer sessions: The sessions must be open-ended, must be longer when necessary, take 2 hrs, if required, etc.;
- request phone numbers: The clients must be able to reach the therapist in case they feel bad, so they need the therapist's private phone number, cell phone number, etc. They promise to call only in emergencies, but it would be a great relief for them if they had the number.

Therapists must always keep in mind: The principle of availability is a devil's principle. If you give an inch, they'll take a yard! And the strategy of "I'll just call in emergencies" is a foot-in-the-door technique. If you open the door just a little bit, the client will be inside! Thus, it is essential to apply the strict rule: "*obsta principiis* – resist the beginnings!"

9.2.5 Tests

Relationship tests are part of strategic behavior and result from the fact that the relationship structuring of the therapist establishes hopes on the part of the client to get their long-frustrated motives satisfied, but also activates the belief that their own motives will never be satisfied. Consequently, doubts arise whether the positive relationship messages the therapist presents are honest. To remove their doubts, the clients make use of tests. In therapy with HPD clients, tests are very likely to occur, especially in the first five sessions. Tests are usually meant to attack the therapists in their role as therapist (criticize them, demand other procedures or approaches, etc.).

Direct control is also a popular test tried by HPD clients. In this case, the client uses direct control at the beginning of the therapy (Sessions 1–4). This is not about criticizing the therapist, nor grappling with the therapist. It is intended to test and find out:

- Is the therapist on my side?
- Do they stay friendly and approachable when I attack them?
- Are they willing to accommodate?

The typical test situation for HPD clients is to criticize the therapist – for example,

- You don't understand me!
- You ask questions that worsen my condition!
- You consult your watch!
- You just let me go on in this condition!
- You don't see how badly off I am!

Such criticism will be realized in three different parts of the therapy process:

- during the session,
- after the session, and
- most effectively, before the next session starts.

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The last can be dramatically combined with massive blame:

- You have let me go on in a miserable condition, and I had an awful week!
- I was worried the whole week!
- I had a severe migraine the whole week!

The therapist must never be mistaken here: The client is not interested in criticism, neither in discussing principles or strategies of the therapy. This is all about a test. Clients want to ascertain whether the therapist stays friendly, respectful, and approachable in spite of the massive criticism, and/or they want to induce the therapist to be more attentive, care more, and be there even more for the client.

The therapist should try to pass these tests, which means they should

- Not react aggressively
- Not be offended
- Not react defensively
- Stay approachable
- Praise the client for their honesty
- Ask the client to tell the therapist what makes them unsatisfied

Then the therapist should start addressing the client's central motives: What does the client really want from the therapist? Why and for what purpose do they adopt this behavior? That is, the therapist continues their therapeutic efforts and works therapeutically on the client's test behavior.

9.2.6 Characteristics of HPD

Dramatics

An outstanding feature of HPD is its dramatics. That involves being at the center of attention, getting attention, and inducing others to perform actions. HPD clients use different strategies to put themselves of stage – for example, by acting entertainingly at a party, being sexy, but also by having a "heart attack" or something similar during a party.

Some HPD clients succeed in telling a totally boring story in a way that everybody is carried away; they succeed in drawing attention through their erotic appearance. They may touch their chest dramatically and groan as if to conceal a heart attack but not want to bother anyone with it. All with the desired effect that suddenly everybody cares about them and is attentive.

When HPD clients act dramatically, they do so on all communication channels. They use facial expressions, gestures, posture, and intonation, with a view to emphasizing the contents of their story. In this context, one could call HPD clients a comprehensive "work of art."

They even succeed in exaggerating everything. A relationship is "totally great," their suffering is "totally awful," a situation is "unbearable." Differentiations only impair the drama, and for that reason must not be used (the so-called impressionistic way of talking results from this.)

HPD clients are easily capable of obsessing over all kind of things, and for this reason, panic perfectly qualifies as an approach to controlling interaction partners (thus, panic is a frequently observed comorbidity). Presentations are overemotionalized, exaggerated, and consequently appear overstated and false. HPD clients also control by turning their crying on and off. Especially, the act of turning off crying arbitrarily implies that this behavior is not about real sadness

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but rather about manipulation. When the clients have achieved their objective, they simply turn off their crying. The dramatics behavior pattern is a central characteristic of HPD. On the behavior surface, it may even be the main characteristic.

Costs

Clients with HPD especially incur relationship costs. Partners react angrily, there are arguments, partners are no longer willing to make concessions, and finally interaction partners end the relationship. What is more, the actual, authentic motives of the person are never satisfied, not even when they achieve their interactional objectives at the game level, and normative and rule schemas have been fulfilled. This leads to fundamental dissatisfaction.

Ego-Syntony

The disorder is highly, to very highly, ego-syntonic. The clients are usually confident that they are acting correctly and that the interaction partner is causing the problems. At the begin of therapy, the therapist must therefore expect that the client's motivation to change will be low and that the client will have only a minimal insight into the fact that they themselves may be part of the problem.

Internal Perspective

It is difficult for HPD clients to take an internal perspective. For that reason, it is difficult to initiate any clarification processes for the client when therapy begins. By frequently asking internalizing questions, therapists have to consistently guide the client to adopt an internal perspective. Over time, clients must understand

- that an internal perspective is important for understanding the problem,
- that they should adopt such a perspective, and
- how such a perspective can be adopted.

Avoidance

HPD clients can avoid facing unpleasant aspects of their problem. Avoidance primary relates to two aspects: The unpleasant elements of schemas and aspects of their rule-setter structure and manipulation. With respect to aspects of schemas, avoidance usually is not very distinct, so that it should normally be sufficient to counteract it and consistently address the topic (with regard to coping with avoidance, see Sachse, 2003, 2006b; Sachse, Fasbender, & Sachse, 2011).

For rule-setter structures and manipulation there are basically two therapeutic strategies: confrontations and careful explications. Careful explications are not confrontational and consequently cause less avoidance. The therapist rather emphasizes empathically that from the viewpoint of the client's system, they can comprehend that they have a tendency toward setting rules, making claims, and controlling their interaction partner. The therapist neither validates these aspects nor mentions costs that may be incurred. They merely attempt to empathically comprehend these aspects and emphasize that they understand them (i.e., the therapist stays perfectly neutral). As a rule, this strategy is not confrontational, and clients acquiesce to this strategy if there is enough relationship credit. In this way, the therapist may succeed relatively

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early in the therapy process with working out rule schemas, expectations, demands, and sometimes even manipulative strategies, without the client showing a reaction. This is very favorable for the subsequent therapy process, because many aspects will have already been made transparent and can be used later on.

Alienation

Another essential aspect of HPD clients is their high degree of alienation. They show alienation of their motive system. If persons show high alienation,

- They have no valid representations of what they actually want, what is good for them, and what their implicit motives are. They are unaware of their preferences and consequently do not know which actions and effects would really prove satisfactory for them.
- They have no current access to their motive system, so they cannot determine in a concrete situation what they really want, which needs are essential, and which effects would be good for them, etc.

9.2.7 Successful and Unsuccessful HPD

As has been stated earlier (Section 9.1), it appears to be quite reasonable to differentiate the various subtypes of HPD. At this point, we would like to elaborate in more detail, the distinction we make between successful and unsuccessful HPD (SHPD and UHPD).

Characteristics of SHPD

SHPD clients are persons who have succeeded in receiving full attention through their behavior and get their compensatory objectives fulfilled (which nonetheless causes high interactional costs!).

Clients with SHPD have positive manipulative strategies as well as negative strategies. They are able to use these strategies flexibly as required in a given situation.

To begin with, they are able to use positive strategies very competently to receive attention. For example, such strategies comprise

- being good-looking, attractive, eye-catching;
- being entertaining, telling stories, having a propensity for small talk;
- feeling good, passing a positive mood on to others;
- being sexy, looking enticingly, flirting.

If positive strategies no longer suffice for manipulation purposes, then SHPD clients make use of negative strategies – that is, those which have a high impact. However, they use these cautiously, considerately, and in doses, taking care to not overdo things. Such strategies are especially aimed at producing symptoms such as panic, anxieties, depressions, and migraine.

In this way, SHPD clients are capable of

- covering a wide range of strategies with relatively good chances of achieving their goals;
- using strategies in a way that interaction partners may only be amused and rarely annoyed by. In this way, SHPD clients avoid having to face negative interaction consequences for quite a long time;

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- having reserve strategies of high impact up their sleeves should the positive strategies fail.

All this makes clients quite successful interactionally: They have many friends and acquaintances, are appreciated, and welcomed, and in this way, they receive many signals of importance (nonetheless, this will not satisfy their importance motive).

Characteristics of UHPD

UHPD clients are people who do not, or no longer, succeed in receiving attention and (despite great effort) cannot satisfy their compensatory objectives.

In contrast to SHPD clients, UHPD clients only (or mainly) use negative strategies.

UHPD clients have available only a very few positive strategies or are unable to use them skillfully; they mainly use negative strategies – that is,

- whining,
- complaining,
- nagging,
- controlling,
- producing symptoms, etc.

They do this in such a way that sooner or later they will annoy their interaction partners. As a consequence, they are often deserted, avoided by friends, and encounter similar experiences. However, this will not bring them to reconsider their strategies, rather they understand it as an affirmation of their schemas, which results in even more moaning and complaining.

Typically, UHPD clients show the following characteristics:

- They mainly adopt negative interaction strategies in everyday life as well as in therapy.
- They have often been deserted by their partners.
- They have only a few social contacts: Friends have terminated their relationship, their children are avoiding them, etc.
- They do not, however, reflect on the situation and are totally unaware that they might be part of the problem.
- They often bring about self-fulfilling prophecies: They alienate interaction partners through their behavior, attribute these effects to their "unimportance," and consistently confirm their negative schemas with this.
- They use this to moan and complain even more, which consequently aggravates the problem even more.
- In a relationship, they appear to be very demanding and controlling. Everything has to be about them, and they frequently break the reciprocity rule.

UHPD clients often appear to be "moaning depressives:" They seem to be depressive because they are lamenting and complaining all the time, see everything as negative, show many symptoms, etc. They are active, highly dramatic, do not accept therapeutic interventions, and at the same time they exert a great deal of pressure on the therapist, demanding to be helped quickly and effectively.

This causes many therapists to be very frustrated – that is, by the moaning, the tendency to disregard proposals for change, refusing to work constructively, and claiming to be unable to

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face the problem or take action, "because it's no use doing something anyway." Therapists often consider this behavior to be an act of sabotage.

Therapists react especially aversively to the client's massive double-binding behavior. The clients send an obvious double-bind message which brings therapist into trouble:

- Help me (save me) and do it quickly and effectively!
- Would you please leave me alone and not worsen my condition!

All interventions that would clarify or analyze the problem fall within this category. From the client's point of view, these interventions force them to face their problems and consequently worsen their condition. Therapists who do not understand the functionality of UHPD clients consequently become at first helpless and then angry.

It is obvious that classical behavioral therapy approaches inducing the client to approach a problem directly and to quickly find solutions are not only not indicated but even contraindicated. They will most certainly cause massive problems in the therapy process.

9.3 Therapeutic Strategies for HPD in General

9.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

In the case of HPD clients, complementary behavior at the motive level is essential. When behaving correctly, the therapist may obtain relationship credit relatively easily from HPD clients and be able to use it to establish a trustful therapist–client relationship (see Sachse, 2000b, 2001a, 2006d, 2010; Sachse, Sachse, & Fasbender, 2010). Complementary behavior, for example, means:

- Giving special attention to the client, listening attentively, emphasizing that one is interested in what the client says.
- Taking the client seriously: not confronting the client at the beginning, accommodating, and taking contents seriously. If the client demonstrates suffering, the therapist takes it seriously and understands it. One may say: An HPD client will be unwilling to work with the therapist until they are convinced the therapist understands and accepts the extent of their suffering and woes.
- Emphasizing that it is important for the therapist to work with the client, exactly understand the client, and be there for the client (within the framework of the therapy).
- Understanding and accepting the client's statements; not devaluing the client, not trivializing content.
- Avoiding everything that could be understood as a sign of disregard: not being late, not forgetting important contents, not consulting one's watch ostentatiously, not being distracted by the client, etc.

Achieving complementarity with HPD clients often means to be very active as therapist to signal the client that the therapist is interested and willing to understand. This is especially important for clients who talk fast and a lot. It might be necessary to interpose short statements of understanding without stopping the client's flood of words. Sometimes clients do not react at all to therapeutic statements at the beginning of therapy. It is important for the therapist to not give up from frustration but rather to continue verbalizing actively, etc. In our experience, the positive effect of complementary behavior only shows after a while.

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Therapists should understand that complementarity also means triggering the client's schemas as little as possible. Because if you trigger schemas, the client will take away relationship credit. So that the therapist should try not to do anything that could trigger dysfunctional or compensatory schemas.

However, a therapist should also know that this cannot be avoided completely. If the client's schemas are known, unpleasant surprises can be circumvented; however, as there are a great many of them, a schema may be activated anyway at some point. If this happens, it is important to apologize and to explain right away what was really meant. It is okay if this happens once in a while, and this will not impair the relationship, provided the therapist behaves appropriately. However, it should not happen too often.

Some problems can be alleviated a priori. As clients usually react aversively if a therapist looks at a clock during the session (and seems to allow their attention to be diverted away from the client), the therapist may try to place the clock in a more convenient location where it may be consulted inconspicuously.

Additionally, the therapist may also send explicit relationship messages to the client within the framework of the complementary relationship structuring. In this way, therapists directly tell the clients that they want to satisfy their motives. Appropriate messages for HPD clients are:

- For the **importance motive**:
  It is important for me to understand you exactly.
  I take it very seriously, or I take you very seriously.
  You are very important to me.
  I'm interested how you feel.
- For the **solidarity motive**:
  I would like to support you.
  I'm here for you, if you feel bad.
  It is important for me to support you.
- For the **dependability motive**:
  I stand by you.

Explicating Relationship Motives

We assume that people, to a varying degree, have central relationship motives that strongly influence their behavior in relationships. We also assume that many people with PDs do not know their central relationship motives very well or do not know them at all, and we think knowing them is important for clients to be able to again establish authentic behavior (and consequently reduce their manipulative behavior) (see Sachse, Sachse, & Fasbender, 2010).

When working with HPD clients, it is essential to start with an explication of relationship motives early in the therapy process. Clients often do not know their central motives, but they should know them. However, the main purpose of explication is that the therapist obtains a great deal of relationship credit for it. The explication of central relationship motives is almost never confrontational; quite the contrary! The client will feel deeply understood, valued, and attended to by it. The therapist emphasizes that they understand the client, and that builds up trust.

Explication often has another important effect. The clients are expelled from their game level for a short time and interact authentically with the therapist. They are even able to work

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constructively with the therapist (to the extent it is possible for them). This means, the clients open up a "processing window" through which one can work well for a while, but only for a while and until the client closes the window again. Nonetheless, the therapist establishes contact, builds an authentic relationship, and shows the client how therapy can work.

No Complementarity at the Game Level

While a therapist may in a way behave complementarily at the game level when treating narcissists, this is not possible in the case of HPD clients, as they try to induce the therapist to behave nontherapeutically. Complementary behavior at the game level with a HPD almost always means the therapist allows themselves to be functionalized and thus stabilizes the client's dysfunctional system. It is, therefore, important for the therapist to behave complementarily within the framework of the therapeutic rules.

Complementarity always means that the therapist behaves complementarily within the framework of the therapeutic rules. These rules are imposed by the therapist, if necessary substantiated, but never discussed, and they must by no means be violated.

As has been said, HPD clients make demands as to availability, and they do it according to the "devil's principle." The therapist must *never* behave complementarily to this manipulative behavior of the client, otherwise they will be completely functionalized for the client's dysfunctional system. Therefore, the therapist must never bargain away the therapeutic rules, and that means:

- A session lasts 50 min and will not be extended.
- There are no extra appointments.
- The client will not be given the therapist's private phone number.
- The therapist is not available for crisis interventions.

If clients cannot accept these terms, they cannot be treated ambulantly in therapy but instead must be hospitalized.

For example, the client asks: "Why can't I have your private phone number?"

Therapist: "You need to make yourself independent of the therapist. That is an objective we want to achieve. If you call me between sessions, chances are you might become heavily dependent on what I say, and this would surely contradict our therapeutic objectives."

The therapist states in a friendly but nevertheless clear way that the rules are not negotiable. The client accepts the rules, otherwise there will be no therapy. The therapist may clarify why these conditions are so important for the client and how important it is for the client that the therapist demands the rules to be observed. The therapist may behave highly complementarily while clarifying this. Still, on the rule level, the therapist will never make concessions.

As said earlier (see section 6.3.2: Complementary Relationship), it is better not to be complementary regarding a client's game level. However, therapists are confronted with the images and appeals of the client and must ask themselves how to deal with them. Here we suggest carrying out an extensive analysis of the images and appeals and finding out for every aspect whether the image can be affirmed or the appeal followed without it being detrimental to the therapy.

If the images and appeals can be affirmed or followed without impairing the therapy, this should preferably be done. This is, for example, to be recommended in case of positive

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strategies. The therapist may then make compliments and appreciate the exceptional case. With regard to other images, however, it may be unfavorable to affirm the image or to follow the appeal. In this case, the therapist can verbalize the client's impressions. This may, for example, be the case when clients emphasize that they are badly treated. Assuming the therapist affirms this, they would implicitly say that others and not the client are to blame for the client's problems. Consequently, the client would not think it necessary to work on themselves, and therapy would not make sense. Instead of affirming the image, the therapist may, for example, say, "You feel badly treated," "You think everybody treats you badly." For many clients, this will be sufficient. However, there are a few clients who demand that the therapist follows up by responding to the appeal: "Affirm me and show solidarity with me!" ("What is your opinion? That's unfair, isn't it?"); or they will insist that it is not just an impression but rather a reality ("It's not my feeling, it is true!"). Then the therapist must make clear: "As I haven't been there, I can't say whether it's real. But this doesn't make a difference since all that interests me is how you experience things." Some clients may persistently state that the therapist actually must have an opinion. In this case therapists should point out it would be unfavorable for the client if they expressed an opinion, and that the client can be helped best by a neutral and empathic therapist.

If the therapist handles most of the images and appeals in the above-described way, behaves complementarily, and sends explicit relationship messages, they may ignore certain images. This may, for instance, be the case if clients on account of their PD behave dysfunctionally to their children but at the same time insist that they are an excellent father or mother.

Dealing With Tests

HPD clients may also test the therapist (see Section 2.6: Tests): They criticize the therapist to see how they will react. If the therapist stays approachable, they will have passed the test, and the client (with a bit more certainty) will know that the therapist continues to act respectfully even in critical situations. If the therapist reacts aggressively and deprecatingly, the client will know the therapist will not stay respectful, in which case the test has been failed. Although the client has lost the relationship, they have definitely attained clarity!

It is important for the therapist to not react to these tests of HPD clients that seemingly are meant to criticize the therapist – that is, not to deal with the arguments and objections the client raises. The therapist should rather focus on the client's motives and schemas and attempt to work with the client on issues regarding what they really want and why the client uses such behavior.

Clients with HPD have a strong importance motive: They want an interaction partner (e.g., therapist) to take them seriously, be attentive, show respect, listen carefully – that is, clients expect from therapists that they signal that the client is important to them. This is the client's hope for the therapeutic alliance. But due to their negative schemas, the client at the same time fears that they will not be respected, will not receive attention, and will fail to be important to others, and that all this also applies to their relationship with the therapist.

If the client now meets (in the first therapy sessions) a therapist who behaves highly accepting, respectfully, and empathically, this will trigger both the client's relationship motive of seeking importance and the negative schemas. The client will then be in a dilemma and will not know whether they should follow their motive and let themselves be involved in a relationship, or whether they should listen to the schemas and avoid the relationship. They will be in doubt and distrustful and not believe that the therapist is really serious about their accepting behavior.

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This uncertainty on the part of the client can be eliminated by testing the therapist. If the client creates a difficult interaction situation for the therapist in which they might react unsurely, with anger or annoyance, such a situation will be a good test scenario for the client. Because if the therapist continues to be approachable and respectful even in such a difficult situation, then it may be assumed quite safely that the therapist really means it.

Criticism is ideal for such test situations: The therapist can be massively criticized to see how they react. In this way, the therapist is blamed for not understanding the client, worsening the client's condition through their interventions, acting irresponsibly, etc. Certainly, it is not really about criticism, because in actual fact there is nothing the therapist is to be blamed for, and contents shall by no means be discussed. Neither shall the therapist be made angry or offended. It is all about performing a test, getting information about the therapist, finding out whether the therapist remains approachable and respectful.

Accordingly, the therapist will fail the test if they

- react angrily, aggressively, or deprecatingly,
- discuss contents or justify themselves for their behavior.

However, the therapist must not behave complementarily at the game level or comply with any demands – including those that are not therapy related – by granting special rights or extra appointments or the like.

The correct therapeutic strategy is complex:

- The therapist must take the client's criticism seriously and emphasize they are aware they are being criticized, but that they will not evade the criticism but rather counteract it.
- However, the therapist cannot admit to having made any mistakes they have not made; what they can do is to say they understand their behavior has affected the client in a certain way, but they must not accept they have acted incorrectly.
- In this situation, the therapist must take the client very seriously, be highly attentive, and send many importance signals to let the client know their motive will be satisfied.
- What is most difficult in the therapy process is that the therapist must attempt to get the client to realize that what they experience and do with the therapist is something they actually experience and do with many people.
- That means, the therapist must get the client to believe that their behavior is related to and stems from their own, old schema.

9.3.2 Therapeutic Strategies in Phase 2

Establishing Motivation to Change

Confrontational Interventions

Clients with HPD usually start therapy without any motivation to change concerning their interaction difficulties. Thus, the clients are often unaware of the fact that

1. They have unrealistically high expectations of and demands on their interaction partners.
2. They try to enforce these expectations through nontransparent, manipulative strategies and punish interaction partners if they do not comply with these intentions and expectations.

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3. There are negative, long-term consequences (costs) for them.
4. They generate these costs through their behavior.

To be able to process the interaction disorder in a motivated manner, it is important for the client to understand these aspects and relations and develop a representation of them. However, this is only possible if an empathic, collaborative, and nonjudgmental therapist carefully points this out.

It has to be borne in mind that making the game structure of HPD clients transparent is often very confrontational. However, at some point in the therapy process, the therapist must make the client's games transparent, because if the client is unaware of what they are doing and of the consequences of their behavior, they cannot decide to change their behavior and will not do so.

If the therapist makes confrontations systematically in the therapy process, they will notice the process passes through different phases:

- In the first phase, clients react strongly to confrontations, culminating in interactional crises the therapist has to deal with; the therapist must follow up every crisis by allowing for a longer phase of complementary relationship structuring so that their relationship credit can again be replenished.
- In the second phase, clients react less sharply, because they realize the therapist has not meant the confrontation to be offensive but rather wanted the client to deal with respective aspects. Nevertheless, a reaction is still triggered, and the amount of relationship credit is reduced (albeit to a lesser degree).
- In the third phase, clients have completely understood the purpose of the confrontation and are now convinced the therapist did not want to be offensive. A confrontation no longer means a loss of relationship credit, and the therapist may now confront the client with any aspects of their PD without it being detrimental to the relationship. The therapist is now in a position to work constructively with the client.

At the beginning of therapy, confrontations may even entail relationship crises; the client may think the therapist is trying to impute something to them or wants to disparage them, etc. It is important for the therapist to

- keep calm and not take offense;
- react highly complementarily, be highly attentive, treat the client respectfully, and make clear they are willing to enter into disputes with the client;
- try to clarify what exactly upsets the client;
- emphasize that they are under a certain impression, they might be wrong though, but for now they will not waive their impression;
- refrain from forcing the client to share their viewpoint;
- let it rest if the client puts up resistance.

However, if the therapist gets the same impression again after a short time, they again confront the client by

- always maintaining an accepting behavior;
- confronting empathically, not reproachfully;
- only emphasizing their own impression;
- always allowing objections to be raised;
- explaining why they make certain things transparent.

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Therapists should not be afraid of interactional crises: If a therapist confronts the client, which eventually they *must*, such crises will in all probability be encountered when treating HPD clients. If the therapist avoids these crises, they avoid confrontations and as a consequence the client will be unable to perceive and recognize important aspects of their problem. It is highly nontherapeutic behavior to dispense with confrontations. Thus, therapists should think about their own avoidance behavior and, if necessary, work on it therapeutically.

In this phase, the therapist pursues the objective of bringing the client to realize and understand the relevant aspects of their game level. In accordance with the dual action regulation model, therapists may confront clients with the three game level aspects:

- compensatory schemas (i.e., interactional objectives, expectations, intentions),
- manipulative strategies, and
- costs.

Additionally, it is very important to confront the client with their rule-setter behavior and the respective costs.

Clients must also realize that they want to receive attention and that they want it everywhere and from anybody at any time. Clients should also be clear about and aware of other interactional objectives – that is, that they wish

- to be the most important,
- to associate with others,
- to be heard,
- to have adventures.

Furthermore, they should also accept their feeling of being entitled to have their interactional objectives satisfied (and that they angrily punish others when they think this right is taken away from them). In this context, the client should also realize how arduous their demands are for their interaction partners, since these often imply that partners must neglect their own needs (like hobbies and friends) in favor of the needs of the client. Interaction partners may thus be induced to act explicitly against otherwise important family members and completely renounce their own needs by fulfilling the demands of the client. It may thus be said that HPD clients implicitly expect the complete subordination of their partners.

Essential aspects of confrontation involve addressing games and manipulations. The client must realize

- they are behaving in a nontransparent way,
- they act strategically,
- which strategies they use, and
- how others react to these strategies.

For example: the therapist may confront a client with symptom productions: “I noticed you always get a migraine when your husband wants something from you. One might think you do this to block your husband.” And “You do this, as I noticed, indirectly. You don’t say: ‘Leave me alone’ but instead ‘I have migraine.’” And “That makes you nontransparent for your husband. Your husband is forced to react, but he actually doesn’t know why.”

The therapist may confront the client by focusing on details the client reports, but the therapist may also confront them with what they do with the therapist. For example, if the therapist asks a question, and the client immediately starts crying, and if the therapist regards this as direct control, they might say, “I think you cry, to tell me I should never ask you this kind of question again.”

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The client may also be confronted with the fact that they hand over responsibility to others: "Please correct me if I'm wrong, but I get the idea you want me to solve your problem, that I should take responsibility and tell you what to do."

The therapist makes the costs of the system salient to make it very clear to the client that the system is not completely free from faults. Clients must realize they do not get what actually satisfies them in relationships, they rather make interaction partners angry, ruin relationships, suffer from a bad condition, and will not get better. What they are doing is exhausting and often not successful anymore.

Also important here is that the therapist creates associations between the client's behavior and the resulting costs, so that the client is able to perceive and understand they have caused these costs themselves.

Procedures for Confrontations

The order according to which the client is confronted with the three aspects of their situation (interactional objectives, strategies, and costs) is not fixed. Basically, it is advisable to confront the client with what has been offered up to that time. Usually, a confrontation with costs is most easily accepted by clients (in any case where it is not yet clear to them that they are causing these costs themselves).

However, there are clients who take their demands on interaction partners for granted, so that this is no longer confrontational for them at all. (It gets difficult for these clients, however, when the therapist contradicts the right asserted by the clients to substantiate such demands). In such a case, a confrontation with relevant expectations may occur quite early.

The fact that the therapist has three different aspects to confront the client with is advantageous in that it is possible to change between these aspects during the confrontation process. Usually, a confrontation with a single aspect – for example, expectations – can only be done up to a certain point when the client starts being evasive as the situation gets uncomfortable. However, this may give rise to the emergence of another aspect – for example, a cost factor – that the therapist can deal with. This confrontation may then work for a while as well, until the client again starts being evasive. In this case, the name of the game is to be flexible, and do not struggle for power.

Another important principle of confrontations is that the therapist must proceed in small steps. It is usually impractical to overwhelm the client by illustrating their entire system at one go, such as "Your relationship develops so badly because you demand from your partner that they care for you on a 24/7 basis and you try to enforce this with your lamentations and 'I don't feel well' strategies." An HPD person will react at best with incomprehension to such interventionsually even angrily. It is important to confront the client little by little with aspects that are relatively close to their current view of the problem – that is, present it as their own impression and establish whether the client accepts this step or not. Only if they accept the step, may the next, maybe harder confrontation take place.

The last and important aspect always requires that the client must perceive and understand that the therapist has no intention to use confrontations because they negatively value the client's behavior but rather because the therapist thinks the client suffers from certain aspects of their situation. The therapist thus offers their insight to show that the client is important them. This implies that aside from confrontational interventions, the therapist must also make relationship-promoting interventions.

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9.3.3 Therapeutic Strategies in Phase 3

Clarification Process

In Phase 3, the therapist should work with the client on clarification of relevant dysfunctional and compensatory schemas. Such clarification serves to deliberately represent these schemas cognitively for creating the conditions required for therapeutic schema processing (Phase 4). As clients' clarification processes and the therapeutic interventions made to direct the clarification process have been described in detail elsewhere (see Chapter 3), mention is made here only of relevant literature: For a closer look, see Sachse (2003, 2005, 2006e, 2008b), as well as Sachse and colleagues (Sachse & Breil, 2011; Sachse, Breil, & Fasbender, 2009; Sachse & Fasbender, 2010; Sachse, Fasbender, & Breil, 2009; Sachse & Sachse, 2011; Sachse & Takens, 2003).

Clarification processes are difficult when dealing with HPD clients who often have difficulty in assuming an internal perspective and in working out what is going on inside themselves. Thus, therapists must practice and train clarification processes systematically – that is, they must

- consistently ask deepening questions aiming at clarification of cognitions, emotions, and aspects of schemas, even if they know the client is unable to answer them at that time: By doing so, the therapist sets to set markers showing the client what to do;
- explain to the client what to do and also explain why this procedure is useful;
- emphasize through well-phrased explanations that they understand what is going on inside the client, and offer the client orientation for the understanding of their feelings.

Explication by the Therapist

Therapeutic explications especially are very helpful for the client – although they are difficult for the therapist since they are not allowed to speculate but must rather understand verifiably. As HPD clients have problems in assuming an internal perspective, the therapist can support the clarification process by making therapeutic explications. The therapist translates and states in other words what they (verifiably!) understand from the client and in doing so may go beyond what the client has said explicitly. If the client is able to understand the explication, this will bring the clarification process forward to a high degree. For a closer look, see Sachse and Sachse (2011), as well as Sachse, Fasbender, and Breil (2009).

Biographical Work

Biographical work mainly serves to show the client that they have not chosen their structures (i.e., schematas, motives, manipulations, etc.) voluntarily and that they are only responsible for it to some extent. Especially for HPD clients, it is important that they understand that their manipulative strategies once had an important function in their biography, but that they now experience more costs than benefits. They should perceive and understand that they experienced much less affection from their parents than they actually needed, and that they had no choice but to develop strategies to get attention. Sometimes HPD clients tend to blame themselves for having ruined their lives when they realize they behave in a manipulative manner and are partly responsible for failures in a relationship. It is essential to emphasize that

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- the client is not to blame for their structure,
- the client is now able to change, and
- it is better to change things now than let things continue as they are.

9.3.4 Therapeutic Strategies in Phase 4

With HPD clients, central schemas have to be processed therapeutically as well, and again, this is only possible when clients have developed a therapeutic goal and their schemas have been clarified. Especially the "I'm not important" schema must be processed – for example, via OPRP (see Section 3.2.4: Therapeutic Processing of Dysfunctional Schemas). Usually clients have experiences of being regarded as important, but they have not yet come to terms with them. In the therapist position of the OPRP, the client therefore systematically searches for persons and situations where

- they experienced being important;
- they belonged to a group, team, etc.;
- they were taken seriously, noticed, and respected;
- they played an important role in the life of others.

This serves to systematically review and reconstruct the schema. For a more detailed description of this method, see Sachse, Püschel, Fasbender, and Breil (2008; see Breil & Sachse, 2009; Sachse, 1983, 2006f; Sachse, Fasbender, Breil, & Sachse, 2011).

9.3.5 Therapeutic Strategies in Phase 5

During this phase, therapist and client work out how the client can adequately present their needs to interaction partners and how they can deal with frustration. Sometimes in this phase, it is necessary to dispute and analyze the manipulative strategies again. Clients need a precise representation of their previous, dysfunctional behavior. When this has been achieved, alternative actions can be discussed, elaborated, and then practiced in a role play. The clients test these strategies in everyday life, with the results being evaluated together.

9.4 Processing Alienation

As HPD clients show a high degree of alienation, it is essential to perform alienation training with them (see Section 6.5.2: Dealing With Alienation).

9.5 Therapeutic Strategies for UHPD

Problems with UHPD clients are especially encountered in Phase 1 of therapy. The clients start therapy and are totally "ravenous" with respect to their central importance motive and their normative schema objectives. They want

- to receive attention;
- someone to tell them they are important;

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- someone to care about them;
- someone to be interested in them, want to listen to them, and spend time with them.

However, due to their schemas, they are led to believe no one will satisfy these needs unless they use strategies to induce interaction partners to comply. As a consequence, they adopt highly manipulative strategies also directed toward the therapist.

The specific problem in therapy is that virtually all of these strategies are negative:

- The clients complain and lament.
- They present themselves as suffering and helpless.
- They keep realizing constructs of unsolvableness and inevitability.
- They send strongly worded appeals, such as help me, care for me, rescue me, and release me.
- They also make appeals such as "Don't demand anything from me," "Don't burden me," and "I can't do it. "

It is obvious that it makes no sense then to clarify any of these contents with the client – this only leads to an aggravation of the situation and more lamentations – or to work on solutions with the client – this only leads to increased complaining and a massive sense of helplessness.

The therapist should therefore forget about any therapeutic exploration of behavior, programs, manuals, solutions etc., because they cannot expect that the client will adjust to the therapist's demands. Instead, it is vital for the therapist to meet the client in their present state of mind.

Moreover, the therapist must have a lot of patience and, as the case may be, must practice with endurance a comprehensive complementary relationship structuring without being able to pursue any objectives regarding aspects of the disorder. The therapist should not want to achieve anything for the client, should not want them to change, should not want to rescue them, etc. They should instead only offer a relationship and try to obtain relationship credit.

The only helpful therapeutic measure at this point is to drastically promote relationship structuring and send a dual therapeutic message by means of which the client's dual message can be intercepted.

The therapists must behave complementarily most stringently – that is, they

- must attentively listen to the client and give them their undivided attention (even, or especially, during the client's lamentations);
- must be maximally approachable, mentally and in terms of body language (sitting face to face, not defensively);
- should themselves remain relaxed, not take part in catastrophic situations or scenarios, not stay tuned in to this aspect, so that the client may perceive that their constructs are not confirmed verbally or otherwise by the therapist;
- should try to understand what the client really means – an understanding based on the client's point of view. The therapist should never affirm the client's constructs. The therapist emphasizes that they understand the constructs when (and only when) assuming the client's viewpoint;
- should emphasize that they understand (from the client's point of view)
  - how extremely miserable the client feels,
  - how extremely helpless the client feels,
  - how desperate the situation appears to be,
  - how strongly the situation burdens the client;
- not question these aspects, not behave confrontationally;

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- not exert pressure on the client, refrain from saying the client must change, contribute to therapy, or answer questions;
- allow the client to remain as they are; the client is allowed to have problems, suffer, and complain;
- signal that they are staying approachable, will not quit the relationship, stay respectful, and will not be angry;
- but signal as well that they will not let themselves be manipulated and will be strictly compliant with the therapeutic rules.

However, this is only one part of the dialectic therapeutic procedure: The client must realize that the therapist is and remains approachable, is and remains respectful, that they are there for the client (within the framework of the therapeutic rules), etc.

Aside from this, the therapist should also make sure that another issue is observed, one that offers to the client a different and new kind of information and makes them see that things may also be perceived, constructed, and be dealt with in another way. This therapeutic step is necessary to slowly and carefully bring the client to part with their ingrained ways of thinking ("I'm helpless, and others must rescue me").

For this to happen, the therapist offers the client alternatives to be considered; however, they do this in a way that ensures the client will not perceive it as an instruction, pressure, criticism, or abasement. In this way, the therapist emphasizes that they

- understand that the client feels miserable, but that they worry about the client and do not want the client to keep feeling that way;
- understand that the client does not see any solution at the moment, but that they think finding a solution is necessary for the client to put an end to their sorrows;
- understand that the client presently lacks confidence to do something, but that they have trust in the client's capabilities and resources to do this.

On the other hand, the therapist also emphasizes that they

- are worried, but it will always be up to the client to make a decision;
- think it is reasonable for the client to think about the problem, solution, etc., but that it is always the client who makes the relevant determinations;
- think the client (with the therapist's help) can find a way, but that the client must really want it themselves.

In this way, the therapist gives incentives but does not exert pressure. They offer alternatives without questioning the client, and they emphasize that they only do this because they care about the client.

The therapist thus sends a dual message:

- You, the client, may remain as you are. I'm prepared to offer you a relationship and let you be as you wish.
- I, the therapist, believe that you, the client, could feel better if you tentatively agreed to my proposals.

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Chapter 10

Dependent Personality Disorder

10.1 Description of DPD

DPD is a proximity disorder: DPD clients are strenuously looking for proximity and nearness and relationships; they do a lot to have and maintain a relationship. The same as other PDs, DPD can also be viewed as a continuum that may range from a mild personality style to a serious disorder (see Widiger & Frances, 1987; Widiger & Sanderson, 1987; Wiggins & Pincus, 1989). DPD includes an aspect that causes the client to incur costs and an aspect that implies that the client has resources to draw on (see Bornstein, 1998; Sachse, Sachse & Fasbender, 2010).

10.1.1 Therapeutic Problems

Dependent personality disorder (DPD) is less frequently found in ambulant psychotherapeutic work than NPD or HPD; however, it still plays a significant role. Clients with DPD are highly demanding of therapists and not at all easy to treat. Due to their disorder, they often seem to be easy handle at first: They seem to cooperate, strive to contribute, follow suggestions, etc. Later on, however, it becomes clear that their motivation to change is actually poor, that they have trouble perceiving themselves as part of the problem. They also find it difficult to internalize their perspective, and their access to their own motive system is impaired. At second glance, they prove to be difficult clients who pass on a high degree of responsibility for the process and for their own changes to the therapist, and who are very persistent.

At first, it seems that these clients show few interaction problems and interaction games in the therapy process. However, after a while it turns out that they just disguise their games more effectively than other clients. They are not less manipulative; they are manipulative in another way. Therapists therefore need a high degree of expertise to deal with DPD clients constructively. This chapter shall provide the knowledge base for this expertise (see also Sachse, 2014e; Sachse, Breil, Sachse, & Fasbender, 2013).

10.1.2 Criteria for DPD

Most of the research on DPD is based on DSM-IV, and this has not changed significantly, because the criteria have changed very little in the new DSM-5. On the one hand, the aspects specified in DSM-IV, for example, have been proven empirically (Agrawal & Rai, 1988; Beck et al., 2001; Beitz & Bornstein, 2006; Berg, 1974; Birtchnell & Kennard, 1983; Bornstein et

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al., 1986, 1987, 1996a, 1996b; Emery & Lesher, 1982; Goldberg et al., 1989; Gude et al., 2004; Hirschfeld et al., 1977, 1991, 1995; Jackson et al., 1991; Keinan & Hobfoll, 1989; Masling et al., 1981; Perry, 2005; Reich, 1996; Rossman, 1984; Sinha & Watson, 2004; Tribich & Messer, 1974; Weiss, 1969). Bornstein (1993, 1997), on the other hand, showed that DPD patients have more distinguishing characteristics than those specified in DSM-IV. Chen, Nettles, and Chen (2009) have found that the characteristics of DPD are highly related to cultural factors and that the DSM characteristics are typical of Western industrial societies (see also Chen, 2006).

10.1.3 Gender Bias

On average, women are given higher scores for dependence than men (Banu & Puhan, 1983; Bornstein, 1997; Conley, 1980; Evans, 1984; Hedayat & Kelly, 1991; Jackson et al., 1991; Kass et al., 1983; Lao, 1980; Loranger, 1996; Oldham et al., 1995; Overholser et al., 1989; Selvey, 1973; Steele, 1978; Vats, 1986). Women are significantly more frequently diagnosed with DPD than men (70% female vs. 30% male; Alnaes & Torgerson, 1988; Bornstein, 1996; Jackson et al., 1991; Kass et al., 1983; Loranger, 1996; Oldham et al., 1995; Reich, 1987; Sharp, 1998). The level of dependence correlates highly with Western gender stereotypes (Anderson, 1986; Cherniss, 1972; McClain, 1978; Watson et al., 1989; Zuroff et al., 1983).

10.1.4 Helplessness and Interactional Behavior

DPD clients have a concept of their own helplessness and assume they must get support from others (Bornstein, 1995b, 1996, 1997; Overholser, 1996). They also to a greater extent resort to actually seeking help (Bernardin & Jessor, 1957; Fisher & Greenberg, 1985; Flanders et al., 1961; Shilkret & Masling, 1981; Sroufe et al., 1983), and they assume that it is better to comply and go along with the wishes and needs of those people who are giving that support (Main et al., 1985; Sroufe et al., 1983).

DPD clients also use passivity as a strategy to appear helpless and in this manner receive help (Bornstein et al., 1996a; Caspi et al., 1989), and they adjust their behavior toward maintaining relations and being protected (Bornstein et al., 1996a; Simpson & Gangestad, 1991). Finally, they show a strong orientation toward meeting expectations, by trying to comply with the wishes and expectations of their interaction partners to a high degree (Agrawal & Rai, 1988; Biaggio et al., 1984; Bornstein & Masling, 1985; Gorton, 1975; Griffith, 1991; Levitt et al., 1962; Masling et al., 1981; Weiss, 1969).

10.1.5 Partner Reactions

Persons with DPD more often cause negative reactions in their marriage partners and more dissatisfaction of in those partners, than persons having other PDs (with the exception of borderline disorder; South et al., 2008). They are also under the impression that they receive insufficient affection from their partners (Birtchnell & Kennard, 1983; Zuroff & DeLorimier, 1989). They fall victim to marital rape more frequently than persons without DPD or persons having other PDs (again with the exception of borderline disorder; Bornstein, 2006; Loas, Cormier, & Perez-Diaz, 2011).

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10.1.6 Emotions

Persons with DPD tend to feel the emotion of shame more intensely than those with other PDs (with the exception of avoidant PD; Schoenleber & Berenbaum, 2012). They also experience more feelings of guilt than persons having other or no PD (Sinha & Watson, 2006) and they experience alexithymia to an especially high degree (which may be viewed as an indication of alienation; Loas & Cormier, 2008). Families of DPD clients show a lower expressivity and higher control than families of other disorder groups (Head et al., 1991).

10.1.7 Comorbidities

DPD persons exhibit a higher inclination toward depression (Andrews & Brown, 1988; Chevron et al., 1978; Coyne & Whiffen, 1995; Hokanson & Butler, 1992; Johnson et al., 2005; Loranger, 1996; Nietzel & Harris, 1990; Overholser, 1996), and they are the most likely to be subject to eating disorders (Jacobson & Robins, 1989; Lenihan & Kirk, 1990; Tisdale et al., 1990; Wonderlich et al., 1990). They are also the most likely to develop somatization disorders (Hayward & King, 1990; Rost et al., 1992). Clients with DPD also show comorbidities with social anxiety, agoraphobia, and panic disorders, and display a tendency toward obsessive-compulsive disorders (Alnaes & Torgerson, 1988; Barzega et al., 2001; Bienvenu et al., 2009; Bornstein, 1995a; Chambless et al., 1992; Jansen et al., 1994; Lilienfeld & Penna, 2001; Loas et al., 2002; McLaughlin & Mennin, 2005; Ng & Bornstein, 2005; Reich, 1996; Skodol et al., 1995; Torres & Del Porto, 1995). Finally, they show a higher inclination toward alcoholism (Bertrand & Masling, 1969; Evans, 1984; Greenberg & Bornstein, 1988b; O'Neill et al., 1984; Overholser, 1992; Vogelsang, 1999).

10.1.8 Further Characteristics of DPD

DPD clients are subject to higher suggestibility (Jakubczak & Walters, 1959; Ojha, 1972; Tribich & Messer, 1974; Zuckerman & Grosz, 1958), show higher levels of fear of being assessed (Devito & Kubis, 1983; Goldberg et al. 1989; Schlenker & Weigold, 1990; Singh, 1981), and more often than not they have anxiety disorders and show increased passivity regarding the formation of their own way of life compared with nondependent persons in the general population (Greenberg & Bornstein, 1988a). DPD is especially frequent in people from the lower classes (Reich, 1996).

DPD forms of interaction may already be noticeable in short interactional sequences (Leising et al., 2006). DPD children were often more unpopular in school than other children (Gordon & Tegtemeyer, 1983).

Bornstein (1995a, 1997, 2007; Bornstein et al., 1996a) proceeds from the assumption that DPD persons engage in their strategic behavior with a view to achieving their interactional goals. He assumes DPD persons adopt strongly passive, submissive strategies if these serve their purposes, but also that some DPD persons are able to behave actively and assertively when they think this is conducive. Therefore, "active dependence, in his view, is merely a behavioral strategy aimed at achieving interactional goals (Bornstein et al., 1996b).

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10.1.9 Therapeutic Work

DPD clients seek psychotherapeutic help sooner than other persons with PDs (Greenberg & Fisher, 1977; Poldrugo & Forti, 1988), adhere more stringently to therapeutic rules than other clients (Poldrugo & Forti, 1988), tend to see their therapists as acting in a more controlling way (Trouw, 1997), need more emergency sessions (Emery & Lesher, 1982), and experience more difficulties with the termination of psychotherapy (Greenberg & Bornstein, 1989). Therapies based on cognitive or interactional therapy reduce characteristics of DPD significantly (Borge et al., 2010).

The main problem of DPD clients is that they have a strong need for reliable relationships but do not deem them reliable, due to their schemas. They always fear being deserted by important interaction partners, even without previous warning. Their strong desire for making relationships reliable results from these fears. They try to achieve this by making themselves especially important for the partner, trying to behave submissively and fulfilling all of their wishes. Consequently, they appear highly dependent, and give the impression they cannot live without the partner.

To make their relationships reliable, they waive their own desires and motives. As a consequence, they have bad access to their own motive system and thus high levels of alienation. They don‘t know what they want anymore, resulting in difficulties in making decisions. To please the partner and avoid conflicts, they pass decisions on to their partner who makes decisions for them. As a consequence, they become highly dependent on the partner, which increases their fear of being left, and at the same time fosters their belief that they cannot live without the partner. They get into a cycle of self-fulfilling prophecies.

The alienation becomes obvious in their everyday life. DPD clients are no longer sure about what they like, what they want to eat, what to wear; they do not have any representation of their preference structure and no current access to it. Thus, they do not have available any internal standards they could use as the basis for decisions. DPD clients prove to be highly conflict avoidant in their everyday life. They avoid arguments, do not dare to say no, and are virtually unable to criticize others or give them negative feedback. As they are especially conflict avoidant in close relationships, they spare others any kind of argument and appear to be very easy care. Sometimes partners have the impression that there is no opponent, no one to argue with, and as a consequence, DPD clients, despite their relationship commitment, sometimes become boring and uninteresting for their partners.

In therapy, therapists often do not notice DPD right away. The client appears inconspicuous, highly cooperative, and agreeable. They do what the therapist says, praise the therapist, are friendly, and appear to be hardly bothered with problems. They do not even seem manipulative, so that, as a consequence, therapists do not really pay attention to what is going on on the relationship level.

However, manipulative strategies actually start relatively early in the therapy processually by a handover of responsibility. The therapist shall give advice, shall make decisions for the client, shall serve as mediator, etc. All this appears to be relatively harmless, but it slowly and gradually snowballs on the therapist if they are not attentive.

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10.2 Definition of DPD, Based on Dual Action Regulation Model

10.2.1 Relationship Motives

The **central relationship motive** of the DPD is **dependability**: People with a DPD show a very strong desire to rate relationships as consistent, stable, outlasting, and reliable. A relationship, as they desire it, shall last forever, shall outlast problems, shall be stable; the partner shall stay in the relationship and not question the relationship. That partner should not threaten to break off but rather emphasize that they will maintain the relationship whatever happens.

Another important relationship motive is **solidarity**: the desire that the partner stays by me when I need them, that they will help me whenever I need them, that I can feel safe. It is the desire that the partner will harbor me, provide a "nest" into which one may withdraw when necessary. DPD clients shows the strongest need for emotional security.

10.2.2 Dysfunctional Schemas

Self-Schemas

Self-schemas of DPD clients especially contain beliefs that one cannot bond other people to oneself. These are schemas such as:

- I have nothing that I can offer others.
- I have no positive characteristics that would bond me to others.
- If I make trouble, I endanger the relationship.

Sometimes even

- I'm not important.
- I'm not attractive.

Due to biographical experiences of being ignored and being left alone, as well as due to their increasing, self-established dependency, clients with a DPD show another important schema: "I can't live on my own!" They believe that they cannot manage their life alone (although they usually know that this is not true) and especially that they cannot emotionally bear the condition of being alone. Sometimes even toxicity schemas can be noted such as: "I alienate others" or "I have characteristics which alienate others," etc.

Relationship Schemas

Also with DPD, the central assumptions, the learned schemas, result from the frustration of central relationship motives; thus, their schemas are basically the negation of the central relationship motives. One central schema is "Relationships are not reliable!" The person is convinced that relationships can be terminated anytime. Due to certain biographical experiences (and their processing of them), the person believes they will be deserted without any prior warning, suddenly, out of the blue, so that they cannot prepare for it and are unable to do anything about it. The DPD client also believes relationships are not strong, that any conflicts or arguments may lead the partner to walk out of it. The second assumption is that relationships do not show solidarity: "Partners are not there when needed, and I can't expect to get support if needed."

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10.2.3 Compensatory Schemas

Normative Schemas

The normative schemas of DPD clients especially refer to aspects that serve to not endanger relationships and aspects of becoming especially important for the partner and in this way increasing the reliability of the relationship. The interaction objectives pursued here are "Avoid being deserted and left alone!" The second objective is "Tying the partner to yourself as strongly as possible!" and the third, "Get the partner to be there for you 24/7, make sure they stand by you!"

Schemas develop such as

- Make sure to avoid conflicts and arguments!
- Subordinate yourself to your partner!
- Always try to please your partner!
- Don't do anything that upsets your partner!
- What is important is what your partner wants; your own needs are not important!
- Show absolute solidarity with your partner!
- Show solidarity, then others will show solidarity as well!

Sometimes there are schemas like

- Don't distance yourself from your partner!
- Don't defend your territory!

Rule Schemas

Clients with DPD barely have any rule schemas, as setting and asserting rules is strongly linked to conflicts and controversy which the clients strongly avoid. However, the clients often have two (rather implicit) expectations: the expectation that a relationship becomes reliable if one subordinates oneself to the partner, and the expectation that a partner will show solidarity if one shows solidarity oneself.

10.2.4 Manipulative Strategies

Part of the manipulative strategies of DPD persons can be termed rather passive. The DPD client behaves helplessly, is needy, unable to make decisions, thus inducing interaction partners to actively take responsibility (Bornstein, 1995a, 1995b), to carry out tasks for them, and to be there for them. However, there are manipulative strategies which can be termed "rather active" (Bornstein, 1995a, 1995b). Sometimes DPD clients may indeed defend their viewpoint, persistently pursuing objectives or behaving assertively. These strategies are usually aimed at meeting the same objectives as the passive ones: The person emphasizes that they are centrally important for the interaction partner and that the partner shall stay with them. This strategy also serves to make the relationship reliable.

Active strategies are adopted – by people who have such strategies available (which we think does not apply to all dependent persons) – when persons believe that they offer advantages or more advantages than passive strategies used with the interaction partner. DPD clients who pursue passive as well as active strategies are, according to the dual action regulation model, people with high manipulative competences. Thus clients with a DPD are basically highly

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manipulative. However, other than HPD clients, DPD clients are less salient in their manipulations. The manipulation is always labeled as "I just do this for my partner. I just want the best for them. I just want to please them." This disguise usually works pretty well. It is hard to recognize that the client is not at all altruistic with their strategies. but rather pursues their own objectives.

Essential images of DPD are

- "I do anything for you!"
- "I answer all your wishes!"
- "I'm indispensable for you!", but also
- "I'm needy!"
- "I'm always there if you need me!"
- "I'm always available!"
- "You can rely on me!"

Appeals are, for example:

- "Stand by me!"
- "Don't leave me!"
- "Be there for me!"
- "Care for me!"
- "Take responsibility for me!"
- "Make decisions for me!"

Clients with DPD often play a variant of the "poor sucker game" (see the subsection Images, Appeals, and Interaction Games, in Section 9.2.4), functioning as follows: "I'm weak, I can't make any decisions, I badly need a strong partner, please assume responsibility for me, tell me what to do, guide me, make decisions for me," and thus "Dominate me!"

The image component takes the form of "I'm basically a helpless little child who badly needs a grown-up leader," and the appeal component implies "Be there for me, care for me, take responsibility!" "In return I will fulfil all your wishes: I even try to fulfil wishes you haven't said or maybe haven't even thought of yet."

Usually, DPD clients are unaware that they play manipulative games. They are rather deeply convinced of their altruism and their own simplemindedness. They think of themselves as naïve rather than clever, as submissive rather than controlling.

10.2.5 Tests

The tendency of DPD clients to test interaction partners and therapists is rather low. If at all, they rather try to hand responsibility over to their partner and see how they handle it.

10.2.6 Characteristics of DPD

Costs

The costs of their disorder are, on the one hand, that the client impairs their occupational success: They cannot assert themselves and thus cause damage to their career. They cannot say no and consequently get overloaded with additional work, and they cannot handle or resolve conflicts. They might recognize this and suffer from it.

Unfortunately, they are often aware only of the costs and do not realize that they produce these costs themselves: A DPD client may tell the therapist they are bullied and badly treated

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by colleagues, without realizing that it is their own behavior that greatly contributes to the problem (usually, clients are not "mobbing victims," rather they are participating in an interaction problem). At the beginning of therapy, clients are interested in reducing these costs but not in changing their system.

Proximity and Bonding

Due to the motives and schemas of the DPD, these clients sometimes show a strong desire for proximity and bonding; sometimes they cannot get enough proximity. Keeping their distance is often strange to them. Interaction partners often perceive the client's behavior as too close and too constrictive, especially when they have a greater need for autonomy. Occasionally, partners want to do something on their own, have their own space and domain, take their own decisions, etc. They may react aversively to statements like "As a couple we should do everything together!" This may then give rise to conflicts with partners.

The result of their schemas is (complete) subordination, sometimes even "unconditional surrender to the partner." The result is also a tendency toward putting up with (much) too much from the partner. Especially the fear of being deserted and unable to cope with this situation can make highly disturbed DPD people stay in a relationship, even when they are treated very badly, beaten, or abused. In extreme cases, the subordination and lack of any kind of rebellion may take on grotesque forms, with people enduring extremely aversive relationships for a very long time without leaving the partner or without defending themselves against them.

Alienation and State Orientation

Their strong external perspective together with the strategy of not taking their own needs seriously, disregarding those needs or shelving them for the partner's sake, result in DPD clients having only poor access to their own motive system and thus to its indicators, the affective reactions. Therefore, these clients can hardly understand their own needs, motives, objectives, and preferences. Accordingly, they have no cognitive representation of their objectives and motives. As a consequence, they are subject to alienation to a great (even very great) extent. They are, for example unaware of

- what is important to them,
- what they want,
- what is good for them,
- what they do not want,
- what is not good for them,
- what objectives they have.

Since DPD clients strongly focus on the needs of their partner, this also leads to situations in which they do not think their own needs are as relevant and thus make no efforts at all to find out what they actually want themselves. In this way, they do not do anything conducive to diminishing their alienation. Over time, their alienation will get worse because they systematically impede their ability to access their system. This may often cause DPD clients to drift through life without paying any heed to their own needs. Usually, DPD clients are so busy with fulfilling their partner's needs that they do not even notice that they know nothing about their own needs. It thus follows that despite their high alienation, they show only a low degree of (represented) dissatisfaction.

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However, the intrusion error frequently occurring with high alienation is strongly pronounced. Clients with DPD cannot tell anymore whether they pursue an objective they have chosen themselves, whether it reflects their own preferences, or whether they pursue it because someone told them to. Therefore, they are strongly inclined to take adopted missions as their own objectives, and as a consequence, it is extremely difficult for them to distance themselves from the demands of others.

Clients with DPD show an extremely high degree of state orientation (Kuhl, 1994). On the one hand, this is due to massive alienation; if one does not know one's own wishes, one will not be able to make a decision. On the other hand, this is due to the fact that one keeps thinking about how one could please the partner. The state orientation even makes things more difficult and impedes decision making further. For example, a client with DPD might be standing in front of their closet and will not know what to put on. They consider all options carefully but cannot make up their mind. Finally, they try to solve the problem by asking their partner what to wear. This is not so far away from asking: "Darling, what kind of food do I like?"

Assumed Expectations

As stated earlier, clients with DPD try very hard to fulfill the expectations of their interaction partners. They even attempt to track down, recognize, and nearly telepathically detect such expectations. Accordingly, they usually refrain from asking their partner what they actually want. Rather, they think that they really should know it. As a consequence, DPD clients do not meet the real expectations of their partner but rather what they think are the expectations. More often than not, fulfilling those expectations is an illusion.

Ego-Syntony

DPD is a **highly ego-syntonic disorder**. These clients usually do not suffer from the dependency itself but rather from its costs: overstressing, difficulties at work, and problems in their relationships, as well as comorbid disorders such as gastrointestinal problems, depression, and sleep disturbances.

As a rule, the clients regard their system itself as all right. Normally, they cannot even imagine an alternative to their behavior. In many cases, they perceive an alternative behavior as a threat and think if they behave differently than they do right now, the partner could be angry, and they might endanger the relationship. Thus, they do not even think about alternatives.

Self-Ideologization

Clients with DPD tend strongly to deceive themselves about their behavior and the reasons for it. They assume things about themselves which actually are in sharp contrast to their schemas and their behavior. We call such a construction self-ideologization: The client establishes a belief system about themselves, their behavior, and motivations which is positive, meets their own ideals or social expectations, and helps them to ignore unfavorable behavior, schemas, and intentions systematically.

DPD clients show a high degree of avoidance and also use manipulative strategies as an avoidance strategy.

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The therapist needs to keep an eye on avoidance strategies and should use appropriate intervention strategies.

10.3 Therapeutic Strategies

10.3.1 Therapeutic Strategies in Phase 1

Complementarity

Complementarity at the Motive Level

The therapist should especially establish complementarity concerning reliability and solidarity (within the framework of therapeutic rules): The client must have the feeling that they can rely on the therapist, and feel secure and cared for.

Here, the therapist might feel like they are caught between a rock and a hard place: On the one hand, they should be loyal to the client but, on the other hand, never collaborate with the client against others. They should stand by the client's side, but must also emphasize that it is allowed to challenge and not just agree with the client's assumptions. They must make it very clear that they are reliable, but that this only applies to therapy and not to everyday situations.

As DPD clients strongly tend to misunderstand the messages of the therapist (due to their schemas), it is essential for the therapist to be transparent to a high degree. They must often tell the client explicitly and clearly what they mean, what they want, and what therapy means.

The therapist should signal to the client:

- I, the therapist, am here for you during therapy and will support you.
- It is my duty to help you find out what you want and what you do not want, clarify what your problems are, and help you with finding solutions.
- It is not my duty to solve problems for you or make decisions for you, otherwise you would become dependent on me, and that is not desirable.
- I am standing by your side but to be helpful, I must challenge your assumptions; this is not meant to annoy you but rather to help you understand your problems.
- I am reliable during therapy – that is, I offer you a therapeutic relationship, no matter what you do or fail to do in therapy.

No Complementarity at the Game Level

We advise therapists against behaving complementarily at the game level, because this may easily intensify an unfavorable interactional behavior of the client. The therapist must carefully deal with the client's strong inclination to pass responsibility on to others. They must not frustrate the client, who might otherwise be led to think the therapist is unreliable and does not show solidarity. On the other hand, they should not accept passed-on responsibilities, because in this way they would behave complementarily at the game level and, consequently, stabilize the client's disorder.

For example, if the client says, "I can't make a decision concerning XYZ. What would you do?" the therapist may reply, "I'd like to work that out with you to help you. Therefore, we

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should find out what you want. What are the pros and cons in this case?" etc. The therapist thus uses the judo technique or the yes-but strategy: Yes, I help you, but I help you by showing how you can solve the problem yourself. No, I don't help you by solving the problem for you! This approach is always important when dealing with clients with DPD. The therapist accompanies the client and guides them toward their own problem processing. This is the most important basic strategy to be adopted for clients with DPD.

It is also important that the therapist refrains from exerting pressure on the client. The therapist makes offers, PPs, but leaves it to the client's discretion to accept those proposals. The client should realize from the beginning that the therapist

- tries hard to help the client,
- actively supports the client,
- actively helps the client,
- stands by the client's side,
- does not patronize the client,
- helps the client by showing them how they can solve their problems themselves.

Resource Activation

As has been described above (see the subsection Self-Ideologization, in Section 10.3.6), clients with DPD show strong avoidance tendencies. They avoid confronting themselves with thoughts that trigger anxieties. It is essential, therefore, to strengthen these clients to enable them to face aversive contents. They need to strengthen their self-confidence, perceive their own competences and abilities, and strengthen their self-efficacy expectations This will increase their motivation to change (see Sachse, Langens, & Sachse, 2012) and their willingness to deal with their problems.

Resource activation means that the therapist

- underlines and repeatedly makes salient all aspects of what the client is capable of achieving, the abilities they have, what they can withstand, etc.;
- underlines all aspects of what the client has already achieved and done (positively), showing that they are also able to repeat the same achievements in future; and
- praises the client for all aspects of their disorder they are positively and constructively dealing with at the moment.

In addition, the therapist and client elaborate the strategies the client can concretely learn and develop, and where, how, and from whom the client can get help.

Explicating Relationship Motives

Explicating central relationship motives is of great importance with DPD clients. The therapist emphasizes in an accepting way that they understand the client's desire for reliability and solidarity and suggests that the client should endeavor to understand these motives even better.

Clarification on the Edge of Possibilities

A therapist should start clarification of problems and schemas as soon as possible. They should emphasize which contents are central, ask concretizing and deepening questions, always up to the limits of what is possible. However, in the first therapy phase, the limits of what is possible are easily reached when dealing with DPD clients.

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Same as with HPD clients, the therapist uses deepening interventions as markers. The therapist knows the client is unable to follow interventions at the moment but makes them anyway to show the client what they think is important and to keep directing the client toward relevant aspects of future changes.

10.3.2 Therapeutic Strategies in Phase 2

Making the Game Level Transparent

This form of intervention is very difficult with clients having DPD, as these clients can barely recognize their manipulative behavior (it greatly contradicts their self-concept). The therapist must makes these confrontations again and again, first in a somewhat softer manner, then even harder later on.

When confronting the client, the therapist does best by using the "It is my impression" strategy: The therapist makes confrontations, the client rejects them. The therapist explains that it is their impression …, but also concedes they might be wrong; they never discuss this with the client and never force the client to grapple with it. Never discuss with a client and never impose an interpretation on the client; however, repeat that it is the therapist's impression and explain why.

Here again, it is important to make the client's costs salient. If possible, the therapist links the costs to the client's behavior. For example, "Your husband always gets upset if you do XYZ: You kind of seem to provoke his anger."

However, therapists should keep in mind that it is not easy to make DPD clients see their costs. If they do perceive the costs they are incurring, internal conflicts are inevitable and will cause them to be frightened so that their avoidance inclination becomes stronger.

The therapist may also confront the client with manipulative intentions and games. These confrontations with intentions at the game level should be very harmless at first. Initially, the therapist should advise the client to note intentions that appear to be quite acceptable – for example, "You want to do something good for your husband," etc. Following this, the therapist should carefully and slowly introduce manipulative confrontations by saying, for example, "You want your husband to care for you" or "You want your husband to believe you're indispensable." If the client acts evasively, the therapist may confront them with this evasive behavior – for example, by saying, "It is hard for you now to have this thought about wanting to influence your husband."

Dealing With Avoidance

A specific strategy to address avoidance has been developed in COP (see Sachse, 1995, 1997b, 1998, 2006g, 2007b; Sachse, Fasbender, & Sachse, 2011). This shall briefly be described here.

As has been described earlier (see the subsection Self-Ideologization, in Section 10.3.6), high avoidance must be expected from clients with DPD. The clients avoid dealing with any aspect that contradicts their self-ideologization. They do not want to realize or elaborate the fact that they

- are afraid of being deserted or left;
- consequently, would put up with a lot from, and do much for, their partner;

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- avoid conflicts and never have their way;
- do not satisfy their needs and in doing so must absorb high costs;
- let themselves be manipulated and exploited because of their schemas;
- often do not fulfill their partner's wishes at all but rather seem to be intrusive and violate boundaries; and
- are basically highly manipulative.

As the clients also avoid conflicts in the therapy process, their most popular avoidance strategies are

- creating tangental scenes;
- answering questions no one has asked;
- trivializing, relativizing; and
- saying, "I don't know."

However, the most popular strategy is to ignore the therapist's interventions. The therapist asks or says something, and the client just continues with what they said before the intervention.

Provided the therapist has enough relationship credit available, they should start processing the avoidance behaviorually by adopting the counteracting strategy. Counteracting means that in case of avoidance, the therapist either repeats the intervention immediately or goes along with the client and repeats the intervention after a while. In this manner, the therapist makes sure the client sticks to conflict areas. According to Dollard and Miller (1950), it may be assumed that clients show an "approximation tendency," a tendency to clarify problematic contents and face up to them, as well as an avoidance tendency – that is, a tendency to evade these contents. As relevant contents are approached, both tendencies grow stronger, with the avoidance tendency growing faster than the approximation tendency. The point where the two tendencies intersect is called the conflict point, and surrounding this conflict point is the conflict area. If a client enters this conflict area, their active avoidance behavior starts, which is indicative of the therapist having reached the edge of what is possible: The therapist cannot go any further, otherwise they would cause reactance (Figure 4).

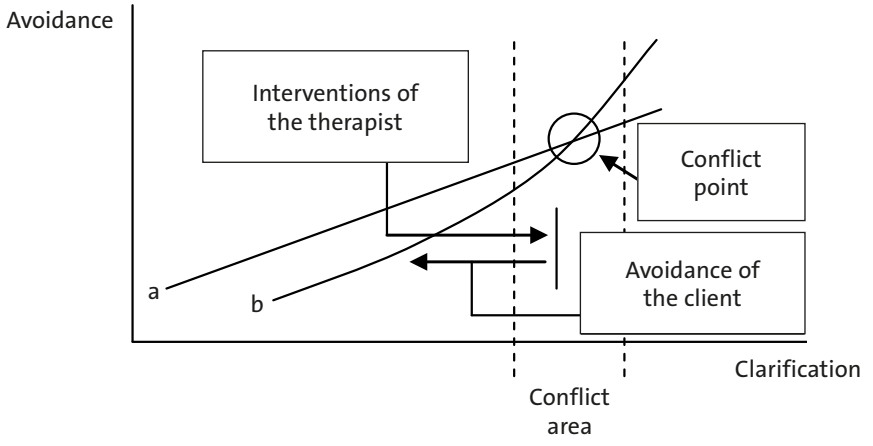


Figure 4
The conflict area and upward trends of tendency to clarify problematic contents and face up to them (a), and avoidance tendency (b).

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Keeping the client to the conflict area means the therapist always leads the client back to the hot topic area, so that the client may eventually realize

- that the contents are not that bad,
- that nothing bad happens there,
- that they will not be overwhelmed by emotions,
- that they can endure the contents and can work on them.

If the client has these experiences systematically through therapeutic interventions, the avoidance tendency will shift to the left. The client will allow problematic contents more and more (Figure 5).

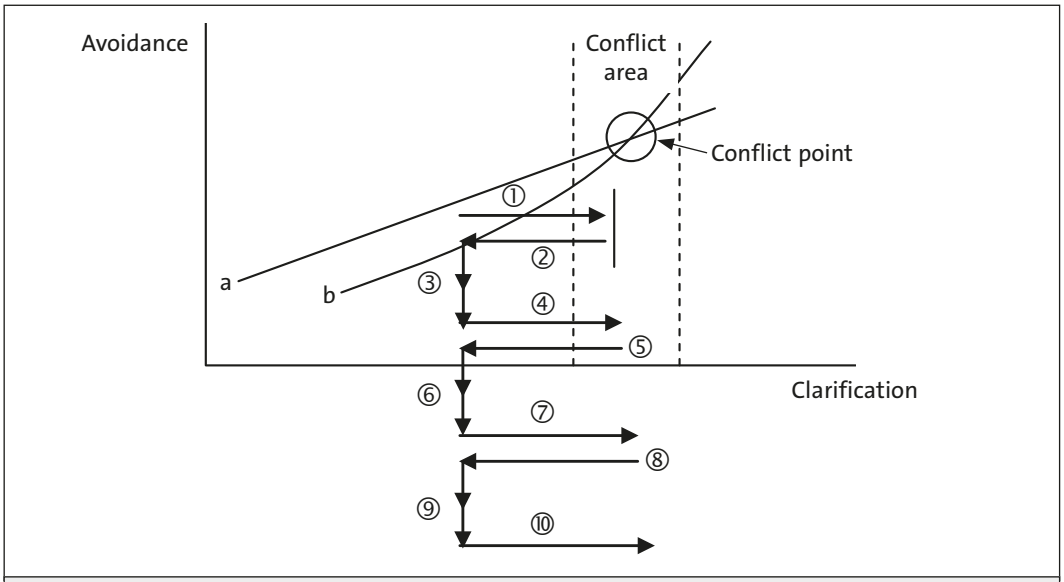


Figure 5
Keeping the client in the conflict area (i.e., between the dashed lines). Procedure: therapist makes intervention toward conflict area (arrows 1, 4, 7, 10), client avoids it (arrows 2, 5, 8), therapist temporarily sticks with the client's chosen topic (arrows 3, 6, 9).

Therefore, the approach is as follows (Figure 5): The therapist makes an intervention leading to the hot topic (1); the client avoids it (2); the therapist stays for one or two rounds with the topic indicated by the client (3), and then realizes another intervention leading to the hot topic (4), and keeps doing this (5–10). After a while the client goes along step by step, and the conflict point moves (see Figure 6).

This procedure requires from the therapist

- to always make interventions that are acceptable and empathic;
- to always remain patient, never hurry, never put pressure on the client;
- to be persistent, keep trying, maybe allow for longer breaks, and then repeat the interventions.

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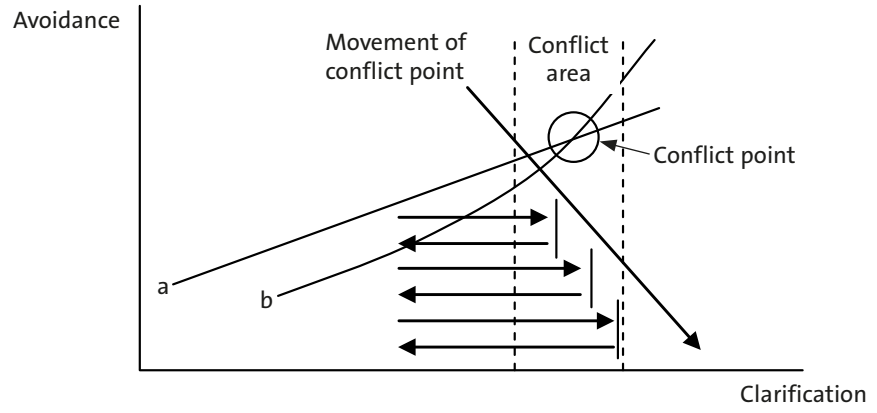


Figure 6
The client's movement toward the clarification objective. Arrows to the left indicate that the therapist follows the client out of the conflict area; arrows to the right indicate that the therapist leads the client into the conflict area.

10.3.3 Therapeutic Strategies in Phase 3

The clarification of relevant schemas is essential with DPD clients as well. These clients have to understand relevant schemas to be able to understand how their problem works. The schemas must be validly cognitively represented to enable them to be therapeutically processed in the next step. The problem of achieving clarification with DPD is that these clients have trouble assuming an internal perspective. It is therefore advisable for the therapist to keep setting markers They keep asking internalizing questions, not because they think the client can answer them, but because they want the client

- to realize that these kinds of questions may come up,
- to realize that these questions are important,
- to realize that it is sensible to address these questions,
- to gradually learn to pay attention to internal aspects of their condition.

The fact that the therapist consistently asks these questions serves as client training. In doing so, the therapist explains time and again why these questions are asked, why it is important to pay heed to internal aspects, why the client should follow those questions, etc.

A therapist must realize that adopting such an approach for a long time is a prerequisite for the client to start internalizing their perspective. However, clients will only do this if the therapist follows this approach consistently and strictly for a long time.

In DPD clients as well, therapists may try to understand the client's schemas and assumptions and translate what has been understood into explicit language for the client. For this, the therapist uses explications that translate what the client means but cannot yet express (Sachse & Sachse, 2011). Explications made by the therapist are essential for clients who have hardly any access to their schemas. Through explications, the therapist emphasizes which aspects the client should pay attention to and which tracks they should follow.

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10.3.4 Therapeutic Strategies in Phase 4

Processing Schemas

For the processing of dysfunctional and compensatory schemas, we use strategies defined by OPRP (Section 3.2.4: Therapeutic Processing of Dysfunctional Schemas). For clients with DPD, it is essential to look at and reappraise dysfunctional schemas. For example, the schema "I can't live on my own" should be processed and reappraised – for example, via OPRP using questions such as:

- What are good reasons for me being able to live on my own?
- Do I have any experience of living on my own? etc.

Processing Alienation

It is vitally important for DPD clients that their massive alienation is therapeutically processed. As long as the client is unable to access their motive system, they are unaware of what they really want and will find decision making difficult. They will display a strong tendency to give in to the expectations of others. The therapist should, therefore, motivate the client to systematically work on questions such as:

- What is important to me?
- What do I long for?
- Which do I need in relationships?
- With what do I feel comfortable?
- What is unpleasant for me?
- Which behavior of partners makes me feel uncomfortable? etc.

10.3.5 Therapeutic Strategies in Phase 5

In clients with DPD, the process of transfer calls for the clients to practice with partners, friends, or colleagues in concrete situations to say no, provoke conflicts, express what they want even if they know their interaction partner thinks otherwise, and especially to say what annoys them or what the partner should change. Through this training, clients learn to act differently and make the experience that conflicts, though unpleasant, are bearable and that conflicts do not endanger the relationship but strengthen it over time. This training is difficult for DPD clients, and they must be extra motivated to realize it; however, the more positive experiences they have, the easier it gets for them.

A special problem may occur if the partner feels threatened by the emancipation of the client and tries to sabotage the therapy. In such cases, couple therapy may be indicated (see Sachse, Breil, & Fasbender, 2013).

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Chapter 11

Avoidant Personality Disorder

11.1 Description of AvPD

11.1.1 Differentiation of AvPD from Social Phobia

There is great controversy about the question of whether **avoidant personality disorder** (AvPD) can actually be differentiated from social phobia, social anxiety disorder, or generalized anxiety disorder, or whether a PD should be diagnosed here at all. It is clear that the definitions of AvPD and social phobia overlap, and it is, furthermore, clear that AvPD is not a typical PD, as it is hardly at all characterized by game behavior. Some authors speak against giving an AvPD diagnosis and rather understand AvPD as a part (or strong form) of social phobia (Chambless et al., 2008; Cox et al., 2011; Dahl, 1996; Herbert et al., 1992; Hofmann et al., 1995; Holt et al., 1992; Hope et al., 1995; Huppert et al., 2008; Kose et al., 2009; LaFrentere, 2009; Mendlowicz et al., 2006; Rettew, 2000; Tillfors et al., 2001, 2004; Turner et al., 1992; Widiger, 1992).

Other authors (Boone et al., 1999; Carter & Wu, 2010; Cox et al., 2009; Hummelen et al., 2007; Kantor, 2010; Ralevski et al., 2005; Van Velzen et al., 2000; Wilberg et al., 2009) argue that AvPD is reasonably and pragmatically distinguishable from social phobia because AvPD is

- more generalized,
- more serious,
- related to heavier functional impairments,
- and more therapy resistant than social phobia and also related to greater social stress.

In our experience, the latter position pertains: Clients with AvPD may show similarities to social phobia; however, they clearly exhibit more generalized and more therapy-resistant schemas than clients suffering from social phobia. Thus, I would like to recommend that AvPD be understood as a PD.

11.1.2 Criteria for AvPD

Studies focusing on the DSM criteria for AvPD indicate that the criteria are valid in most cases. Empirically confirmed (Abraham, 2005; Alden et al., 2006; Eggum et al., 2009; Herbert, 2007; Hofmann, 2007; Leising et al., 2006; Mahgoub & Hossain, 2007; Meyer, 2005; Müller, 2000, Robin et al., 2007; Rosenthal et al., 2011; Ye et al., 2011), essential aspects of AvPD are

- low self-esteem;
- avoidance of social situations;

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- fear of being rejected;
- sensitivity to criticism regarding socially relevant aspects of the person;
- negatively distorted self-assessment of the self as social object;
- a high degree of shyness;
- a high degree of social inhibition and social retraction;
- problems in correctly interpreting emotional expressions of other people.

Persons with AvPD show an increased comorbidity with depression (Alpert et al., 1997; Bockian, 2006a). As regards AvPD, there is an equal distribution between men and women (Becker et al., 2009). Persons suffering from AvPD feel deeply worried about the question, "Am I or is my behavior socially acceptable?"

11.1.3 Further Characteristics of AvPD

AvPD is a proximity disorder: Clients crave contacts and relationships but do not dare to establish them, or when clients are in a relationship, they are afraid they might lose it because they think they would be unable to find a new one (Büker, 2006; Cremer, 2010; Karschti, 2006; Müller-Bruhnke, 2008; Pralat, 2001; Thurow-Hartmann, 2010; Wiesener-Kalveram, 1997). It can be assumed that AvPD develops during puberty and primarily stems from negative feedback from, or negative experiences with, peers (Bloemer, 2000).

AvPD clients realize that they are (strongly) impeded through this disorder, which makes AvPD the most ego-dystonic PD. This PD is also the least manipulative one. Therapeutic problems are especially associated with the processing of highly resistant schemas.

Basically, the disorder is always about the question of whether the person can meet essential social expectations. Due to their schema assumptions, these clients assume that

- they can not fulfil essential social expectations as a person or with their behavior,
- other people recognize this and will assess them negatively,
- this evaluation leads to depreciation and rejection, and
- this social depreciation entails social expulsion.

Social evaluation or depreciation causes massive insecurity and shame. Such a situation is highly embarrassing for these people; the anticipation of evaluation and depreciation causes insecurity and anxiety and consequently an intense avoidance behavior (therefore the term avoidant personality disorder).

AvPD clients avoid approaching people, showing themselves, and engaging in small talk. If they go to a party, they take a glass of wine, lean against the wall, and sort of merge into the color of the wall. They do this because they think that mistakes in social surroundings are quickly noticed and cannot be undone: A wrong joke at the wrong time cannot be corrected; an obvious uncertainty is conspicuous and cannot be compensated for, so it is better not to do anything rather than make mistakes that are easily perceptible.

People with a general AvPD are extremely focused on their deficits and on what could go wrong. They focus totally on negative aspects and consequently lose track of their own strengths and resources. A person with AvPD often has **negative assumptions** about their attractiveness and think they are not good looking and that they are boring, and they are under the impression that they do not have anything to offer to potential partners. They think they are awkward, and that when in contact with potential partners, they will disgrace themselves. Often these clients show active imaginations regarding massive disasters that could or will happen when they make a fool of themselves.

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With respect to AvPD, we define a continuum that ranges from *generally avoidant* to *particularly avoidant*. People who are generally avoidant normally show uncertainty concerning their social acceptability; this includes many different social situations, even insecurity toward people they are sexually attracted to. People who are particularly avoidant do not display social insecurity in general: Often, they are socially (very) competent and behave with confidence in society and in occupational contexts. But when it comes to getting in contact with potential partners, their negative attractiveness assumptions activate great uncertainty and depreciation anxieties. In successful narcissists, this type of AvPD sometimes occurs as a comorbidity.

Massive self-fulfilling-prophecies are very characteristic of clients suffering from AvPD. Due to their schemas and resulting anxieties, these clients behave very awkwardly socially. They attend parties but refrain from making contact with others and avoid attracting any attention; they remain in the background and signal by their countenance and gestures that they would prefer not to be disturbed. As a consequence, nobody approaches them, and they leave the party without having achieved whatever they might have had in mind. They attribute their failure to their unpleasant appearance, etc. (confirming their schemas) rather than to their inappropriate behavior. As they do this very often, therapists have to expect **extremely persistent schemas**.

Usually, the disorder is highly ego-dystonic, as AvPD clients know they stand in their own way; it is quite obvious that they generate the costs themselves. The disorder is slightly manipulative as a rule, which does not come as a surprise, since social practice fields are needed to develop manipulative strategies. Accordingly, many aspects of other PDs are less pronounced with these clients. Due to their massive and persistent schemas and because of the plurality of self-fulfilling prophecies, the present author nevertheless conceives, however, of AvPD as a PD.

In therapy, AvPD clients are usually not very manipulative; they only make a few tests and are relatively motivated to change. However, male AvPD clients tend to misunderstand the empathic therapy situation and sometimes fall in love with their therapist.

The biggest problem for the therapist are the strongly implicating and very persistent schemas of the clients. It is often very difficult to process these, and the therapist must be able to reveal the hidden implications to get to central schema assumptions.

11.2 Definition of AvPD, Based on Dual Action Regulation Model

11.2.1 Central Relationship Motives

A central motive of AvPD is the desire for appreciation. The desire to be appreciated and valued as a person. Very important here is

- appreciation as a person: Feedback that they are okay as a person, loveable, and valuable;
- appreciation as having positive social qualities, being welcome, and
- appreciation as being attractive for potential partners, being masculine or feminine, and having attractive characteristics.

As with narcissists, appreciation is most important for AvPD persons. However, these

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motives relate to other contentual aspects: For AvPDs it is especially important to receive positive feedback concerning relevant social aspects as well as attractiveness. If the schema admits it, feedback of this nature has especially positive effects and feeds the relationship motive.

Another central motive is the desire for importance: The desire to be relevant in the life of potential partners and have characteristics and skills that make the person important for potential partners. This involves the desire for feedback such as

- "You are (highly) important to me (because of your traits)"
- "As a person, you play an important role in my life."

11.2.2 Dysfunctional Schemas

Self-Schemas

Clients with AvPD have had biographical experiences that cause negative schemas to develop that are diametrically opposed to their motives. A client with AvPD believes, for example, that

- "I'm socially incompetent."
- "I can't fulfil important social expectations."
- "I can't assess social situations correctly."
- "I'm not ok as a person."
- "I have no positive social characteristics."
- "I appear repulsive to others."
- "I can't entertain others, and I have nothing to offer."
- "As soon as I say something, I make a fool of myself."
- "I'm not attractive."
- "I'm not masculine (or feminine) (enough)."
- "I have no traits that may be interesting for potential partners."
- "I'm unable to impress others."
- "I'm boring."
- "If I attract attention, it's always negative attention."

It is essential for the therapist to realize that the schemas of AvPD include many hidden, implicit assumptions that therapists must tediously reconstruct. However, these should be clarified as the implicit assumptions contain highly dysfunctional beliefs and imaginings that lead to very unfavorable behavior. If these assumptions remain unclear, they cannot be processed therapeutically, and they will keep affecting the client's behavior unfavorably.

Relationship Schemas

Relationship schemas of AvPD clients are less distinct than their self-schemas. Clients with AvPD mostly show assumptions such as

- " Interaction partners constantly assess and watch me."
- "Attracting negative attention will have negative consequences immediately."
- "My behavior is critically assessed by others."
- "Others don't tolerate social flaws."
- "When attracting attention in groups, it is negative attention."
- "One makes a fool of oneself in groups."

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- "It is dangerous to be the focal point."
- "Potential partners have high expectations."
- "If one can't meet expectations, massive depreciation will result."
- "When making a mistake, there will be no second chance."

11.2.3 Compensatory Schemas

Normative Schemas

Clients with AvPD show significant avoidance assumptions:

- "Disclose as little of yourself as possible, then you can't make any mistakes."
- "Avoid making a fool of yourself."
- "Avoid social contacts that are insecure."
- "Avoid being the center of attention."
- "Only talk about well-known topics."
- "Only contact potential partners if you're sure it will work."
- "Avoid disgracing yourself in front of a potential partner."

Rule Schemas

Clients with AvPD have (due to their anxieties resulting from their schemas) practically no rule schemas: Rule schemas develop when clients think on account of their competence they have social control and that they deserve this control. Since clients with AvPD do not have these assumptions, they cannot develop rule schemas.

11.2.4 Games and Tests

Of all persons with PDs, clients with AvPD are the least manipulative. This is, on the one hand, due to their high ego-dystonia: These clients take a high degree of responsibility for their problems. On the other hand, this may possibly also be due to the fact that with this kind of PD, manipulative strategies can hardly be developed at all, because manipulation always causes attention and that is what the clients want to avoid, and, moreover, manipulation requires proximity and relationships which the clients neither possess nor have available.

Thus, one sees at most appeals to the therapist such as:

- "Do you think I'm ok?"
- "Do you think I appear masculine?"
- "Do you think I'm boring?" etc.

Real tests are rarely made by clients with AvPD. These clients are not aggressive and do not put therapists to the test. Lack of cooperation usually results from resistant to change schemas, not from interactional games. The only thing they might do is question the therapist's affection, positive appreciation, and interest, to find out whether the therapist is serious.

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11.2.5 Characteristics of AvPD

Costs

Usually, clients are aware of most of the costs their behavior causes; however, there are costs that must be explained in more detail. Usually, the clients even know themselves that they generate these costs through their behavior. However, what they do not know is that their schemas are constructs and that dysfunctional schemas are producing their costs.

Avoiding Contacts

Due to their motives, clients with AvPD have a strong need for proximity and relationships. These clients desperately want social contacts, are looking for a partner, and suffer from being single. The longer clients are single, the higher the degree of deprivation.

The problem is that due to their schemas, AvPD clients assume they cannot succeed in behaving appropriately in social situations or are unable to establish positive social contacts in a relaxed way. Thus, their dysfunctional schemas and rule schemas result in a very high anxiety level. AvPD clients enter social situations with a high degree of anxiety, and they expect to be constantly observed and evaluated. As a result, they are never relaxed or easygoing, but are always behaving ambivalently when having to deal with social situations.

AvPD clients view situations in which they can make contact with potential partners as very dangerous and highly aversive; they greatly fear that their schemas are (fully) affirmed, and they will disgrace themselves so badly that they will never be able to be in social situations again. Therefore, when trying to get in contact with other people, the client will no longer dare to act on their own initiative.

A person with AvPD can only display initiative if they are positive that their action will not fail. And that is the problem, because most AvPD clients need very conspicuous signals for that; they do not respond to the usual flirting signals. This strongly reminds one of the lyrics to the song "Living Next Door to Alice" ("'Cos for twenty-four years I've been living next door to Alice"). Obviously, this involves two persons with AvPD desperately waiting for clear signals from each other, and none of them had the courage to come forward to say "Who the fuck is Alice?" (as is often sung in the refrain in different versions to the original song and at concerts).

Self-Fulfilling Prophecies

All PDs create self-fulfilling prophecies; however, clients with AvPD are in the lead. They are shy and distanced; consequently, they appear repulsive, hardly open, and uninterested in relationships. They send signals such as

- "Please, don't speak to me!"
- "I want to be alone!"
- "Please don't disturb!"
- "I'm not interested in relationships!" etc.

Most likely, potential interaction partners react to these signals and leave clients with AvPD alone. As a consequence, they will understand this as

- "Nobody is interested in me!"
- "This shows how unattractive I am!"
- "I shouldn't do anything anymore!"

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As a result, AvPD clients themselves through their behavior and interpretation of behavior effects initiated by their schemas will end up gaining relevant experience, which they cannot accept and perceive, however; they are of the opinion their experiences are obvious, imperative, and valid, which, of course, is wrong altogether. Such experiences do not prove anything, only that the person has a problem. People regard these experiences as conclusive and that they always affirm their negative assumptions. However, "experience" in this respect must be considered as nonsensical. On account of the proof thus obtained, the client sticks persistently to their dysfunctional schemas, which constitutes the greatest problem in therapy when dealing with AvPD clients.

Ego-Syntony, Perspective, and Avoidance

AvPD is probably the least ego-syntonic of all PDs. The clients perceive their needs very well, and they perceive very well that they do not succeed in satisfying their needs. What is more, they understand very clearly that they stand in their own way, meaning they are aware of the interaction between their own behavior and its costs. As a result, AvPD clients already at the beginning of the therapy are quite motivated to bring about change. However, this does not mean changes can be achieved easily. As has been said earlier, their schemas are relatively resistant.

The perspective of AvPD clients is very much an external one. They are highly attentive to the behavior of other people and the signals potential partners send. Therapists thus have trouble internalizing the client's perspective. As a result of their high ego-dystony, their level of (intratherapeutic) avoidance is comparatively low. However, the treatment of anxieties and assumptions may still be unpleasant for AvPD clients and should therefore be carried out by the therapists in a way that will not be embarrassing.

11.3 Therapeutic Strategies

11.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

The therapist's complementarity at the motive level is less crucial with AvPD clients compared with those with other PDs. However, it helps to obtain relationship credit. The therapist should

- take the client very seriously, including their difficulties, assumptions, and anxieties;
- treat the client respectfully as a person having positive characteristics and resources;
- signal sympathetically that they understand the problems, that such problems may arise, and that it is not unusual to have these problems;
- signal that it is important to them to understand the client, work with the client, and that they are personally committed to the client.

The therapist should appear competent, as somebody who understands problems and can be of help in solving problems.

Additionally, it is important that the therapist does not exert pressure on the client: The client sets the pace, and the client decides on what shall be done. The motivation for the therapist's

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therapeutic work must always be based on the intentions of the client. Work shall be done if the client wants to change something; changes shall be made if something bothers the client, etc. This is important, as resistance to changing schemas may induce the therapist to put the client under pressure, but such changes cannot be enforced.

Explicating Relationship Motives

The intervention to explicate relationship motives is not that important when dealing with clients suffering from AvPD: The clients usually know their motives. However, the therapist may obtain relationship credit by showing interest in the relevant motives.

Clarification to the Edge of Possibilities

When treating AvPD, it is very important to clarify dysfunctional schemas – that is, by representing them in a valid cognitive manner. The client must realize and come to understand

- that they are subject to schemas, that these schemas are assumptions, that these schemas are questionable and changeable;
- what these schemas are called exactly or how they can be labeled;
- that these schemas include many implicit assumptions, and what these implicit assumptions are.

Questions asked by the therapist are of limited help only. Because clients are to some extent unaware of their schemas, they cannot name them when asked to by the therapist. Thus, the therapist must make the implicitly addressed, deducible schemas explicit, must label them, *pinpoint and identify* them for the client. This helps the client to realize their schemas, understand, and represent them. Building representations of relevant schemas is a vital prerequisite for therapeutic processing.

11.3.2 Therapeutic Strategies in Phase 2

Problem Definition

This is about defining exactly the client's problem through clarification processes and finding out what the client wants to achieve with therapy (to the extent that this can be clarified at this point).

Making the Game Level Transparent

As the game level does not play a significant role in the behavior of clients with AvPD and is hardly noticeable in the therapeutic interaction, the therapeutic processing of the game level is more or less irrelevant. If game aspects appear, the therapist may confront the client with them.

The therapist must frequently confront the client with

- their strong risk avoidance; the client avoids all risks and thus ruins any chance of being successful; if they want a relationship they must be willing to take risks;
- their countless self-fulfilling prophecies and the consequences of those, especially the artificial stabilization of schemas.

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11.3.3 Therapeutic Strategies in Phase 3

In this phase, the clarification of schemas is essential. It is absolutely necessary for the therapist to elaborate the implications of schemas.

Many schemas contain hidden, implicit assumptions that the client cannot consciously perceive and is unable to express explicitly. However, these schemas direct their processing of situations and their behavior to a great extent. Therapists must therefore detect implicit assumptions and elaborate them together with the client or explicate them. For example, a frequently seen implication of the assumption "I'm unable to entertain others" is "I must entertain others." When making contact or having a first date, the client assumes it is solely their own responsibility, and the partner need not make a contribution at all. They must make and keep up the conversation alone, must entertain the partner, and must make a good impression. Consequently, they are under the impression they are being tested.

As regards the assumption "I have no value on the relationship market," clients have absolutely unverified assumptions about what potential partners want. Again, clients assume that partners are only interested in physical attractiveness, and no other characteristics are worthy of note or of any valuable (e.g., as this is fostered by the Hollywood film and advertising industries). Thus, most clients possess many resources they think are altogether irrelevant. They are of the opinion that what they have to offer will not be of interest to others and that the things that really matter and count, the "hard currency," are good looks, a well-toned physical appearance, and a model-like appearance. Kindness, the ability to listen, being empathic, humorous, and reliable, and showing solidarity are not worth anything; these are a valueless currency, on the relationship market.

11.3.4 Therapeutic Strategies in Phase 4

Processing Schemas

Based on many years of experience, we can say that processing dysfunctional and normative schemas is the crucial part of therapeutic work when dealing with AvPD clients. Compared with clients suffering from other PDs, AvPD clients present barely any problems to therapists in building up a constructive relationship. They are not even involved in interaction games. Therapeutic problems which are characteristic of other PDs hardly occur.

However, the schemas of AvPD clients prove to be very resistant. They contain many hidden, implicit assumptions, are highly resistant to change, and clients with AvPD to some extent also show many affective schema components.

We suppose that one reason for the high levels of resistance change are the many self-fulfilling prophecies by means of which the clients themselves apparently affirm their schemas again and again. These serve to persistently explain that all situations where attempts to contact potential partners were not successful were due to their own, stable deficits. As a consequence and from the client's point of view, the schemas are again and again "empirically validated." The clients even formulate this in sentences such as "I look back on 20 years of experience of being unattractive," and they strongly believe in such assumptions. However, what they are really looking back on are 20 years of experience of self-fulfilling prophecies!

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These constant validations of, and reflections on, the reasons that have led to the failures are most likely also responsible for the schemas always being enriched with new, partly implicit assumptions. Clients develop more and more differentiations – for example, of "I'm not attractive":

- "I'm a bore."
- "I can't entertain women."
- "I have absolutely nothing to offer."
- "I'm physically too weak."
- "I'm a loser sexually."
- "I can't compete with other men."
- "I have no charisma, etc."

They also develop dysfunctional definitions such as

- Attractiveness is exclusively defined by physical attractiveness.
- Only physical attractiveness counts on the relationship market.
- Attractiveness is an absolutely defined construct: either you have it or you don't.
- Attractiveness is an invariable characteristic of a person.

As well as dysfunctional assumptions such as

- I must take the initiative for any relationship contact.
- On dates, I'm fully responsible for the conversation.
- If I make even the slightest mistake, or the potential partner will turn away.

Most of these assumptions are implicit. The client has them and believes in them, and the assumptions direct processing activities and behavior; however, the client is unaware of having these assumptions and for that reason is unable to express them, and when asked cannot name them.

In this context, OPRP has proven to be helpful (Section 3.2.4: Therapeutic Processing of Dysfunctional Schemas). It is very important to motivate the client to assume the position of the therapist and systematically change their perspective, step out of their frame of reference, and distance themselves from their assumptions. In this way, the client participating in OPRP may address and pursue questions such as:

- What proof is there – for example, of my unattractiveness?
- How has this evidence been obtained?
- To what an extent did my behavior produce this evidence?
- How coercive are my conclusions?

During OPRP, the client may also systematically analyze assumptions about situations – for example, the assumption „When, after all, I do succeed to ask a woman out for dinner, I'm sure I will not be able to entertain her appropriately, and she will think I'm boring and awful." In OPRP, the client may find out, for example, that

- They can definitely be entertaining in situations where it doesn't matter.
- They can talk about many topics.
- They are not solely responsible for the conversation.
- When starting talks, being particularly smart is not that important.
- Listening carefully may also be a sign of being interested in others without the need for having to actively take part in a conversation.

In the end, the client should realize they do not bring about a change of their situation when they refrain from taking the initiative, but instead they must be prepared to take the initiative themselves and be proactive. And the client must also realize that the risk of being rejected can never be ruled out completely. In OPRP, the client may discover that

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- Rejection doesn't necessarily mean their assumptions are true.
- There are many different reasons an offer is turned down.
- Many of these reasons have nothing to do with them.
- It is not true that the right person will be met at first sight.
- It is also untrue that only one of many attempts can be successful.
- Rejections are thus completely normal, must be expected, and are not to be overestimated.

The restructuring of these dysfunctional schemas is therapeutically highly important: Without changing the schema, a change in behavior cannot be expected.

Competence Training

If clients have systematically avoided social interactions for a long time, deficits in their competence must also be expected. If they do not practice the competence qualities they have, they will lose these qualities and will not develop new competence. However, in some cases and in the presence of certain comorbidities, clients do not or may not have any competence deficits at all. Therefore, therapist must carefully examine and analyze this.

For this reason, with AvPD clients, social competence training is often required. For example, the clients must learn

- how to make small talk,
- how to smile,
- how to flirt,
- how to make compliments,
- what to do or not to do in certain situations, etc.

In this case, appropriate role plays, including in groups, are indicated. There may also be a problem in that the client does not put into practice in everyday life what they have learned. It is thus important to make a deal with the client about how exactly this is to be put into practice, let them do homework, and supervise their compliance.

11.3.5 Therapeutic Strategies in Phase 5

Especially for clients with AvPD, the explicit transfer of what has been learned into reality is very important. The clients eventually must dare to approach and contact potential partners, speak with other people, engage in small talk, etc. It is exactly this that proves to be the crucial hurdle: Clients refrain from transferring new behavior patterns into their everyday life. It is thus important for the therapist to

- start setting homework early in the therapy (after initial success in OPRP);
- talk about the homework with the client and work out why they failed to do it (if they did);
- adopt certain motivation strategies to induce the client to do their homework

(see Sachse, Langens, & Sachse, 2012).

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Chapter 12

Passive-Aggressive Personality Disorder

12.1 Description of PAPD

12.1.1 Description and Empirical Results

The characteristics of **passive-aggressive personality disorder** (PAPD) are described by Benjamin (1993), Costa and Widiger (1993), Mahrer (1983), Millon (1993), Millon and Radanov (1995), Millon, Weiss, Millon, & Davis (1994), and McCann (2009), as well as Wetzler and Morey (1999). Empirical research on the DSM-IV criteria has shown that these criteria are valid to a great extent (Fossati et al., 2000; Hopwood et al., 2009; Rotenstein et al., 2007). Individuals with PAPD show a high degree of authority conflict (Vereycken et al., 2002) as well as a high degree of alexithymia (Nicolò et al., 2011) and an increased comorbidity with depression (Yanes et al., 2010). Some authors argue that the negativistic part of the disorder should not be considered as an integral component of PAPD (Hopwood & Wright, 2012).

PAPD was not included in the PD section of the DSM-5 because the evidence base was considered to be insufficient. The author of this present work finds this be a disadvantage as, although PAPD is one of the least encountered PDs in outpatient psychotherapy, it is one of the most challenging for therapists.

PAPD is a distance disorder: Individuals with this disorder or personality style are rather distrustful and keep their distance from other people; it takes longer for them to trust others (Sachse & Sachse, 2017).

The core of passive-aggressive personality disorder is a boundary problem: The person is afraid that others might cross their boundaries and do harm in their territory. These schemas of the kind "Others violate my boundaries" are usually the result of corresponding experiences in the client's biography. Important caregivers behaved transgressively, controlling the client's bedroom, reading their diaries, disposing of things without permission, etc.

If the person only had the schema "Others violate my boundaries," they could protect their boundaries openly and directly, which they do not do. The fact that they do not do this results from a schema such as "If I openly defend my boundaries, everything will get worse, the intruders will intensify their actions and resort to stricter controls." Thus, open or openly aggressive

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protection is out of the question. The defense used is rather, indirect, nontransparent, and consequently highly manipulative.

Two factors appear jointly in this: First, as a result of their schemas, individuals with PAPD are extremely sensitive and hyperaversive to all actions they interpret as transgressive. This may be the case with a question that the client thinks "is none of the questioner's business," or it may be an instruction given by a boss of which the person thinks "they have no right to do that," etc.

Second, as a result of the schema "It gets worse if I defend myself," the PAPD client is unable to distance themselves openly, but rather defends themselves in a way that they cannot be blamed for it. Accordingly, they realize two contradictory actions:

- They reply directly to the instruction from their boss – for example, saying, "Yes, sir, you will have the file by tomorrow."
- The next day they say, "I'm sorry, sir, I was just busy getting the file done when my husband called. He fell off the ladder! I wasn't able to finish the file."

This **indirect and nontransparent sabotage** quickly results in interactional problems because after a few such incidents, the person with PAPD will be viewed as

- unreliable,
- not showing solidarity,
- not trustworthy (or even as devious),
- a saboteur, etc.

Therefore, even a PAPD personality style may quickly lead to high interactional costs. Individuals with PAPD tend to see the resulting negative effects (which are mostly self-generated) as unfair, unjustified, impairing, etc., which induces them to complain and evaluate situations negatively, which is why the disorder is also called negativistic. It is also because of these actions that individuals with PAPD are regarded as rather unpopular. Others get tired of their lamentations, complaints, and accusations, especially because they highly tend to attribute the causes externally. It is always the fault of other people. With this kind of behavior, PAPD clients keep their distance from others.

12.1.2 Further Characteristics of PAPD

Millon (1996, 2011) describes further characteristics of PAPD as follows:

- Demands or requests on PAPD clients may easily cause massive reactance.
- Reactance primarily occurs in the form of passive resistance, which becomes manifest in a variety of actions and can be triggered in many different sorts of situations.
- PAPD clients complain of being devalued by authorities, wrongly judged, and badly treated.
- PAPD clients vigorously try to avoid being controlled by others.
- They keep their distance, are rather mistrustful, and rarely establish bonds.
- Their tolerance of frustration is low: They are quickly discouraged and disappointed, and tend to quickly react aggressively.
- They feel (great) envy of more successful persons and are very resentful.
- They are often under the impression others demand too much or expect too much, and this makes them angry.
- They tend to devalue or criticize other people they depend on or who are more powerful.

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- They take actions aimed at deceiving, confusing, or doing injustice to other people.
- They let others "fall into traps."
- They try to be unpredictable and inscrutable.
- They show a strong tendency toward nagging, cynicism, and skepticism.
- They very rarely praise others.
- They always like to say but.
- PAPD clients often have negative feelings.
- They find it very hard to bring themselves to a positive emotional condition.
- They often feel misunderstood and that they are badly treated by others.
- Their view of the future is pessimistic.
- They constantly think they must defend and justify themselves.

12.1.3 Prevalence and Comorbidity

The prevalence of PAPD in the population has been estimated to be 10.5% (Black et al., 1992). However, there are other studies that give figures of 1% (Lenzenweger et al., 1997; Samuels et al., 1994). Frequent comorbidities in clients with PAPD include paranoid personality disorder, AvPD, generalized anxiety disorder, and somatoform disorders (Millon, 2011).

12.2 Definition of PAPD, Based on Dual Action Regulation Model

12.2.1 Central Relationship Motives

The central motive of PAPD is the inviolability of one's own boundaries. Biographically, the clients have had a long experience with boundary violations: Mother reads diaries and letters and afterwards depreciates the daughter; father controls the bedroom and depreciates the son because of the sex magazines he finds. One client once told the author the following story: After her first day at school, she proudly returned home and could not find her stuffed animals anymore in her room. Asking her mother about it, the daughter was told that she had decided that she was now too old for stuffed animals, and they had all been disposed of. The client described this experience as highly traumatic, a massive boundary violation, ignoring her as a person, and a massive intervention and injury. However, this had only been one of many boundary violations in her life.

Clients with PAPD have usually experienced a variety of such incidents:

- Adults violated their boundaries when they were children.
- Adults used the information obtained to do harm: they depreciated the person, hurt them, disgraced them, etc.

Thus, the motive of seeking the inviolability of one's boundaries ranks high in the motive hierarchy: One's boundaries must be protected and maintained.

Another central motive is **autonomy**. Usually, boundary violations are linked with the restriction of autonomy in the client's biography and the using of controls, surveillance, paternalism, and prohibitions. The results today are a strong need for making their own decisions about their own life without any controls or restrictions imposed and a need to retain their own

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autonomy. High autonomy makes clients with PAPD highly **reactance prone**. Therapists must expect that all interventions the client might understand as restricting their freedom (which happens quite easily) will cause reactance.

The third central motive is **appreciation**. Boundary violations are often linked to depreciation signaling of "You're not ok." The consequence is a high need for appreciation and respect as a person. In the client's biography, boundary violators have used all of the information they have obtained within the territory of the client wrongfully, against the client:

- "In reading your diary I learned you are meeting X, although I ordered you not to."
- "You have sex magazines in your closet. You are a pig!" etc.

12.2.2 Dysfunctional Schemas

When treating PAPD clients, relationship schemas play a more central role than do self-schemas.

Self-Schemas

Clients with a PAPD usually show a deficient self-concept, with many self-doubts – for example:

- "Will I be able to accomplish my work?"
- "Am I good enough?"
- "Am I ok?" etc.

Self-doubts often lead to ruminating, concentration disorders, and work and performance impairments.

Relationship Schemas

A central relationship schema stemming from the biographical experiences of PAPD clients is "Others don't respect my boundaries" or "Others are violating my boundaries." Due to these schemas, PAPD clients expect boundary violations; they are sure these violations will occur and, consequently, are extremely sensitive to everything that could be interpreted as a violation. In contrast to paranoid individuals, PAPD clients start from the assumption "Others are not mindful of me or don't respect me" rather than "Others want to harm me." Due to their extreme sensitivity in this respect, the clients often mistake completely harmless interactions of partners for boundary violations which the partners are unable to understand, and they react highly aversively to that perceived violation.

Another schema can be described as "Others will restrict my autonomy." On the basis of this schema, clients with PAPD also react hypersensitively in that they may understand a totally harmless demand as a restriction of their autonomy and show massive reactance in response. A client with PAPD once told the author that he simply could not watch a movie if someone told him before to "make sure you watch that movie, it's great." This remark alone would always cause reactance in him so that he was unable to follow the recommendation. Although the client found his own reaction ridiculous, he could not change it.

The third relevant schema is "I don't get appreciation" or "I will be devalued" or "whatever I say or do can be held against me." In this context as well, clients with PAPD are highly sensitive. They easily understand objectively meant criticism as depreciation. The schema also induces clients, even in therapy, to make attempts to show as little as possible of themselves.

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In their biography, clients with PAPD have had the experience that directly defending their own boundaries or their autonomy is dangerous. Actions to this effect were punished by parents, resulted in an aggravation of conflicts, or in long-lasting relationship impairments. Parents refused to communicate and resorted to depreciation, rejection, even beatings, confinement to one's room, etc. Consequently, the clients were led to assume "I cannot defend my boundaries openly" or "Open opposition worsens the problem." Assumptions of this kind are the basis for the development of **passive strategies:** If someone cannot defend themselves openly, they must defend themselves nontransparently, indirectly.

On the open communication level, the PAPD client is friendly, cooperative, and approachable. If the boss asks for something the person regards as a boundary violation or autonomy restriction, the person remains friendly and approachable. On the indirect level, however, the client will sabotage the job and refuse to do what has been requested. This strategy is called aggressive because it is to be viewed as a kind of sabotage which harms the partner and therefore has aggressive aspects. It is called passive because the sabotage is an indirect, concealed, nontransparent act for which the client is unwilling to assume responsibility. However, under certain conditions, the PAPD client may also be very actively aggressive, in that they yell, rage, and attack others directly.

However, it is important to understand that there is a moderator variable which determines whether a client with PAPD is **passive-aggressive** or **active-aggressive**: This moderator variable is the client's status relative to that of the interaction partner. If the client thinks the interaction partner (who apparently violates boundaries) has a higher status than themselves (and thus is potentially dangerous), the client will adopt passive-aggressive strategies of being openly submissive and, subliminally, attempting sabotage. If the client thinks they have the same or a higher status than the interaction partner (who is thus regarded as nondangerous, nonintrusive), they may indeed act very actively aggressively. Thus, clients with PAPD are absolutely capable of defending themselves openly.

For therapy this has an almost paradoxical consequence: As soon as the PAPD client starts attacking the therapist openly, this is an indication that the therapist–client relationship has significantly improved. The therapist may even be glad that the client shows this behavior. It is important that the therapist does not understand this behavior

- as personal depreciation,
- as an offense,

and that the therapist reacts

- in a relaxed manner.
- with empathy and understanding.

Due to their biographical experience, PAPD clients often have an additional schema "I'm treated unfairly" or "I'm being impaired or I'm aggrieved by others, and that is unfair." Sometimes this even turns into "I begrudge others that everything comes naturally to them" or even "I want to get my revenge for what has been done to me." These schemas are the basis of nagging and depreciation: The person is dissatisfied with their fate and demonstrates this constantly through negative statements, depreciation, expressing dissatisfaction, etc. The person begrudges others any kind of success or an easy life. They thus create an aura of dissatisfaction around them.

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12.2.3 Compensatory Schemas

Normative Schemas

Essential schemas in PAPD are:

- Protect your boundaries!
- Don't permit any kind of boundary violation!
- Only let very "safe" people into your territory!
- Don't disclose anything about yourself!
- Don't protect your boundaries openly!
- Protect your boundaries in a way that you can't be held responsible for it!
- Avoid open confrontation!
- Avoid criticism and depreciation!
- Avoid being controlled and restricted!

These norms represent avoidance objectives to a very great extent. The attention of PAPD clients is strongly focused on such avoidance behavior.

It is thus not surprising that this disorder is also called negativistic: PAPD clients are concentrating on things that do not work, on what fails, and not on what is going smoothly! For that reason, they rarely make positive statements, they rather nag (which does not make them particulaly attractive to interaction partners!).

Rule Schemas

Individuals with PAPD have stringent rule schemas, such as

- "I expect that others will accept my boundaries!"
- "I expect that others will not violate my boundaries!"
- "I expect that others don't try to control or restrict me!"

On the basis of these rule schemas, PAPD clients also assume they are entitled to punish rule violators for having violated those rules. However, whether they react openly aggressively depends on the moderator variable: The estimated threat potential of the rule violator (see the subsection Relationship Schemas, above). If the rule violator is classified as "dangerous," then the PAPD client will react passively aggressively; if the rule violator is classified as less dangerous, PAPDthey may react openly aggressively. The therapist should thus bear in mind that PAPD clients can indeed behave openly aggressively under certain circumstances.

12.2.4 Manipulation

General Remarks

Clients with PAPD are highly manipulative and in this context they keep others at a distance. They protect their boundaries, their autonomy, in a way even their identity, by adopting manipulative strategies, because they may even fear that boundary violations and autonomy restrictions may eventually lead to an impairment of their identity.

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PAPD clients are especially manipulative in performing acts of sabotage in a way that they cannot be held responsible for. It may well be that they invent stories and use excuses that are absolutely untrue. Thus, one may expect that PAPD clients will tend to diverge from the truth to a greater or lesser extent.

Most PAPD clients have at least some idea that they behave manipulatively; many of them are indeed aware of their strategies. Some even know the costs they are incurring; however, their boundary protection is so important to them that they stick to their strategies despite those costs.

Images and Appeals

One image that PAPD clients use is "I'm cooperative" and "you can rely on me." Furthermore, they use images such as "you can talk about anything with me" and "you don't need to argue with me." However, it will not take long for interaction partners to realize that the clients do not act or behave in line with these images. Clients with PAPD send their appeals rather indirectly through their actual behavior. The following examples are the appeals that are indirectly communicated:

- "Hold your distance!"
- "That's not your business!"
- "Don't give any instructions to me!"
- "Leave me alone!"
- "Respect me!"

Interaction Games

Preferred interaction games for PAPD clients include

- *Poor sucker game* (see the subsection Images, Appeals, and Interaction Games, in Section 9.2.4): Presenting oneself as weak, helpless, in need.
- *Heroic poor sucker:* "I'm weak and helpless, but I'm fighting against it in an almost heroical manner."
- *Always me*: "I've been strongly impaired and am still impaired by others. I always get the short end of the stick so I'm allowed to complain vigorously."
- *Victim of other people or of circumstances:* "I'm impaired or treated unfairly by other people or by circumstances."
- *Martyr:* "Despite all impairments and adversities, I have fought bravely, struggled through, and done well. I have suffered for my convictions and beliefs and managed to stand my ground."

All of these games blame others or fate (the fault lies with others, others are unfair, etc.), and these give sufficient cause for persistent nagging; and they prompt others to assume responsibility and care for the PAPD client, and may even prompt others to admire and praise them. The PAPD client thus avoids criticism and dissatisfaction, and instead attracts attention, gains affection and appreciation, and protects themselves against unreasonable demands and encroachments. These games are very effective but at the same time highly manipulative as well.

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12.2.5 Tests

Tests are performed in a rather indirect way: PAPD clients will address certain questions to evaluate interactions, such as:

- Does the interaction partner exert pressure on me?
- Do they give instructions that violate my boundaries or restrict my autonomy?
- Do they respect me as a person?
- Do they remain friendly and approachable even if I withdraw?

Interaction partners will not pass these tests when they (massively) violate boundaries or behave disrespectfully, and they will not therefore succeed in establishing a relationship with the client who suffers from PAPD.

During the therapy process, tests may also be made through sabotage or directly aggressive attacks. For example, after a few therapy sessions, a client of the author's suddenly reacted very aggressively to a totally harmless questions, by saying, in this case, "It's always the same, you go to a therapist in good health and when you are leaving, you are sick!" The author kind of felt like a Roman in the hands of Obelix. However, it could be seen that this would be part of the problem and so it was important to be able to stay calm, explain the question, remain friendly and approachable, and thus pass the test (see also the subsection Dealing With Tests, in Section 12.3.1, above).

12.2.6 Characteristics of PAPD

Proximity, Distance, and Bonding

Clients with PAPD keep their distance from people unknown to them. This is some sort of distance for the sake of safety, as people who are kept away can hardly violate boundaries. This distance will be maintained until proof has been established that these people will not violate those boundaries, or restrict the PAPD client's autonomy, etc. If interaction partners have proven this, they are allowed to come a bit closer. In fact, clients with PAPD may even permit very close bonds and stable relationships, but this will only happen with a few, carefully selected interaction partners.

All of this also applies to the therapist who may establish a trustful client–therapist relationship if they succeed in convincing the client that they

- will not violate the client's boundaries;
- will accept the client's autonomy;
- will not force, induce, or talk the client into something;
- will not put pressure on the client;
- will not do anything the client does not want.

Ego-Syntony, Perspective, and Avoidance

Clients with PAPD are often aware of the costs of their behavior, but they still think they are necessary because of the protection they give. However, all told, the disorder is only moderately ego-syntonic and significantly more ego-dystonic than dependent PD, for example. The interactional problems the therapist faces at the beginning are rather that the client perceives all interventions as boundary violations.

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PAPD clients show a rather external perspective as they must always scrutinize their interaction partners. However, in the experience of the author, their perspective can be internalized much more easily than is the case for HPD clients.

At the beginning of therapy, avoidance by the client involves disclosing as little as possible of themselves; thus, the client will not really avoid certain topics and contents but rather any personal information of relevance. With increasing relationship credit, however, this tendency decreases.

Costs

Many clients with PAPD know about the high costs of their system. They know that colleagues and partners react angrily. However, they continue with their strategies because they do not know of any alternatives and especially because giving up these strategies appears to be threatening. The indirect costs may also be very high – for example,

- Lack of self-confidence
- Problems at work
- Fear of failure
- Examination and evaluation anxieties
- Concentration disorders
- Brooding
- Depressive episodes, etc.

Clients with PAPD often come into therapy because they perceive they stand in their own way with their strategies. They realize, for example, that they suffer job impairments due to their constant acts of sabotage, annoy colleagues with their unreliability, and may strongly annoy partners as well. Accordingly, they are subject to negative feedback, encounter conflicts in relationships, are avoided and depreciated. Sometimes clients realize the correlation between their behavior and the costs incurred; however, they also may occasionally pretend that colleagues are mobbing them.

Clients with PAPD have a relatively poor self-concept. They have little trust in themselves, avoid failures, do not accept challenges, ruminate about possible failures, etc. As a consequence

- They avoid important work.
- They suffer from examination anxieties.
- They shirk work which is subject to assessment.
- They have difficulties concentrating and suffer from performance anxieties, etc.

All of these aspects of their disorder induce them to undergo therapy. They want to train their concentration etc., which in fact would be of no help to them.

Therapists who behave correctly observe that when therapy starts, these clients are difficult to deal with, but are found to be very cooperative as soon as a good therapist–client relationship has been established. It is important for therapists to understand the interaction problems to be part of the client's problem: The client does all this because they are unable to act otherwise and not because they want to anger the therapist! As soon as the therapist has recognized this, it will be easier for them to stay calm and friendly and be attentive in the event the client does not cooperate, tries to sabotage, or reacts aggressively.

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12.3 Therapeutic Strategies

With a view to working constructively with PAPD clients, therapists should take a certain basic attitude which will allow them to avoid mistakes and avoid triggering schemas. This basic attitude is not only essential for PAPD clients but rather for all clients who suffer from distance disorders. This attitude can be described as follows:

- *Be patient:* The client will not allow the therapist to exert pressure, and thus the process cannot be forced. The therapist operates under the motto "I do my very best" and accepts that the process will take as long as it takes. The client may circumvent the therapist's strategies for a long time, which should be okay for the therapist.
- *Be very respectful:* The client will be respected, appreciated, and accepted by the therapist. The client are allowed to be as they are, and the therapist only points out that
  - they are incurring costs;
  - if they want to reduce those costs, they may want to follow the therapist's suggestions;
  - if they do not want that, it's ok.

The therapist highlights aspects, questions constructs, etc., but always maintains the attitude that the client may change, and they think the client is capable of changing, but the client need not change. This involves *giving a lot of control to the client*: If the client signals "this far and no further," the therapist accepts this; if the client thematizes superficial topics, then the therapist accepts this but always tries to reach for the "edge of what is possible" to find out what the client is prepared to accept at the moment. Further, the therapist must not react as if triggered: If the client gets aggressive, the therapist understands this as an aspect of the disorder. They should not react as if triggered or offended but rather remain friendly, caring, respectful. and accepting of the boundaries the client has set.

12.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

When treating clients with PAPD, complementary therapeutic behavior at the motive level means to avoiding boundary violations, autonomy restrictions, and any disrespectful behavior. Here, for example, it is significant that the therapist *never* puts pressure on the client. They may, for example, say, "It's completely up to you how quickly we proceed in therapy. You decide on a topic to be worked on. If you don't want to work on it, we will stop it."

If the client attempts any kind of sabotage, the therapist may say,

> It is totally okay if you don't want to work on this point. It is your time, and you spend it as you like. If you come here and want to keep silent throughout the session, it is okay for me. I'm here, I stay here, and if you want to talk about something, we talk and if you want to be silent, we are silent.

The therapist emphasizes that it is solely the client who says how the session is to be spent. However, in such a case, they must, of course, persevere. It may happen that the client indeed tests the therapist and keeps silent for one or two sessions, in which case the therapist should sit near the client in a relaxed manner, should not observe the client, should follow their own

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thoughts, and be ready if and when the client decides the work should go on. In the experience of the author, the client will terminate such a test after two sessions at the latest.

The therapist formulates their interventions as proposals and makes everything transparent: They briefly explain why they ask a question, what they are aiming at with an intervention, etc. Transparency! Transparency! Transparency! The therapist does not make lengthy explanations, only gives short statements, but a lot of them.

Openness to objection is essential as well (Fiedler, 1981). The therapist says, „If you have any questions, please ask them right away. If there is anything you don't want, please tell me right away. If you have any criticisms, please express them right away." Then the therapist addresses questions, criticisms, and objections very empathically, in a friendly and caring way, and meets the wishes of the client to the greatest extent possible.

The therapist should also point out the following:

> When I ask questions or make interventions, it is very important for me to respect your boundaries. But since I do not have any telepathic capabilities, a boundary violation may nevertheless occur with one of my interventions. Please let me know if this happens. I will make sure to observe it.

This procedure has proven to be successful with PAPD clients. The client will get the impression that

- The therapist takes them seriously, greatly respects them, deals intensively with their issues.
- The therapist strives to respect boundaries and autonomy.
- The therapist does not put pressure on the client.

We call this strategy **reactance relief,** as it probably reduces the client's reactance greatly. The therapist must deal with sabotage, criticism, etc. in an accepting, friendly, caring, and respectful manner (otherwise they should not work with PAPD clients). The therapist shall depreciate the client, be sarcastic or upset, or exert pressure. The therapist must understand that the client does all this because they have no alternative – because it is part of the problem and not because they want to anger the therapist.

Explicating Relationship Motives

When explicating the relationship motives, it is important to make the negatively expressed objectives transparent to the client so that they know which objectives they are actually trying to reach with their behavior. It is important to emphasize that they pursue negative objectives and avoidance objectives, and explain the consequences these objectives entail.

It is also essential to explicate that they also have positive motivations. The client should be aware that they have a strong desire for others to respect their boundaries and that there is no need for them to protect them; what is more, respecting boundaries should be regarded as an interactional matter of course. It should also be clear that clients have a strong desire for others to respect their autonomy – most of clients even wish that interaction partners foster their autonomy – and that clients wish to be respected by others as a person.

These insights are very important for PAPD clients because in the therapy process, the therapist can make perfectly clear that the client, by adopting their sabotage strategies, will achieve

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quite the opposite of what they actually wanted; in particular, they lose the respect they eagerly desire, because their behavior often brings others to simply classify them as "assholes." In this way, the client's attention will be directed to quite different cost aspects, so they can see they make self-fulfilling prophecies.

Dealing With Tests

As has been said (see Section 2.6: Tests; see also Section 12.2.5: Tests), therapists can deal with tests best by

- understanding tests as being part of the problem,
- accepting and respecting the behavior of the client,
- trying to understand what the client means and wants,
- trying to accommodate the client as far as possible, and if necessary
- commenting on and making their own behavior transparent.

Fewer tests will be made as the therapist's relationship credit increases. Tests used by PAPD clients are never meant as personal insults, so that most therapists can deal with them very well.

Resource Activation

When treating PAPD clients, a systematic resource activation is essential: The therapist can emphasize that the client in fact is quite competent to protect their boundaries openly and directly and is also capable of dealing with greater social pressure resulting from boundary protection. The therapist makes obvious that they are positive the client has the required competence and offers the client help in improving their competence level even further through training.

12.3.2 Therapeutic Strategies in Phase 2

Establishing a Motivation to Change

The strategy of making the game structure transparent can only be followed if the client has complete trust in the therapist. The client must allow the therapist to make such interventions. When this requirement has been met, the interventions are not that problematic anymore, provided the therapist makes them in a very accepting, empathic, and objection-enabling manner. As most PAPD clients are aware of most of their structures, interventions aimed at obtaining transparency will not be unduly confrontational.

Confrontation With Costs

Although the costs of their disorder are quite clear to PAPD clients, the therapist may again make associated costs salient and especially explain the correlation between behavior and costs. What is also important here is to refrain from being reproachful and to always show respect: "If you want to change something, it would be good for you to see aspects where changes could reasonably be made."

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Confrontation With Intentions

If a therapist confronts a client with their intentions – for example, with their intention to protect their own autonomy, the therapist should always proceed as follows:

1. Make it perfectly clear the therapist understands that this is important to the client (and why it is important to them).
2. Provide elucidation for the client by pointing out the costs they incur, costs they do not want and that in fact contradict their own intentions.

The client should not be confronted with the therapist‘s standards but with their own: If the client wishes to leave everything the way it is, they may do so. But if they want to make changes, they may readily work on those changes together with the therapist.

Confrontation With Games and Manipulations

These confrontations are very important for clients with PAPD, nevertheless, they are difficult to realize, and the therapist can only make them if enough relationship credit is available. The clients must be led to understand why they play these games and that these stem from schemas that should be examined. And the clients have to realize these games are very cost-intensive over the long term: They alienate interaction partners, especially because of the massive negative and indictment character of the games.

12.3.3 Therapeutic Strategies in Phase 3

Usually, the clarification of schemas can only begin when the client trusts the therapist – that is, when the therapist has gained relationship credit. Before that, any explication may be misunderstood as a massive boundary violation. Therefore, the therapist should decide carefully about when to start the explication of schemas. It is relatively important to combine the explication of schemas with biographical work: The client should realize as soon as possible

- that they were exposed to pathogenic conditions,
- where they had to develop certain strategies to survive,
- that these strategies are totally understandable and appropriate,
- but that they can now check whether these strategies are needed any longer,
- and whether these are still useful now.

In this context, the therapist can be very empathic and may concede that it is not at all defamatory to have developed such biography-related strategies.

12.3.4 Therapeutic Strategies in Phase 4

Through cognitive techniques or OPRP, it shall be pointed out to the client that

- They no longer have to cope with the interactional situations they experienced as a child.
- Now people as a rule are not violating their boundaries the way they did before.
- And most notably, now they are much more competent and resourceful in defending their boundaries and autonomy.
- Now they are able to effectively defend themselves and thus no longer need to use non-transparent strategies.

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- Openly and directly defending themselves no longer entails desperate situations, but they can handle conflicts properly now.
- And they may now resort to and use open strategies for protection.

12.3.5 Therapeutic Strategies in Phase 5

A transfer into everyday life is difficult for PAPD clients, as actions and behavior to be modified are rather inconspicuous. It is especially important to propose that the client does concrete practice that requires openness. They should learn to say no openly and openly explain why they make a refusal; they should tell partners openly what upsets them and in doing so stay friendly and caring.

Therapist and client can go through important situations in important contexts and develop concrete alternative behaviors; the client should then look for these situations and realize the new behavior. Not only is the behavior essential, but an appropriate revaluation and good alternative cognition are as well.

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Chapter 13

Schizoid Personality Disorder

13.1 Description of SzPD

13.1.1 Description

Severe **schizoid personality disorder** (SzPD) presents as conspicuous and serious; nevertheless, Yontef (2001) thinks that a minor schizoid personality style is widespread. Almost everyone, so he thinks, has facets of this. Empirically, several characteristics of SzPD have been proven to be valid (Harper, 2004; Katz, 2004; Martens, 2011; Mittal et al., 2007; Nirestean et al., 2012; Rasmussen, 2005; Yan et al., 2011):

- a deficiency in bonding with other people;
- low involvement in social contacts;
- deficient social competences and deficient abilities to understand the behavior and emotions of others, a high extent of anhedonia, and difficulties in experiencing pleasure and joy.

Nonetheless, individuals with SzPD show a strong desire for proximity (Thylstrup & Hesse, 2009), and they show an increased comorbidity with depression (Bockian, 2006d; Skodol et al., 2011). Some authors hold the view that severe patterns of SzPD are hardly distinguishable from Asperger's syndrome (Attwood, 2007; Mittal et al., 2007; Wolff, 1998, 2000).

Due to their schemas, individuals with a schizoid personality disorder assume that "Relationships are not worth it" and that relationships are rather dangerous as well. As a consequence, they establish a believe system that they do not need others: In this way, they create a strong self-ideologization of the kind "I can get along well by myself" (see Sachse, 2014f).

However, it is a natural fact that all people need relationships – albeit to a different extent, there is nobody who does not need any relationships at all. Therefore, apart from self-ideologization ("I don't need anybody") there is also a (more or less strong) desire to participate in relationships, but this is (more or less successfully) blanked out of consciousness. Often clients realize this desire themselves as therapy takes its course. It is thus untrue to assume SzPD clients do not need any relationships and are not interested in social contacts and sex; they are, however, rather successful in convincingly making others and themselves believe this.

Individuals with SzPD establish few social contacts and as a result they usually lack social training; therefore they often have major deficits in social competence. Like avoidant individuals,

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their deficits are so huge that they are even unaware of social rules and do not understand social interactions (Millon, 1996), which in turn strengthens their avoidance behavior.

They establish social contacts when they have to, but in doing so they appear awkward, sometimes even arrogant (which is mostly a misinterpretation of their uncertainty). Their social behavior is partly indicative of these deficits, but also indicative of their strategy to hold others at a distance. They may enter interactions but often do not show affective expressions. They do not smile, do not respond to the smiles of others, show no facial expressions, only a few gestures, etc. To their interaction partners, they appear like zombies, which irritates those partners and leads them to avoid the SzPD individual.

Most of the activities they are involved in, they do on their own. Even if they are in a relationship, it is important to them to be frequently active without the partner participating; they do not perceive being alone as aversive or as loneliness, and hate collective activities like school excursions, office parties, or even mass gatherings, etc. Individuals with a schizoid personality style have contacts and relationships, but they enjoy being alone; they only have a few friends and even if they have good social competences, they still do not like events where on the average more than four people are present, especially if they do not know them. They observe others who are in lasting relationships and perceive they enjoy it; this may cause a diffuse desire and diffuse dissatisfaction.

13.1.2 Prevalence and Comorbidities

The prevalence of SzPD is estimated to be 1% of the normal population (Samuels et al., 2002; Torgersen, 2009; Weissman, 1993). Frequent comorbidities of SzPD are AvPD, DPD, obsessive-compulsive disorder, and anxieties (Millon, 2011).

13.1.3 Further Characteristics of SzPD

Some characteristics of SzPD have been proven to be valid empirically (see Ahmed et al., 2012; Carrasco & Lecic-Tosevski, 2000; Esterberg et al., 2010; Grant et al., 2004; Kalus et al., 1993; Kosson et al., 2008; Kramer & Meystre, 2010; Pérez-Álvarez, 2003; Triebwasser et al., 2012; Wiggins & Pincus, 1992). Persons affected by SzPD

- are in want of bonds with other persons,
- show little commitment with respect to social contacts,
- often show insufficient social competence and lack of an ability to understand the behavior or the emotions of other people,
- show a high degree of anhedonia and have difficulty in feeling joy

(Harper, 2004; Katz, 2004; Martens, 2011; Mittal et al., 2007; Nirestean et al., 2012; Rasmussen, 2005; Yan et al., 2011).

However, persons with SzPD nevertheless feel a strong desire for proximity (Thylstrup & Hesse, 2009), and they have an increased comorbidity with depression (Bockian, 2006d; Skodol et al., 2011). Some authors hold the assumption that severe forms of SzPD can hardly be distinguished from Asperger's syndrome (Attwood, 2007; Fitzgerald & Corvin, 2001; Mittal et al., 2007; Remschmidt, 1997; Tantam, 2000; Wolff, 1998, 2000).

As a result of their schemas, SzPD clients assume that "Relationships are not worthwhile" and also that relationships are rather dangerous. And based on this assumption, they create a belief system for themselves that they do not need other people: In this way, they produce a very strong, self-deceiving concept of the sort "I'm doing quite well on my own" (Sachse, 2014g).

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Millon (1996, 2011) and Nirestean et al. (2012) specify further characteristics as follows:

- SzPD persons show energy and vitality to a minor degree.
- They keep in the background in social situations.
- They are hardly at all interested in the life of others.
- Their social interactions are formal and functional.
- They lack social competences.
- They are hardly at all interested in social activities; they prefer to be on their own.
- They demonstrate a low level of introspective ability.
- Their emotional expression and emotional experiences are greatly reduced.
- They have difficulties in seeing things from the perspective of other people.
- They have difficulty distinguishing relevant from irrelevant social stimuli.
- They experience major difficulties in describing themselves, and their intentions and preferences.
- They are hardly able to differentiate or perceive their own and others' emotional states.

13.2 Definition of SzPD, Based on Dual Action Regulation Model

13.2.1 Central Relationship Motives

One may assume that clients with SzPD have met with **emotional neglect** in their biography. For example, an SzPD client once told the author that he had grown up in a family with two siblings, but that there had been barely any contact between the family members. There was no physical contact, no affection, barely any emotional devotion, barely any praise and appreciation, and barely any communication. The parents cared for everything: food, clothes, nannies, private tutoring, etc. However, they devoted hardly any care to their children. Everybody in the family was on their own; everybody had to care for themselves. The atmosphere was cold and frosty, expedient and not affectionate.

Clients with a SzPD are therefore in great need of **appreciation**: the need to be viewed and defined as positive and to be appreciated as a person. Another motive is **seeking after importance**: the need to play a crucial role in the life of another person, to mean something to another person. There is also a strong need for **reliabilit***y,* to be able to rely on others, and have a stable and lasting relationship. And there is a strong need for a **supportive relationship** to get help and support from others.

The SzPD client mentioned here had completely blanked out all of these motives from their consciousness for reasons that will be described subsequently. These clients do not even know they have these needs. In this context, DSM-5 (APA, 2013) assumes that these clients have no need for relationships. However, if the clients in the course of the therapy regains access to their motives, their strong need will again be brought to the surface. It is the clients' assumption that they do not have these needs, and they establish an image of the kind "I don't need other people." Therapists should never believe these images. They are constructions, which serve as self-protection. And they are wrong: These clients in actual fact have strong motives concerning relationships. In the therapy process, it may take a while until the SzPD client is able to access their motives again.

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13.2.2 Dysfunctional Schemas

With SzPD clients as well, relationship schemas are of greater importance than self-schemas.

Self-Schemas

In their biography, SzPD clients have not been subject to direct depreciation, but rather they lacked appreciation. It can be assumed that schemas have developed such as:

- "I don't know what I'm able to do."
- "I don't know what I mean to others."
- "I don't know what I can offer others."
- "I don't know whether I'm attractive."
- "I don't know how to deal with others."

That means the self-schema in all probability is subject to feelings of doubt and uncertainty, and this does not make it easier for the person to establish relationships, even if there are no other problems.

SzPD individuals systematically avoid social interactions and must cope with great social competence deficits, as well as with deficits concerning their knowledge of social rules. To some extent, they are aware of this, which adds to their social uncertainty: "I can't approach others because I don't know how to behave."

Relationship Schemas

A central schema for SzPD clients is "Relationships are unfavorable and not conducive to well-being. There are assumptions such as:

- "One doesn't get appreciation in relationships, there is no affection."
- "I'm not important to others in relationships, I don't play a crucial role."
- "Relationships are not reliable; relationships are at best alliances of convenience."
- "No one will help in relationships, you can't really rely on others."
- "Relationships are altogether cold, strenuous, unfriendly, and useless."

From this, clients are induced to make other assumptions such as:

- "I can only rely on myself."
- "I get by very well on my own."
- "Avoid relationships, as they are cold, empty and frosty."
- "In relationships I feel unwell, forlorn, and alone."
- "Stay alone and care for yourself."

As a consequence, something emerges that one might call an **escape into autonomy.** In fact, clients with a schizoid PD do not really need autonomy; they have no real desire for autonomy. They rather choose autonomy because there is nothing else they can do. Relationships are rather aversive; partners cannot be relied on in relationships; relationships are cold and unfriendly; one will not get any help and support in relationships. Accordingly, living on one's own and only relying on oneself is the most reasonable and best thing to do. One has to be autonomous; any other alternative is clearly insecure and unpleasant.

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Apart from this, SzPD clients also adopt the **sour grapes strategy**. As relationships are assumed to be pointless and negative, but the clients in actual fact have the desire for a relationship, this dilemma is circumvented by establishing an image

- "I don't need relationships."
- "I get along by myself"
- "I don't want relationships."
- "I want to stay alone."

This gives rise to the typical image of a client with SzPD: The cowboy who lives in Utah's Monument Valley with horses and dogs, and says, "Come to where the flavor is." This makes the image perfect.

13.2.3 Compensatory Schemas

Normative Schemas

Essential normative schemas are:

- "Only rely on yourself!"
- "Avoid being dependent!"
- "Avoid being controlled!"
- "Avoid contacts as much as possible!"
- "Don't give anything away of yourself!"

Rule Schemas

Usually, SzPD clients do not have distinct rule schemas. Relevant schemas may include:

- "I expect from others that they keep their distance."
- "I don't want to be restricted or patronized."

13.2.4 Manipulation

The extent of manipulation by clients with SzPD is less pronounced, because they avoid contact and thus barely use (or learn!) manipulative behavior. One may also assume that manipulative strategies were not very effective in the biography of these clients: They were hardly able to exercise control over their environment so that manifest manipulative strategies could never develop. The only strategy open to them is to hold others at a distance – for instance, by reducing their nonverbal behavior systematically. But even this strategy is barely manipulative, as it is pretty much transparent. Interaction games are barely played at all.

13.2.5 Tests

The only test conducted by SzPD clients is to find out whether the therapist understands them. They intend to find out:

- Can therapists see things from my perspective?
- Do they understand my motives and behavior?
- Do they remain caring and friendly despite my behavior?

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- Are they able to comprehend what I am experiencing?
- Do they give help and support reliably?

In this respect, the whole distant interactional behavior of the client is a test:

- Does the therapist also think I'm arrogant?
- Does the therapist also keep their distance?
- Does the therapist also waive attempts to make contact?

13.2.6 Characteristics of SzPD

The Inner Dilemma

Clients with SzPD mostly live alone; they go on vacation alone, choose jobs which have little to do with social interactions, and they avoid people. They often go into therapy because of problems with their jobs, concentration disorders, depression, and sometimes anxieties. In the therapy process it becomes obvious that the image "I don't need anybody" is a facade. A SzPD client of the author's once explained how he "perceived his existence": He stands near the end of an ice cave; the cave is open at the front but surrounds him on all sides and continues toward the rear. He stands at the cave's edge still inside it and is able to look outside. What he sees is a sunny meadow where many people play. He would like to go outside and join these people but is afraid to leave the cave. Although it is cold, the cave provides protection and safety. On the other hand, he cannot retreat any further into the cave as this would mean losing contact with the outside altogether, which he does not want. Thus, he keeps his position standing rooted to the ground at the cave entrance unable to go outside and unable to retreat any further into the cave.

This picture nicely illustrates the dilemma of SzPD clients: There is a strong need for contacts, for relationships, but all that is very insecure, unpredictable. Aloneness provides protection and safety but is also cold. A solution cannot be found, everything must stay as it is.

The clients show a high degree of alienation: They have barely any access to their emotions and no access to their motives and needs. Instead, they show a high degree of **self-ideologization**: They effectively deceive themselves with images.

Proximity, Distance, and Bonding

Due to their schemas, SzPD clients do not allow for any proximity, and consequently, they hardly form any bonds. At best, they may allow a friend to get a little bit closer. Their main interactional strategy is to maintain a distance, and they do this especially by completely putting their nonverbal interactional behavior on ice: They do not smile, they maintain a straight face when talking, do not use gestures but sit rigidly, do not show any emotions. Usually, this interaction behavior scares others away; the SzPD individual appears to be aloof, standoffish, arrogant, and unapproachable, and thus causes others to react accordingly. They also keep their distance, do not make relationship offers, and stay away from the person.

In fact, the whole emotionality of SzPD clients is reduced to an absolute minimum: Everything is only expedient, one does not feel any needs and allows no proximity. One does not feel anything – no anger, no joy, nothing. Everything is gray. As a consequence, SzPD clients turn

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into a "living sleeping pill": They are unable to entertain, be cheery, or charming; do not have charisma; and consequently are totally unattractive. Even many therapists are irritated by the SzPD client's interaction behavior, in that they react by minimizing their nonverbal behavior as well. They do not smile, they keep a straight face, they do not make gestures, and they behave like a mummy having a therapy mission.

Ego-Syntony, Perspective, and Avoidance

SzPD is in fact highly ego-syntonic if clients are capable of avoiding the representation of their relationship motives. The more the clients are aware of their needs, the more ego-dystonic the disorder becomes. The increasing representation of their needs in therapy can therefore make the disorder highly ego-dystonic.

Whether the client assumes an internal perspective depends on the extent of their ego-dystony: If the disorder is very ego-syntonic, it may be very difficult to internalize the perspective. Later on in the therapy process, it will be more and more easy for the client to focus their attention on internal processes. However, due to their high alienation, it will still be difficult for the client to gain access to their emotions and needs.

In the early stages of the therapy when the client does not yet trust the therapist, they barely talk about relevant contents; the extent of avoidance is high. However, as mutual trust is developed, the extent of avoidance decreases distinctly. As soon as the therapist has accumulated relationship credit, the client starts working on relevant issues.

Costs

At the beginning of therapy, clients with SzPD are not aware of the costs they incur from their disorder; however, these costs do become salient over time. Slowly the client is capable of perceiving the sour grapes strategy they have adopted to make their situation bearable.

13.3 Therapeutic Strategies

13.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

With SzPD clients, complementarity at the motive level primarily means that the therapist approaches the client in a friendly and empathic manner. This also implies that the therapist endeavors not to restrict their nonverbal behavior: The therapist keeps smiling even if the client does not smile; the therapist remains friendly even if the client keeps a straight face; the therapist is still interested in the client and their statements even if no relevant information is given; the therapist underlines their statements with gestures even if the client sits totally rigid. Through their nonverbal behavior, the therapist can show the client that

- They are and remain friendly and empathic.
- They are still interested and respectful.
- They are not scared away, will not retreat, do not think the client is arrogant – in brief: They are not behaving like other interaction partners.

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In doing so, the therapist signals on all communication channels that they respect and appreciate the client and are interested, loyal, and reliable. In this way, they are behaving complementarily to the most important relationship motives.

It is especially important for the client to experience that the therapist intensively strives to understand the client and tries to empathize with the client's perception and processing. The client should also experience that the therapist really understands them. And in this context, it is important that the therapist verbalizes and explicates a great deal of what they do understand. That they actively signal to the client that they understand what autonomy means for the client, how the client feels about their decisions, and the conflicts and dilemmas the client faces. Understanding and sympathy probably bring the most relationship credit. In this way, the client gradually builds up trust in the therapist and by and by abandons some of their autonomy and allows the therapist to provide help. All of these processes are difficult for SzPD clients, and their involvement with the therapist develops slowly and tentatively. It follows that the therapist must not exert pressure on the client. The therapist must emphasize that

- It is the client who determines the pace of the therapy.
- Nothing happens that the client does not want to happen.
- The client may use the session in whatever way they want.

If the client wants to be silent in the session, the therapist will be silent as well; they will stay with the client, be attentive, not observe the client but just look at them once in a while. If the client wants to talk, the therapist is ready to participate. Otherwise, the therapist accepts their silence entirely. If the client wants to engage in small talk, the therapist is ready to participate. Accordingly, in the first phase of the therapy (sessions 5–15), the therapist primarily makes relationship offers instead of addressing contents and making PPs. The objective, therefore, is to get in contact with the client, impart the fact that contrary to the client's assumptions, contact with the therapist would be good for them, and the relationship may be important for them. Only if these objectives are achieved, can contentual issues – for example, schemas – be processed and worked on.

Explicating Relationship Motives

The explication of relationship motives should be done carefully and only if proper contact has been established with the client. The client has been avoiding their motives, so that their representations are ambivalent. A very early and massive elaboration of motives could be very confrontational. However, the explication of relationship motives is vitally important for SzPD clients. The clients need a representation of what is really important to them; without this knowledge, they lack any motivation to change. Basically, the main costs of their disorder are that they are unable to satisfy their needs. Once these needs have been clarified, it is possible to work on what can and must be changed with a view to meeting them.

Nonetheless, it must be expected that an explication of motives may massively intensify inner conflicts. The more the person realizes that they actually want proximity, the more painful it is for them to realize that they stand in their own way. Thus, a recursive procedure is required in therapy. As soon as the client is aware of details of their motives, any barriers that may be obstructive should be identified. An explication of schemas should be worked on, uncovering strategies such as those of sour grapes and the escape into autonomy, and thus revealing what the clients are doing and that they are possibly able to change something. The better their motives are clarified, the greater will be the client's desire to work on their schemas. Thus, the explication of motives and schemas and the processing of schemas should go hand in hand. In

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contrast to those of NPD and HPD clients, the motives of SzPD clients cannot be entirely explicated before schemas are processed. Processing takes place in parallel and always depends on how much relationship credit the therapist has acquired.

Resource Activation

The activation of resources is essential with SzPD clients, as these clients often underestimate their abilities and skills. However, the therapist must expect real deficits with respect to their social competence. More often than not, social competence must first be built up by training.

13.3.2 Therapeutic Strategies in Phase 2

When treating SzPD clients, the handling of the game level is of minor significance. The only thing the therapist should do is to refrain from reacting to any distancing strategy – that is, not letting themselves be scared away by the client. There are no special tests that have to be passed.

13.3.3 Therapeutic Strategies in Phase 3

The client's assumptions are revealed slowly, gradually, step by step. By explication, the therapist greatly helps their client with the representation of the schemas. The client must know in particular that autonomy is not what they actually strive for on account of any attractiveness it might have, but rather because they believe they have no choice. The clients must know this, to be induced to work on their assumptions – that is, it must be clarified how much of their autonomy they are willing to give up to achieve other objectives – for example, establishing contacts with other people. The client should also get it straight in their own mind that they only think they do not need relationships because they think relationships are unpleasant. They must question this self-image, otherwise they will not be able to come out of their shell. But this image can only be disputed if the client realizes what they are doing.

For SzPD clients, biographical work can be essential to understand how they acquired their assumptions and strategies. This will usually help the client realize these assumptions are not coercive and do not represent the reality.

13.3.4 Therapeutic Strategies in Phase 4

A systematic processing of their schemas is only possible later on in the therapy. Apart from sufficient relationship credit, it is also absolutely necessary for the SzPD client to know their motives, their schemas, and that they actually want a change. OPRP is a very effective means for the processing of schemas. The client themselves can put assumptions to the test, such as "Relationships are pointless." They can convince themselves that this assumption stems from past experiences with their family and that they see many people to whom this does not apply. Clients can also prove the correctness of the assumption that relationships are pointless, but to do so they must at least tentatively be willing to participate in a relationship.

When in the therapist position in OPRP, the client should realize they have constructed their system themselves: the reason for escaping into autonomy, the advantages this offers, but

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also the costs incurred by this. And they should work on the question of whether they actually want to incur these costs. They should also be aware they do not have to give up all of their autonomy; that there may be a compromise between proximity and bonding, on the one hand, and autonomy and relying on themselves, on the other. Probably, the client will always need certain aspects of their autonomy. However, the assumption that an individual is either entirely autonomous or entirely bound can be revealed as dichotomous thinking.

13.3.5 Therapeutic Strategies in Phase 5

Work with these clients can commence by doing very easy practice: Smiling at a shop assistant, asking people for directions in a friendly way, and smiling back if someone smiles at you. As these clients often have competence deficits and as competence training is necessary, all of these training practices may also be defined as homework.

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Chapter 14

Obsessive-Compulsive Personality Disorder

14.1 Description of OCPD

14.1.1 Description

Obsessive-compulsive personality disorder (OCPD) is characterized by high accuracy, fear of making mistakes, low spontaneity, and strong orientation toward standards. See APA (2013) for the full DSM-5 criteria. The prevalence of OCPD is estimated to be 7.8%, with men and women being equally affected (Grant et al., 2012). DSM-5 estimates the prevalence in the population to range between 2.1% and 7.9% (APA, 2013).

OCPD individuals show high levels of comorbidity with paranoid, avoidant, and borderline personality disorders (Pfohl & Blum, 1995), and with depression (Hardy et al., 1995; Bockian, 2006b) and anxiety disorders (Kasen et al., 2001). They also have problems enduring insecurity (Gallagher South, & Oltmanns, 2003); occasionally, they also tend to aggressive outbursts (Villemarette-Pittman et al., 2004).

Persons with obsessive-compulsive disorder are more likely to show aspects of OCPD (Eisen et al., 2006; Garyfallos et al., 2010; Lochner et al., 2011; Ruppert et al., 2007). On the other hand, persons with OCPD are not affected disproportionately by obsessive-compulsive disorder (Baer et al., 1990; Joffe et al., 1988; Rasmussen & Tsuang, 1986). In cases where someone is affected by a comorbid OCPD their suicide risk increases (Diaconu & Turecki, 2009; Raja & Azzoni, 2007).

Obsessive-compulsive personality disorder is primarily characterized by an extremely strong norm orientation (Millon, 1996): These individuals have adopted massive norms or standards to which they very persistently adhere and which impair them (sometimes even very gravely). OCPD clients are thus living their lives exceedingly according to compulsive "I must" and "I must not" mantras.

OCPD clients have adopted the norms they cling to, from caregivers or significant others in their biography – they had no free choice in this respect. It is to be noted that they do not cling to these norms because they find the norm contents extremely convincing but because conforming to these norms protects them against anxiety. Accordingly, the issue is never really about contents: It is all about psychic functions.

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Moreover, OCPD persons set strict rules. However, in contrast to how they are handled by narcissists, these rules are not of an egocentric or self-involved nature, but are generally applied rules in the form of "One must..." Similar to narcissists they consider themselves as guardians of the rules and proceed under the assumption that they are entitled to punish those who violate or disregard those rules (see Sachse & Kiszkenow-Bäker, 2016; Sachse, Kiszkenow-Bäker, & Schirm, 2015).

OCPD clients basically fear that they may do something wrong, and such wrongdoing mainly consists of violating the norms. To make sure they do not commit wrongs, they follow strategies such as

- *Don't be spontaneous!* Being spontaneous could easily lead to doing things that may violate the norms. Thus, it is essential to "Always think first before you act!" A person always must think and weigh things up, which gives rise to a high degree of state orientation.
- *Control emotions!* Having emotions, showing emotions, and acting on emotions can easily lead to a violation of norms. The person therefore tries to strongly control emotions: *Emotions are dangerous!*
- *What I must do is more important than what I want to do!* Even one's own needs can lead to disregarding and violating the norms. So, one must act on the basis that "one's own needs are immaterial." In this way, one's own needs must not be dealt with, and neither is it necessary to know about them. This attitude consequently results in extremely high alienation.

Due to these strategies, OCPD individuals often appear

- rigid and awkward: unrelaxed, controlled, inauthentic;
- not spontaneous: The OCPD client can hardly be activated, barely reacts to jokes, and can hardly be motivated to join in with social interactions;
- unemotional, cold, and unapproachable: The OCPD client barely smiles, does not show joy, is emotionally barely responsive.

Individuals with OCPD are less flexible (can't turn a blind eye), insist on compliance with rules, and in doing so often annoy other people.

Individuals with OCPD often distinctly perceive the costs of their systems: their lack of spontaneity and emotionality, and the negative impression they make on others. They often think they are living in a straightjacket. However, these costs usually do not make the disorder ego-dystonic, because they also think

- it has to be this way;
- there is no alternative;
- if I don't follow the norms, chaos will break out.

So they whitewash the costs:

- It is not that bad.
- I can live with this.

They even succeed in finding honey in the system by thinking:

- I'm much more ethical than others.
- I'm the only one who actually follows norms.
- If everyone were like me, the world would be a much better place etc.

As a result of these cost-compensating efforts, OCPD individuals often appear to display a certain degree of arrogance, which is not at all conducive to their popularity with interaction partners.

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14.1.2 Biographical Experience

The biographical experiences of OCPD clients seem to have significant relevance because they may serve to illustrate and make understandable how the extreme norm orientation of these clients has developed.

Certainly, all assumptions about the client's biographical experiences are based on experience imparted by individual clients. For that reason, they must always be viewed as hypothetical. They nevertheless may be useful, especially to improve the therapist's understanding of the disorder. These assumptions clarify that the client had not deliberately chosen to be as they are, and that their systems are solutions they inevitably had to develop.

When considering the biography of individuals with OCPD, it turns out that these individuals have often been exposed to massive **conformity pressure**. In their upbringing, central interaction partners have acted according to rules such as "If you do what we say, if you do exactly what we expect from you, you'll get appreciation and importance, and we will show solidarity and exert as little control as possible; but if you don't comply and if you don't do what we expect from you

- you will be devalued: told you are immoral, worthless, harmful, selfish, etc.;
- you will be defined negatively as someone who isn't able to do anything right, who isn't any good, who is worthless;
- you will be defined as 'negatively important,' as toxic, annoying, as someone we do not want at all;
- any show of solidarity will be cancelled: no one will stand by you, they won't help you, won't care about you, you will be alone (probably the worst conceivable disaster and threat for a child);
- you will be controlled and regimented and monitored, you won't be able to do anything without control, you will be totally restricted;
- you will be punished, confined to your room, subject to physical violence, etc."

What is left to the person is choosing between rebellion and adaptation: If rebellion is not possible (e.g., because the person estimates it as futile), only adaptation remains and must be strictly complied with, as otherwise any positive responses will be forfeited and highly aversive consequences must be faced instead.

To be on the safe side, the OCPD person has to comply with predetermined norms. They person must deduce what is expected in different situations and must act accordingly, must anticipate what is good and right according to the norms, and must develop **anticipatory obedience**. To avoid making mistakes, they must *completely* internalize the norms. To ensure they act absolutely correctly, they must make the norms their own norms. Their own system must prescribe the standards: This is safe, quickly enough, reliable, makes it possible to always do the right thing. The more the norms become their own norms, the more secure the system is.

As the interaction partner's prescribed norms are sometimes difficult to reconstruct and often inconsistent, it is helpful to make one's own norms so strict and rigid that they meet any expectations without fail. Accordingly, the norms of OCPD clients may become more rigid than the norms pre

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scribed by interaction partners. OCPD individuals install a security system, a mechanism intended to make sure one is on the safe side. Out of the whole threat situation which affords only a little control, an overwhelming desire for control and predictability arises: Following norms creates control; orientation toward internalized norms improves control; and orientation toward stringent and rigid norms improves control even more. Aversive consequences are avoided, and positive feedback is obtained. The system works.

However, control also means that everything that could impede norm fulfillment must be avoided. This includes meeting one's own needs and complying with emotions that indicate what one wants, what is important, and what one does not want. Such behavior may easily lead to norm violations and is therefore highly dangerous and, accordingly, must be strongly controlled. OCPD clients try to abandon everything that might cause alternative actions and give rise to norm violations: The OCPD person consistently defines their own needs and emotions as disturbing – which in fact they are, because they lead to violations of norms. So, one's own needs and emotions must no longer be tolerated, not be taken seriously; they are not perceived and not followed. In this manner, the individual gradually loses access to their own motive system. They systematically minimize their own emotions – for example,

- by avoiding situations that may activate emotions and needs,
- by diverting attention from emotions,
- by not taking emotions seriously,
- by not reflecting on one's own needs, etc., systematically.

The consequences are that

- Massive alienation develops: The OCPD client can no longer represent their own needs.
- Their own motives can hardly be represented anymore.
- Their own emotions can no longer be interpreted but are rather irritating and disturbing.
- They have no standards of their own anymore, which further increases the probability of orientating on external standards.

When in the course of time, their own emotions get really irritating, disturbing, and aversive, the OCPD client will entirely avoid and circumvent emotional processing so that a vicious cycle is created that becomes more and more severe over time.

It is also important to note that OCPD clients do not orientate themselves toward any current expectations of other people, as clients with DPD do. DPD clients try to find out what their interaction partner currently wants and try to comply flexibly with those wishes. This strategy would be useless for an OCPD client: They do not want to adjust themselves to the varying expectations of interaction partners but rather to comply with generally and validly defined and always applicable norms. Children are not supposed to do what the interaction partner wants or what would be good for themselves, they are supposed to do "what one does," what is "good and right"; they shall follow strict and generally applicable rules. The standards which have to be followed are not flexible, they are rigid and stringent. And it thus follows that compliance that is as strict as possible is relatively secure.

The only option available to OCPD clients is fulfilling the norms, regarding these norms as their own norms, which are wanted and defended, and which form the exclusive basis of the client's actions and behavior. Following norms creates control and with it security, whereas disregarding norms creates anxiety and fear.

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Usually, OCPD clients are no longer aware of the reasons they suffer the anxieties they do. Even reflections on their anxieties are avoided, which causes the reasons for the anxiety to remain inaccessible, the same as all other emotion processes. The consequence is that OCPD individuals cannot even explain what they are actually afraid of. Their anxiety becomes rather diffuse, intangible but not less aversive. Quite the contrary: As the anxiety is intangible, nothing can be done about it anymore. A diffuse, incomprehensible anxiety or fear makes OCPD clients much more helpless than if they suffered a concrete anxiety that could be described. Such a diffuse anxiety massively violates the need for control and must therefore be avoided. Again, the only thing that can be done in this case is fulfilling the norms: It is the only way to control the anxiety. The system is highly isolated and self-stabilizing.

However, it can be assumed that the isolation of the system is not totally complete as a rule. Otherwise, the system would be massively ego-syntonic, and if it were, OCPD clients would probably not undergo therapy at all. And if they did go into therapy because of comorbid disorders, it would probably be impossible to work out a motivation to change for someone disturbed by OCPD. Therapists should, therefore, take this into account: OCPD clients are not approachable, and should not be influenced against their will. If the therapist succeeds in working an motivation to change out of the client's system, resulting in the client deciding to work on making changes, the therapy measure is ethically justified; however, if this is not successful, the therapist has no right to relieve the client from their suffering if they refuse to be relieved. Every person has the right to have problems. And if a person decides to go along with those problems, they have every right to do so.

However, let us now examine OCPD clients who are not totally isolated. It is to be assumed that these clients live in an environment where they perceive how others live. And they see that others are happier, live an easier life, are more satisfied, do things the client refrains from doing, etc. Accordingly, we may assume that

- The person's authentic motives are frequently triggered and have not been deactivated completely.
- Emotional processes cannot be completely blocked either.
- Clients are consistently confronted, more or less rudimentarily, with a discrepancy between their life script and what could be.
- Dissatisfaction is consistently triggered through that.
- Clients consistently see that there is another way, that a different life can be lived, that a different behavior is conceivable.

Thus, the assumptions in such OCPD clients are that these are clients whose disorder is not totally ego-syntonic and who feel ambivalence and discrepancies. And these are the clients who are basically approachable through therapy.

These OCPD clients often go into therapy because of a comorbid depression or general dissatisfaction. Their extremely constrictive and extremely nonhedonistic behavior makes depression as a comorbid disorder very likely (if the discrepancies are noticed!). It can be said that OCPD clients who undergo therapy because of this comorbidity, already have a distinct, initial motivation to work therapeutically. Their system is not totally isolated and unapproachable, it is open to some extent, and this is exactly what the therapist must use.

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14.2 Definition of OCPD, Based on Dual Action Regulation Model

14.2.1 Central Relationship Motives

The biographical scenarios described in the previous section have generally led to the fact that several central relationship motives have been frustrated and consequently rank high in the motive hierarchy of the OCPD client. An essential motive is the desire for **appreciation**, to be respected as a person, positively defined, and praised. OCPD clients will probably have constantly been devalued, negatively defined, and rejected. The motive of seeking appreciation will consequently be central. Another motive is the desire for **importance**, to play a crucial role in the life of others. It may be assumed that OCPD clients have been defined as annoying, disturbing, toxic if they violated norms, and thus they have experienced little importance as a person. Another probably very important motive is the desire for **solidarity**, to be able to rely on others, and get help, protection, and support. Solidarity is probably a strongly frustrated motive and thus ranks high in the motive hierarchy. Furthermore, there is probably a very strong desire for **autonomy**: to determine one's own life, decide for yourself what is right and wrong. Autonomy has probably been strongly restricted through controls, observations, and regimentations.

However, as has been described, it may also be assumed that there is a great alienation problem with OCPD clients: Their most important motives are barely represented anymore. And it is to be assumed, furthermore, that there is **strong avoidance**. Their own needs are regarded as reasons for noncompliant behavior and therefore as causes of massive punishment. Admitting one's own emotions (that are indicative of needs) and having needs are prohibited. The OCPD client consequently defines their own needs as aversive, disturbing, selfish, unacceptable, and irrelevant. They establish a contra-image which includes

- not having one's own needs,
- not following one's own needs,
- regarding one's own needs as irrelevant, and
- regarding one's own needs as dangerous.

It must theoretically be assumed, however, that needs, as central as they are, can be easily triggered and easily satisfied by the therapist. And it is to be further assumed that as long as the satisfaction of needs through the therapist takes place implicitly without emphasizing explicitly what it is about, avoidance will probably not occur. It should thus be relatively easy to satisfy these needs through the therapist's complementary behavior. However, when explicating motives, avoidance may be triggered as soon as the client realizes what it is about. Nonetheless, theoretically there is nothing to be said against complementary behavior; this should be a rather conducive way of approaching the client.

14.2.2 Dysfunctional Schemas

Self-Schemas

It can be assumed that biographical experience has basically resulted in negative self-schemas:

- "I'm not okay."
- "I have poor abilities."

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- "I'm basically not acceptable."
- "I'm immoral, selfish, reckless."
- "I'm toxic, dangerous, harmful for others."
- "I'm not important."
- "I have nothing to offer."
- "I can't decide for myself."
- "I have no control over my life."

It may also be assumed that these schemas consistently create self-doubts, and consistently produce self-depreciation to be constantly acted against by the person.

Self-Esteem Enhancement

Self-esteem enhancement, necessarily arising because of the massive self-depreciation, is a strategy which distinctly aggravates the problem of OCPD clients. Clients who deeply doubt themselves and perceive themselves as being unattractive and unimportant can extract something positive from norm fulfillment and especially from fulfilling norms strictly and stringently, because the OCPD person is the only one in the universe with the right norms, the only one who really knows what one should do, the only one who really and correctly fulfills these norms (actually the First Fulfiller of the norms!). This uniqueness is something special: It characterizes the person and makes the person superior to others.

It is clear that this construction does not make it easier for the therapist to shake the OCPD client's system: Obviously, the norms constitute a hardly vulnerable fortress. Thus, the question arises of whether the therapist should make any attempts at all. After all, should a therapist deal with norms? Based on therapeutic experience and for theoretical reasons, the author of this present work would urgently advise against this.

Relationship Schemas

The clients probably have never experienced relationships as friendly, caring, and showing solidarity, so that the therapist may proceed from the assumption they have relationship schemas such as:

- Relationships are cold, unfriendly, pointless.
- That one is rather devalued in relationships.
- Relationships do not show solidarity.
- It is better to rely on oneself.
- It is better to disclose little of oneself.
- It is better to keep away from others, as distance provides security.

14.2.3 Compensatory Schemas

Normative Schemas

In particular, the interactional objectives of OCPD clients are

- "Follow norms, then you'll be on the safe side!"
- "Follow norms to avoid severe consequences!"

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- "Follow norms, then everything will be okay!"
- "Not following norms results in disaster!"
- "Never follow your own needs and emotions!"
- "Never cease in fulfilling norms, as this alone would be disastrous!"
- "Never allow people to question your system; this would be dangerous!"
- "Be prepared to incur all costs as there is no alternative to norm fulfillment!"
- "Disclose as little as possible of yourself, as this results in depreciation!"

Obviously, the norms are thus an aspect of the control level; there is no need for norm fulfillment at the motive level. Norms provide solutions for a massive interactional problem. And, meanwhile, norms offer strong protection against anxieties.

The fulfillment of norms means interactional objectives are achieved, but does not satisfy the OCPD client's motives in any way. Norms create control and security, but as they do this they simultaneously create a small, narrow, rigid box that also functions as a prison.

Norm orientation can be so strong that all areas of life are affected. Work has to be done in compliance with certain norms, and even in relationships, norms play a role. Here OCPD clients often act in line with the rule "Norms are more important than relationships." They do not want any relationship with interaction partners who do not fulfill certain norms. In extreme cases, their own children are rejected because they behave indecently.

Rule Schemas

OCPD clients are rule setters: They determine the dos and don'ts of other people and what is right and wrong. In contrast to how they are handled by narcissists, their rules are not directly ego-oriented. OCPD clients always set generally valid rules: Citing what is stated in the Bible, the Torah, the Koran, or what the legal code says, or saying "it's self-evident" or "it's just common decency." All of these are allegedly universal rules that need not be substantiated.

And again in contrast to narcissists, OCPD clients are highly missionary: Everyone shall follow their rules, which would make the world a better place and ethically sounder, and would eliminate problems, etc. And those who do not follow the rules are to be depreciated as being immoral, unsocial, selfish, vermin, etc.

By acting this way, OCPD clients usually do not make many friends: Most people think they do not need a high priest around them and refuse to be constantly judged and depreciated.

In contrast to narcissists, OCPD clients obey their own rules themselves. They in fact are the first rule followers in that they adhere to the rules much more stringently, more precisely than anybody else.

The rules are obviously aspects of the game structure: They serve to enhance safety, make the environment more controllable; if everyone would follow the rules, they

- would no longer be in danger or threatened;
- would not be confronted with alternatives and would experience fewer discrepancies and less tension.

Basically, this may appear to be quite a clever strategy to create a world where problems are eliminated. It is only too bad that other people see this differently, which means it is exactly this strategy that causes the problems.

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14.2.4 Manipulation

General remarks

OCPD clients set social rules. And they do this quite openly and outspokenly. They are highly dominant and determined and they hardly endeavor to disguise their strategies. Accordingly, the manipulative quality of their interactions is rather low.

Images and Appeals

Images projected by OCPD clients include
- "I know the ropes!"
- "I can tell you what to do!"
- "I'm vastly superior morally!"
- "You can all follow me!"

Appeals include
- "Everyone shall leave me alone!"
- "Do what I tell you!"
- "But keep your distance!"

Interaction Games

The author calls the game played by OCPD clients the **Moses game**: The clients proclaim the law to other people, and for this they want appreciation and praise. They alone are capable of bringing others salvation – if only others were listening to them. The problem is that usually the game is not very convincing and does not impress anyone (it may even be annoying).

14.2.5 Tests

Tests usually regard allegiance: "You certainly will agree that … You don't think so? Then you should be seriously worried about your moral integrity!" The therapist is tested as to whether they share the client's opinion. They are not expected to form an alliance with the client against others, but they should rather be in complete agreement with the client, thus confirming the client's assumptions. However, it can be said from past experience that tests of this kind are not made very often; they may take place if the therapist has unfortunately challenged norms directly, but if they avoid actions of this nature, tests will rarely occur.

14.2.6 Characteristics of OCPD

Proximity, Distance, and Bonding

Clients with OCPD keep their interaction partners at a distance. Distance prevents details about oneself from being disclosed, and it rules out surrender and vulnerability. Only a few people are allowed to come near, and this only with reservations, so that they may be rejected whenever it is thought expedient. OCPD clients may even turn others into a "nonperson," may "delete" a person from their environment, or mentally "evaporate" them.

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The OCPD person never fully trusts others, never gives themselves to others, does not fully open up – that is, they never really deepen a relationship. Additionally, they show limited emotionality and spontaneity. Everything must be verified (for compliance with norms), elaborated, and planned, and spontaneous behavior is potentially dangerous. A person with OCPD is quite the opposite of a Latin lover: They are stiff, awkward, inhibited, and totally unromantic. In brief: They are the ideal partner for an annual tax declaration.

Keeping a distance is even noticeable in the relationship with the therapist. The client is reserved, rather mistrustful, and keeps the therapist at a distance. This means that establishing a reasonably functioning therapeutic alliance is a central task.

Ego-Syntony, Perspective, and Avoidance

OCPD is highly ego-syntonic, because the client is convinced the fulfillment of norms is the most important thing in the world and that they really want them fulfilled. However, the disorder is not entirely ego-syntonic in some clients. These clients may perceive the costs incurred and as a result suffer; they often wonder "I didn't do anything wrong, why isn't everything going well?"

Sometimes it seems as if the OCPD client does not really care about the costs they incur and are prepared to accept the consequences just to be able to keep the system working. However, a more precise analysis shows that this is only partly true. The client's firm insistence that "the costs must be borne no matter what they are" is in the opinion of the author due to reactance. For example, the author previously exercised supervision during which a client with OCPD had faced a problem: He worked as a technical inspector in a metal-working company. Due to financial difficulties, he was asked by the management to lower the inspection quality standards for a while because jobs could depend on sales. In therapy, the client expressed his distress about this imposition and said he would never comply with a request to lower standards even if this caused jobs to be lost. But as it subsequently turned out, his colleagues did matter to him, after all. However, he was still unable to change his behavior and act differently. He felt under pressure from the request to deviate from his stringent norms. For him, this was a highly threatening situation: He was asked to abandon everything that provided security, control, and protection against anxieties. Consequently and in response, the client acted by showing reactance: He did not relent and did not move an inch.

This emphasizes something very important concerning contacts with OCPD clients: Acting directly against the norms, causes massive reactance! This provokes a protective reaction in the client. It is thus not true that OCPD clients do not care about costs; usually this reaction is to be viewed as a response to a direct or indirect assault on norms. And this moreover attests to the finding that therapeutically it makes little sense to concentrate on the client's norms. They are an invulnerable fortress that must be attacked and conquered from the rear because any unconcealed frontal attacks will be in vain.

The perspective of the OCPD client is strongly external. They are concerned with whether they do everything correctly but not concerned with themselves. Any attempts to internalize the perspective may prove difficult because avoidance strategies may then take effect. The client also uses arguments such as:

- "It doesn't matter what I want!"
- "I'm not selfish!" etc.

The therapist should expect this, otherwise they will easily be stymied.

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Costs

As has been described earlier, OCPD clients often perceive their costs as diffuse: feelings of constriction, exertion, lacking joy, and an impaired quality of life. All this is usually not very salient but does exist.

14.3 Therapeutic Strategies

14.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

It may be difficult for the therapist to establish a relationship with a client with OCPD but it is not altogether impossible; a complementary relationship structuring is an essential prerequisite, however.

First of all, the therapist should ensure that the OCPD client's behavior does not trigger their self-schemas. Because if the therapist reacts aggressively toward the client's rule setting, for example, or toward the missionary behavior of the client, it is hardly conceivable that a therapeutic relationship can be established. Therefore, the therapist should ask themselves if they wish to work with OCPD clients at all. They should be sure they are able to respect these clients and are willing to work with them. If this is not the case, they will not be capable of behaving really complementarily. It is helpful for the therapist to realize that the client most likely was forced to choose this strategy; this may improve their understanding of the client.

The first thing the therapist should do is to treat the client with respect: respect for the client as a person and respect for their solutions, views, and behavior. The therapist accepts such solutions as the client's solutions and as something they do not depreciate but also do not value either – they just accept them as aspects of the client. The therapist also emphasizes that they are interested in the client and that they would like to work with the client; and this also means that they want to understand the client: They want to understand what the client does and why they do it, how the client feels and thinks, and what they think.

It is essential for complementarity that the therapist totally respects the client's autonomy and boundaries. The client does not have to change for the therapist; the therapist does not demand standards of the client. If the client wants to stay the way they are, the therapist fully accepts this. The client need not even work on certain issues, and may address and deal with whatever they want. The client shall understand everything that happens in therapy. If the client does not understand something, they shall ask the therapist right away. The OCPD client will be extremely afraid of losing control. Accordingly, the therapist allows them maximum control over the process. The client should see that they do not have to fight for control, do not have to fight for appreciation and importance, and that they do not have to fight to be understood. Instead, all of this will, of course, be provided by the therapist.

This also means, for example, that if the client makes small talk, the therapist joins in, and this is even positive because it enables the therapist to learn something of the client and send relationship messages on unproblematic issues. For a while, it is also beneficial to talk about what the client likes to do, the hobbies they have, what their daily routine is, and what they are interested in. The client will refuse to be urged into talking about themselves as long as they

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still keep their distance, and the therapist should by all means bear this in mind. Initially, contentual work is prohibited, if the clients do not want it themselves: Establishing the relationship comes first.

It is also important for the therapist to readily perceive and talk about the client's strengths and resources and make it clear they are really convinced of the client's competence, capabilities, etc. In this way, the therapist emphasizes that they do not at all regard the client as deficient, weak, toxic, etc.

Explicating Motives

Explicating central motives should not yet take place when therapy starts. This is due to the specifics of OCPD, in that access to one's own motives may be prohibited. Accordingly, such explications made early in therapy may (quite unusually) be confrontational. For that reason, the therapist should only use explications when sufficient relationship credit is available and the costs of the system have been made salient.

Explicating the central relationship motives should also take place carefully at first and subsequently be intensified. The therapist emphasizes that the client has a strong need for control but at the same time needs appreciation, solidarity, and to be considered important. And they also have a strong need for autonomy. All these explications may deepen the client's ego-dystonic experience. The client realizes more and more how far away they are from their needs and that they entirely ignore in their life of what they really want. Accordingly, another therapy step must be again and again be followed: the step of finding solutions (see the subsection Solution Orientation, in Section 14.3.3).

Dealing With Norms

As has been described above (see Section 6.6 Therapeutic Strategies in Phase 4: Working Through Schemas), the therapist (in the first therapy phase and later on) should never assault the norms. They do not address the norms and rules at all; whenever the OCPD client refers to them, the therapist respects the client's view. The therapist should never discuss the client's norms with the client. If they do, they will soon get into very deep water and drown. Do not get involved in arguments about norms! And refrain from

- assaulting the contents of norms,
- questioning the contents of norms,
- suggesting alternatives, etc.

Norms are put up with by the therapist and accepted as the client's views, and nothing more. This should not be the therapeutic entrance. Norms are not attended to, addressed, or processed by the therapist.

If the OCPD client insists that their norms are correct and wants the therapist to confirm this – for instance, asking, "You agree that one must do things this way, don't you?" the therapist may reply "No, I don't, but I do respect your way of thinking." If the client further insists, "But if it isn't done this way, society will be thrown into chaos, so, obviously, something must be done this way!" the therapist may answer, "I can't say if this is so, and actually I don't want to discuss this. Because what happens to society is not for me to decide, I rather care about what you think; I would like to understand you, that is important to me."

The therapist should not let themselves be distracted from their offer to understand the client; they should not let themselves be involved in discussions but insist that they

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- cannot judge the consequences the client has described,
- do not want to judge the consequences,
- do not want to talk with the client about society,
- are only interested in the client.

The therapist may also express astonishment: that they are astonished the client needs the therapist's confirmation and they think the client capable of judging and deciding these things on their own. And the therapist will not make decisions for the client. The client regards this as a boundary violation which they do not want to take place at all.

The therapist should by no means agree with the norms here, but neither should they question them. They should just leave them untouched and make clear they are not willing to deal with working on norms; their aim is to understand and be interested in the client. However, the therapist's interest also is in no way associated with pressure or demands, and the therapist only makes an offer to the client. They also emphasize what they will not do: They will not have a discussion about sense and nonsense, about consequences or contents of norms! Therapists may carefully and unobtrusively emphasize that they will not follow these norms and that they think no one must follow these norms, but that any decision to follow them is left to each person's choice. In this way, the therapist offers an alternative orientation and emphasizes that norms must not be necessarily followed. If asked by the client, the therapist may say that they think everybody can develop own standards, and everybody is responsible to reconcile their standards with society's standards. They may also say they have no intention of dictating what people have to think or do. In this way, the therapist demonstrates an alternative, a model for non-obsessive-compulsive behavior, which the client may follow if they want.

Therapists should always bear in mind that norms are a means of protection for the client. The client can only discard them in the event their security and safety increases. Norms cannot just be taken away from the client. You cannot "saw off the tree branch on which one is sitting" before the client has found another one to sit on!

Norms and Anxieties

What can be emphasized relatively early to the OCPD client is that fulfilling norms is actually about avoiding anxieties. This may already be emphasized when the client arrives for therapy and says, "I was afraid of being late!" or if they occasionally speak themselves about the correlation between norms and anxieties. This should immediately be followed up by the therapist saying, "If you violate the 'be-on-time rule,' you become afraid." If the client confirms this, the therapist may offer: "I would like to clarify with you what actually makes you afraid." The therapist may also reveal anxieties when asking the client, "What if you do not follow norm X, but instead do Y? What would this arouse in you?" If the client is willing to participate in this cognitive experiment, they will quickly notice that it triggers anxiety.

Therapists who constantly point out that there is a correlation between norms and anxieties elucidate step by step to the client that they fulfill norms because

- nonfulfillment causes anxiety,
- fulfillment creates control and security.

Clients should be brought to realize they do not fulfill norms because one must fulfill them but rather because they have personal reasons for fulfilling them. When clients realize this sufficiently, questions may be asked like "Why do norms have to be followed?" "Why does it cause anxiety when they are not obeyed?" "Why do I actually have these norms?" etc. When

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following this approach, the client has embarked on an internal clarification track. This is a prerequisite to further strategies in therapy.

Resource Activation

Individuals with OCPD often regard their own emotions as dangerous because they lead to spontaneous behavior, bring up aspects which cannot be understood and controlled, etc. To enable the client to admit, regard, and understand emotions, it is essential to emphasize that the client has many resources in dealing with emotions: They can definitely control emotions if they want to. And, as this is a fact, they may allow themselves to experience emotions and see what they mean to them. At this point, the client can still decide whether to follow them or not, whether to pay attention to them. OCPD clients can learn in this way that they are able to deal with impulses, wishes, and affects, without being overwhelmed, and they can also learn that it is one of their special strengths to be able to control emotions so that emotions are only allowed to be experienced when the client admits them.

14.3.2 Therapeutic Strategies in Phase 2

Making Costs Salient Empathically

It is always important to extract the motivation to change out of the client's system – that is, making it clear to the client that they themselves have reasons to change and work on changes. As has been said earlier (Section 14.1.1: Description), the client knows about their costs, but these have not been made perfectly salient, so that they have no real effect on their motivation to change. For that reason, the therapist should carefully and very empathically provide elucidation of the costs of their disorder already known to the client. The therapist should at first only deal with costs the client is addressing themselves or which the client can easily accept, because these interventions shall not be confrontational. And the therapist should be very empathically and in this way signal their understanding of the client. For example, to clients talking about norm fulfillment, the therapist may say:

- "I imagine all this to be very exhausting."
- "Surely, this must be very exhausting for you."
- "I could imagine that you must always take care that you are acting correctly, and you must always be on your guard. I think that must be very exhausting."

Usually, the client will feel themselves understood by this intervention. Therefore, not only do interventions of this kind make costs salient, they also create relationship credit.

The more relationship credit the therapist has accumulated, the more effectively they can make costs salient – for example, by saying

- "Permanently following rules must be as if you were in a cage."
- "I ask myself whether you sometimes have the feeling that you restrict yourself."

In this manner, there will be growing awareness on the part of the client about what they are doing to themselves, and that they restrict and even enslave themselves.

The strategy of making costs salient gets easier when the OCPD client starts therapy because of depressions or general dissatisfaction. It can be emphasized to these clients relatively quickly that something must be changed and that detailed knowledge must be obtained to exactly understand how their system works and how and where it can be changed.

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Making the Game Structure Transparent

Clients with an OCPD need hardly be confronted with their nontransparent behavior. However, should this be necessary, it should take place late in the course of the therapy. Other tests than those already described (Section 14.2.5: Tests) rarely occur.

14.3.3 Therapeutic Strategies in Phase 3

Clarification of Schemas and Rules

If the client knows there are reasons for their norms, that the norms fulfill a function for them, and if the client tries to find out which function they have for them, then and only then may the therapist start explicating the client's schemas and rules. This should be performed as empathically and respectfully as possible. The therapist may point out step by step that

- The client follows norms because otherwise they would feel toxic, worthless, etc.
- They have strong self-doubts, doubt their value and importance, etc.
- They also have strong doubts about relationships.
- Therefore, they keep others at a distance, etc.
- They set social rules because they provide security and are conducive to the avoidance of internal discrepancies.

It is important that these insights are elaborated gradually, because proceeding too fast can easily overstress the client. Moreover, the therapist should only emotionalize the client to a certain degree and allow for sufficient time for trying to understand those emotions. If clients are emotionalized too fast or too much, this may get very aversive for them, and they will avoid such strategies in future! The therapists should therefore use deepening work carefully, should always allow the client to gain control of their emotions. Never emotionalize OCPD clients by exerting pressure on them!

Biographical Work

If the client is motivated, biographical work may begin. In this way, the client can gain knowledge of the reasons they have developed such a high norm orientation. This may be helpful to make the meaning of the norms more relevant and less stringent.

Solution Orientation

When treating clients with OCPD, solution-oriented work should always be performed on every level of the clarification process. This is necessary to constantly give the client the feeling that they are able to gain control. Therefore, questions should frequently be addressed such as:

- What could I change?
- What could I do?
- What could help me?

Even very small steps should be accepted, and the client should be encouraged to also strive for small changes.

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14.3.4 Therapeutic Strategies in Phase 4

If schemas are explicit and the client's motivation to change is adequate, those schemas can be systematically processed. This may take place by way of OPRP, during which clients can, for example, focus their attention on their own resources, convince themselves that nothing will happen to them anymore today if they do not fully follow a norm, that they in fact may take the risk and enter into a relationship, that they are not toxic for other people etc. Clients may especially focus their attention on dichotomous thinking, and thus realize that fulfilling norms by 80% does not mean a complete loss of control or that total chaos will prevail.

14.3.5 Therapeutic Strategies in Phase 5

A transfer to reality can now occur in small steps and to the extent that the client is able to implement them. Being a bit spontaneous here, going along without following a rule there, paying attention to what one wants oneself, admitting to or showing an emotion. Small steps are often possible and helpful in bringing some relief to the client. The therapist should motivate the client but should always refrain from exerting pressure. If the client is not yet ready to transfer aspects of their thinking and behavior then the therapist has reached the limits of what can possibly be achieved at this point.

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Chapter 15

Paranoid Personality Disorder

15.1 Description of PPD

15.1.1 Description

In the author's opinion, **paranoid personality disorder** (PPD) is the most cost-intensive and most therapy-resistant of all pure PDs. In addition to the criteria listed in DSM-5 (APA, 2013), the following characteristics of PPD have been described in the literature (see Beck, Freeman & Davis, 2004; Bernstein et al., 1993; Bernstein, Useda, & Siever, 1995; Blaney, 1999; Carroll, 2009; Costa & Widiger, 1993; Edens et al., 2009; Falkuma, Pedersen, & Karterudb, 2009; Knecht, 2013; Miller et al., 2001; Millon, 1996, 2011): Patients with PPD

- are extremely distrustful;
- constantly anticipate criticism, depreciation, and impairment;
- are hypervigilance: always attentive, watchful, and on guard;
- always think they must defend themselves;
- assume they cannot rely on others;
- show feelings of intense jealousy (Kingham & Gordon, 2004);
- are convinced they must rely on themselves, because of intense jealousy;
- assume others threaten their autonomy and boundaries;
- assume (based on biographical experience) they are unable to protect their boundaries sufficiently;
- think they must protect their boundaries effectively and very quickly, because they assume they are unable to protect their boundaries;
- becoming dependent on others must be avoided;
- dependence on persons of authority must be avoided in particular (these are especially dangerous!);
- react with extreme sensitivity to criticism, control, restrictions, or boundary violations;
- feel threatened by everything and everyone; threats are thought to be present in all vague and unclear stimuli;
- think they must develop a feeling of strength and vividly demonstrate this strength externally;
- do not allow other people to get close to them, and they try to maintain their independence;
- due to their isolation, they can no longer socially verify their constructions; they lose more and more contact with reality;

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- are constantly on the alert, constantly try to discover potential impairments, and for that reason are hypersensitive;
- feel at a disadvantage, overlooked, betrayed by fate, and begrudge others their success;
- are unable to perceive their own weaknesses and deficits but easily spot a weakness in others;
- tend to behave querulously (have a rigid interpretation of the law, feel easily and quickly at a disadvantage and therefore go to court very often, even in the event of minor offenses).

15.1.2 Prevalence and Comorbidity

The prevalence of PPD in the normal population is estimated at 2%; in psychiatric populations at 1–5% and 20% in permanent psychiatric patients (Bernstein et al., 1995; Bockian & Jongsma, 2001). Frequent comorbidities of PPD exist with NPD and AvPD (Millon & Martinez, 1995; Morey, 1988).

Research has shown that the DSM criteria for PPD are generally valid (Falkuma et al., 2009; Useda, 2002). With PPD clients, the proportion of men to women is 11:4 (Fryd Birkeland, 2011). Individuals with PPD show an increased comorbidity with depression (Bockian, 2006c).

The essence of the paranoid personality disorder (PPD) is **mistrust:** These individuals assume that "Others have bad intentions toward me."

From this basic assumption, they derive further assumptions:

- "I must always be attentive."
- "I must always be careful and should not give away too much about myself, as everything I say or do may be used against me."
- "I have to react quickly and strongly so that danger cannot arise and potential offenders are scared away from the start."

Notably, individuals with PPD react quickly and highly aggressively to "apparently" harmless incidents – for example, "stealing" a cherry may already result in a lawsuit: Obsta principiis is an important motto.

PPD individuals find it very difficult to trust other people, and they never trust others fully; they often question the loyalty and the good intentions of other people. Many interaction partners may easily see this as a personal assault. The social behavior of PPD individuals quickly causes high interaction costs, in that others turn away and do not want to have anything to do with the person. Basically, individuals with PPD do not deem themselves strong or competent. If they did, they would not have to be on the alert permanently as if danger were imminent but could trust in their ability to cope with problems. However, this is exactly what they do not do: They regard relationships as dangerous, and they regard themselves as weak. This makes it (apparently) necessary to be distrusting and careful – always and toward everyone.

And due to the fact that PPD clients think of themselves as deficient and since being deficient is extremely dangerous (because any weakness you have will immediately be used against you), even the knowledge of being deficient cannot be allowed. Accordingly, PPD individuals develop a self-ideologization of the kind:

- "I am okay!"
- "I understand!"

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- "My view of things is correct!"
- "My interpretations are correct!"
- "I behave correctly!"
- "It's always others who misbehave!"

Self-doubts are threatening to self-esteem and are strongly avoided. This makes therapeutic clarification processes extremely difficult, because every change begins with the insight that one is oneself part of the problem. For PPD clients, it is extremely difficult to gain this insight. And therapists who confront clients with such insights are quickly categorized as being one of the enemies.

As PPD individuals regard mistakes and flaws as an unacceptable weakness, they tend strongly to shift responsibility onto others. They frequently play the games "Victim of circumstances and other people" and "Always me." It is also this behavior that makes them unpopular with interaction partners and especially colleagues. Others quickly get the feeling that in the end they are left holding the bag.

Paranoid personality disorder appears to the author to be the most cost-intensive of all pure PDs, even more cost-intensive than the OCPD (which closely follows, though). Individuals with PPD are always sort of on the alert; they can never trust, never feel secure and safe, can never relax. They are unable to really engage in relationships and enjoy being together with others. This disorder makes life very hard for these persons.

For that matter, effective therapeutic help is urgently be needed, but providing this help turns out to be extremely difficult. Due to the deep mistrust that is encountered, even establishing a trustful, therapeutic relationship, which is an essential prerequisite in therapy, is impeded. As a consequence, the therapist often cannot work very effectively. The number of therapy sessions paid for by health insurance funds will usually not be sufficient to yield satisfactory results. Therapists, nevertheless, should not, of course, give up but do everything they can. It is to be noted that even softening the client's system may constitute remarkable progress.

15.2 Definition of PPD, Based on Dual Action Regulation Model

15.2.1 Central Relationship Motives

A central relationship motive of clients with PPD is **defending their own boundaries and territory**. It is to be assumed that these clients have experienced serious boundary violations, possibly even more massively than clients with PAPD. Significant others (caregivers, etc.) have controlled, punished, depreciated them, sometimes even violently. As a result, the motive of desiring to protect their boundaries became centrally important and ranked high in their motive hierarchy. There is a strong desire for

- boundaries to be respected, regarded, not violated by others;
- their own territories to be regarded and respected as such;
- defining their own territories and not having these definitions questioned.

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Another central motive of PPD clients is the desire for **autonomy**. Here as well it may be assumed that the PPD individual will have experienced massive restrictions of their autonomy: controls, regulations, being made responsible for everything, insufficient freedom of choice, etc. This also results in this motive ranking high in their motive hierarchy. It is the desire to

- make one's own decisions about issues and one's own life;
- be allowed sufficient room for one's own actions;
- do things without being controlled, patronized, restricted, and regimented.

Boundary violations and autonomy restrictions most likely occurred together with depreciation, criticism, and rejection so that it may also be assumed that the need for appreciation is also a central motive of clients with PPD. This is the desire to be

- respected;
- viewed positively;
- deemed competent, capable, and, especially, competent to organize one's own life and make one's own decisions.

Another strong motive is the desire for **solidarity**, that other people stick by one, are there for, give support to, help, and care for one.

15.2.2 Dysfunctional Schemas

Also with PPD individuals, relationship schemas are more significant than self-schemas.

Self-Schemas

The self-schema of clients with PPD is probably negative and will includes assumptions such as:

- I'm a failure,
- I'm not ok,
- I'm not respectable, etc.

Moreover, the self-schema most likely includes assumptions like "I'm unable to adequately protect my boundaries," and even if I protected my boundaries adequately, this would be ineffective. However, this negative self-schema is not represented by the person. Their self-images rather express and illustrate that they are strong, do not put up with things, are dangerous, etc.

It is thus evident that the demonstration of strength the PPD client is constantly making is not at all the result of a self-confident self-schema: It is all show, an inflated dragon intended to intimidate others and prevent others from seeing the no-name insignificancy walking behind the dragon (and it also serves to prevent the client from seeing that themselves). However, once inflated, the PPD person must continue to keep the fire-breathing dragon company, otherwise, the others who have been provoked would immediately tear the person to pieces. Therefore, the dragon must be inflated more and more. All this terminates in a fatally vicious cycle the person cannot escape without help.

Relationship Schemas

A central schema of clients with PPD is "My boundaries are not respected." There are assumptions such as

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- "Everyone violates my boundaries."
- "No one respects my boundaries."
- "Everyone invades my territory."

Probably also due to biographical experiences and overgeneralization as well, assumptions go so far as to ascribe malicious intentions to other people:

- "Others don't want to respect my boundaries."
- "Others try to harm and use me."
- "Others want to impair me."

As a consequence, PPD persons assume they must protect themselves:

- "If I don't defend myself, I will be used"
- "If I'm not permanently attentive, I will be impaired."

There are similar assumptions concerning autonomy:

- "Others want to patronize, restrict, and regulate me."
- "If I don't watch out from the start, I will be constrained, etc."

Concerning solidarity, there are assumptions such as

- "Nobody is on my side."
- "I can't rely on anyone."
- "Nobody will speak up for me, help or support me."

Therefore,

- "I can only rely on myself."
- "I must take care of my own affairs."

Further assumptions resulting from this are

- "I always must be attentive, watch out not to be impaired."
- "I must always be ready to fight so that I can nip in the bud any attempts to violate my boundaries."

Concerning appreciation there are assumptions like

- "No one respects me."
- "No one takes me seriously."
- "No one thinks I'm strong and competent."

The result of these assumptions is an especially problematic behavior based on the assumptions that "as no one takes me seriously, it is not enough to just express my wishes. Instead, I must protect myself efficiently, defend myself massively and make clear how dangerous it is to mess with me."

The result is that clients with paranoid personality disorder are unable to defend themselves appropriately (fail to make a flexible response). They rather see themselves forced to react directly in a powerful way and therefore, from the interaction partner's point of view, their reaction very much exaggerated. They think anything else would be understood as a sign of weakness, and they cannot admit any weakness that could invite their opponents to do them more damage. PPD clients do not realize that in this way, they even create a typical self-fulfilling prophecy: Naturally, most of their interaction partners react vigorously after a while, as they feel wholly misunderstood, rejected, and provoked. However, clients with PPD perceive the interaction partner's behavior as confirmation of their schemas. Others are dangerous, and one must defend oneself heavily.

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15.2.3 Compensatory Schemas

Normative Schemas

The normative schemas of PPD are very pronounced and relatively strict, including

- "Be attentive, observe everything!"
- "Don't trust anyone!"
- "Always keep in mind that others can blemish, deceivee, take advantage of and impair you!"
- "Keep others at a distance!"
- "Talk as little as possible about yourself!"
- "Defend yourself directly, don't show weakness!"
- "Defend yourself massively, don't allow others to prevail!"

Rule Schemas

PPD individuals also show significant rule schemas. Violations of these schemas trigger fierce anger and aggressive behavior (unless suppressed through other normative schemas).

Rule schemas are

- "Nobody must harm me."
- "Nobody must impair me at all"
- "Nobody must control me."
- "Nobody must restrict or patronize me."
- "Nobody must enter my territory without permission."

15.2.4 Manipulation, Images, and Appeals

The strategies of clients with PPD are usually very explicit: The interactional objectives are clearly identifiable and barley hidden. The only thing that is manipulative is that the clients "inflate" themselves – that is, they try to appear stronger, more powerful, and more dangerous than they actually are. They permanently act according to the image "I'm a dangerous wolf and tear to pieces every offender." If presented well, this image can be very effective, cause anxiety in others, and induce people to avoid the client, circumvent confrontations, and give in. The appeals used include

- "Leave me alone!"
- "Don't mess with me!"
- "If you do mess with, you will lose!"
- "Don't get too close!"
- "Never violate my boundaries!"

These clients especially play the game of "Victim of other people" but also "Victim of circumstances" and "Always me" (see Sachse, 2000a, 2000b). These games especially convey the impression that

- The person is right to behave aggressively.
- Their assumptions and interpretations are accurate.
- It is not their fault; they are only acting defensively and preventively.

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15.2.5 Tests

Therapists treating PPD clients are frequently tested to a high degree. The PPD client assumes the therapist is exactly like everybody else or even worse, is only friendly and approachable because they want to be obsequiously flattering. In the end, they will show their true colors and harm the client massively. The client thus tries to provoke the therapist to reveal their true colors – that is, be aggressive, depreciating, patronizing, regulating, restrictive, etc.

15.2.6 Characteristics of PPD

Impressions on Others

PPD individuals appear to interaction partners not only as unapproachable but as dangerous – as someone it's best not to tangle with. As a consequence, they are extremely isolated. This isolation in turn means they are unable to socially validate their constructs and interpretations. Their interpretations become more and more absurd and increasingly detached from reality. Additionally, the PPD individual's behavior causes negative effects, resulting in other people avoiding them, being angry, and rejecting them. The individuals thus make self-fulfilling prophecies confirming their interpretations and assumptions. PPD clients are stuck in a vicious cycle.

Sensitivity

It has to be borne in mind that the existence of the schemas they have causes the PPD person to be highly sensitive, even aversive to certain stimuli. A schema of the kind "Everyone violates my boundaries, impairs, and constrains me" means, they will react very intensely to any incident that even allusively may be interpreted in this sense. Thus, clients react hypersensitively to any potential impairments (someone picked a cherry from my tree), potential intrigues (when entering a room two other people stop talking), potential insults (traffic warden books them for illegal parking), etc. What is more, PPD clients not only react intensely, they also react quickly and to the most trivial things. On account of this, others consider them a loose cannon, and as a result, they think they are neither companionable, charming, nor popular at all. More likely, they are regarded as the "big asshole" disliked by everybody. However, it should be kept in mind that it is not the client's intention to be seen this way; it is rather the cost factor the client incurs because they think their behavior is without alternative.

15.2.7 Proximity, Distance, and Bonding

With these assumptions, the PPD client can hardly allow other people to be any closer to them. If an interaction partner approaches them, this may easily trigger interpretations like "They are just toadying up to me, and when close enough they will impair me." It thus follows that the PPD client will perhaps only accept people who have passed many different tests. From all others, they keep their distance. And even those who are allowed to come closer, may not be permitted to readily form a bond, as bonding makes them dependent, restricts their autonomy, opens up boundaries, makes them vulnerable, and very likely causes disappointment. Thus, clients with PPD will have great difficulties with bonding. Relationships are rather loose, open, able to be cancelled; their own autonomy must be ensured.

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15.2.8 Ego-Syntony, Perspective, and Avoidance

Due to their schemas and resulting avoidance, clients with PPD expect that others are potentially threatening them, that they must defend themselves, and that they are entitled to defend themselves. Therefore, the disorder is ego-syntonic: The clients think their constructions are reality and that any change in behavior would aggravate the problem.

As prescribed by the schemas, their perspective is extremely external: Other people and situations must constantly be checked for potential threats as the worst must always be expected. Since the PPD client is convinced they are not part of the problem, they disregard any effects of their own behavior. Accordingly, it is quite difficult to internalize their perspective.

Their avoidance is strong. Any attempt to make the client aware of their own problem aspects may be perceived as a threat and as an attempt to hold the client responsible. In therapy, the PPD client, therefore, will for a long time avoid dealing with their own responsibilities or with the fact that the costs of their disorder are incurred through their own behavior.

15.2.9 Costs

The client may indeed perceive the costs of their disorder, such as

- rejection,
- no contacts, no friends,
- no partner,
- social conflicts, etc.

However, the PPD client does not think that they contribute to these costs themselves: They think these costs arise because others impair them. They even think that their own behavior minimizes the costs. If they behaved more permissively, costs would even be considerably higher. It is thus obvious that it is very difficult to convince the client that they are causing the costs in the first place.

15.3 Therapeutic Strategies

15.3.1 Therapeutic Strategies in Phase 1

Complementarity at the Motive Level

First of all, it is important for the therapist to treat the PPD client respectfully. The therapist must show they appreciate the client and emphasize that they deem them to be competent, circumspect, and efficient, and someone who cares about themselves, who can decide, structure, and live their own life. The therapist should again and again mention these aspects and refer to them explicitly: The client is competent and able to self-determine their life.

Furthermore, it is essential for the therapist to constantly emphasize the following aspects:

- “I don’t want to do anything here in therapy that you don’t want to do.”
- “I would like to explain everything that we do, why I do it, and what objectives I have.”
- “If you have any questions, ask me immediately.”
- “If there is something you don’t like, let me know immediately; I will deal with it at once.”

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- "It is also important to me that I don't violate your personal boundaries; I will try hard to not violate them."
- "But as I'm not telepathic, it may happen that I still violate a boundary sometime, in which case, please let me know right away."
- "I can't and don't want to prescribe anything in therapy; I can only make suggestions; you definitely may decide whether you want to follow the suggestions."
- "Check this precisely; if you remain skeptical, nothing can happen that you don't want."

The therapist consequently follows this line: Nothing happens that the client does not want. The therapist does not want anything for the client; they make proposals but do not give instructions. They trespass within boundaries as carefully as possible and never constrain the client's autonomy.

Complementarity at the Game Level

The biggest problem for therapists is that the PPD client demands they show solidarity to a very high degree. Clients describe events where they allegedly experienced injustice. And in doing so, they may directly or indirectly request the therapist to show solidarity with them against the perceived offender. For the therapist, this poses a dilemma – that is,

- if they collaborate with the client, they stabilize the client's system thus confirming, in their role as an expert, the client's assumptions;
- if they do not collaborate with the client, they will lose the client's trust and not be able to gain relationship credit.

The therapist may try a compromise, saying,

- "I don't really know the situation you talk about and I don't know Mr. X. That's why I can't really assess what X wanted or why he behaved like that. Therefore, I can't and don't really want to comment on this."

And moreover, they may say,

- "It is essential that I as therapist stay neutral. Other than a friend, I can fulfill my duty best if I remain unbiased. Only then will I be able to analyze the situation with you in an unprejudiced way."
- "But let's assume the situation is like you said, then I would like to improve my understanding of what you did, what you wanted, what you thought, etc."

The therapist should *never* collaborate with the client against others; such behavior is totally antitherapeutic! However, the therapist can be loyal and offer to analyze the situation with the client and accept the client's appraisal as the assessment given by the client (not as reality!). If the client insists that the therapist as therapist should collaborate with them against others with statements like "Either you're on my side and collaborate with me or you're against me," then the therapist should refrain from working with the client, because they should do not let themselves be induced to break central therapeutic rules. In such a case, the therapist would have to tell the client that they will not comply and the reason for this.

Establishment of Relationship Credit

Before a therapist does anything that goes beyond the realization of basic variables, they should have some relationship credit. However, this is exactly the most significant problem with PPD clients. Thus, in the first 3–6 sessions, the therapist does nothing else other than establishing relationship credit. The therapist

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- signals acceptance of the client;
- signals that they respect the client, respect their boundaries and autonomy;
- signals that they understand how the client feels, that they think they must defend themselves, etc.; the therapist understands and comprehends all constructions as constructions of the client (not as reality) without showing solidarity with the client;
- feeds the client by emphasizing that
  - they think the client is competent, autonomous, etc.,
  - they think some skepticism in the part of the client is appropriate,
  - they perceive that the client cares for themselves very well and efficiently protects their boundaries,
  - they think the client is autonomous, is someone who knows what they want and can take good decisions,
  - the client is someone who doesn't put up with everything, etc.

The therapist should try to positively raise as many aspects as possible to emphasize to the client that they

- do not reject but rather accept them;
- do not think they are weak but instead are competent;
- think many characteristics of the client are okay.

This is important as the client expects offense, depreciation, etc.; therefore, the therapist must not confirm any those assumptions.

Dealing With Tests

At the beginning of therapy, dealing with tests is also essential (see Section 2.6: Tests). The client may

- impute malicious intentions to the therapist,
- impute that the therapist wants to influence them,
- impute that the therapist actually thinks badly of them but disguises this, etc.

The therapist should react calmly and amicably, and should

- verbalize the client's statements in a way that shows they know those statements are meant personally (e.g., "You think that I hold something against you. . . ."); they should "face the dragon" and not shy away, and then
- say what they really mean: "No, I don't hold anything against you, I just would like to . . ., and then
- "But I can understand if it's hard for you to believe me. All I can do is explain this often, and so I'd like to ask that you try me. Keep your skepticism but try me. I want you to be totally sure before you decide to work with me."

The therapist thus makes clear that they

- are reacting positively to the test, not aggressively;
- think the test is okay;
- want to pass the test;
- do not want to talk the client into something.

Explicating Relationship Motives

In the treatment of clients with PPD, this intervention takes place comparatively late – not until the therapist has gained some relationship credit. Usually, the explication of relationship

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motives is not very confrontational either for clients with PPD; however, it may be asking too much of the client if they are not yet ready to disclose anything about themselves. When ready, explication may result in more relationship credit earned, as the client may feel truly understood by the therapist. The therapist understands how important the protection of the client's own territory is, what impairing their autonomy would mean to them, etc. It is also important to represent their motives to the client and point out that the motives as such are absolutely okay. The client may and shall keep their motives; they must not give up their need for autonomy.

Resource Activation

Basically, clients with PPD feel weak and unable to cope with (alleged) threats.

> It is thus an essential therapeutic objective to strengthen and support the client: The client should know their possibilities, strengths, competences, etc. to improve their introspection and thus be able to face threats better. Such an evaluation can also serve to diminish their avoidance behavior and enable them to at least cope with some of their own personal problems.

It is thus an essential strategy of the therapist to activate client resources right from the start and during the entire therapy process.

For this purpose, therapists may

- underline all the client's strengths and competences whenever they perceive them,
- insist that they recognize and perceive these strengths even if the client does not,
- express astonishment at the clients being unaware of their strengths and competences,
- raise the question of why the client doesn't see or believe in these strengths,
- express astonishment at the client being easily triggered in situations where thanks to their strength they could react calmly.

This strategy is not confrontational as a rule; however, therapists should make sure their opinion does not deviate too much from that of the client, so as not to give rise to any suspicion they might want to deceive or mislead the client. It is thus important for the therapist to only provide feedback of this nature when this can be done authentically without misleading the client.

15.3.2 Therapeutic Strategies in Phase 2

Making the Game Level Transparent

It seems questionable to the author whether therapists treating clients with PPD should reveal any manipulative behaviors at all. At most, interactional objectives should be made transparent and worked on, with a view to inducing the client to either abandon them or try to achieve them using better strategies. All other interventions concerning transparency are very dangerous, because they might meet with massive reactance. Accordingly, the therapist should refrain from associating the client's motivations with costs or the knowledge of unfavorable strategies, but rather motivate the client to find more favorable strategies. Certainly, it would be advantageous if the client established a representation of what they are doing and the effects resulting therefrom; this would improve the effectiveness of self-regulation. It appears, however, that this therapy objective is difficult to achieve with PPD clients, and the strategies required for this may endanger the entire therapy. For that reason, they are better not adopted.

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Dealing With Tests

As has been described (see the subsection Passing the Tests, in Section 6.3.7, above), therapists should deal with tests by staying friendly, obliging, and explanatory.

15.3.3 Therapeutic Strategies in Phase 3

The explication of dysfunctional schemas is significantly more difficult with PPD clients than with those with other PDs and should take place much later in therapy. Therapists should always work at the edge of the possibilities, thus verbalizing and explicating schemas and schema aspects tentatively, and checking whether the client can yet accept them or still avoids them. If the client reacts aggressively, the therapist can leave the statement as "This was my impression, but I might be wrong" and not touch again on such verbalizations for a while. If the client accepts the explication, the therapists carefully and gradually elaborates it further. (A combination of schema explication and biographical work may prove useful here: Through processing biographical aspects, the therapist may provide elucidation to the effect that the client had to develop certain assumptions considering the situation they had been in, and that it is not defamatory to have these assumptions.)

The explication of potentially confrontational schemas should preferably be combined with a genuine complementary relationship. For example:

> I have the impression that you react very sensitively even to minor infringements, and this actually astonishes me as you appear to be very strong and determined. There is no need for strong people to react to every triviality, actually one should keep cool. So, I actually wonder about your quick reaction, and I would like to examine this issue with you.

It is important here that the therapist finds paraphrases for the client's behavior which do not sound depreciating, indeed not even valuing: Euphemisms and softeners are needed. Whenever possible

- the client's behavior should be expressly stated positively;
- the client's behavior should be compared with (even better) alternatives;
- the therapist should express astonishment about the reaction the client has made in spite of the alternatives that could be resorted to;
- the therapist should use **Trojan horses** to connect the interventions to the client's system (see, e.g., the subsection Trojan Horses, in Section 8.3.2.5).

Therapists must necessarily question the client's schemas. However, they must do this in a way that does not offend the client. Therefore, the therapists wonders why the client reacts like this although they could easily react differently. The therapists thinks the client is capable of resorting to another kind of behavior, is competent, strong enough to do this. And they just want to understand and clarify together with the client why the client reacts like this and not differently. It is left to the client's discretion how they react, and the therapist has no intention to alter this. But making attempts to understand the reaction is surely allowable.

And just like with NPD clients (see Chapter 8), the therapist emphasizes here as well that the motivation to work on one's own assumptions and behavior patterns does not result from the client having behaved incorrectly or because the client has problems. This motivation rather aims at making the client understand that they probably could improve things, especially

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because the therapist thinks they are strong and competent. And someone like the client who can care for themselves quite well, may care for themselves even better when they take a closer look at what they are really doing, why they do it, and how they could act even more effectively. Such an approach may indeed be acceptable to the client (provided the client already trusts the therapist; and trust also seems to be a prerequisite for this); the client may accept it without losing face, without the feeling of being threatened, without the feeling of being limited in their decisions. This is most essential because if the client think they are being criticized, depreciated, or constricted that will immediately cause reactance, Therefore, the therapist needs strategies that bring reactance relief. And these strategies consist of making it very clear to the client that a person with their capabilities and options can make more of their life: The therapist must make the client an offer that they cannot refuse.

15.3.4 Therapeutic Strategies in Phase 4

If the PPD client decides to develop different strategies to protect their boundaries, etc., the therapist can systematically develop and practice them with them. This is important as it can be expected that clients with PPD have competence deficits concerning "normal" interaction strategies.

15.3.5 Therapeutic Strategies in Phase 5

The same as with OCPD clients, transfer is difficult for PPD clients and may only take place in small steps. It is especially important that the therapist formulates homework only in the form of suggestions and should avoid arousing the client's mistrust. The client can best be motivated by underlining that they are capable of performing something and that relevant actions are a sign of strength. However, more often than not, even this does not result in achieving compliance.

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List of Abbreviations

AvPD = avoidant personality disorder
COP = clarification-oriented psychotherapy
CP = client position
DPD = dependent personality disorder
DSM = *Diagnostic and Statistical Manual of Mental Disorders*
FNPD = failed narcissistic personality disorder
ICD = *International Classification of Diseases*
NPD = narcissistic personality disorder
OCPD = obsessive-compulsive personality disorder
OPRP = one-person role play
PAPD = passive-aggressive personality disorder
PD = personality disorder
PPD = paranoid personality disorder
SHPD = successful histrionic personality disorder
SNPD = successful narcissistic personality disorder
SzPD = schizoid personality disorder
TP = therapist position
UHPD = unsuccessful histrionic personality disorder
UNPD = unsuccessful narcissistic personality disorder

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